

PASSREADY PREP™ · NCMHCE SERIES



THE COMPLETE NCMHCE STUDY GUIDE

Clinical Reasoning, Diagnostic References and
Practice Simulations for the Mental Health Clinical Exam

92 DIAGNOSES · 18 DIAGNOSTIC FAMILIES
3 FULL-LENGTH EXAMS · 429 PRACTICE QUESTIONS

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GA INTEGRATED THERAPEUTIC PERSPECTIVES LLC

The Complete NCMHCE Study Guide

PassReady Prep™

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2026

The Complete NCMHCE Study Guide — *PassReady Prep*™

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For every licensed counselor candidate who has stared at a table of diagnoses and wondered: "But how do I actually use this on the exam?"

Preface

If you are holding this book, you have already done the hard part.

You sat through graduate seminars that ran long past the end of a full workday. You wrote case conceptualizations at midnight, drove across town for practicum and internship hours, and sat with clients whose pain you carried home with you. You finished the coursework, logged the supervised hours, and kept going on the nights when stopping would have been easier. None of that was small, and none of it was guaranteed. All of it was you choosing, again and again, to become the counselor your future clients need.

That road was not easy — and it was never meant to be. Becoming a clinician asks something real of a person: patience, humility, and the willingness to keep showing up for other people's hardest moments. You have already proven you can do that. The credential ahead of you is not the beginning of your competence; it is the formal recognition of competence you have been building for years.

So let us be clear about what the NCMHCE actually is. It is not a verdict on whether you belong in this profession — you answered that question long ago, with your training and your hours. It is one last structured hurdle: a test of clinical reasoning under exam conditions. It rewards a specific way of thinking — gathering information before acting, prioritizing safety, and matching each intervention to the client's stage — and that way of thinking can be learned, practiced, and mastered. That is exactly what this book is built to do.

Everything ahead is designed to meet you where you are and carry you the rest of the way: the reasoning behind each diagnosis, the traps the exam sets and why they work, and worked cases that show how a strong clinician moves through a scenario. And when you are ready to pressure-test that reasoning under realistic conditions, **PassReady Prep™** (www.passreadyprep.com) puts everything you learn here into full-length case simulations built for the exam's current format — the natural next step after the pages that follow.

Read at your own pace. Trust your preparation. And when the doubt shows up — because it will — remember how far you have already come to arrive here.

You have carried yourself through the hardest years of this journey.
This is the final stretch. Let's finish it together.

— *Kejuiana Johnson, MA, LPC, NCC, BC-TMH, CPCS*

How to Use This Book

This guide was built on a single observation: most NCMHCE prep resources teach you **what** to know but not **how to think**. They give you tables. The exam gives you a human being in crisis — and four options that all look plausible.

The chapters ahead do something different. Every diagnosis family gets:

- A **narrative overview** that explains why the exam tests this family and what the examiners are actually looking for
- **Diagnosis reference tables** (retained as quick-reference tools)
- **“The clinical picture”** — prose describing how these disorders actually present inside a case simulation
- **Differential discussion** — not just “rule out X,” but *why* X loses and how to detect the deciding feature
- **Assessment tools in context** — when you reach for each instrument and what the score tells you
- **“What the exam rewards / What it punishes”** — scored priorities and the traps, each with a reason
- **Worked mini-cases** — a vignette, the reasoning chain, and *why each distractor loses*
- **Practice questions** with expanded rationales

The Priority Ladder

One framework governs every scored decision in the NCMHCE. Memorize this and apply it before you pick any answer.

Rung	Priority	Clinical Translation
1	Imminent safety	Suicidality, homicidality, abuse, danger — nothing else happens first
2	Medical / substance rule-outs	Could this be organic? Could substances explain it? Rule out before diagnosing
3	Stabilization	Acute symptom relief, grounding, crisis reduction
4	Alliance and validation	Before any intervention, the client must feel heard

Rung	Priority	Clinical Translation
5	Clarify the picture	Assessment, history, collateral info
6	Evidence-based treatment	The right modality for the right diagnosis
7	Ethics throughout	Confidentiality, mandated reporting, competence — woven into every rung

When two answers both seem correct, ask: which rung is higher?
The higher-rung action wins.

How This Book Maps to the NCMHCE

The NCMHCE evaluates clinical reasoning through case simulation — not isolated recall. Every scored decision falls into one of five clinical competency areas. The table below shows where each area is taught and practiced in this book.

Content Area	Exam Share*	Taught In (Part One & Two)	Practiced In (Part Three)
Intake & Assessment	~18 %	Ch 1 Helping Relationship · Ch 3 Skills & Techniques · Ch 5 Worked Case · All diagnostic chapters (6–23)	All 33 exam cases: intake narrative + assessment questions
Core Counseling Attributes	~20 %	Ch 1 Helping Relationship · Ch 2 Theories & Orientations · Ch 3 Skills & Techniques · Ch 4 Group Work	Differential diagnosis questions across all 3 exams
Treatment Planning	~27 %	Ch 2 Theories · Ch 5 Worked Case · Ch 6–23 each include first-line treatment tables and decision flow	Treatment sequencing questions · 116 items across 3 exams
Counseling Skills & Interventions	~16 %	Ch 3 Skills & Techniques · Ch 5 Worked Case · Appendix B Theories of Psychotherapy	In-session response questions · 70 items across 3 exams
Ethics & Legal Compliance	~18 %	Ch 23 Ethical and Legal Decision Points · Ch 17 Crisis, Risk & Safety · Appendix A Crisis & Legal Duties	Ethics domain questions · 78 items across 3 exams

**Approximate distribution based on question domain analysis across the 33 print cases (429 scored questions). Actual exam weighting varies by case. The NCMHCE does not publish fixed domain percentages for the current format.*

How the Book Is Organized

Part One — Foundations, Theories & Skills (Ch 1–5)

Builds the clinical reasoning infrastructure the exam scores: the helping relationship, counseling theories, intervention skills, group work, and a fully worked case walkthrough that models the Priority Ladder decision sequence from intake through disposition.

Part Two — Diagnostic Reference (Ch 6–23)

92 diagnoses across 18 diagnostic families. Each chapter presents the clinical picture, key differentials, discriminating features, first-line treatments, and counselor scope — exactly the information the exam tests through case presentation.

Part Three — Practice Exams

Three full-length NCMHCE-format exams: 33 cases, 429 scored questions, complete answer keys with rationales and identified clinical errors. Each exam spans all five content areas and all 17 diagnostic categories. Work them under timed conditions before consulting the key.

PART ONE: FOUNDATIONS, THEORIES & SKILLS

These chapters turn the reference tables into teachable, testable knowledge.

Chapter 1 — The Helping Relationship

Overview: Why Alliance Outpredicts Technique

Here is something the research has been saying for decades that exam writers clearly believe: **the therapeutic alliance is the single strongest predictor of outcome**, accounting for more variance in client improvement than the specific technique used. This is not a soft finding. Meta-analyses consistently show that alliance explains 5–7 times more outcome variance than treatment modality. The exam knows this. You will see questions that tempt you to pick an intervention — confront the distortion, assign the homework, interpret the dream — when the clinically correct answer is first to *repair the relationship*.

Understanding why the alliance works also helps you spot alliance items in disguise. The exam rarely says “the alliance is rupturing.” Instead, it shows a client who “seems disengaged,” “pushes back on the homework,” “misses appointments,” or “says the counseling isn’t helping.” Your job is to recognize these as alliance signals and respond accordingly — before you pivot to technique.

The mechanism of rupture and repair. Ruptures happen in two forms. A *withdrawal rupture* looks like silence, vagueness, sudden compliance, or emotional flatness — the client has pulled back without saying so. A *confrontation rupture* looks like direct disagreement, irritation, or challenging the counselor’s approach. Both forms carry the same message: “Something in our relationship isn’t working.” The research-supported repair sequence is: (1) notice the shift, (2) name it without blame (“I want to check in — it feels like something shifted for you just now”), (3) explore the client’s experience, (4) acknowledge your contribution if relevant, and (5) re-contract on the working arrangement. The exam rewards noticing and naming over plowing ahead.

How the exam disguises alliance items. Alliance questions frequently appear as “what do you do next” items in which the client has just expressed doubt, frustration, or disconnection. The trap is to pick the answer that moves the work forward — psychoeducation,

a new technique, a referral. The correct answer almost always validates the client's experience first. "Validate before you challenge" is the cardinal rule (see Chapter 3). It applies to alliance ruptures just as strongly as to skill use.

Rogers' Core Conditions

Carl Rogers proposed that three counselor-provided conditions are both necessary and sufficient for therapeutic change. The NCMHCE tests these by showing you a counselor response and asking you to identify the condition it demonstrates — or by asking which condition is *missing* in a scenario where the client isn't improving.

Unconditional Positive Regard (UPR) is not approval. It is the commitment to value the client's personhood regardless of behavior, choices, or disclosures. A client who describes something the counselor finds morally troubling still receives warmth and non-judgment. Exam tip: UPR is violated when a counselor's response implies the client *should* feel differently, or when the counselor's tone signals disapproval.

Dialogue example: > Client: "I've been drinking every night. I know it's bad, but honestly? It's the only time I feel like myself." > Counselor (UPR): "That's worth understanding — what does feeling like yourself mean to you in those moments?" > (Not: "Have you considered what alcohol is doing to your body?")

Empathy (accurate empathic understanding) requires the counselor to track not just content but underlying emotion — and to communicate that tracking back to the client in a way the client recognizes. It is not sympathy ("I feel sorry for you") and not agreement ("You're right, your boss is terrible"). It is: "I am with you in your experience."

Dialogue example: > Client: "I've tried to talk to my husband about this a hundred times. He just doesn't get it." > Counselor (empathy): "There's something exhausting about explaining yourself over and over to someone you love and still feeling unseen."

Congruence (Genuineness) means the counselor's internal experience, outward expression, and words align. The counselor does not perform warmth while feeling bored. Congruence does not mean self-disclosure of every feeling — it means authenticity, which

sometimes includes saying “I’m noticing something in our conversation I want to name.”

Bordin’s Working Alliance: Bond, Goals, Tasks

Bordin’s model breaks the alliance into three components that must all be intact for treatment to proceed. Knowing which component is strained tells you what kind of repair to attempt.

Component	What It Means	When It’s Strained
Bond	Emotional connection, trust, felt sense of being valued	Client feels judged, misunderstood, or unsafe
Goals	Mutual agreement on what treatment is for	Client doesn’t believe the stated goals are their goals
Tasks	Agreement that the <i>methods</i> used will reach the goals	Client questions why they are doing an exercise or technique

Rupture-repair vignette: A client diagnosed with depression has been doing behavioral activation for three weeks. She arrives and says, “I don’t see why I keep filling out these activity logs. My problem isn’t that I don’t do things — it’s that nothing feels worth doing.” This is a **task rupture**. The repair: explore her understanding of the technique, validate that the tasks can feel meaningless when motivation is low, and collaboratively renegotiate whether behavioral activation is the right fit or whether the approach needs adjustment. Do not defend the technique. Do not skip straight to reassigning the homework.

Stages of Counseling

The exam tests whether you know the *clinical priorities* at each stage — not just the names.

Exploration / Initial Stage: Safety assessment comes first. Alliance-building is the primary task. Do not move to goal-setting before the client feels heard and the presenting problem is adequately explored. Counselors who jump to treatment planning in the first session lose points.

Working / Middle Stage: The treatment plan is active. Resistance appears here (see Chapter 3). Ruptures are most likely here because the work gets harder. The counselor moves between support and challenge.

Termination Stage: The exam tests two things: (1) how far in advance to raise termination (early enough for processing, not so early it disrupts the work), and (2) how to respond when clients regress near termination. Regression near termination is **normal and expected** — it is not a sign of failure and does not warrant extending treatment indefinitely. The correct response is to name the pattern, normalize it, and continue the planned termination.

Cultural Responsiveness

Cultural humility vs. cultural competence. Cultural *competence* implies mastery — that you can learn enough about a group to work with any of its members. Cultural *humility* recognizes that the client is the expert on their own cultural experience. The exam favors humility: the counselor asks, learns, and remains curious rather than applying assumptions. Saying “I know that in many Latino families, mental health treatment carries stigma” is competence language. Saying “I’d like to understand how your family thinks about getting help — can you tell me about that?” is humility language. Humility wins on the exam.

Managing countertransference. Countertransference is the counselor’s emotional reaction to the client — rooted in the counselor’s own history, biases, or unresolved material. It is not inherently harmful; unexamined countertransference is. The exam expects you to: recognize it (name the feeling), contain it (don’t act on it impulsively), use it (mine it for clinical information about the client’s impact on others), and consult or seek supervision when it feels unmanageable. Never act out countertransference by changing the treatment contract, providing dual-relationship services, or disclosing at length about your own experience.

Practice Questions 1–4

Question 1. A client who has been making steady progress in counseling for four months arrives and sits silently for several minutes.

When the counselor asks what's on his mind, he shrugs and says, "Nothing. I don't know if this is even doing anything." The *most appropriate* initial response is:

- A. Reassure him that progress takes time and review recent gains
- B. Explore what is missing for him in the counseling relationship
- C. Administer a symptom measure to objectively track progress
- D. Ask about recent stressors that might explain the regression

Correct: B. The client is signaling a rupture — something in the alliance or the work is not working for him. The first response is to *explore his experience*, not to defend the treatment (A), provide objective data (C), or redirect to external events (D). The rupture must be addressed before any other clinical work can resume.

Question 2. A counselor is working with a client from a cultural background in which family decisions are made collectively. The counselor has been encouraging the client to "set limits with your family about your time." The client has been increasingly disengaged. The *most likely* explanation is:

- A. The client has a characterological difficulty with assertiveness
- B. The counselor's goals conflict with the client's cultural values
- C. The client is ambivalent about change, which is normal in the working stage
- D. The client needs psychoeducation about healthy relational patterns

Correct: B. The disengagement is a goals rupture: the counselor's stated treatment goal (individual autonomy) conflicts with the client's cultural framework. The exam rewards cultural humility — the counselor needs to re-explore what the client actually wants, not double down on a culturally incongruent goal.

Question 3. Which of the following counselor responses BEST demonstrates unconditional positive regard?

- A. "I understand that's how you feel, but most people in your situation would react differently."
- B. "It sounds like you made the best decision you could with what you knew at the time."
- C. "I appreciate you sharing that — let's explore whether there was a better option."

D. "I'm glad you told me. That behavior concerns me, and we should address it."

Correct: B. This response validates the client's experience without judgment and without implying a better choice existed. A implies comparison to others (evaluative). C introduces a search for a "better option" (evaluative). D names concern (conditional).

Question 4. A counselor is terminating with a client who has met all treatment goals. Three weeks before the final session, the client begins arriving late, canceling sessions, and reports that old symptoms are returning. The counselor should FIRST:

- A. Extend the treatment contract by four additional sessions
- B. Assess for the recurrence of the original presenting problem
- C. Name the pattern and explore whether it may be related to the ending
- D. Consult with a supervisor about whether termination is premature

Correct: C. Pre-termination regression is a well-documented and expected phenomenon. The first clinical response is to name the pattern and connect it to the transition. Extending treatment (A) without exploring this dynamic reinforces the avoidance. Assessment (B) is premature — this symptom pattern in the context of ending is the most parsimonious explanation. Consultation (D) may eventually be warranted but is not the first step.

Chapter 2 — Theories & Orientations

Overview: Pattern-Matching a Stem to a Theory

Theory questions on the NCMHCE follow a predictable structure. A vignette describes a counselor doing something — asking a specific question, assigning a specific task, using a specific technique — and asks you to identify the theoretical orientation. The shortcut: **every major theory has 1-2 signature techniques that appear on no other theory's menu.** Learn those signatures and you can identify the orientation from the technique alone, without needing to analyze the whole vignette.

The three traps of theory items: 1. **Conflating humanistic and existential.** Both emphasize meaning and authenticity, but humanistic therapy (Rogers) is non-directive and relationship-focused; existential therapy addresses freedom, death, meaning, and isolation more directly and may be more confrontational about avoidance. 2. **Mislabeling CBT sub-modalities.** REBT (Ellis) uses the ABC model and targets irrational *beliefs*. Standard CBT (Beck) uses thought records and targets cognitive *distortions*. DBT targets emotion dysregulation with skills training. These are not interchangeable. 3. **Choosing the right family interventions.** Structural family therapy (Minuchin) focuses on boundaries and subsystems. Strategic family therapy (Haley, Madanes) uses directives and paradoxical interventions. Bowenian therapy focuses on differentiation and genograms.

The Five Theoretical Traditions

Psychoanalytic / Psychodynamic

Core premise: Unconscious conflicts, early attachment patterns, and unresolved developmental material shape present functioning. Change comes through insight into the unconscious.

Signature Techniques	What They Do
Free association	Access unconscious material by removing censorship
Dream analysis	Examine manifest vs. latent content
Transference interpretation	Use the therapeutic relationship as a window into relational patterns
Defense analysis	Identify and gently challenge protective mechanisms

How this shows up on the exam: Questions mention the counselor “exploring childhood experiences,” “interpreting a dream,” “noticing the client is relating to the counselor as though they were the client’s father,” or “helping the client understand how the past affects the present.” The exam also tests defense mechanisms (see Part Three).

Adlerian (Individual Psychology)

Core premise: People are motivated by social interest and a striving for superiority (overcoming inferiority). Birth order, lifestyle, and fictional final goals shape behavior.

Signature Techniques	What They Do
Lifestyle assessment	Understand the client’s characteristic pattern of engaging the world
Early recollections	Access the client’s guiding life narrative
“The question” (“What would be different if you were well?”)	Distinguish psychological from organic factors; reveal avoidance
Encouragement	Counteract discouragement, which Adler saw as the root of dysfunction

How this shows up on the exam: Look for birth order questions, early childhood recollections, and the phrase “social interest.” Adlerian therapy is goal-directed and often shorter-term than classical psychoanalysis.

Humanistic / Person-Centered

Core premise: People have an innate actualizing tendency that is blocked by conditions of worth imposed by others. The therapeutic relationship is the change mechanism.

Signature Techniques	What They Do
Reflective listening	Communicate empathic understanding
Non-directive exploration	Follow the client's lead; resist the expert role
Congruent self-disclosure	Authentic presence when relevant

How this shows up on the exam: Non-directive stems, emphasis on the relationship over technique, Rogers' three conditions (see Chapter 1). Person-centered questions usually ask you to identify the response that is *most empathic* or *least directive*.

Cognitive-Behavioral Traditions

Core premise: Thoughts, feelings, and behaviors are interconnected. Changing distorted cognitions and maladaptive behaviors changes affect.

Approach | Signature Technique | Core Target |

|———|—————|———|| | CBT (Beck) | Thought record, Socratic questioning | Cognitive distortions | | REBT (Ellis) | ABC model, disputation | Irrational beliefs | | DBT (Linehan) | Skills training (TIPP, DEAR MAN, STOP) | Emotion dysregulation | | Behavioral (Skinner/Wolpe) | Exposure, systematic desensitization, token economy | Conditioned responses |

How this shows up on the exam: The ABC model is REBT, not CBT. Thought records are CBT. DBT is specifically indicated for borderline personality disorder and chronic suicidality. Behavioral techniques (desensitization, exposure) appear with anxiety disorders.

Worked example: A vignette describes a counselor asking a client: "What evidence do you have that your coworkers think you're incompetent?" This is **Socratic questioning** — a core CBT technique (Beck) targeting a cognitive distortion. It is not REBT (which would target the underlying *belief*, e.g., "I *must* be competent or else I'm

worthless"). It is not person-centered (which would reflect, not challenge). The answer is CBT.

Systemic / Family Systems

Core premise: The client's symptoms are best understood as expressions of a system's (family's, couple's) dynamics. Change the system; the symptoms resolve.

Approach | Signature Technique | Core Concept |

|-----|-----|-----|| | Structural (Minuchin) | Joining, enactment, boundary-setting | Subsystems, boundaries, hierarchy |
| Strategic (Haley) | Directives, paradoxical intention | Problem-maintaining sequences | | Bowenian | Genogram, coaching differentiation | Differentiation, triangulation, emotional cutoff | |
Solution-Focused | Miracle question, scaling, exception-finding | Client strengths, brief therapy | | Narrative | Externalization, re-authoring | Dominant narratives, unique outcomes |

The miracle question (SFBT): "Suppose tonight, while you slept, a miracle occurred and the problem that brought you here was solved. When you woke up, what would be different?" This is the single most tested SFBT technique.

Externalization (narrative): "When did the anxiety first try to take over your life?" The problem becomes an entity separate from the person. This is narrative therapy, not any other modality.

Practice Questions 5–8

Question 5. A counselor asks a client struggling with procrastination: "On a scale of 1-10, how confident are you that you can complete the first step this week? What would make it a 7 instead of a 5?" This technique is MOST consistent with:

- A. Motivational interviewing
- B. Solution-focused brief therapy
- C. Adlerian therapy
- D. Rational emotive behavior therapy

Correct: B. Scaling questions are a signature SFBT technique. Motivational interviewing also uses scaling (readiness rulers), but the

addition of “what would make it a 7” is specifically solution-focused. Adlerian therapy uses encouragement. REBT uses ABC disputation.

Question 6. A family therapist observes that the parents speak for the identified patient, who remains silent. The therapist asks the child to respond directly to one parent while the therapist watches. This is BEST described as:

- A. A paradoxical directive
- B. An enactment
- C. Externalization
- D. Differentiation coaching

Correct: B. Enactment is the signature structural family therapy technique in which the therapist asks family members to interact in session so that the boundary/hierarchy problems become visible and can be restructured. Paradoxical directive (A) is strategic. Externalization (C) is narrative. Differentiation coaching (D) is Bowenian.

Question 7. A counselor says, “It sounds like the depression has been trying to convince you that you’re worthless. When did you first notice it doing that?” This is MOST consistent with:

- A. Cognitive behavioral therapy
- B. Psychodynamic therapy
- C. Narrative therapy
- D. Existential therapy

Correct: C. Externalization — separating the problem from the person’s identity — is the hallmark narrative therapy technique. CBT would examine the thought (not personify it). Psychodynamic would explore its developmental roots. Existential would explore meaning, not externalize.

Question 8. According to REBT, which of the following is the MOST direct target of intervention?

- A. The activating event (A) that triggered the emotional response
- B. The consequences (C), including emotional distress and behavioral reactions

C. The belief (B) the client holds about the activating event

D. The environmental reinforcers maintaining the behavior

Correct: C. The ABC model holds that it is not the activating event itself but the client's *belief* about it that produces emotional consequences. Changing the irrational belief (disputation) is the core REBT intervention. A and B are components of the model, not the target. D describes a behavioral, not REBT, approach.

Chapter 3 — Skills & Techniques

Overview: The Microskills Hierarchy as a Sequence

Counseling microskills are not a menu to pick from randomly. They form a **developmental sequence** with a clear clinical logic: you must earn the right to move up the hierarchy by doing the lower levels well first.

Attend → Listen → Influence

Attending (nonverbal and physical presence) must come before anything else. A counselor who looks distracted, avoids eye contact, or creates physical distance has already failed before speaking. Culturally responsive attending recognizes that eye contact norms vary across cultures — sustained eye contact communicates respect in many Western contexts but can signal aggression or disrespect in others.

Listening skills (reflection of content, reflection of feeling, paraphrase, summarization) communicate understanding and build the alliance. These are the engine of the middle phase of every session. The exam wants you to know which listening skill to use when: - **Paraphrase:** captures the cognitive content of what the client said in slightly different words. Use to demonstrate you heard the *facts*. - **Reflection of feeling:** names the emotion underneath the content. Use when the affect is stronger than the content. - **Reflection of meaning:** captures the deeper value or belief the feeling points to. Use when you want to go deeper. - **Summarization:** draws together themes across a segment of the session or across sessions. Use to signal transition, to punctuate insight, or to open a session.

Influencing skills (confrontation, self-disclosure, interpretation, immediacy, information-giving) require that the alliance be solid before they will be effective. A confrontation delivered without a strong alliance lands as an attack. An interpretation without sufficient exploration is a guess. The exam frequently tempts you to intervene (influence) before the listening foundation is laid.

“Validate before you challenge” is the exam’s cardinal rule.

When a client expresses distress, the first scored response reflects *feeling*, not content. When a client is resistant, the first scored response acknowledges the resistance, not argues against it. When a client disagrees with the counselor, the first scored response explores the client’s perspective, not defends the counselor’s position.

Skills Discrimination Drills

These four skills are the most-confused group on the exam:

Skill	What It Is	What It Is Not
Paraphrase	Restate <i>content</i> in your words	Does not name emotion
Reflection of feeling	Name the <i>emotion</i> the client is experiencing	Does not summarize content
Immediacy	Address the <i>counselor-client relationship</i> in the here-and-now	Not about the client’s outside relationships
Confrontation	Point out an <i>incongruity</i> between two things the client said, or between words and behavior	Not attacking; the tone is gentle and curious

Drill:

Client: “I keep telling my wife I’m fine, but honestly, I don’t sleep, I can’t concentrate, and I feel like I’m carrying this weight I can’t put down.”

- **Paraphrase:** “You’re telling her things are okay even though you’re struggling with sleep and focus.”
- **Reflection of feeling:** “There’s something exhausting — and maybe lonely — about carrying all of this privately.”
- **Immediacy:** (*Not applicable here unless the counselor-client relationship is the topic*)
- **Confrontation:** “I’m noticing something — you say you’re fine to your wife, but what you’re describing to me sounds like you’re far from fine. What do you make of that difference?”

When to Use Influencing Skills

Confrontation is appropriate when: (1) the alliance is strong and established, (2) a discrepancy exists between what the client says and what they do (or between two things they say), and (3) the discrepancy is maintaining the problem. Never confront in the first session. Never confront when a client is acutely distressed.

Interpretation is appropriate when: the data has been gathered (sufficient exploration), the client is in a receptive state, and the interpretation is offered tentatively (“I wonder if...”), not as a pronouncement.

Self-disclosure is appropriate when: it normalizes the client’s experience, strengthens the alliance, or models a coping response — and it is brief and client-focused, not counselor-centered. The exam punishes lengthy self-disclosure that shifts the focus from client to counselor.

Immediacy is appropriate when: the in-session relational dynamic is more clinically important than the content the client is discussing — for example, when the client begins to replicate a problematic relational pattern with the counselor.

Practice Questions 9–12

Question 9. A client describes her conflict with her mother in detail, then adds: “I’m sorry — I just talk too much.” The MOST clinically appropriate response is:

- A. “What do you think makes you feel that your sharing isn’t welcome?”
- B. “Not at all — please continue telling me about your mother.”
- C. “I’m noticing that you apologized for taking up space — I wonder if that happens in other relationships too.”
- D. “It sounds like you feel uncomfortable sharing your feelings.”

Correct: C. This is an **immediacy** response — the counselor names something happening *in the room* (the client’s apology for speaking) and connects it to a broader pattern. It is clinically rich without being intrusive. B is reassurance (not clinically useful). A is exploratory but misses the relational parallel. D is a reflection of feeling but is weaker than C because it doesn’t leverage the here-and-now moment.

Question 10. A client says: “I want to stop drinking, but then I just feel so anxious when I don’t that I pour myself a drink to calm down.” The counselor’s MOST appropriate response at this stage in treatment (second session, alliance being established) is:

- A. “It sounds like alcohol has become your primary way of managing anxiety.”
- B. “Have you considered that the alcohol may actually be making the anxiety worse?”
- C. “There’s a real tension there — you want to stop, but not drinking brings something painful.”
- D. “Let’s talk about what we could put in place to help you manage the anxiety without alcohol.”

Correct: C. The alliance is being built (second session). The correct response reflects the client’s ambivalence using *empathy*, not information (B) or premature problem-solving (D). A is a paraphrase-adjacent response but interprets rather than reflects. C holds the ambivalence and communicates that the counselor understands both sides — which is what MI-informed and alliance-building responses do.

Question 11. A client who usually speaks openly arrives and gives one-word answers, avoids eye contact, and crosses her arms. The BEST response is:

- A. Continue with the session agenda and monitor whether she opens up
- B. Ask directly whether something is wrong
- C. Reflect the nonverbal discrepancy: “I notice you seem different today — quieter, maybe more guarded.”
- D. Ask about any stressors that may have occurred since the last session

Correct: C. This is a **confrontation** of an incongruity between the client’s usual presentation and today’s nonverbal behavior. It names the observation tentatively and invites the client to explain. B is too blunt. A ignores clinical data. D redirects to external causes before the relational communication has been addressed.

Question 12. Which of the following BEST describes an appropriate use of counselor self-disclosure?

- A. “I know exactly how you feel — I went through a divorce myself several years ago.”
- B. “I want to be honest with you: when you describe your situation, I feel a sense of sadness for what you’ve been carrying.”
- C. “This reminds me of a client I worked with once who had a very similar story.”
- D. “I’ve struggled with anxiety too, so I understand why this is so hard for you.”

Correct: B. This self-disclosure is brief, genuine, client-focused, and uses the counselor’s felt response to deepen the alliance. A (claiming “I know exactly how you feel”) overidentifies and shifts focus. C (referencing another client) breaches confidentiality norms and shifts focus. D is well-intentioned but the length and framing make it counselor-centered.

Chapter 4 — Group Counseling & Group Work

Overview: The Group as Client

The single most important conceptual shift in group work is this: **the group is the client, not the collection of individuals.** The leader’s role is not to do individual counseling with multiple people watching. It is to tend the group as a living system — its norms, its cohesion, its stage, its safety.

Under pressure, the exam will test your ability to prioritize the group’s needs over an individual member’s immediate wishes. When one member dominates, the leader’s first responsibility is to the group’s ability to function. When conflict erupts between members, the leader’s job is to help the group process the conflict, not to adjudicate it. When a member discloses a serious personal crisis, the leader must both tend to that member and tend to the group’s response to the disclosure.

Leader priority order under pressure: 1. Safety of all group members 2. Group cohesion and the ability of the group to function 3. The therapeutic progress of individual members 4. Efficient use of group time

Group Types

Type	Purpose	Leader Role
Psychoeducational	Skill-building, information	Teacher/facilitator
Skills-training	Practice specific skills (DBT, parenting)	Coach/trainer
Cognitive-behavioral	Change distorted cognitions and behavior patterns	Active, structured
Support	Normalize, provide peer support	Facilitator, low-directive

Type	Purpose	Leader Role
Process/interpersonal	Initial group dynamics as change mechanism	Process observer, catalytic
Psychotherapy	Deep personality/relational change	Clinician, boundary-holder

Tuckman's Stages of Group Development

Stage	Group Task	Leader Focus
Forming	Inclusion, safety, initial trust	Establish norms, provide structure
Storming	Conflict, challenge, jockeying for position	Normalize conflict, protect safety, don't shut it down
Norming	Cohesion, established roles, deepened trust	Step back, facilitate member-to-member interaction
Performing	Productive work, self-disclosure, insight	Catalyst, minimal direction
Adjourning	Termination, integration, grief	Process endings, transfer of learning

Exam trap: in the **storming** stage, the instinct is to suppress conflict. The correct response is to *name and normalize* conflict while keeping it safe — because navigating conflict is part of the therapeutic work. A leader who deflects all conflict keeps the group in perpetual forming.

Yalom's Therapeutic Factors

Yalom identified 11 curative factors unique to group therapy. The exam most commonly tests these five:

Factor	What It Means
Universality	"I'm not alone in this" — the relief of shared human experience
Cohesion	The group's felt sense of connection and belonging (the group equivalent of alliance)
Altruism	Members gain from <i>giving</i> help to others, not just receiving it
Instillation of hope	Seeing others improve provides evidence that change is possible

Factor	What It Means
Interpersonal learning	The group becomes a social microcosm in which members can learn about their relational patterns

When the exam asks why group therapy is particularly powerful for clients with long-standing relational difficulties, the answer is **interpersonal learning** — the group provides live data about how the client relates to others that no individual therapy can replicate.

Group Ethics: The Confidentiality Problem

This is the most important ethical principle specific to group work: **the counselor can promise confidentiality from themselves but cannot guarantee it from other group members.** This must be disclosed in pre-group screening and in the group agreement. The counselor can establish a confidentiality *norm* and *expectation*, but cannot enforce it legally among members.

Exam implication: if a group member asks “Is what I share confidential?”, the correct answer acknowledges the norm and its limits — it does not simply say “yes.”

Leadership Skills with Mini-Scenarios

Linking connects what one member said to something another member expressed: “I notice that what Danielle is describing — feeling invisible at home — resonates with what Marcus shared last week about his work relationships. I wonder if there’s something there for the group.”

Blocking stops behavior that would harm the group: a member who gives unsolicited advice, asks intrusive questions, or dominates speaking time. “Let me pause you there, Derek — before you respond, I want to hear from others about their reactions to what Sarah just shared.”

Drawing out invites silent members to participate: “Jordan, you’ve been very still during this conversation. I’m curious what’s coming up for you.”

Cutting off — distinct from blocking — ends a member's sharing when it becomes too lengthy or is keeping the group stuck. Done respectfully but firmly.

Practice Questions 13–16

Question 13. During the sixth session of a process group, two members begin arguing about whose disclosure is “more serious.” Other members look uncomfortable. The group leader's BEST response is:

- A. Remind the group that all experiences are valid and redirect to the agenda
- B. Ask one of the arguing members to step back and listen
- C. Name the tension in the room and invite the group to explore what is happening between them
- D. Take a break and regroup after a few minutes of silence

Correct: C. This is storming behavior. The therapeutic response is to use the conflict as material — to name it, hold it, and invite the group to process it together. A deflects the conflict (misses the therapeutic opportunity). B singles out a member (not leader's adjudication role). D avoids (the conflict will persist).

Question 14. A new member joins an established process group and immediately begins sharing extensive personal history, causing other members to shift uncomfortably. The leader should:

- A. Allow the new member to finish, as interrupting would damage the alliance
- B. Provide a brief empathic reflection and redirect by checking in with the group
- C. Ask the new member to schedule an individual session to share the history
- D. Remind the new member of the group's disclosure norms at the next session

Correct: B. The leader must balance warmth toward the new member with responsibility to the group's functioning. A brief empathic reflection honors the disclosure; redirecting to the group restores the shared process. A ignores the group's discomfort. C is not the right mechanism. D is too delayed to address the current session dynamic.

Question 15. Which Yalom therapeutic factor is MOST specifically activated when a group member who struggled with addiction for years shares that she is celebrating two years of sobriety?

- A. Universality
- B. Altruism
- C. Instillation of hope
- D. Cohesion

Correct: C. Seeing a member succeed provides evidence that recovery is possible — this is the definition of instillation of hope. Universality is about shared pain, not observed success. Altruism involves giving help. Cohesion is the relational bond, not a specific event.

Question 16. Before a new member joins a therapy group, the counselor explains that what members share in group is confidential. Which statement MOST accurately represents the counselor's ethical obligation?

- A. "Everything shared in this group is confidential by law."
- B. "I will keep what you share confidential, and I strongly encourage other members to do the same — though I cannot legally bind them."
- C. "Group members have agreed to confidentiality, so you can be assured your disclosures are protected."
- D. "Confidentiality in groups works the same way as in individual therapy."

Correct: B. This is the only response that accurately distinguishes what the *counselor* can guarantee (their own confidentiality) from what they *cannot* guarantee (member confidentiality). All other options overstate the protection.

Chapter 5 — Putting It Together: Worked Case Walkthrough

The Case

Presented during the intake session:

Marcus is a 38-year-old African-American male, self-referred, who presents with “stress at work and feeling disconnected from everything.” He reports that for the past six weeks he has had difficulty sleeping (waking at 3–4 a.m. and unable to return to sleep), low energy, difficulty concentrating at work, and a persistent feeling of “emptiness.” He denies suicidal ideation but says, “Some days I wonder what the point is.” He drinks 3–4 beers most evenings, which he says “takes the edge off.” He has a primary care physician but has not had a checkup in two years. He is currently taking no medications. He is married with two children and describes his marriage as “strained.”

Decision Point 1: Safety

The question: What do you assess first?

Marcus said “I wonder what the point is.” This phrase **requires direct safety assessment**. The counselor cannot assume it is not suicidal ideation. The Priority Ladder places imminent safety at rung 1.

Correct action: Conduct a direct, empathic inquiry — “When you say you wonder what the point is, I want to make sure I understand. Are you having any thoughts of harming yourself or ending your life?”

Why the distractors lose: - Moving directly to depression screening (PHQ-9): premature until safety is established - Exploring the

marital strain: premature — safety first - Asking about the drinking: important but lower rung than safety

Decision Point 2: Medical and Substance Rule-Outs

The question: After safety is established (he denies active SI with no plan or intent), what is next?

Two rung-2 issues are present: 1. **Medical rule-out:** Sleep disturbance, fatigue, and cognitive difficulties can have thyroid, metabolic, or other medical causes. He hasn't had a checkup in two years. 2. **Substance consideration:** 3–4 beers nightly is above the NIAAA low-risk threshold. Alcohol disrupts sleep architecture (explaining the early awakening), worsens depression, and must be quantified before diagnosing a mood disorder.

Correct action: Refer for a medical evaluation. Assess the alcohol use with a standardized tool (AUDIT or CAGE).

Why this matters: If the depression is alcohol-induced, treating depression with psychotherapy alone while ignoring the substance use will fail. The diagnosis cannot be confirmed until substance contribution is ruled out.

Decision Point 3: Differential

The leading question: Is this Major Depressive Disorder, Persistent Depressive Disorder, Alcohol-Induced Depressive Disorder, or a normal stress response?

Feature	MDD	PDD	Alcohol-Induced	Normal Stress
Duration	≥2 weeks (episode)	≥2 years (chronic low-grade)	Present only during/after use	Days to weeks; resolves with stressor
Severity	Moderate-severe	Mild-moderate, persistent	Varies	Mild-moderate

Feature	MDD	PDD	Alcohol-Induced	Normal Stress
Key feature	Episodic; Never often had a higher base-line	free of depression for >2 months	Temporally linked to alcohol	Stressor-responsive
Marcus's fit	✓ (6 weeks, moderate symptoms)	Possible if longer history	Possible — alcohol use is significant	Less likely given severity

The 6-week timeline fits an MDD episode. However, without knowing whether he was depressed before the drinking escalated, the alcohol question must be resolved first (rung 2 before rung 6).

Decision Point 4: Alliance

Marcus has mentioned “feeling disconnected.” He has not previously sought mental health treatment. His cultural background and self-referral both matter: self-referral suggests motivation, but “feeling disconnected” may mean the therapeutic relationship itself feels unfamiliar or unsafe.

Correct action before introducing any assessment tools: validate the courage of coming in, reflect the experience of carrying all of this privately, and explicitly invite his input on what kind of help would feel useful.

Decision Point 5: Treatment Planning

Once safety is cleared, medical rule-out is initiated, substance use is assessed, and the alliance is established:

- If MDD confirmed: CBT or behavioral activation is first-line evidence-based treatment. If alcohol use disorder is also present, treat both (integrated treatment is superior to sequential).

- If PDD confirmed: CBT + medication consultation (dysthymia responds to combined treatment).
 - Involve the marriage — the marital strain is both a stressor and a maintenance factor.
-

Scoring Summary

Decision	Priority Rung	Correct Action
Safety	1	Direct SI inquiry before anything else
Medical / substance	2	Refer for checkup; administer AUDIT
Alliance	4	Validate before assessing
Assessment	5	PHQ-9, AUDIT, further history
Treatment	6	CBT/behavioral activation + substance treatment if indicated

The case illustrates that clinical sophistication on the NCMHCE is not about knowing more diagnoses — it is about holding the Priority Ladder correctly under the pressure of a complex presentation.

End of Part One

PART TWO: DIAGNOSTIC REFERENCE

Chapters 6-14

Each chapter follows the standard rhythm: Overview → Diagnosis table → Clinical picture → Differentials → Assessment tools → What the exam rewards/punishes → Worked mini-case → Practice questions.

Chapter 6 — Depressive Disorders

Overview

Depressive disorders are the most commonly tested diagnostic family on the NCMHCE. They appear in nearly every case simulation — either as a primary diagnosis, as a comorbidity, or as part of a differential. The exam’s highest-yield question about this family is: **how severe, how long, and what was present before?** Those three dimensions separate the diagnoses from each other and from normal grief or adjustment.

The second key point: **the NCMHCE 2022+ format typically provides the diagnosis.** Your job is to demonstrate what comes after the diagnosis — the correct assessment sequence, the correctly prioritized interventions, the right evidence-based treatment, and the awareness of what could go wrong. This means you do not need to memorize DSM criteria for their own sake; you need to know what each diagnosis *looks like in a case* and what to do about it.

Depression questions on the exam specifically test: 1. The distinction between MDD and PDD (chronic but lower-grade) 2. The role of grief — when grief becomes MDD 3. Medical and substance rule-outs (hypothyroidism, B12, sleep disorders, alcohol) 4. The priority of safety assessment (suicidality is always rung 1) 5. The evidence base: CBT and behavioral activation for MDD; combined CBT + medication for PDD

Diagnosis Reference Table

Diagnosis	ICD-10	Key Feature	First-Line Treatment
Major Depressive Disorder (MDD)	F32.x / F33.x	≥5 symptoms ≥2 weeks, including depressed mood or anhedonia	CBT, behavioral activation, medication

Diagnosis	ICD-10	Key Feature	First-Line Treatment
Persistent Depressive Disorder (Dysthymia)	F34.1	Depressed mood ≥ 2 years (adults), never symptom-free >2 months	CBT + medication (combined superior)
Disruptive Mood Dysregulation Disorder	F34.8	Severe recurrent temper outbursts + persistent irritability, children 6–18	Multimodal (therapy + psychoeducation)
Premenstrual Dysphoric Disorder	F32.81	Mood symptoms in the week before menses, resolve after onset	SSRIs, lifestyle, CBT
Substance/Medication-Induced Depressive Disorder	F10.14	Temporally linked to substance use or medication	Treat underlying substance use
Depressive Disorder Due to Another Medical Condition	F06.31	Etiologically linked to medical condition	Treat underlying medical condition

The Clinical Picture

In a case simulation, a client with **MDD** typically presents with a cluster of changes over the past two to four weeks. The narrative usually emphasizes: the *change from baseline* (this person used to function differently), the *pervasiveness* (multiple domains — sleep, appetite, concentration, social withdrawal), and the *distress or impairment*. The client may not use the word “depressed” — more often they describe feeling “numb,” “empty,” “like I’m going through the motions,” or “just tired of everything.” Tearfulness is common but not universal. Men with depression more frequently present with irritability than sadness; missing this is a common exam trap.

Anhedonia — the inability to feel pleasure from previously enjoyable activities — is one of the two cardinal symptoms. When a case description says “she used to love hiking but hasn’t left the house in weeks,” that is anhedonia.

A client with **PDD** (persistent depressive disorder) presents differently: the story is longer, more chronic, and often normalized. The client may say “I’ve just always been kind of a low-energy person”

or “this is just how I am.” They may never have experienced a symptom-free period they can remember. The depressive state feels like their personality, not an episode. This normalization of chronic depression is a key clinical marker — and it means these clients often present later and are harder to engage in treatment.

Grief vs. MDD: Normal grief includes waves of sadness, longing, and impaired function — but the bereaved person can typically be consoled, finds moments of positive emotion, and does not experience the pervasive anhedonia and worthlessness of MDD. The DSM-5 removed the bereavement exclusion, recognizing that grief can co-occur with or precipitate MDD. The exam expects you to know: persistent grief that meets MDD criteria for two weeks or more *is* MDD and warrants treatment.

Differentiating Within the Family

MDD vs. PDD: The deciding feature is *duration*. MDD is episodic — there was a baseline the client fell from. PDD is chronic — the client has been depressed, at a lower severity, for at least two years without a two-month remission. A client can have both simultaneously (“double depression”): a PDD baseline with a current MDD episode layered on top. In the exam, “double depression” usually appears when a chronically dysthymic client suddenly deteriorates significantly.

MDD vs. Bipolar Depression: This distinction is critical — wrong diagnosis leads to wrong treatment (antidepressants alone can destabilize bipolar disorder). Look for: prior hypomanic or manic episodes (elevated or expansive mood, decreased need for sleep with increased energy, grandiosity, impulsivity). A thorough history is essential. The exam often tests this by presenting a depressed client with a *history* — buried in the vignette — of a period of feeling unusually energized, creative, needing little sleep, making impulsive decisions. Missing that history means missing the bipolar diagnosis.

MDD vs. Adjustment Disorder with Depressed Mood: Adjustment disorder occurs within three months of an identifiable stressor, is proportionate to the stressor, and resolves within six months of the stressor ending. If the symptoms meet MDD criteria regardless of the stressor’s presence, the diagnosis is MDD.

Assessment Tools in Context

PHQ-9 (Patient Health Questionnaire-9): Nine items, each scored 0-3. Total score 0-27. - 0-4: minimal depression - 5-9: mild - 10-14: moderate - 15-19: moderately severe - 20-27: severe

When to use: initial screening, ongoing monitoring of treatment response. Note: Item 9 asks directly about suicidal ideation — a positive response requires safety assessment regardless of total score.

Beck Depression Inventory-II (BDI-II): 21 items. Widely used in clinical and research settings. Distinguishes cognitive (worthlessness, guilt) from somatic (sleep, appetite) symptoms.

Hamilton Depression Rating Scale (HAM-D): Clinician-administered (not self-report). Used to rate severity in research and inpatient contexts. Know that it is clinician-rated.

Columbia Suicide Severity Rating Scale (C-SSRS): Not a depression measure — a suicidality measure. Always use when suicide risk is being assessed (see Chapter 17 for full treatment).

What the Exam Rewards / What It Punishes

Rewards: - Assessing suicidality *before* anything else when hopelessness, worthlessness, or passive death ideation is present (“what’s the point,” “everyone would be better off without me”) — rung 1 always - Ruling out medical causes and substances before confirming the diagnosis — thyroid, B12, sleep disorders, alcohol, steroids, beta-blockers - Recognizing the grief-MDD distinction and treating grief-triggered MDD as MDD - Identifying PDD’s chronicity and recommending combined treatment (therapy + medication) - Recognizing male depression may present as irritability, not sadness

Punishes: - Moving to treatment without safety assessment - Diagnosing MDD without exploring prior manic/hypomanic episodes - Dismissing chronic dysthymia as “just personality” - Prescribing antidepressants alone for PDD without recommending psychotherapy - Missing the somatic presentation of depression in medical comorbidity cases

Worked Mini-Case

Intake vignette: Carla, 44, is referred by her primary care physician following a routine visit. She reports she has “never really been happy” — as long as she can remember, she has felt “kind of flat.” Over the past three weeks, however, she has stopped showering, has called out of work twice, is sleeping 10+ hours but feels no more rested, and told her sister “I’m not sure I want to be here anymore.”

Reasoning chain:

Step 1 — Safety: “Not sure I want to be here anymore” is passive suicidal ideation. This requires immediate, direct assessment before any other clinical action. (*What is the severity? Is there a plan? Intent? Means?*)

Step 2 — Differential: Carla has a long history of low-grade depression (PDD criteria: years of depressed mood) *plus* a recent acute deterioration (hypersomnia, hygiene breakdown, work absence, passive SI — MDD episode criteria). This is likely *double depression*.

Step 3 — Medical rule-out: Has her PCP already evaluated thyroid function? B12? Other metabolic causes of the acute change? The referral was from her PCP, so coordinate.

Step 4 — Treatment: Double depression requires combined treatment — CBT or IPT plus medication consultation. Behavioral activation alone is insufficient for the chronic PDD layer. The acute safety risk must be managed first (safety plan, evaluate need for higher level of care).

Answer selection logic: On an exam question about “first action,” the answer is always the safety assessment. On a question about “appropriate treatment,” the answer is combined therapy + medication with safety planning. On a question about “what complicates the prognosis,” the answer is the chronic PDD substrate.

Practice Questions

Question 17. A client with a confirmed MDD diagnosis has been in CBT for six weeks. She reports improvement in sleep and concentration but continues to endorse a “nagging sense that nothing

matters.” PHQ-9 score has dropped from 17 to 9. The counselor should:

- A. Discontinue CBT and refer for medication evaluation
- B. Continue CBT and reassess; a score of 9 suggests mild residual symptoms
- C. Add a focus on behavioral activation and meaning-making
- D. Refer to a psychiatrist because medication is required for full remission

Correct: C. A score of 9 represents mild, not remitted, depression. Anhedonia and loss of meaning often respond well to meaning-focused work and behavioral activation. Neither discontinuation nor mandatory medication referral is indicated at this stage. B is partially correct but doesn’t offer clinical direction. C correctly names what to add.

Question 18. A 52-year-old man presents with fatigue, low mood, cognitive slowing, weight gain, and hair loss over four months. He attributes his symptoms to “getting older.” The MOST appropriate next step is:

- A. Administer the PHQ-9 and begin CBT
- B. Refer for a medical evaluation, including thyroid function tests
- C. Diagnose MDD and initiate a treatment plan
- D. Provide psychoeducation about depression and schedule the next session

Correct: B. The symptom cluster — fatigue, cognitive slowing, weight gain, hair loss — is consistent with hypothyroidism. Medical rule-out (rung 2) must precede any mood disorder diagnosis. Proceeding directly to CBT or MDD diagnosis (A, C, D) before ruling out a medical cause violates the priority ladder.

Question 19. Which of the following presentations MOST suggests the need to assess for bipolar disorder before diagnosing and treating MDD?

- A. A client who reports feeling “numb” and has lost interest in activities she previously enjoyed
- B. A client who endorses depressed mood, hypersomnia, and significant weight gain
- C. A client who reports a three-week period six years ago when she

felt “on top of the world,” barely slept, and made several impulsive financial decisions

D. A client who has had two prior MDD episodes that responded well to antidepressants

Correct: C. The three-week period of elevated mood, decreased need for sleep, and impulsivity is a possible hypomanic or manic episode — which would indicate Bipolar I or II. A and B are classic MDD presentations without bipolar features. D suggests recurrent MDD with good treatment response, not bipolar.

Question 20. A client who lost her spouse eight weeks ago is unable to work, cannot stop crying, feels that life is meaningless, and endorses passive death ideation (“I just want to be with him again”). The counselor should:

A. Diagnose adjustment disorder with depressed mood, as grief is a normal response to loss

B. Provide supportive counseling only, as medication is contraindicated in grief

C. Assess fully for MDD and suicidality; grief-triggered depression meeting MDD criteria warrants treatment

D. Refer for complicated grief treatment and avoid any diagnostic label during the grieving period

Correct: C. The DSM-5 removed the bereavement exclusion. When grief meets MDD criteria, the diagnosis is MDD and treatment is indicated. Passive death ideation (“I want to be with him”) requires direct safety assessment. A mislabels the severity. B is incorrect — medication is not contraindicated in grief-triggered MDD. D delays needed intervention.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 46-year-old man reports three weeks of depressed mood, anhedonia, insomnia, poor concentration, and worthlessness following a job loss. He has no prior psychiatric history and denies any period of elevated mood or decreased need for sleep. His functioning at home is impaired. The most likely diagnosis is:

- A. Adjustment Disorder with Depressed Mood
- B. Major Depressive Disorder
- C. Persistent Depressive Disorder
- D. Bipolar II Disorder

Answer: B. Five or more symptoms for at least two weeks with impairment meets MDD; a precipitating stressor does not downgrade it to Adjustment Disorder once full MDD criteria are met (A is the trap). PDD requires a two-year course (C loses on duration). No hypomania history rules out Bipolar II (D). *What would change the answer:* a past hypomanic episode buried in the history → Bipolar II; subthreshold symptoms or duration under two weeks → Adjustment Disorder.

Assessment — what to do next. To establish a baseline and monitor response, the most appropriate assessment step is:

- A. Administer the PHQ-9 and directly follow up Item 9 (suicidal ideation)
- B. Administer the GAD-7 as the primary measure
- C. Administer the MDQ and treat as bipolar
- D. Order thyroid labs yourself

Answer: A. The PHQ-9 is the standard depression measure, and a positive Item 9 requires direct SI assessment regardless of total score. The GAD-7 (B) targets anxiety; the MDQ (C) screens for bipolarity but is not the depression baseline. Ordering labs (D) is outside counselor scope — the counselor refers for medical rule-out.

Treatment Decision Flow

Treatment items rarely ask “what is first-line?”; they ask what to do for *this* client, in what order, and what to avoid.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
MDD with passive suicidal ideation	Safety plan, increase session frequency, begin behavioral activation	Starting the full CBT protocol without addressing the SI
Moderate-to-severe MDD	Combined CBT + medication referral (superior to either alone)	Offering CBT alone as sufficient
Client cannot identify or articulate automatic thoughts	Lead with behavioral activation (a behavioral entry point)	Forcing cognitive restructuring the client cannot yet engage

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Double depression (MDD layered on PDD)	Combined therapy + medication referral; manage acute risk first	Behavioral activation alone for the chronic PDD substrate

Sequencing & caveats. Behavioral activation is specifically indicated when apathy and withdrawal are the primary barriers, but it must be paired with a safety plan when passive SI is present. For moderate-to-severe presentations, combined treatment outperforms monotherapy.

Avoid / contraindicated. Do not begin antidepressant-oriented planning without first screening for a history of hypomania or mania (see Chapter 11) — antidepressant monotherapy can destabilize an undetected bipolar disorder. CBT alone is insufficient when a cognitive deficit prevents the client from accessing automatic thoughts.

Scope. The counselor provides psychoeducation about combined treatment and makes a formal referral to a psychiatrist or psychiatric NP; the counselor does not select or recommend a specific agent.

Chapter 7 — Anxiety Disorders

Overview

Anxiety disorders are the most prevalent mental health conditions in the United States and among the most frequently appearing on the NCMHCE. The exam tests two primary skills with this family: (1) distinguishing the specific anxiety diagnoses from each other based on the *trigger and focus of the fear*, and (2) knowing the evidence-based treatment hierarchy (which always includes exposure-based work).

The central organizing principle: **anxiety disorders differ by what the person fears and what they avoid**. The feared stimulus, the physiological response, and the behavioral response (avoidance) together define the diagnosis. Knowing the feared stimulus is your key to differentiating.

Diagnosis Reference Table

Diagnosis	ICD-10	Core Fear / Trigger	First-Line Treatment
Generalized Anxiety Disorder (GAD)	F41.1	Pervasive, uncontrollable worry about multiple topics	CBT, relaxation training, medication (SSRI/SNRI)
Panic Disorder	F41.0	Fear of panic attacks themselves (interoceptive conditioning)	CBT with interoceptive exposure, medication
Social Anxiety Disorder	F40.10	Fear of negative evaluation in social/performance situations	CBT, exposure, medication (SSRI)
Specific Phobia	F40.2xx	Fear of specific object/situation (heights, blood, animals, etc.)	Systematic desensitization, exposure
Agoraphobia	F40.00	Fear of situations from which escape might be difficult or embarrassing	Exposure therapy, medication

Diagnosis	ICD-10	Core Fear / Trigger	First-Line Treatment
Separation Anxiety Disorder	F93.0	Fear of separation from attachment figures	CBT, family involvement, graduated exposure
Selective Mutism	F94.0	Consistent failure to speak in specific social situations despite capability	Behavioral treatment, exposure, school collaboration

The Clinical Picture

GAD presents as a worry that does not settle — the client cannot stop, cannot turn the volume down, and has been this way for months or years. The worry shifts topics: health, finances, relationships, the future. The client often describes the worry as “out of control” and may be surprised to learn that not everyone worries this persistently. Physical symptoms include muscle tension, fatigue, irritability, sleep difficulty, and difficulty concentrating. The DSM-5 requires six months of excessive worry about multiple areas.

Exam tip: GAD clients are often high-functioning. They appear at intake because the worry has finally crossed a threshold — it is causing relationship problems, physical health effects, or occupational impairment. The chronicity of the worry (it has always been there to some degree) is key.

Panic Disorder is distinguished by the presence of unexpected panic attacks *plus* persistent concern about having more attacks *or* significant behavioral change to avoid them. The panic attack itself — a sudden surge of intense fear peaking within minutes — is not sufficient for the diagnosis. The disorder requires the secondary fear of the attack, or avoidance. A client who has had several panic attacks but is not particularly worried about having more and has not changed behavior does not have panic disorder.

Key exam detail: **Panic attacks can occur in multiple disorders** — they are not unique to panic disorder. A client with social anxiety disorder may have situationally triggered panic attacks in social settings. A client with PTSD may have panic attacks triggered by trauma reminders. The presence of a panic attack does not equal panic disorder.

Social Anxiety Disorder (formerly social phobia) is the fear of scrutiny, humiliation, or negative evaluation in social or perfor-

mance situations. The client avoids situations where they might be observed, judged, or embarrassed. The exam sometimes presents a client who seems depressed (withdrawn, isolated, not engaging with others) — the key is whether the withdrawal is driven by low mood (depression) or by fear of judgment (social anxiety).

Specific Phobia is the most focused of the anxiety disorders — one discrete feared stimulus (or category). The exam sometimes tests the subtypes: animal, natural environment (heights, storms, water), blood-injection-injury, situational (elevators, planes), and other. The blood-injection-injury subtype is notable because it produces a *vasovagal response* — a drop in heart rate and blood pressure, sometimes leading to fainting — rather than the sympathetic arousal typical of other phobias.

Differentiating Within the Family

GAD vs. Depression: Both involve fatigue, sleep problems, concentration difficulties, and dysphoria. The deciding feature: in GAD, worry is the central symptom; in depression, anhedonia and dysphoric mood are central. The client with GAD typically worries about bad things that might happen in the future; the client with depression ruminates on loss, failure, and worthlessness about the past and present.

GAD vs. Panic Disorder: In GAD, anxiety is chronic and diffuse (many topics, constant low-level worry). In panic disorder, anxiety is episodic and intense (discrete attacks). A client can have both.

Social Anxiety Disorder vs. Schizoid Personality Disorder: Both involve social withdrawal, but the mechanisms differ entirely. Schizoid PD involves genuine indifference to social relationships — the person does not want them and is not distressed by their absence. Social anxiety disorder involves a *desire* for social connection combined with *fear* of it. The client with social anxiety disorder wants to attend the party but cannot; the person with schizoid PD doesn't want to go.

Social Anxiety Disorder vs. Avoidant Personality Disorder: These exist on a continuum. AvPD is essentially more severe, pervasive social anxiety with greater functional impairment and a more ingrained self-view of being inadequate. The treatment is the same (CBT, exposure) but longer.

Assessment Tools in Context

GAD-7 (Generalized Anxiety Disorder 7-item scale): Seven items, scored 0–3. Scores of 5, 10, and 15 are thresholds for mild, moderate, and severe anxiety. Commonly used in primary care. Know cutoffs.

Beck Anxiety Inventory (BAI): 21 items, self-report. Focuses on somatic and cognitive anxiety symptoms. Developed alongside the BDI-II; often used together.

Panic Disorder Severity Scale (PDSS): Seven items rating panic frequency, distress, anticipatory anxiety, avoidance, and impairment. Clinician-administered.

SUDS (Subjective Units of Distress Scale): A 0–100 rating of current distress/anxiety. Not a standalone measure — used within exposure therapy to track anxiety throughout a session.

What the Exam Rewards / What It Punishes

Rewards: - Using exposure as the core treatment for all anxiety disorders (with systematic desensitization as a variant for specific phobias) - Recognizing that avoidance maintains anxiety — treatment must address avoidance, not just symptoms - Distinguishing GAD's chronicity and breadth from panic disorder's episodic intensity - Identifying the secondary fear (fear of the attack) as what defines panic disorder, not the attack itself - Knowing that blood-injection-injury phobia has a different physiological response (vasovagal, not sympathetic arousal) and requires adapted exposure (applied tension technique)

Punishes: - Treating anxiety with relaxation alone without addressing avoidance - Diagnosing panic disorder based on panic attacks alone without the secondary apprehension or behavioral change - Missing comorbid depression in anxiety clients (depression very commonly co-occurs) - Recommending prolonged benzodiazepine use as first-line (this maintains avoidance and produces dependence) - Confusing social anxiety disorder's motivated-but-fearful presentation with schizoid PD's indifferent withdrawal

Worked Mini-Case

Intake vignette: Jordan, 28, presents describing episodes of heart racing, shortness of breath, dizziness, and a sense of unreality that come “out of nowhere” and last about ten minutes. He has had four such episodes in the past two months. Since the first episode, he has been monitoring his heart rate constantly, avoids exercise (because it raises his heart rate), and has stopped taking public transit because “I had one there once and I couldn’t get off.” He has visited the emergency department twice and been medically cleared each time.

Reasoning chain:

Step 1 — Safety/Medical: Already medically cleared twice. This is relevant — it rules out cardiac causes and supports the anxiety diagnosis.

Step 2 — Differential: Jordan has unexpected panic attacks (confirmed), persistent concern about having more attacks (heart rate monitoring), and significant behavioral changes to avoid them (no exercise, no transit). This is Panic Disorder.

Step 3 — Avoidance pattern: His avoidance has expanded (transit joins exercise). This is the hallmark of panic disorder’s progressive course — without treatment, the avoidance spreads.

Step 4 — Treatment: First-line is CBT with *interoceptive exposure* — deliberately inducing panic-like sensations (through exercise, spinning, breathing exercises) in a controlled setting to break the interoceptive fear conditioning. Psychoeducation about the physiology of panic (the alarm is false; the sensations are not dangerous) is essential. Avoidance of feared situations must also be addressed with situational exposure (transit, exercise).

Answer on treatment: The exam will offer options including relaxation training, CBT with exposure, medication referral, and supportive counseling. The correct answer is CBT with exposure. Relaxation alone is insufficient because it does not address avoidance. Medication alone does not produce durable change without addressing avoidance.

Practice Questions

Question 21. A client with GAD has been in treatment for three months. She has learned progressive muscle relaxation and diaphragmatic breathing. She reports that the exercises help “in the moment” but that the worry always comes back. The counselor should:

- A. Teach additional relaxation strategies and increase practice frequency
- B. Add a cognitive component targeting the function of worry and addressing avoidance
- C. Refer for medication evaluation, as CBT has been insufficient
- D. Begin exposure therapy targeting the specific situations she worries about

Correct: B. Relaxation addresses the somatic symptoms of GAD but does not address the cognitive maintenance factors (intolerance of uncertainty, metacognitive beliefs about worry, worry as avoidance of deeper feared images). The treatment is incomplete. D is tempting but is more directly applicable to phobias/panic — for GAD, the cognitive restructuring of worry beliefs and worry postponement are key additions. C is premature at three months.

Question 22. Which feature MOST clearly distinguishes a specific phobia diagnosis from a normal fear response?

- A. The fear is disproportionate to the actual danger
- B. The feared stimulus is present and identifiable
- C. The fear causes the person to avoid the feared stimulus
- D. The fear is persistent, causes significant distress, and the avoidance or endurance causes functional impairment

Correct: D. Many fears are disproportionate (A) and many people avoid things they’re scared of (C) — neither alone makes a diagnosis. The distinguishing features are: the persistence, the significant distress, and the functional impairment. B describes a trigger, not a diagnostic criterion.

Question 23. A client reports severe fear and avoidance of blood draws, to the point that she has avoided necessary medical care for two years. She describes that when exposed to needles or blood,

she initially feels panicky but then becomes lightheaded and has fainted twice. The counselor should:

- A. Apply standard systematic desensitization, as this is a specific phobia
- B. Use applied tension technique, as the vasovagal response requires a different approach
- C. Refer to a physician first, as fainting suggests a medical issue
- D. Treat with interoceptive exposure, as this presentation resembles panic disorder

Correct: B. Blood-injection-injury phobia produces a vasovagal (fainting) response rather than the sympathetic arousal typical of anxiety. Standard exposure would drive the heart rate up and then allow the vasovagal drop — potentially reinforcing fainting. Applied tension (tensing large muscle groups to maintain blood pressure) is the evidence-based adaptation. C is not needed — vasovagal fainting in this context is a known response to the phobia. D is incorrect — this is not panic disorder.

Question 24. A client endorsed daily, pervasive worry about finances, health, relationships, and work — for the past three years. She says “I’ve always been a worrier.” She also reports that she frequently lies awake at night and feels “keyed up” throughout the day. The MOST likely diagnosis is:

- A. MDD
- B. Panic Disorder
- C. GAD
- D. OCD

Correct: C. Three years of pervasive, uncontrollable worry about multiple domains, with insomnia and hyperarousal, is the clinical picture of GAD. MDD (A) would feature anhedonia and depressed mood as the central feature. Panic disorder (B) involves episodic attacks. OCD (D) involves intrusive obsessions and compulsions, not general worry.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 29-year-old reports recurrent, unexpected surges of intense fear that peak within minutes — pounding heart, shortness of breath, and a fear of dying — followed by a month of worrying about the next surge and avoiding the gym where the first one occurred. The most likely diagnosis is:

- A. Generalized Anxiety Disorder
- B. Panic Disorder
- C. Specific Phobia
- D. Social Anxiety Disorder

Answer: B. Recurrent *unexpected* panic attacks plus at least a month of anticipatory worry and behavior change define Panic Disorder. GAD (A) is chronic, diffuse worry without discrete attacks. The avoidance is cued by the panic itself, not by a specific object (C) or by social scrutiny (D). *What would change the answer:* attacks only in social-evaluative settings → Social Anxiety; only around a specific object → Specific Phobia.

Assessment — what to do next. Before beginning treatment, the most important assessment step is:

- A. Administer the GAD-7 as the primary measure
- B. Rule out medical contributors (thyroid, cardiac, caffeine/stimulants) via referral and screen substance use
- C. Begin interoceptive exposure to gauge tolerance
- D. Administer a personality inventory

Answer: B. Panic symptoms overlap with hyperthyroidism, arrhythmia, and stimulant or caffeine use; these are ruled out before diagnosing and treating. The GAD-7 (A) is the wrong target; exposure (C) is treatment, not assessment.

Treatment Decision Flow

The exam tests whether you know that relaxation is a *component*, not a treatment, and that avoidance is the target — not something to accommodate.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
GAD	CBT with cognitive restructuring, worry postponement, and behavioral experiments that test catastrophic predictions	Choosing relaxation-only or muscle relaxation as the standalone treatment
Panic disorder	Interoceptive exposure plus cognitive restructuring	Accommodating avoidance or providing repeated reassurance
Specific phobia	Graduated in-vivo exposure	Long-term benzodiazepine use or avoidance
Client refuses exposure	Validate the fear, provide psychoeducation on how avoidance maintains anxiety, collaboratively build the hierarchy	Switching to relaxation-only or terminating

Sequencing & caveats. Relaxation training is an adjunctive skill, not a standalone treatment. Exposure is first-line for phobia and panic; for GAD the active ingredients are cognitive restructuring and worry exposure, not relaxation alone.

Avoid / contraindicated. Long-term benzodiazepines interfere with the extinction learning that exposure depends on. Relaxation-only for GAD is the classic exam trap.

Scope. Where medication is indicated (SSRIs are typical first-line pharmacotherapy), the counselor refers for evaluation rather than recommending an agent.

Chapter 8 — Trauma and Stressor-Related Disorders

Overview

The trauma chapter is among the highest-yield diagnostic chapters on the NCMHCE — not because PTSD is complicated to identify, but because the exam tests the nuances: subthreshold presentations, the difference between acute stress disorder and PTSD, the distinction between PTSD and adjustment disorder, complex trauma presentations, and the critical treatment hierarchy.

The non-negotiable clinical principle: **trauma clients require safety and stabilization before any processing work begins.** Moving to trauma memory processing before the client has adequate distress tolerance skills, a stable therapeutic alliance, and a safe living environment is a clinical error that causes harm. The exam will present scenarios where the wrong answer is “begin trauma processing immediately” — always stabilize first.

Diagnosis Reference Table

Diagnosis	ICD-10	Key Feature	Duration	First-Line Treatment
PTSD	F43.10	Re-experiencing, avoidance, negative cognitions/mood, hyperarousal following Criterion A trauma	>1 month	Trauma-focused CBT (TF-CBT), EMDR, CPT, PE
Acute Stress Disorder	F43.0	Same symptom domains as PTSD	3 days - 1 month post-trauma	Stabilization, CBT, crisis support

Diagnosis	ICD-10	Key Feature	Duration	First-Line Treatment
Adjustment Disorder	F43.2x	Emotional/behavioral symptoms in response to stressor; does not meet criteria for another disorder	Within 3 months of stressor; resolves within 6 months	Supportive therapy, brief CBT
Reactive Attachment Disorder	F94.1	Disturbed social relatedness in children following inadequate caregiving	Early childhood	Caregiver-focused intervention, dyadic therapy
Disinhibited Social Engagement Disorder	F94.2	Indiscriminate attachment in children; approaches strangers with little hesitation	Early childhood	Caregiver stability, attachment-focused work

The Clinical Picture

PTSD presents across four symptom clusters. Understanding these helps you identify PTSD in a case sim where the diagnosis isn't named:

1. **Re-experiencing (intrusion):** Flashbacks, nightmares, intrusive memories, psychological distress when reminded of the trauma. The client “goes back” — not just remembers, but *re-lives*.
2. **Avoidance:** Avoids trauma-related thoughts, feelings, people, places, situations, conversations. The avoidance maintains the disorder by preventing natural processing.
3. **Negative alterations in cognition and mood:** Distorted blame, persistent negative emotions, diminished interest, feeling detached from others, inability to experience positive emotions (“emotional numbing”), persistent negative beliefs (“the world is dangerous,” “I am permanently damaged”).
4. **Hyperarousal and reactivity:** Hypervigilance, exaggerated startle, sleep disturbance, irritability, reckless or aggressive behavior, concentration problems.

The exam sometimes presents PTSD in non-stereotypical ways. A

combat veteran is the clichéd presentation — but PTSD also follows childhood abuse, sexual assault, medical trauma, accidents, witnessing violence, and workplace incidents. The Criterion A trauma does not have to be experienced directly — it can be witnessed or learned about (if it affects a close person).

Complex PTSD (not in DSM-5 as a standalone diagnosis, but recognized in ICD-11 and clinically important): results from prolonged, repeated trauma (childhood abuse, domestic violence, captivity). Presents with affect dysregulation, dissociation, disrupted self-perception, disturbed relationships, and loss of systems of meaning *in addition to* core PTSD symptoms. Treatment must include a sustained stabilization phase before any exposure-based work.

Acute Stress Disorder is PTSD's time-limited precursor — the same symptoms, but within the first month. ASD predicts later PTSD. Early intervention (trauma-focused CBT within the acute phase) can reduce PTSD incidence.

Differentiating Within the Family

PTSD vs. Adjustment Disorder: Both follow a stressor. The key distinction: PTSD requires a Criterion A traumatic stressor (actual or threatened death, serious injury, sexual violence) and specific symptom clusters (re-experiencing, avoidance, negative cognitions, hyperarousal). Adjustment disorder occurs after a stressor of *any* type (job loss, divorce, moving) and doesn't require those specific symptom clusters. Adjustment disorder is, in effect, a clinical response to a stressor that does not meet criteria for any other disorder.

PTSD vs. MDD: Depression and PTSD commonly co-occur. The differentiating features: PTSD has re-experiencing (flashbacks, nightmares), avoidance of trauma-related cues, and hyperarousal — none of which are cardinal MDD features. When both are present, both are diagnosed (comorbidity).

PTSD vs. Dissociative Disorders: Dissociation can occur in PTSD (as a symptom within the negative alterations cluster or as the dissociative subtype of PTSD). The dissociative subtype of PTSD presents with depersonalization and derealization alongside PTSD symptoms. True dissociative disorders (DID, depersonalization/derealization disorder) are distinct entities (see Chapter 20).

Assessment Tools in Context

PCL-5 (PTSD Checklist for DSM-5): 20-item self-report measure aligned with DSM-5 criteria. Frequently used for screening and monitoring treatment response. Scores ≥ 31 –33 suggest probable PTSD (cutoff varies by context). Each item maps to a DSM-5 symptom — clinically useful for tracking specific clusters.

CAPS-5 (Clinician-Administered PTSD Scale): The gold standard — a structured clinician interview that assesses PTSD diagnosis and severity. Used in research and for formal diagnosis. Know that it is *clinician-administered*.

Trauma Symptom Inventory (TSI): Broader trauma-related assessment including depression, anxiety, anger, dissociation, sexual concerns. Used when complex trauma is suspected.

ACE Score (Adverse Childhood Experiences): Not a diagnostic tool — a cumulative risk indicator based on 10 categories of childhood adversity. Higher scores correlate with more severe adult health and mental health outcomes. Used in psychoeducation and treatment planning.

What the Exam Rewards / What It Punishes

Rewards: - The three-phase treatment model: (1) safety and stabilization, (2) trauma memory processing, (3) reintegration — never skip phase 1 - Evidence-based trauma treatments: TF-CBT (children), CPT (cognitive processing therapy), PE (prolonged exposure), EMDR - Direct but compassionate safety assessment when suicidality or self-harm is present - Recognizing that avoidance is central — treatment must approach what the client is avoiding - Cultural sensitivity around trauma disclosure (especially for sexual trauma or politically-motivated trauma)

Punishes: - Beginning trauma memory processing (exposure, EMDR) before stabilization - Forcing trauma disclosure before trust is established - Missing that PTSD and MDD, or PTSD and substance use disorder, commonly co-occur and both require treatment - Diagnosing adjustment disorder when the stressor meets Criterion A and symptoms meet PTSD criteria - Treating ASD with watchful waiting only when early CBT can reduce PTSD risk

Worked Mini-Case

Case vignette: Diane, 34, is referred following a car accident eight weeks ago in which she was not injured but witnessed another driver's death. She reports nightmares about the crash nightly, avoids driving (her husband takes her to work), becomes distressed when she sees news coverage of accidents, and has been "on edge" at all times since the accident. She endorses feeling detached from her family and reports she "can't feel anything." Her husband reports she has become irritable and startles easily at sounds.

Reasoning chain:

Step 1 — Criterion A: Diane witnessed another person's death. This meets Criterion A (witnessed death).

Step 2 — Symptom clusters: - Re-experiencing: nightmares ✓ - Avoidance: avoids driving, avoids accident news coverage ✓ - Negative alterations in cognition/mood: emotional numbing, detachment from family ✓ - Hyperarousal: on edge, irritability, exaggerated startle ✓

Step 3 — Duration: Eight weeks (>1 month) → PTSD, not ASD.

Step 4 — Safety: Brief assessment — no SI mentioned, but irritability and numbing warrant asking about self-harm.

Step 5 — Treatment: Begin with Phase 1 (stabilization, psychoeducation about PTSD, grounding skills, safety). Then CPT or PE to address the trauma memory and avoidance. Involve the husband (he is already a support resource).

Key exam point: The exam might present this and ask whether driving avoidance should be "respected" (normalized and left in place) vs. addressed. The correct answer: avoidance is a maintaining factor; treatment must eventually address it. Permanently allowing the avoidance = maintaining the disorder.

Practice Questions

Question 25. A client with PTSD and co-occurring alcohol use disorder (she drinks to suppress nightmares) is beginning outpatient treatment. The counselor should FIRST:

- A. Begin prolonged exposure to the trauma memories while simultaneously addressing the alcohol use
- B. Address stabilization, safety, and reduce drinking before initiating trauma-focused therapy
- C. Refer to a substance use treatment program and defer trauma work until sobriety is established
- D. Focus exclusively on the PTSD, as the drinking will resolve once the nightmares stop

Correct: B. Stabilization precedes processing. Initiating prolonged exposure while the client is actively drinking is contraindicated — the client lacks the distress tolerance and sobriety needed to process traumatic material safely. C over-sequences treatment — integrated (dual) treatment is more effective than sequential. D ignores the substance use, which is a primary maintaining factor.

Question 26. Which of the following MOST clearly differentiates PTSD from Adjustment Disorder?

- A. PTSD symptoms are more severe
- B. PTSD requires a Criterion A traumatic stressor and specific symptom clusters including re-experiencing
- C. Adjustment disorder resolves without treatment
- D. PTSD symptoms are more disabling

Correct: B. Criterion A (traumatic stressor — actual/threatened death, serious injury, or sexual violence) plus the specific symptom clusters (re-experiencing, avoidance, negative cognitions/mood, hyperarousal) define PTSD. Adjustment disorder can follow any stressor and does not require re-experiencing. Severity (A, D) is not the defining distinction. Adjustment disorder may resolve on its own (C), but that is not the key differential.

Question 27. A client who experienced childhood sexual abuse is three sessions into treatment. The counselor suggests beginning EMDR to process the traumatic memories. The client agrees but dissociates during the initial session and is unable to complete it. The MOST appropriate response is:

- A. Continue with EMDR and teach grounding techniques between sessions
- B. Pause the EMDR and extend the stabilization phase; teach distress tolerance and grounding first

- C. Refer the client to a higher level of care
- D. Switch to CPT, which is better tolerated than EMDR

Correct: B. Dissociation during processing is a clear indicator that the stabilization phase was insufficient. The clinical response is to pause processing and return to Phase 1 (stabilization, distress tolerance, grounding). The client needs to be able to stay present during processing for it to be effective and safe. A risks retraumatization. C is premature. D may eventually be considered but the immediate issue is the phase sequence, not the modality.

Question 28. A child, age 7, was removed from a neglectful home at age 3 and has since been in stable foster care. The foster parent reports that the child approaches strangers at the park as though they are familiar, invites them to hold her hand, and has attempted to leave with strangers twice. The MOST likely diagnosis is:

- A. Reactive Attachment Disorder
- B. Disinhibited Social Engagement Disorder
- C. PTSD
- D. Social Anxiety Disorder (rule-out)

Correct: B. Disinhibited Social Engagement Disorder is characterized by indiscriminate, disinhibited attachment behavior with unfamiliar adults — approaching and engaging strangers without appropriate reticence, a result of early attachment disruption. RAD (A) presents with emotional withdrawal and inhibited attachment behavior (the opposite). PTSD (C) could co-occur but doesn't explain the disinhibited attachment pattern. Social anxiety (D) is the opposite presentation.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 34-year-old witnessed a fatal collision ten days ago. Since then she has intrusive images, nightmares, a sense of being detached and “in a fog,” hypervigilance, and she avoids driving. The most likely diagnosis is:

- A. Post-Traumatic Stress Disorder
- B. Acute Stress Disorder

- C. Adjustment Disorder
D. Panic Disorder

Answer: B. Criterion-A exposure with intrusion, dissociation, avoidance, and arousal from three days to one month post-trauma defines Acute Stress Disorder. PTSD (A) requires symptoms beyond one month. Adjustment Disorder (C) does not fit a full trauma-symptom picture. *What would change the answer:* the identical presentation persisting past one month → PTSD.

Assessment — what to do next. The most appropriate assessment approach now is:

- A. Detailed, repeated recounting of the trauma to process it immediately
B. A validated screen (e.g., PCL-5) plus safety and current-symptom assessment, paced to avoid re-traumatization
C. Single-session critical-incident debriefing
D. Defer all assessment for three months

Answer: B. Validated screening with safety assessment, paced carefully, is appropriate. Immediate detailed recounting (A) and single-session debriefing (C) can worsen outcomes; deferring (D) forfeits early intervention.

Treatment Decision Flow

The high-yield trauma decisions are about *stabilization vs. delay* and *integrated vs. sequential* care.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
PTSD, stabilized and safe	Trauma-focused therapy (PE, CPT, or EMDR)	Indefinite supportive therapy that never approaches the trauma
PTSD with comorbid SUD	Integrated/concurrent treatment (e.g., COPE)	“Get sober first, then we’ll do trauma” — sequential delay is outdated
Acute intoxication or withdrawal Recent trauma, symptomatic (ASD window)	Stabilize medically before trauma processing Watchful waiting with brief trauma-focused CBT if indicated	Beginning exposure during acute withdrawal Mandatory single-session psychological debriefing

Sequencing & caveats. Stabilization comes first, but do not delay trauma work indefinitely once the client is safe. For PTSD + SUD,

integrated treatment is superior to sequential — the “achieve sobriety first” rule applies only during acute intoxication or withdrawal.

Avoid / contraindicated. Single-session psychological debriefing (CISD) is not supported and can worsen outcomes. Exposure is contraindicated during active psychosis, severe dissociation, or acute suicidality without a safety plan.

Scope. Standard counselor scope; coordinate with medical/addiction providers as needed.

Chapter 9 — Substance-Related and Addictive Disorders

Overview

Substance use disorders occupy a unique position on the NCMHCE because they appear in two ways: as primary diagnoses, and as co-occurring conditions that complicate every other disorder. The exam tests whether you can (1) recognize the difference between use, misuse, and disorder; (2) apply the diagnostic criteria correctly; (3) identify the appropriate level of care; and (4) avoid the two classic errors — over-pathologizing normal use and under-recognizing serious use disorders in high-functioning clients.

The single most testable substance question: **what is the appropriate first clinical action with a new client who reports substance use?** The answer is: *assess the severity and functional impact before doing anything else*. Do not assume use = disorder, and do not assume the client knows whether they have a problem.

The second key concept: **co-occurring (dual) disorders require integrated treatment**. Treating only the mental health issue while ignoring the substance use, or treating only the substance use while ignoring the mental health issue, produces consistently worse outcomes than treating both simultaneously.

Diagnosis Reference Table

Diagnosis	ICD-10	Severity	Key Criteria
Alcohol Use Disorder (mild/moderate/severe)	F10.10/.20/.20	2-3 Sx: mild; 4-5: moderate; ≥6: severe	≥2 of 11 criteria in 12 months
Cannabis Use Disorder	F12.10	Same 11-criterion scale	Includes withdrawal (irritability, sleep, appetite)
Opioid Use Disorder	F11.10	Same scale	Tolerance, withdrawal, compulsive use despite consequences

Diagnosis	ICD-10	Severity	Key Criteria
Stimulant Use Disorder	F15.10	Same scale	Cocaine, amphetamine, methamphetamine
Sedative/Hypnotic/Anxiolytic Use Disorder	F13.10	Same scale	Includes benzodiazepines
Gambling Disorder	F63.0	Only behavioral (non-substance) addictive disorder in DSM-5	Persistent, problematic gambling with 4+ of 9 criteria

The 11 diagnostic criteria for substance use disorders (applied to each substance): 1. Taking larger amounts or longer than intended 2. Persistent desire or unsuccessful efforts to cut down 3. Great deal of time spent obtaining, using, or recovering 4. Craving 5. Failure to fulfill major role obligations 6. Continued use despite social/interpersonal problems 7. Important activities given up 8. Use in hazardous situations 9. Continued use despite knowledge of physical/psychological problems caused by it 10. Tolerance (needing more for same effect) 11. Withdrawal

Tolerance and withdrawal are criteria 10 and 11 — they do not automatically indicate a substance use disorder when they occur in the context of medically prescribed use.

The Clinical Picture

The clinical picture of a substance use disorder often conceals itself. High-functioning individuals with moderate or severe alcohol use disorder may appear polished and organized. Cannabis use disorder clients may resist the diagnosis (“it’s legal now”). Opioid use disorder often presents first in medical settings.

In a case simulation, look for: - **The erosion pattern:** things the person used to do that they no longer do (work performance declining, relationships deteriorating, hobbies abandoned) - **The explaining-it-away pattern:** the client attributes every problem to something other than the substance (“my boss is terrible,” “my marriage was already struggling”) - **The all-or-nothing disclosure:** the client’s first report minimizes; the full picture emerges only with direct, non-judgmental questions

Motivational interviewing (MI) is the evidence-based approach for the pre-contemplation and contemplation stages. MI does not

argue with the client, does not lecture, and does not try to convince. It explores ambivalence (the “yes, but” of addiction) through open questions, reflections, affirmations, and summaries — the OARS skills.

FRAMES model (Brief Intervention context): - **F**eedback (personalized) - **R**esponsibility (emphasize client’s own choice) - **A**dvice (clear recommendation) - **M**enu of options - **E**mpathy - **S**elf-efficacy support

The ASAM Levels of Care

The American Society of Addiction Medicine (ASAM) criteria guide placement in addiction treatment. Know the levels:

Level	Name	Setting
0.5	Early Intervention	Brief outpatient
1	Outpatient Services	<9 hours/week
2.1	Intensive Outpatient (IOP)	9–19 hours/week
2.5	Partial Hospitalization (PHP)	≥20 hours/week
3.1–3.7	Residential	24-hour therapeutic community
4	Medically Managed Intensive Inpatient	24-hour medical supervision

The exam expects you to match severity to level of care. A client with moderate alcohol use disorder and no medical complications goes to outpatient or IOP, not residential. A client with severe opioid use disorder and current withdrawal with medical instability goes to level 4.

Assessment Tools in Context

AUDIT (Alcohol Use Disorders Identification Test): 10 items. Total score 0–40. - 8–15: simple advice - 16–19: brief counseling + monitoring - ≥20: referral for evaluation

AUDIT-C: 3-item abbreviated screen. Scores ≥4 (men) or ≥3 (women) suggest likely AUD.

CAGE: 4-item clinical screen (Cut down / Annoyed / Guilty / Eye-opener). ≥ 2 positive responses = likely AUD. Fast and memorable; less sensitive than AUDIT for lower-severity use.

DAST-10 (Drug Abuse Screening Test): 10-item screen for non-alcohol drug use. Score ≥ 3 suggests need for further assessment.

ASI (Addiction Severity Index): Comprehensive structured interview across seven domains (medical, employment, alcohol, drugs, legal, family/social, psychiatric). Used for treatment planning, not just screening.

What the Exam Rewards / What It Punishes

Rewards: - Matching level of care to ASAM criteria (not defaulting to outpatient for all presentations) - Using MI (not confrontation or lectures) with ambivalent clients in pre-contemplation or contemplation - Recognizing the need to assess both substance use AND mental health simultaneously - Assessing for withdrawal safety — alcohol and benzodiazepine withdrawal can be life-threatening (seizures, delirium tremens); opioid withdrawal is medically uncomfortable but rarely fatal - Integrated treatment for co-occurring disorders, not sequential

Punishes: - Confronting denial without using MI - Diagnosing a substance use disorder without systematic assessment (AUDIT, CAGE, clinical interview) - Ignoring the substance use when treating another primary disorder - Sending all clients to residential regardless of severity - Missing alcohol withdrawal risk in clients who present with co-occurring medical issues

Worked Mini-Case

Intake vignette: Robert, 47, is self-referred. He says, “My wife thinks I drink too much, but I can handle it.” He reports drinking “six or seven” beers most evenings (“maybe more on weekends”). He denies problems at work — he is a high-functioning attorney. He reports that he tried to stop drinking for two weeks last year and experienced sweats, tremors, and difficulty sleeping. He stopped the attempt. He endorses some morning drinking to “feel right.”

Reasoning chain:

Step 1 — Assess severity: Robert endorses 4–5 of the 11 DSM criteria. Morning drinking (using in situations where it’s not customary), attempted withdrawal with physiological symptoms, drinking larger amounts than described (self-minimization), wife noticing the problem, and the failed quit attempt. This is likely **moderate to severe AUD**.

Step 2 — Medical priority: The history of tremors and sweating during the prior cessation attempt signals **withdrawal risk**. Alcohol withdrawal can be medically dangerous (seizures, delirium tremens). Before outpatient treatment begins, a medical evaluation and possibly medical management of withdrawal is needed (rung 2 on the priority ladder).

Step 3 — Motivational stance: Robert is in pre-contemplation (“I can handle it”). MI is the indicated approach. Confronting denial will increase resistance. Reflect ambivalence: “Your wife is concerned, you tried to stop once — what’s your own take on your drinking?”

Step 4 — Level of care: Given severity and withdrawal history, outpatient is not safe until medically stabilized. Consider medically supervised detox (level 3.7 or 4) followed by residential or PHP.

Exam answer: The first step is medical evaluation (rung 2) before any psychosocial treatment begins.

Practice Questions

Question 29. A counselor using motivational interviewing with a client who is ambivalent about stopping marijuana use should:

- A. Challenge the client’s rationalizations about the effects of marijuana
- B. Explore both the benefits and costs of marijuana from the client’s own perspective
- C. Provide psychoeducation about the health effects of cannabis
- D. Set a goal for abstinence within the first two sessions

Correct: B. MI explores ambivalence — the client’s own perceived benefits and costs — without imposing the counselor’s agenda. The change talk that emerges from this exploration is more durable than change that comes from being lectured (A, C) or pressured (D).

Question 30. A client in early recovery from opioid use disorder says, “I know I said I wanted to stop, but sometimes I wonder if it was really that bad.” The counselor’s BEST response using MI is:

- A. “It was that bad — your last overdose required hospitalization.”
- B. “What are some of the things you’ve noticed that made you decide to come in?”
- C. “It sounds like you’re not ready to commit to sobriety.”
- D. “That kind of thinking is a sign that your addiction is talking.”

Correct: B. This open question evokes the client’s own change talk — their reasons for seeking treatment. MI amplifies the client’s own motivation rather than providing external arguments (A), labeling ambivalence negatively (C), or pathologizing the thinking (D).

Question 31. A client with alcohol use disorder is medically cleared after completing detox. She has moderate AUD with stable housing, employment, and family support. She is motivated for recovery. The MOST appropriate level of care is:

- A. Inpatient residential treatment (Level 3.5)
- B. Intensive outpatient program (Level 2.1)
- C. Outpatient services (Level 1)
- D. Partial hospitalization (Level 2.5)

Correct: B or C depending on context. Given moderate severity, stability, and strong support, IOP (2.1) or outpatient (1) is appropriate. The ASAM placement decision considers six dimensions; with all stabilizing factors present, less intensive levels are preferred. If there are any additional complicating factors (prior relapse, limited coping skills), IOP is the stronger choice. Residential (A) is not indicated.

Question 32. Alcohol withdrawal is potentially life-threatening and may require medical management. Which symptom presentation indicates the HIGHEST level of medical concern?

- A. Irritability, sleep disturbance, and mild anxiety 24–48 hours after last drink
- B. Visual hallucinations, disorientation, elevated temperature, and autonomic instability 48–72 hours after last drink
- C. Intense cravings, dysphoric mood, and fatigue in the first week

of abstinence

D. Insomnia, vivid dreams, and mild tremor in the first 48 hours

Correct: B. This presentation describes **delirium tremens** — the most severe and potentially fatal form of alcohol withdrawal. It typically develops 48–72 hours after last drink and involves autonomic instability (tachycardia, hypertension, hyperthermia), confusion, and hallucinations. A and D describe early, less severe withdrawal. C describes symptoms more consistent with opioid or stimulant withdrawal (no medical emergency).

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. Over the past year a client needs more alcohol to feel the effect, has repeatedly failed to cut down, reports strong cravings, has missed work several times, gave up his weekend soccer league in order to drink, continues despite escalating marital conflict, and becomes shaky and sweaty when he stops for a day. The most likely diagnosis is:

- A. Alcohol Use Disorder, mild
- B. Alcohol Use Disorder, moderate
- C. Alcohol Use Disorder, severe
- D. Alcohol withdrawal

Answer: C. Tolerance, failed attempts to cut down, craving, role failure, activities given up, use despite interpersonal problems, and withdrawal total six or more criteria — severe. Withdrawal (D) is a physiological state, not the disorder itself. *What would change the answer:* two to three criteria → mild; four to five → moderate.

Assessment — what to do next. Given the shakiness on stopping, the most appropriate assessment and safety step is:

- A. Administer the AUDIT and assess withdrawal risk (e.g., CIWA-Ar); coordinate medical evaluation for possible detox
- B. Begin relapse-prevention CBT
- C. Require abstinence as a condition of continuing treatment
- D. Administer the PHQ-9 only

Answer: A. Autonomic withdrawal signs raise the risk of complicated withdrawal, so severity and withdrawal are assessed and med-

ical detox is coordinated. CBT (B) is treatment; requiring abstinence (C) is punitive and premature.

Treatment Decision Flow

The single biggest overgeneralization to correct here: not every client is in the Action stage. Match the intervention to the stage of change.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Precontemplation (“I can handle it”)	Motivational interviewing (OARS); explore ambivalence	Confronting denial, or requiring abstinence as a condition of treatment
Contemplation	Decisional balance; develop discrepancy	Assigning action-stage coping plans prematurely
Preparation / Action	Match the ASAM level of care; add relapse prevention (CBT)	Recommending residential care or groups before readiness
Opioid use disorder	Refer for MAT (buprenorphine or methadone) plus counseling	Framing MAT as “trading one addiction for another”; abstinence-only

Sequencing & caveats. Recommending abstinence-based groups, coping skills, or residential treatment in precontemplation is clinically premature and exam-punished. Only after the client reaches Preparation/Action do you match the ASAM level and introduce relapse-prevention skills.

Avoid / contraindicated. Confronting denial ruptures the alliance and is the classic trap. Do not require abstinence as a precondition for engaging a precontemplative client.

Scope. MAT is evidence-based for OUD; the counselor refers for and supports it rather than prescribing.

Chapter 10 — Personality Disorders

Overview

Personality disorders are among the most challenging chapters on the NCMHCE — not because the diagnosis is hard to recognize, but because the exam tests your understanding of *how personality disorders change the clinical picture* and what adaptations they require in the therapeutic relationship.

The foundational principle: a personality disorder is a **persistent, pervasive, inflexible pattern** of experiencing and relating to the world that causes distress or impairment. The key word is *pervasive* — the pattern appears across multiple contexts (work, relationships, the counseling relationship itself), is stable over time, and is ego-syntonic (the client experiences the pattern as “just who I am,” not as symptoms to be treated). This ego-syntonic quality is why personality disorder clients often do not present seeking change in their personality — they present because the world keeps responding badly to them.

For exam purposes, know the clusters, know the highest-tested diagnoses (Borderline, Narcissistic, Antisocial), and know what DBT targets and why.

Diagnosis Reference Table

Cluster A — “Odd/Eccentric”

Diagnosis	ICD-10	Core Pattern
Paranoid PD	F60.0	Pervasive distrust and suspicion Detachment from social relationships; restricted affect; indifferent to others
Schizoid PD	F60.1	
Schizotypal PD	F21	Social isolation + odd beliefs, perceptions, speech, and behavior

Cluster B — “Dramatic/Emotional/Erratic”

Diagnosis	ICD-10	Core Pattern
Antisocial PD	F60.2	Pervasive disregard for and violation of others’ rights (≥ 18 ; conduct disorder before 15)
Borderline PD	F60.3	Instability in relationships, self-image, affect, and impulsivity
Histrionic PD	F60.4	Excessive emotionality and attention-seeking
Narcissistic PD	F60.81	Grandiosity, need for admiration, lack of empathy

Cluster C — “Anxious/Fearful”

Diagnosis	ICD-10	Core Pattern
Avoidant PD	F60.6	Social inhibition, feelings of inadequacy, hypersensitivity to negative evaluation
Dependent PD	F60.7	Excessive need to be taken care of; submissiveness; fear of separation
Obsessive-Compulsive PD	F60.5	Preoccupation with orderliness, perfectionism, control (distinct from OCD)

The Clinical Picture

Borderline Personality Disorder (BPD) is the highest-yield personality disorder on the exam. The clinical picture: a client whose relationships are intense and unstable (idealization followed by devaluation), who experiences profound fear of abandonment (real or imagined), who has a markedly unstable self-image, who engages in impulsive behaviors that are self-damaging (spending, sex, substance use, self-harm), and whose mood shifts rapidly. The counseling relationship itself will often reproduce the BPD relational pattern — the client may idealize the counselor early and devalue them sharply after a perceived abandonment (e.g., a canceled session or a moment when the counselor seems less attuned).

In the counseling relationship: A client with BPD will test the therapeutic frame (late cancellations, frequent between-session contact, testing limits), will react intensely to breaks and endings, and may

oscillate between idealization (“You’re the only person who’s ever understood me”) and devaluation (“You’re just like everyone else — you don’t care”). The exam expects you to: maintain a consistent frame, respond with validation rather than defensiveness, and use DBT skills-based interventions.

Narcissistic PD in therapy: the client presents with a sense of entitlement, often expects special treatment (phone the counselor directly, session times accommodated to their schedule), becomes irritable or contemptuous when the counselor doesn’t agree or provide validation. Beneath the grandiosity is often profound shame. The clinical task is to neither attack the grandiosity nor collude with it.

Antisocial PD and the limits of individual therapy: clients with ASPD have limited capacity for guilt, remorse, and empathy. Individual talk therapy has limited evidence for ASPD. Court-mandated treatment is common; motivation is typically external. The exam sometimes tests whether you would maintain treatment with a client who is actively deceiving you — the answer involves: monitor honesty, maintain the therapeutic frame, and be realistic about the goals of treatment.

Differentiating Within the Family

BPD vs. Bipolar II: The most common diagnostic confusion. Both involve mood instability, impulsivity, and interpersonal difficulties.

Feature	BPD	Bipolar II
Mood episode duration	Hours to days, reactive to events	Days to weeks, not necessarily reactive
Trigger	Often interpersonal (abandonment, rejection)	May be autonomous
Self-harm	Common	Less central
Identity disturbance	Core feature	Not a feature
Hypomanic episodes	Not part of BPD	Defining feature of Bipolar II

Schizoid PD vs. Avoidant PD: Both involve social withdrawal. The distinction: Schizoid PD is withdrawal *by preference* (the person

genuinely does not want relationships, feels no distress about isolation). Avoidant PD is withdrawal *by fear* (the person desperately wants relationships but is too afraid of rejection to pursue them).

OCPD vs. OCD: OCPD is ego-syntonic — the person sees their orderliness, perfectionism, and control as virtues, not symptoms. OCD is ego-dystonic — the obsessions and compulsions are experienced as unwanted and distressing. A person with OCPD does not experience their rigidity as a problem; a person with OCD wishes they could stop the compulsions.

Assessment Tools in Context

Personality disorder diagnosis is made through clinical interview (comprehensive history, collateral sources) — there is no brief screening tool that reliably diagnoses a PD. The MMPI-2 (Minnesota Multiphasic Personality Inventory) and MCMI-IV (Millon Clinical Multiaxial Inventory) are the most commonly used structured assessment tools. The exam expects you to know their purpose (comprehensive personality assessment) without needing to know specific scales in depth.

McLean Screening Instrument for BPD (MSI-BPD): 10-item self-report, used to screen specifically for BPD. A score ≥ 7 suggests the need for further evaluation.

What the Exam Rewards / What It Punishes

Rewards: - DBT as the first-line evidence-based treatment for BPD (developed specifically for this population) - Maintaining the therapeutic frame consistently with personality disorder clients (consistency is therapeutic) - Validating the client's emotional experience before addressing behavior - Recognizing that personality disorder traits can intensify in times of stress - Understanding that personality disorders are ego-syntonic and that the client often does not identify the PD pattern as the problem

Punishes: - Diagnosing PD in a first session (diagnosis requires pervasive, persistent pattern across contexts and time) - Diagnosing PD under the age of 18 (except ASPD's precursor, conduct disorder) — PD diagnosis in adolescents is generally deferred - Colluding with splitting in BPD by taking sides in the client's devaluation of others

- Using generic supportive therapy for BPD when DBT is available
- Ignoring the client's expressed suicidal ideation in BPD because "it's chronic" — chronic suicidality requires ongoing assessment; each expression must be evaluated

Worked Mini-Case

Intake vignette: Sandra, 26, presents for the third time in 18 months. She was discharged from a prior counselor ("she stopped getting me") and another before that ("he was useless from the start"). She describes cutting on her arms "when things get too intense" and has had two brief psychiatric hospitalizations following "fights with people I trusted." She says you are "the only one who might actually help" and wants to meet three times per week.

Reasoning chain:

Step 1 — Safety: Self-harm is present. Assess current ideation, plan, intent. Distinguish non-suicidal self-injury (NSSI) from suicidal self-harm — in BPD, cutting frequently serves an affect-regulation function (not suicidal intent), but this must always be verified.

Step 2 — Diagnostic picture: The pattern across three counselors (splitting, idealization-devaluation), self-harm, hospitalizations after intense interpersonal conflict, and the immediate idealization ("the only one who might actually help") is a classic BPD presentation.

Step 3 — Frame-setting: Begin with a clear therapeutic agreement. Number of sessions (three times per week is typically not the standard outpatient frame; discuss what's appropriate and sustainable). Crisis plan (what Sandra does before calling the counselor or going to the ER). Limit-setting communicated warmly.

Step 4 — Treatment: DBT — skills training in distress tolerance, emotion regulation, interpersonal effectiveness, and mindfulness. The self-harm requires a behavioral chain analysis to understand its function and to develop alternatives.

Step 5 — Counter-transference: "The only one who might actually help" is an idealization that, if accepted uncritically, will be followed by devaluation. The counselor should receive it warmly but not collude with it.

Practice Questions

Question 33. A client with BPD calls the counselor's office for the fourth time in one day. In the last call, she threatens to harm herself if the counselor doesn't call back. The counselor's BEST response is:

- A. Call back immediately to prevent any potential self-harm
- B. Follow the previously established crisis plan: if the client is in immediate danger, she calls 911 or goes to the ER
- C. Increase session frequency to reduce the client's distress
- D. Terminate the therapeutic relationship due to the client's unmanageable behavior

Correct: B. The crisis plan establishes the protocol in advance — precisely so that the counselor does not need to respond to escalating contact in real time. Responding immediately (A) reinforces crisis behavior as a way to access the counselor. Increasing sessions (C) may be temporarily appropriate but is not the right *crisis* response. Terminating (D) is a clinical decision that would require planning, not an emergency action.

Question 34. Which of the following BEST describes the primary goal of DBT for a client with BPD?

- A. Modify core personality traits to eliminate the BPD diagnosis
- B. Build a life worth living by developing skills to regulate emotions, tolerate distress, improve relationships, and maintain mindfulness
- C. Resolve early childhood trauma that underlies the borderline pathology
- D. Provide consistent support and validation until the client achieves stable relationships

Correct: B. DBT's primary goals are skills development — distress tolerance, emotion regulation, interpersonal effectiveness, and mindfulness — toward building a "life worth living." A is not a realistic or stated DBT goal. C describes trauma-focused work, which is appropriate only after stabilization. D is support without skills, which alone is not sufficient.

Question 35. A client with Narcissistic PD becomes irritated when the counselor is three minutes late to a session and says, "I can't

work with someone who doesn't respect my time — people like you are all the same." The counselor's BEST response is:

- A. Apologize extensively and explain the reason for the delay
- B. Explore what the delay meant to the client without becoming defensive
- C. Point out that three minutes is a minor inconvenience and redirect to the session
- D. End the session early to reinforce appropriate session behavior

Correct: B. The therapeutic move is to explore the meaning and emotional impact of the delay — beneath the contempt is often an experience of being devalued or dismissed. Defensiveness (A, in excess) or minimizing (C) miss the clinical moment. D is punitive, not therapeutic.

Question 36. Which of the following differentiates Avoidant PD from Schizoid PD?

- A. Avoidant PD involves bizarre thinking and perceptual disturbances
- B. Schizoid PD involves fear of rejection; avoidant PD involves indifference to relationships
- C. Avoidant PD involves a desire for close relationships that is blocked by fear of negative evaluation
- D. Schizoid PD involves impulsive, emotionally intense behavior

Correct: C. Avoidant PD = wants connection, fears rejection. Schizoid PD = genuinely indifferent to connection, prefers solitude. A describes schizotypal PD (not schizoid). B reverses the relationship between the two. D describes Cluster B behavior.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 26-year-old has a long-standing pattern of intense, unstable relationships, identity disturbance, chronic emptiness, impulsivity, and recurrent self-harm triggered by fears of abandonment. Her mood shifts last hours and track interpersonal events; she denies discrete multi-day periods of elevated mood with decreased need for sleep. The most likely diagnosis is:

- A. Bipolar II Disorder
- B. Borderline Personality Disorder
- C. Post-Traumatic Stress Disorder
- D. Cyclothymic Disorder

Answer: B. A pervasive, enduring pattern of abandonment sensitivity, identity disturbance, hours-long affective reactivity tied to relationships, and self-harm points to BPD. Bipolar II and Cyclothymia (A, D) involve sustained mood episodes lasting days, not hours-long interpersonal reactivity. *What would change the answer:* distinct multi-day hypomanic episodes → Bipolar spectrum.

Assessment — what to do next. The most appropriate diagnostic approach is:

- A. Diagnose from the single intake session
- B. Gather a longitudinal, cross-situational history and collateral, and assess current self-harm and suicide risk
- C. Administer the MDQ and treat as bipolar
- D. Defer any risk assessment until rapport is established

Answer: B. A personality-disorder diagnosis requires evidence of an enduring, cross-situational pattern, and active self-harm requires immediate risk assessment. A single session (A) is insufficient; deferring risk assessment (D) is unsafe.

Treatment Decision Flow

Personality-disorder items reward correct *sequencing* and correct handling of crisis contingencies.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
BPD with active self-harm	Behavioral chain analysis to identify the trigger and function first	Teaching a distress-tolerance skill before analyzing the chain
BPD crisis call	Follow the pre-established crisis/coaching plan	Calling back immediately in a way that reinforces the crisis behavior
BPD, chronic (non-acute) suicidal ideation	Manage within outpatient DBT	Hospitalizing for chronic SI as if it were acute escalation
Cluster B splitting within the team	Consult; maintain team consistency	Taking sides or absorbing the split

Sequencing & caveats. With BPD self-harm, the chain analysis

precedes the replacement skill (e.g., TIPP, DEAR MAN). Distinguish chronic suicidal ideation (managed in DBT) from acute escalation (which warrants a higher level of care). DBT is the first-line comprehensive treatment.

Avoid / contraindicated. Do not reinforce crisis behavior with contingent attention; do not teach skills before the functional analysis.

Scope. Standard counselor scope; coordinate medication referral when comorbidity warrants.

Chapter 11 — Bipolar and Related Disorders

Overview

Bipolar disorders are among the most serious — and most misdiagnosed — conditions the NCMHCE tests. The exam’s highest concern: **missing a manic or hypomanic history when a client presents in a depressive episode**. Prescribing an antidepressant to someone with unrecognized bipolar disorder can precipitate a manic episode. The exam rewards thoroughness — always take a mood episode history, not just a current symptom snapshot.

The second critical concept: **mania and hypomania differ in severity and impact, not in kind**. Hypomania is a lighter, shorter, and less impairing version of mania. Understanding this distinction separates Bipolar I from Bipolar II.

Diagnosis Reference Table

Diagnosis	ICD-10	Defining Feature	First-Line Treatment
Bipolar I	F31.x	At least one manic episode (with or without depression)	Mood stabilizer (lithium, valproate), psychotherapy adjunct
Bipolar II	F31.81	At least one hypomanic episode + at least one MDD episode; NO full mania	Mood stabilizer, lamotrigine; psychotherapy
Cyclothymic Disorder	F34.0	≥2 years of hypomanic and depressive symptoms; never meeting full criteria for either	Mood stabilizer, psychotherapy
Substance/Medication-Induced Bipolar	F10.x4	Manic symptoms during or shortly after substance use/withdrawal	Treat underlying cause

Diagnosis	ICD-10	Defining Feature	First-Line Treatment
Bipolar Due to Medical Condition	F06.33	Etiologically linked to medical condition	Treat underlying cause

The Clinical Picture

Mania (Bipolar I): A distinct period of abnormally elevated, expansive, or irritable mood *plus* increased goal-directed activity or energy, lasting at least one week (or any duration if hospitalization required). During this period, three or more of: grandiosity, decreased need for sleep (feels rested after 3 hours), pressured speech, racing thoughts, distractibility, increased goal-directed activity, and reckless behavior. The key marker: the change is *distinct from baseline* and represents a *different level of functioning*. The client who is always ambitious and energetic is not manic. The client who has gone three days without sleep, maxed out four credit cards, and started four new business ventures simultaneously — that is mania.

Hypomania (Bipolar II): The same symptom criteria, but lasting at least four consecutive days, not severe enough to require hospitalization, and without psychotic features. The crucial point: **the person feels great during hypomania**. They are productive, creative, charming, and energized. They often do not identify hypomania as a problem — they come in depressed and, when asked about their “high” periods, describe them as their best functioning. This is why hypomanic episodes are missed.

Exam trap: A client presents with MDD. You review the history and she says, “Actually, there are periods — usually in the spring — when I need less sleep and I get a lot done. I feel fantastic during those times.” This is a hypomanic episode. The diagnosis is Bipolar II, not MDD. The treatment changes significantly.

Differentiating Within the Family

Bipolar I vs. Bipolar II: The single distinguishing feature is whether a full manic episode has occurred. Bipolar I requires at least one manic episode (may or may not have depressive episodes). Bipolar II requires at least one hypomanic episode and at least one

major depressive episode — and *never* a full manic episode. A Bipolar II client who later develops a full manic episode becomes Bipolar I.

Bipolar II vs. Cyclothymia: Cyclothymia involves chronic, fluctuating mood over ≥ 2 years, with periods of hypomanic symptoms and periods of depressive symptoms that *never meet full criteria* for either a hypomanic episode or an MDD episode. It is milder but chronically unstable. Cyclothymia in adolescents is a risk factor for developing full Bipolar I or II.

BPD vs. Bipolar II: (See Chapter 10 table.) The critical distinguishing feature is that Bipolar II has discrete episodes (days to weeks) that are relatively autonomous from triggers; BPD mood shifts are reactive to events (hours to days), interpersonally triggered, and accompanied by identity disturbance and self-harm.

What the Exam Rewards / What It Punishes

Rewards: - Taking a full mood history before diagnosing or treating depression (rule out prior manic/hypomanic episodes) - Recognizing that hypomania is often ego-syntonic and not spontaneously reported - Recommending mood stabilizer (not antidepressant monotherapy) as primary pharmacological treatment - Psychotherapy as an adjunct (CBT, IPSRT — interpersonal and social rhythm therapy — are evidence-based) - Psychoeducation about sleep regulation as a critical component of bipolar management

Punishes: - Diagnosing and treating MDD without inquiring about prior elevated mood periods - Recommending antidepressant monotherapy for a bipolar client (can precipitate mania) - Confusing BPD mood instability with hypomanic episodes - Failing to assess for psychosis during manic episodes (Bipolar I with psychotic features requires a different care level)

Practice Questions

Question 37. A client presents for the first time reporting persistent low mood, hypersomnia, and difficulty concentrating for the past three months. He scores 14 on the PHQ-9. Before confirming an MDD diagnosis and recommending CBT, the counselor should:

- A. Administer the BDI-II to confirm the severity
- B. Ask about any prior periods of elevated mood, decreased need for sleep, increased energy, or impulsive behavior
- C. Consult with a prescriber before beginning therapy
- D. Begin CBT immediately, as it is appropriate for both MDD and bipolar depression

Correct: B. Screening for prior hypomanic or manic episodes is necessary before confirming MDD. If the client has had hypomanic episodes, the diagnosis is Bipolar II — with different treatment implications. CBT (D) is not appropriate for bipolar depression without mood stabilization.

Question 38. A client with Bipolar I has been stable on lithium for 18 months. She reports that she “feels so normal” now that she’s wondering whether she actually needs the medication. The counselor’s BEST response is:

- A. Support her interest in autonomy and help her taper the medication with physician oversight
- B. Explore her concerns while providing psychoeducation about the high relapse risk of discontinuing mood stabilizers without supervision
- C. Remind her that Bipolar I is a lifelong condition requiring permanent medication
- D. Refer back to her psychiatrist and defer the discussion

Correct: B. Explore before educating — but psychoeducation is clearly indicated here. Medication discontinuation in Bipolar I dramatically increases relapse risk. This is a collaborative conversation that includes both the counselor and the prescriber. A is too permissive without addressing the risk. C is directive without exploration. D is avoidant (the counselor should engage).

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 24-year-old presents depressed. History reveals a past five-day period of decreased need for sleep, rapid speech, increased goal-directed activity, and uncharacteristic spending — without psychosis and without hospitalization. The most likely diagnosis is:

- A. Major Depressive Disorder, recurrent
- B. Bipolar I Disorder
- C. Bipolar II Disorder
- D. Cyclothymic Disorder

Answer: C. A hypomanic episode (at least four days, no marked impairment, no hospitalization, no psychosis) plus major depression defines Bipolar II. MDD (A) ignores the hypomania. *What would change the answer:* an episode of seven or more days, or hospitalization or psychosis → Bipolar I.

Assessment — what to do next. Before treatment planning, the most appropriate assessment step is:

- A. Screen for a history of hypomania/mania (e.g., MDQ) and obtain collateral
- B. Administer only the PHQ-9 and treat the depression
- C. Proceed to an antidepressant-focused referral
- D. Administer the AUDIT

Answer: A. Because antidepressant monotherapy can destabilize bipolar disorder, screening for past (hypo)mania with collateral is essential before planning. Options B and C miss the bipolarity that changes the whole treatment path.

Treatment Decision Flow

The governing rule: stabilize mood before intensive psychotherapy, and screen for bipolarity before anything antidepressant-shaped.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Acute Bipolar II depression	Refer for mood stabilization first; psychotherapy (IPSRT/CBT) is adjunctive	Beginning intensive CBT while the client is actively cycling
“Depressed” client with a buried history of hypomania	Screen for past hypomania/mania before treatment planning	Proceeding as unipolar and heading toward antidepressant monotherapy
Acute mania	Ensure safety; refer for urgent psychiatric medication management	Outpatient psychotherapy as the primary intervention
Euthymic maintenance	IPSRT (regular social rhythms), psychoeducation, adherence support	Discontinuing maintenance once the client feels stable

Sequencing & caveats. For acute Bipolar II depression the evidence-based sequence is mood stabilization (the psychiatric provider may use agents such as lamotrigine or quetiapine) *before* intensive CBT or IPSRT. Psychotherapy is adjunctive; starting therapy during active cycling risks destabilization.

Avoid / contraindicated. Antidepressant monotherapy is contraindicated when any hypomania history is present — screen first. Do not begin intensive therapy while the client is actively cycling.

Scope. The counselor refers for medication before the therapeutic work and does not select the agent.

Chapter 12 — Neurodevelopmental Disorders

Overview

Neurodevelopmental disorders are characterized by developmental deficits that emerge early in life. On the NCMHCE, the exam primarily tests: recognition of ADHD presentations across the lifespan, awareness of ASD’s clinical picture in functioning adults, appropriate use of psychological testing, and the distinction between intellectual disability and learning disorders.

Diagnosis Reference Table

Diagnosis	ICD-10	Key Feature	First-Line Treatment
ADHD — Combined	F90.2	≥6 inattentive + ≥6 hyperactive/impulsive symptoms, present in ≥2 settings	Multimodal: behavioral, medication (stimulants), psychoeducation
ADHD — Predominantly Inattentive	F90.0	≥6 inattentive symptoms	Same multimodal approach
ADHD — Predominantly Hyperactive/Impulsive	F90.1	≥6 hyperactive/impulsive symptoms	Same multimodal approach
Autism Spectrum Disorder (ASD)	F84.0	Persistent deficits in social communication + re-restricted/repetitive behaviors	ABA (behavioral), speech, OT, social skills; no cure
Intellectual Disability	F7x	Significant limitations in intellectual functioning AND adaptive behavior	Skills-based support, educational planning, family psychoeducation

Diagnosis	ICD-10	Key Feature	First-Line Treatment
Specific Learning Disorder	F81.x	With impairment in reading (dyslexia), writing (dysgraphia), or math (dyscalculia)	Specialized instruction, accommodations
Developmental Coordination Disorder	F82	Marked impairment in motor skills	Occupational therapy
Tourette's Disorder	F95.2	Multiple motor + ≥ 1 vocal tic, present >1 year	Behavioral (CBIT), medication if needed

The Clinical Picture

ADHD in adults: The presenting complaint is rarely “I can’t sit still” — it is more often: “I can’t finish anything,” “I’m always late,” “I lose things constantly,” “I start projects and never complete them.” Adults with ADHD often develop compensatory strategies that mask symptoms; the disorder comes to attention when demands increase (new job, graduate school, parenthood). The exam expects you to know that ADHD diagnosis in adults requires evidence that symptoms were present before age 12 — not just that they are present now.

ASD in higher-functioning adults: These clients may present for anxiety, depression, or relationship difficulties — not for autism. The ASD features become apparent in the clinical relationship: unusual communication style, difficulty with reciprocity, literal interpretation of figurative language, intense interests, rigidity around routines, and sensory sensitivities. The exam may test whether you recognize ASD features in a client who does not carry the diagnosis.

Assessment Tools in Context

Conners Rating Scales: Parent and teacher rating forms for ADHD; multiple informants are essential because ADHD must be present in more than one setting.

ADOS-2 (Autism Diagnostic Observation Schedule): Structured observational assessment for ASD; clinician-administered. Gold standard for ASD diagnosis.

WAIS-IV (Wechsler Adult Intelligence Scale): Administered by a licensed psychologist; provides IQ scores across verbal comprehension, perceptual reasoning, working memory, and processing speed. Used in ID diagnosis.

Practice Questions

Question 39. A 28-year-old client is referred after her employer cited repeated missed deadlines and organizational failures. She reports that she has “always been scattered” but managed through college with extra time accommodations. She meets criteria for ADHD — predominantly inattentive. The counselor’s MOST appropriate first step in treatment planning is:

- A. Refer immediately for a stimulant medication evaluation
- B. Provide psychoeducation, assess for comorbidities (anxiety, depression, sleep), and develop a multimodal plan
- C. Begin CBT focused on distorted cognitions about productivity
- D. Recommend time management coaching as a standalone intervention

Correct: B. Psychoeducation and comprehensive assessment are the first steps in multimodal treatment planning for adult ADHD. Medication evaluation (A) is often appropriate but not necessarily the *first* step. CBT alone (C) is a component, not a complete plan. Coaching (D) is helpful but not a standalone evidence-based treatment.

Question 40. Which of the following behaviors in a 5-year-old child is MOST consistent with an ASD diagnosis?

- A. Difficulty sharing toys with siblings
- B. Repetitive, stereotyped hand-flapping when excited, limited eye contact, and speaking in scripts from memorized TV shows
- C. Hyperactivity and impulsivity in the classroom and at home
- D. Selective mutism in school but normal communication at home

Correct: B. Hand-flapping (stereotyped behavior), reduced eye contact (social communication deficit), and echolalia (scripted speech) are core ASD features. A is developmentally normative. C is more consistent with ADHD. D is selective mutism.

End of Part 2A (Chapters 6–12). Continue in Part 2B.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 7-year-old has difficulty sustaining attention, is easily distracted, loses things, fidgets, interrupts, and cannot wait his turn — in both school and home — since age five, with clear impairment. He has no language delay and no restricted or repetitive behaviors. The most likely diagnosis is:

- A. Autism Spectrum Disorder
- B. ADHD, Combined Presentation
- C. Oppositional Defiant Disorder
- D. Specific Learning Disorder

Answer: B. Inattentive plus hyperactive-impulsive symptoms across settings before age 12, with impairment, define ADHD, combined presentation. The absence of social-communication deficits and restricted/repetitive behaviors argues against ASD (A); ODD (C) is defiance and hostility, not an attention problem. *What would change the answer:* social-communication deficits with restricted interests → ASD.

Assessment — what to do next. The most appropriate assessment approach is:

- A. Diagnose from the office observation alone
- B. Obtain standardized parent and teacher rating scales across settings (e.g., Vanderbilt) and rule out sensory/medical contributors
- C. Administer the MMSE
- D. Begin stimulant medication

Answer: B. ADHD requires cross-informant, cross-setting data. Office observation alone (A) is insufficient; the MMSE (C) assesses adult cognition; medication (D) is treatment accessed by referral.

Treatment Decision Flow

For neurodevelopmental presentations the leverage is in the *system* around the client — parents, teachers, structure — not individual insight.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
ADHD (child)	Behavioral parent training and school coordination; refer for a medication evaluation	Individual insight-oriented therapy as the primary intervention
ASD	Structured skills/behavioral approaches with parent involvement	Unstructured play only, or attempting to “normalize” the child
Comorbid ADHD + anxiety	Coordinate care; target the most impairing condition; refer for medication considerations	Treating one condition and ignoring the comorbidity

Sequencing & caveats. Parent management training and school/system involvement are central and modifiable treatment targets. Stimulants are typical first-line pharmacotherapy for ADHD, accessed by referral.

Avoid / contraindicated. Individual insight therapy as the primary treatment for a young child with ADHD is a poor match.

Scope. The counselor coordinates and refers for medication evaluation rather than prescribing.

PART TWO: DIAGNOSTIC REFERENCE (continued)

Chapters 13-23

Chapter 13 — Neurocognitive Disorders

Overview

Neurocognitive disorders (NCDs) are characterized by a decline in cognitive function from a previous level of performance. The exam tests two key skills: (1) recognizing delirium as a medical emergency distinct from dementia, and (2) understanding the counselor’s role in supporting clients and caregivers rather than reversing the condition.

Diagnosis Reference Table

Diagnosis	ICD-10	Key Feature	Counselor Role
Major Neurocognitive Disorder (Dementia)	F02.xx	Significant decline in ≥ 1 cognitive domain; interferes with independence	Caregiver support, behavioral management, coordination
Mild Neurocognitive Disorder	G31.84	Modest cognitive decline; independence maintained	Psychoeducation, monitoring, lifestyle
Delirium	F05	Acute, fluctuating disturbance in attention and awareness; medical emergency	Refer immediately for medical evaluation

The Clinical Picture

Delirium vs. Dementia: The most important distinction in this chapter.

Feature	Delirium	Dementia
Onset	Acute (hours to days)	Gradual (months to years)
Course	Fluctuating	Slowly progressive
Attention	Markedly impaired	Relatively preserved early

Feature	Delirium	Dementia
Reversible?	Often yes (treat cause)	Rarely reversible
Common causes	Infection, medication, metabolic	Alzheimer's, vascular, Lewy body

A client who “seemed fine last week and now doesn’t know where they are” has delirium until proven otherwise. **Delirium is a medical emergency.** Do not attempt psychotherapy. Refer immediately.

What the Exam Rewards / What It Punishes

Rewards: - Recognizing delirium and referring for immediate medical evaluation - Providing grief-informed support to caregivers (caregiver burden is immense) - Functional assessment to match level of care to current ability - Using structured, predictable environments and routines for dementia clients

Punishes: - Treating delirium as a behavioral or psychiatric problem - Attempting insight-oriented therapy with moderate-to-severe dementia clients - Missing caregiver burnout in the family system

Practice Question

Question 41. An 81-year-old client who has been in counseling for grief presents with his daughter, who reports he has been “confused and agitated since last night” and cannot remember where he is. He is usually oriented and articulate. The counselor should:

- A. Administer a cognitive screening tool and schedule a follow-up
- B. Contact his prescribing physician immediately and facilitate emergency evaluation
- C. Provide reassurance and see whether the confusion resolves by the next session
- D. Increase session frequency to monitor cognitive changes

Correct: B. Acute onset confusion in a previously oriented elder is delirium until proven otherwise. Medical emergency. Refer immediately. A, C, and D all delay necessary medical intervention.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. An 80-year-old is brought in for confusion that began two days ago, fluctuates across the day, and includes prominent inattention and disorientation. She recently started a new medication and has a urinary tract infection. The most likely diagnosis is:

- A. Major Neurocognitive Disorder (dementia)
- B. Delirium
- C. Mild Neurocognitive Disorder
- D. Major Depressive Disorder

Answer: B. Acute onset, a fluctuating course, prominent inattention, and a medical/medication precipitant define delirium — a medical emergency. Dementia (A) is insidious and progressive. *What would change the answer:* a gradual, months-long decline without acute change → a neurocognitive disorder.

Assessment — what to do next. The most appropriate immediate step is:

- A. Administer the MoCA and schedule follow-up in a month
- B. Facilitate immediate medical evaluation to identify and treat the underlying cause; use a delirium-oriented assessment (e.g., the Confusion Assessment Method)
- C. Begin cognitive stimulation therapy
- D. Diagnose dementia and provide caregiver education

Answer: B. Delirium is a medical emergency; the counselor refers immediately, and the CAM screens for it. Cognitive testing (A) and dementia care (D) misclassify an acute, potentially reversible state.

Treatment Decision Flow

The two skills tested: recognize delirium as a medical emergency, and understand that the counselor supports rather than reverses a major NCD.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Acute, fluctuating confusion with inattention (delirium)	Treat as a medical emergency; immediate medical referral to find the reversible cause	Managing it as dementia or beginning psychotherapy

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Major NCD (dementia)	Supportive care, caregiver psychoeducation, environmental and safety strategies	Cognitive restructuring or insight therapy aimed at reversing the decline
Caregiver burnout	Caregiver support, respite, psychoeducation	Focusing only on the identified patient

Sequencing & caveats. Delirium is distinguished by acute onset, fluctuating course, and impaired attention; it demands medical workup for a reversible cause. In major NCD the counselor's role is support and safety, not cognitive recovery.

Avoid / contraindicated. Do not attempt insight-oriented or restructuring work aimed at reversing cognitive decline.

Scope. Refer for medical evaluation; coordinate with the care team.

Chapter 14 — Obsessive-Compulsive and Related Disorders

Overview

OCD and related disorders are characterized by unwanted, repetitive thoughts and/or behaviors that the person cannot stop despite wanting to. The critical exam concept: these disorders are **ego-dystonic** — the client experiences the obsessions or compulsions as unwanted intrusions, not as expressions of their own values. This is what distinguishes OCD from OCPD (which is ego-syntonic).

Diagnosis Reference Table

Diagnosis	ICD-10	Core Feature	Treatment
OCD	F42.2	Obsessions (intrusive thoughts) + compulsions (rituals that reduce anxiety)	ERP (Exposure and Response Prevention), medication (SSRI)
Body Dysmorphic Disorder	F45.22	Preoccupation with perceived defect in appearance	CBT/ERP, SSRI
Hoarding Disorder	F42.3	Difficulty discarding possessions + excessive acquisition + cluttered living space	CBT (specialized), motivational interviewing
Trichotillomania	F63.3	Recurrent hair pulling	HRT (Habit Reversal Training)
Excoriation (Skin-Picking)	L98.1	Recurrent skin picking	HRT

The Clinical Picture

OCD in a case sim: A client describes intrusive thoughts (“What if I hurt someone?”, “What if I left the stove on?”) that they experience as horrifying and foreign to their values. They engage in

rituals — checking, counting, washing, reassurance-seeking — that provide temporary relief but fuel the obsessive cycle in the long run. The checking or washing is not the problem; the problem is the *attempted solution* (the ritual) that prevents habituation to the obsessive thought.

The OCD cycle: Intrusive thought → anxiety spike → compulsion → temporary relief → intrusion returns stronger → repeat

ERP works by breaking the link between the obsession and the compulsion: the client confronts the feared thought without engaging the compulsion, allowing anxiety to naturally reduce through habituation.

Hoarding Disorder is often misunderstood. Clients with hoarding disorder form intense attachments to objects, experience severe distress at the prospect of discarding, and may not perceive the hoarding as a problem (making it partially ego-syntonic). CBT for hoarding is specialized and requires motivational enhancement, not just behavioral strategies.

Differentiating Within the Family

OCD vs. OCPD: OCD = ego-dystonic, unwanted obsessions and compulsions. OCPD = ego-syntonic, the person values their orderliness and perfectionism. These can co-occur but they are distinct.

OCD vs. GAD: Both involve worry. In OCD, the worries are intrusive, often bizarre or ego-dystonic (“What if I’m secretly a violent person?”), and are accompanied by ritualistic responses. GAD worries are about realistic life concerns (finances, health, relationships) and are not accompanied by compulsions.

BDD vs. Normal Appearance Concern: Everyone has appearance concerns. BDD is distinguished by the preoccupation being time-consuming (>1 hour/day), causing significant distress, leading to repetitive behaviors (mirror-checking, excessive grooming, seeking reassurance), and being based on a *perceived* defect that is either absent or minimal. BDD is associated with high rates of suicidality.

What the Exam Rewards / What It Punishes

Rewards: - ERP as the first-line treatment for OCD (not CBT without the exposure/response-prevention component) - Recognizing that compulsions maintain the disorder by providing short-term relief - HRT (habit reversal training) for trichotillomania and excoriation - Assessing suicidality in BDD (high risk population)

Punishes: - Using relaxation techniques to manage OCD (reduces anxiety in the moment, reinforces avoidance) - Treating OCD with standard CBT without the ERP component - Responding to hoarding disorder by immediately pushing the client to discard items (increases reactance)

Practice Questions

Question 42. A client with OCD describes checking the stove 30–40 times before leaving his apartment. He is 45 minutes late to work most days. He says the checking “has to be done or something terrible will happen.” The MOST appropriate evidence-based treatment is:

- A. CBT to identify and restructure the catastrophic belief about the stove
- B. Exposure and response prevention: gradually approaching the feared situation (leaving) without performing the ritual (checking)
- C. Progressive muscle relaxation to reduce the anxiety that triggers checking
- D. Mindfulness meditation to create distance from the intrusive thoughts

Correct: B. ERP is the gold-standard treatment for OCD. Cognitive restructuring (A) can be a helpful adjunct but is not sufficient alone. Relaxation (C) reduces anxiety without confronting the rituals — maintaining the disorder. Mindfulness (D) is an adjunct but not the primary treatment.

Question 43. A 19-year-old woman is referred by her dermatologist after presenting with wounds from picking at her skin for 2–3 hours daily. She describes difficulty stopping and feels distress after picking sessions. She has tried to stop independently multiple times. The counselor should:

- A. Diagnose OCD and begin ERP
- B. Diagnose Excoriation Disorder and use Habit Reversal Training as the primary treatment
- C. Assess for self-harm and refer for psychiatric evaluation
- D. Provide psychoeducation about the dermatological risks and encourage willpower

Correct: B. Excoriation (skin-picking) disorder is treated with HRT (habit reversal training) — awareness training, competing response training, and social support. It is distinct from OCD (different disorder, different treatment). Self-harm (C) should be ruled out but the presentation is more consistent with excoriation. D is not evidence-based.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 22-year-old reports intrusive, unwanted thoughts that her hands are contaminated, causing intense anxiety that she neutralizes by washing for hours each day. She recognizes the fear is excessive but cannot stop. The most likely diagnosis is:

- A. Generalized Anxiety Disorder
- B. Obsessive-Compulsive Disorder
- C. Specific Phobia
- D. Illness Anxiety Disorder

Answer: B. Obsessions (intrusive contamination thoughts) paired with time-consuming compulsions (washing) define OCD. GAD (A) is diffuse worry without ritualized compulsions. *What would change the answer:* preoccupation with *having* a serious illness and reassurance-seeking, without contamination rituals → Illness Anxiety Disorder.

Assessment — what to do next. The most appropriate symptom-severity measure is:

- A. The Yale-Brown Obsessive Compulsive Scale (Y-BOCS)
- B. The PHQ-9
- C. The GAD-7
- D. The AUDIT

Answer: A. The Y-BOCS is the standard OCD severity measure and

guides exposure and response prevention. The others target depression, anxiety, and alcohol use.

Treatment Decision Flow

OCD-spectrum items reward exposure-based methods and punish reassurance in all its disguises.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
OCD	Exposure and response prevention (ERP)	Reassurance, thought-stopping, or cognitive therapy without ERP
Hoarding disorder	Specialized CBT (motivation, sorting, decision-making skills), harm reduction	A forced cleanout
Body dysmorphic disorder	CBT with ERP-style exposure; address checking and avoidance	Referral for cosmetic procedures
Trichotillomania / excoriation	Habit reversal training	Insight-only exploration

Sequencing & caveats. ERP is first-line for OCD; habit reversal training is first-line for body-focused repetitive behaviors. Build the exposure hierarchy collaboratively.

Avoid / contraindicated. Reassurance and thought-stopping feed the compulsive cycle; forced discarding fails and damages the alliance in hoarding.

Scope. Where an SSRI (often at higher doses) is indicated, the counselor refers rather than prescribes.

Chapter 15 — Schizophrenia Spectrum and Other Psychotic Disorders

Overview

Schizophrenia spectrum disorders are among the most severe on the NCMHCE, and the exam tests them differently from most other chapters — less about therapeutic interventions you would personally deliver, more about recognition, appropriate referral, and collaborative care. A key exam principle: **individual psychotherapy is not the primary treatment for active psychosis**. The counselor’s role includes case management, psychoeducation (for client and family), and coordination with the psychiatric treatment team.

Diagnosis Reference Table

Diagnosis	ICD-10	Duration	Key Features
Schizophrenia	F20.9	≥6 months (active symptoms ≥1 month)	≥2 of: delusions, hallucinations, disorganized speech, disorganized behavior, negative symptoms
Schizophreniform Disorder	F20.81	1–6 months	Same symptoms; good premorbid functioning is a positive prognostic indicator
Brief Psychotic Disorder	F23	1 day – 1 month	Sudden onset, often following stressor; full recovery expected
Schizoaffective Disorder	F25.x	Duration variable	Meets criteria for schizophrenia + concurrent mood episode; mood episode present for majority of illness
Delusional Disorder	F22	≥1 month	Non-bizarre delusions; relatively intact functioning outside the delusion

The Clinical Picture

Positive symptoms (things added to experience): hallucinations (hearing, seeing, feeling, smelling things not there), delusions (fixed false beliefs not culturally explained), disorganized speech (loosening of associations, word salad), and disorganized behavior.

Negative symptoms (things removed from experience): flat affect, alogia (poverty of speech), avolition (lack of motivation), anhedonia, social withdrawal. Negative symptoms are harder to treat and more predictive of long-term functional impairment.

In a case simulation, recognize psychosis by: the client reports experiences no one else can verify (hearing voices, being watched by the government), the thinking pattern is tangential or incoherent, the client's behavior is bizarre or self-neglecting, and the client shows flat or inappropriate affect.

The counselor's role when a client presents with active psychosis: **do not engage with the content of delusions.** Neither agree ("Yes, the government may be watching you") nor argue ("That's not real"). Maintain calm, focused, reality-grounded interaction. Refer for psychiatric evaluation and medication management. The antipsychotic medications are the primary treatment.

What the Exam Rewards / What It Punishes

Rewards: - Immediate psychiatric referral for active psychosis - Family psychoeducation (Family-focused therapy / NAMI involvement) to reduce expressed emotion (EE), which predicts relapse - Supported employment and cognitive remediation in the recovery/maintenance phase - Recognizing negative symptoms as distinct from depression

Punishes: - Attempting insight-oriented individual therapy during active psychosis - Arguing with delusional content - Prescribing or managing antipsychotics (outside the counselor's scope) - Missing the medical rule-out (neurological causes, substance-induced psychosis)

Practice Question

Question 44. A client who has been stable on antipsychotic medication tells her counselor she has stopped taking her medication because “the pills are part of the control system.” The counselor’s MOST appropriate response is:

- A. Explore the client’s concerns about the medication in a non-confrontational manner and contact the prescribing psychiatrist
- B. Reassure her that medication is safe and will not harm her
- C. Discharge the client as she is no longer treatment-compliant
- D. Hospitalize the client immediately due to the likelihood of relapse

Correct: A. The counselor explores the concern (does not argue with the delusion) and coordinates with the prescriber. This is the collaborative care model. B argues against the delusional belief. C abandons the client at a critical moment. D may eventually be appropriate but is not the immediate first step.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 20-year-old has had three months of delusions, auditory hallucinations, disorganized speech, and a decline in social and occupational functioning, without a sustained mood episode. The most likely diagnosis (assuming substance and medical causes are ruled out) is:

- A. Brief Psychotic Disorder
- B. Schizophreniform Disorder
- C. Schizophrenia
- D. Schizoaffective Disorder

Answer: B. Active psychotic symptoms lasting one to six months define Schizophreniform Disorder. Under one month → Brief Psychotic Disorder (A); six months or more → Schizophrenia (C). *What would change the answer:* duration of six months or more → Schizophrenia; a mood episode present for the majority of the illness → Schizoaffective Disorder.

Assessment — what to do next. The most appropriate assessment step is:

- A. Rule out substance-induced and medical causes (history and labs via referral) and establish symptom duration
- B. Begin insight-oriented therapy
- C. Administer the MMSE
- D. Challenge the delusions to test conviction

Answer: A. Symptom duration plus substance/medical rule-out define the diagnosis within the psychotic spectrum. Therapy (B) is treatment; directly challenging delusions (D) is contraindicated.

Treatment Decision Flow

Psychosis items reward getting medication in place first and never arguing with a delusion.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Acute psychosis	Ensure safety; refer for antipsychotic medication management (the primary intervention)	Insight-oriented therapy, exposure, or directly challenging the delusion
Stabilized	CBT for psychosis, psychoeducation, family psychoeducation, supported employment	Discontinuing medication or offering unstructured insight therapy
First-episode psychosis	Refer to coordinated specialty care	Delaying treatment
Voices persist despite adherence	CBTp coping strategies to change the relationship with the voices	Framing residual symptoms as treatment failure

Sequencing & caveats. Medication is the primary treatment; psychotherapy is adjunctive. Family psychoeducation reduces relapse and expressed emotion.

Avoid / contraindicated. Do not argue with or directly challenge delusions; avoid exposure; do not support medication discontinuation.

Scope. Refer for antipsychotic management and coordinate care.

Chapter 16 — Feeding and Eating Disorders

Overview

Eating disorders carry the highest mortality rate of any psychiatric condition — a statistic the NCMHCE exam knows and tests. The exam expects you to: assess medical stability first, know the diagnostic criteria, apply the appropriate level of care, and understand the multi-team nature of eating disorder treatment.

Diagnosis Reference Table

Diagnosis	ICD-10	Core Feature	Medical Concern
Anorexia Nervosa — Restricting	F50.01	Restriction, low weight, body image disturbance, fear of gaining	Cardiac arrhythmia, bone loss, lanugo, bradycardia — HIGH mortality
Anorexia Nervosa — Binge/Purge	F50.02	Restriction + binge/purge cycles, low weight	Same as above
Bulimia Nervosa	F50.2	Binge-purge cycles, normal or above-average weight, shame cycle	Electrolyte imbalance, enamel erosion, Russell’s sign
Binge Eating Disorder	F50.81	Recurrent binge episodes without compensatory behaviors; significant distress	Obesity-related metabolic effects
Avoidant/Restrictive Food Intake Disorder (ARFID)	F50.82	Restriction not driven by body image; sensory, fear, or disinterest-based	Nutritional deficiency, growth failure in children
Pica	F98.3	Eating non-nutritive substances	Lead poisoning, GI obstruction
Rumination Disorder	F98.21	Repeated regurgitation of food	Nutritional, dental, social

The Clinical Picture

Anorexia Nervosa (AN): The client presents as underweight but may not perceive themselves as thin — body image disturbance is a core feature. They may be brought in by family (the client often does not self-refer because the disorder is ego-syntonic). They may minimize the problem (“I eat fine”) or be engaged but hide restriction severity. Physical signs: lanugo (fine hair on face/body), bradycardia, cold intolerance, amenorrhea, dental erosion (in binge/purge type).

Bulimia Nervosa (BN): The binge-purge cycle is driven by shame. The client often eats normally in public and purges privately. They are more likely than AN clients to self-refer because the disorder is ego-dystonic — the binges and purges feel out of control and shameful. Russell’s sign (calluses on knuckles from self-induced vomiting) is a physical marker.

The Refeeding Syndrome: A potentially fatal electrolyte shift that can occur when malnourished clients (particularly AN) begin eating again too quickly. It is a medical emergency. Medical monitoring is essential during refeeding.

What the Exam Rewards / What It Punishes

Rewards: - Medical evaluation FIRST for any suspected eating disorder (cardiac monitoring, electrolytes, bone density) - Knowing that higher levels of care (residential, partial hospital) are required for medically unstable clients - Multi-disciplinary team approach (counselor + dietitian + physician + psychiatrist) - Family-Based Treatment (FBT/Maudsley approach) as first-line for adolescents with AN - CBT-E (enhanced) as evidence-based treatment for BN and BED

Punishes: - Attempting outpatient therapy with a medically unstable client with AN - Focusing only on weight restoration without addressing the psychological maintaining factors - Colluding with the client’s body image distortion (e.g., agreeing that they “look a little heavy”) - Missing that purging in BN can cause fatal electrolyte imbalances (hypokalemia → cardiac arrhythmia)

Worked Mini-Case

Intake vignette: Sophia, 16, is brought in by her parents. She has lost 22 pounds over the past 8 months. Her parents became alarmed when her pulse at a school screening was 48 bpm. She says she is “eating fine” and doesn’t understand why everyone is worried. She denies purging. Her BMI is 16.2.

Reasoning chain:

Step 1 — Medical emergency: Bradycardia (48 bpm) in an underweight adolescent with significant weight loss is a cardiac warning sign. Before any psychotherapy, medical evaluation is needed. Sophia may need hospitalization for medical stabilization.

Step 2 — Level of care: Based on the ASAM or eating disorder-specific criteria (MEDA: medical stability, eating disorder behaviors, BMI, psychiatric comorbidities), Sophia may need residential or inpatient medical level before outpatient counseling.

Step 3 — Denial: Her insistence that she is “eating fine” reflects the ego-syntonic quality and body image distortion of AN. The counselor does not argue but maintains the medical priority.

Step 4 — Treatment once medically stable: Family-Based Treatment (FBT/Maudsley) is the first-line for adolescents with AN — parents are empowered to temporarily take control of eating while the adolescent’s brain is malnourished and unable to make rational food decisions.

Practice Questions

Question 45. A client with bulimia nervosa is engaging in CBT. She reports that last week she binged five times — up from two the previous week. She attributes the increase to exam stress. The counselor should:

- A. Review the binge-purge logs and increase CBT session frequency
- B. Evaluate electrolyte levels and physical status, then address the stressor in treatment
- C. Add a relaxation component to address the exam-related stress
- D. Maintain the current treatment without change, as fluctuations are expected

Correct: B. Medical safety is always rung 2. An increase in purging frequency increases the risk of hypokalemia (from vomiting or laxative use) — electrolyte evaluation is warranted. The stressor can then be integrated into CBT work. A skips the medical check. C and D are insufficient responses to an acute escalation.

Question 46. Which of the following presentations warrants IMMEDIATE medical evaluation or hospitalization rather than outpatient counseling?

- A. A client with bulimia nervosa who has been purging 3 times per week for 6 months, maintains normal BMI, and is motivated for treatment
- B. A client with binge eating disorder who has gained 30 pounds and has comorbid depression
- C. A client with anorexia nervosa presenting with BMI of 14.5, heart rate of 44 bpm, and reports of fainting
- D. A client with ARFID who limits food textures but maintains adequate nutrition and growth

Correct: C. BMI 14.5 (severely low), bradycardia (44 bpm), and syncopal episodes indicate medical instability. This client requires immediate medical evaluation and likely inpatient or residential medical level of care before any outpatient therapy begins. A, B, and D can begin in outpatient settings with appropriate monitoring.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 19-year-old restricts her intake, has a BMI of 16, reports intense fear of weight gain, and perceives herself as overweight despite being underweight. She does not binge or purge. The most likely diagnosis is:

- A. Bulimia Nervosa
- B. Anorexia Nervosa, restricting type
- C. Avoidant/Restrictive Food Intake Disorder
- D. Binge-Eating Disorder

Answer: B. Restriction leading to significantly low weight, intense fear of weight gain, and body-image disturbance define anorexia, restricting type. ARFID (C) lacks the weight/shape concern; bulimia

(A) and BED (D) involve binge episodes. *What would change the answer:* normal weight with binge-purge cycles → Bulimia; food avoidance without body-image disturbance → ARFID.

Assessment — what to do next. Given the BMI of 16, the most appropriate first assessment step is:

- A. Administer the EAT-26 and begin outpatient CBT-E
- B. Assess medical stability first (vitals, orthostatics, electrolytes, ECG) via medical referral before psychosocial treatment
- C. Begin family-based treatment immediately
- D. Administer the PHQ-9

Answer: B. At a low BMI, medical stability is assessed first, and psychosocial treatment pauses if the client is unstable. Self-report screening and therapy (A, C) follow medical clearance.

Treatment Decision Flow

Eating-disorder items are decided by *medical stability first*, then the stage-appropriate modality.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Anorexia with BMI < 18.5, HR < 50, or orthostatic hypotension	Pause psychosocial treatment; facilitate immediate medical stabilization/hospitalization	Beginning outpatient CBT-E or FBT
Adolescent anorexia, medically stable	Family-based treatment (FBT)	Individual insight-oriented therapy
Bulimia nervosa, medically stable	CBT-E	Nutrition education alone
Binge-eating disorder	CBT-E or IPT	Weight-loss dieting as the primary intervention

Sequencing & caveats. When the client is medically unstable, medical stabilization precedes all psychosocial treatment. FBT is first-line for medically stable adolescents with anorexia. Watch for refeeding syndrome during renutrition.

Avoid / contraindicated. FBT is contraindicated where there is active parental abuse or the parents cannot supervise meals. Do not begin outpatient therapy while the client is medically unstable.

Scope. Care is multidisciplinary (physician, dietitian, therapist); the counselor coordinates and refers for medical management.

Chapter 17 — Crisis, Risk, and Safety

Overview

This chapter is non-negotiable. Every NCMHCE candidate must have a thorough, fluent understanding of risk assessment and crisis response — because safety (rung 1) applies to every single case simulation that touches on suicidality, homicidality, abuse, or danger. No clinical action precedes safety assessment when risk is present.

The exam tests two specific things about suicidality: (1) your ability to conduct a thorough, direct, and compassionate risk assessment, and (2) your ability to match the appropriate level of care to the assessed risk level.

The Columbia Suicide Severity Rating Scale (C-SSRS) Framework

The C-SSRS is the most widely used structured risk assessment tool and the framework the exam implicitly uses. It assesses:

Ideation: - Wish to be dead (passive) - Active suicidal ideation — nonspecific (without plan or intent) - Active suicidal ideation with plan - Active suicidal ideation with plan and intent to act - Active suicidal ideation with plan, intent, and preparatory behaviors

Behavior (recent and lifetime): - Aborted attempt - Interrupted attempt - Actual attempt - Non-suicidal self-injury (important to distinguish)

The SAL Framework for Risk Assessment

S — Suicidal Ideation: Presence, frequency, intensity, duration. Active vs. passive.

A — Access to means: Does the client have a gun at home? Pills available? Access to lethal means dramatically increases risk.

Means restriction counseling — reducing access to the most lethal means — is a primary safety intervention with high evidence.

L — Lethality of plan: A plan involving firearms is higher risk than a plan involving pills. A specific plan with a time, place, and method is higher risk than vague ideation.

Additional risk factors: - Prior attempt (strongest single predictor of future attempt) - Current substance intoxication - Recent significant loss - Social isolation / lack of support - History of impulsivity - Male sex (higher completion rate); female sex (higher attempt rate) - Access to means

Protective factors: - Reasons for living (children, religious beliefs, future plans) - Social support - Help-seeking behavior (client is in your office) - Engaged in treatment

Level of Care Matching

Risk Level	Features	Appropriate Action
Low	Passive ideation, no plan, protective factors present, no history of attempts	Safety plan, increase session frequency, remove means
Moderate	Active ideation, vague plan, some protective factors, prior attempt history	Safety plan, family/support involvement, possible voluntary hospitalization; possibly intensive outpatient
High	Active ideation with specific plan, intent, means available, prior attempt, intoxicated	Emergency psychiatric evaluation; 5150/involuntary hold if imminent danger
Imminent	States intent to act immediately; has means	Call 911 / contact emergency services; do not leave client alone

The Safety Plan

A safety plan (not a “no-harm contract” — the research does not support no-harm contracts as effective) includes: 1. Warning signs

(what the client notices before crisis escalates) 2. Internal coping strategies (what the client can do alone) 3. Social contacts for distraction (friends/family who don't need to know the reason) 4. People and agencies to contact for help (with numbers) 5. Making the environment safer (restricting means) 6. Reasons for living

Mandated Reporting: Child Abuse and Neglect

All mental health counselors are mandated reporters. When there is *reasonable suspicion* (not certainty) of child abuse or neglect, the report must be made to Child Protective Services. The counselor does not investigate; the counselor reports.

Four types of child maltreatment: - Physical abuse - Sexual abuse - Emotional/psychological abuse - Neglect (by far the most common)

Duty to Warn (Tarasoff): When a client makes a specific, credible threat against an identifiable third party, the counselor has a duty to protect that third party. Actions may include: warning the intended victim, notifying law enforcement, or arranging for involuntary hospitalization. The specific obligations vary by state. The exam tests the principle — specific, identifiable threat, identifiable victim — not state-specific law.

What the Exam Rewards / What It Punishes

Rewards: - Direct, compassionate, non-judgmental suicidal ideation assessment (“Are you having thoughts of suicide?”) - Using the safety plan over the no-harm contract - Means restriction counseling - Matching level of care to risk (not automatically hospitalizing every suicidal client, but not under-responding to high-risk presentations) - Reporting suspected abuse when there is reasonable suspicion — not waiting for certainty

Punishes: - Avoiding direct inquiry about suicidal ideation (“I didn’t want to put the idea in their head” — this is a myth) - Assuming chronic suicidality in BPD doesn’t need ongoing assessment - Failing to report suspected abuse due to fear of disrupting the therapeutic alliance - Using a no-harm contract as a substitute for a thorough risk assessment - Discharging a high-risk client without a safety plan

Worked Mini-Case

Vignette: A client calls between sessions and says, “I’ve been thinking about taking all my sleeping pills tonight. I have them right here. I just don’t see a way out.”

Reasoning chain:

This is a high-to-imminent risk presentation: - Active ideation ✓ - Specific plan (sleeping pills) ✓ - Access to means (has them available) ✓ - Intent (tonight) ✓ - Hopelessness (“I don’t see a way out”) ✓

Response sequence: 1. Stay on the line; do not hang up. 2. Ask her to move away from the pills while speaking with you. 3. Assess whether she has taken anything already (medical emergency if yes → 911). 4. Engage her around reasons for living; explore what led to the escalation tonight. 5. Arrange for emergency evaluation: either the client agrees to go to the ER (and a support person accompanies her), or if she refuses and the risk is imminent, call 911 with her on the phone or with knowledge of her address. 6. Do not end the call until a safety plan is in place or emergency services have been contacted.

Practice Questions

Question 47. A client discloses active suicidal ideation with a specific plan to hang himself using a rope he has already purchased. He denies intent to act “tonight” but says “soon.” He lives alone. The counselor should:

- A. Create a safety plan and schedule a session for tomorrow
- B. Conduct a full risk assessment, remove the means if possible, and arrange emergency psychiatric evaluation
- C. Contact his family to stay with him tonight and follow up next session
- D. Administer the PHQ-9 and document the disclosure in the chart

Correct: B. This is high-risk: specific plan, purchased means, lives alone, intent expressed as “soon.” Emergency psychiatric evaluation is required. A safety plan alone is insufficient at this risk level. Family contact (C) may be part of the plan but not the *primary* action. Documentation (D) is necessary but not a safety intervention.

Question 48. A child client reports that her uncle “touches her in a place I don’t like” when she stays at his home on weekends. The counselor should:

- A. Explore the disclosure further with the child to determine whether abuse occurred before reporting
- B. Contact the client’s parents and let them handle the situation
- C. Report the disclosure to child protective services immediately based on reasonable suspicion
- D. Consult with a supervisor before making any report

Correct: C. Mandated reporting requires reasonable suspicion, not certainty. The counselor does not investigate. A delays a mandatory report. B inappropriately transfers the legal obligation to the parents (and may alert the alleged abuser). D may be appropriate as a simultaneous step, but does not delay the report.

Question 49. Which of the following is the MOST significant single risk factor for a future suicide attempt?

- A. Diagnosis of major depressive disorder
- B. History of a prior suicide attempt
- C. Social isolation
- D. Access to firearms

Correct: B. Prior attempt is the strongest single predictor of a future attempt. All others (A, C, D) are significant risk factors but are less predictive than prior attempt history.

Question 50. A client with chronic BPD and a history of repeated suicidal gestures mentions, during a routine session, “I’ve been thinking about suicide again — like always.” The counselor should:

- A. Acknowledge the statement and continue the session, as chronic suicidality is expected in BPD
- B. Conduct a current risk assessment, update the safety plan, and document
- C. Refer to the crisis line for this type of ongoing ideation
- D. Contact a supervisor to discuss whether to hospitalize

Correct: B. Chronic suicidality in BPD does not excuse the counselor from assessing each expression. Each report requires assess-

ment of current ideation (is this different from baseline?), means, intent, and plan. Ignoring it (A) is a clinical and ethical failure. C deflects responsibility. D may be appropriate but is not the first step — assess first.

Chapter 18 — Disruptive, Impulse-Control, and Conduct Disorders

Overview

This family groups disorders characterized by problems with self-regulation of emotions and behavior that violate the rights of others or bring the person into conflict with social norms. The exam primarily tests this in the context of youth populations and asks you to distinguish between childhood behavioral presentations.

Diagnosis Reference Table

Diagnosis	ICD-10	Key Feature	Age/Context
Oppositional Defiant Disorder (ODD)	F91.3	Angry/irritable mood + argumentative/defiant behavior + vindictiveness ≥6 months	Children/adolescents; directed at authority figures
Conduct Disorder	F91.x	Serious violation of others' rights and societal norms (aggression, destruction, deceit, rule violations)	Children/adolescents; precursor to ASPD
Intermittent Explosive Disorder	F63.81	Recurrent, impulsive aggressive outbursts disproportionate to provocation	Adolescents and adults
Antisocial PD	F60.2	Adult pattern of rights violations (requires CD before 15)	Adults ≥18
Pyromania	F63.1	Deliberate fire-setting with fascination and tension-relief pattern	Rare
Kleptomania	F63.2	Recurrent stealing not for gain; tension-relief pattern	Rare

The Clinical Picture

ODD vs. CD: The critical distinction is severity and type of rule violation. ODD involves defiance, arguing, and irritability — annoying but not seriously harmful. CD involves aggression toward people or animals, destruction of property, deceit/theft, and serious rule violations. A child who argues with teachers and refuses chores has ODD; a child who fights, breaks into homes, and tortures animals has CD.

The developmental pathway: ODD → CD → ASPD is a known risk trajectory, especially with early-onset CD and lack of early intervention. Not all ODD children develop CD, and not all CD adolescents develop ASPD — but the association is clinically important and the exam tests it.

Parenting components: Evidence-based treatment for ODD and CD includes parent management training (PMT). Parents are taught to reinforce prosocial behavior consistently, set clear and enforceable limits, use natural consequences, and reduce inadvertent reinforcement of problem behavior.

What the Exam Rewards / What It Punishes

Rewards: - Parent management training as the core treatment for ODD/CD in children - Functional behavioral assessment to understand the ABCs of the behavior (antecedent → behavior → consequence) - Multisystemic therapy (MST) for severe adolescent CD (targets family, peer, school, community systems simultaneously) - Assessing for co-occurring ADHD, trauma, or mood disorders (ODD rarely appears alone)

Punishes: - Individual insight-oriented therapy as the sole treatment for ODD/CD (insufficient without systemic components) - Diagnosing CD or ASPD in young children (ASPD requires ≥18 and prior CD) - Failing to assess the family system's contribution to the behavioral pattern

Practice Question

Question 51. A 9-year-old boy is referred for aggressive behavior. He has been suspended three times this year for fighting, was

caught setting a small fire in the schoolyard, and his parents report he has stolen money from them multiple times. These behaviors have persisted for over a year. The MOST appropriate diagnosis to consider is:

- A. ADHD — predominantly hyperactive/impulsive
- B. ODD
- C. Conduct Disorder
- D. Intermittent Explosive Disorder

Correct: C. Aggression toward others, fire-setting, and theft are CD criteria — these are serious violations of others' rights, not just defiant behavior (ODD). ADHD may co-occur but doesn't account for the pattern of rights violations. IED involves impulsive aggression but not the deceit and rule-violation pattern.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 15-year-old has a 14-month pattern of physical fights, bullying, shoplifting, staying out past curfew, and cruelty to a neighbor's pet, with onset at age 13. The most likely diagnosis is:

- A. Oppositional Defiant Disorder
- B. Conduct Disorder, adolescent-onset
- C. Intermittent Explosive Disorder
- D. Attention-Deficit/Hyperactivity Disorder

Answer: B. Aggression toward people and animals, theft, and serious rule violations define Conduct Disorder; onset at or after age 10 is the adolescent-onset type, which carries a better prognosis. ODD (A) is defiance and anger without rights-violating aggression or theft; IED (C) is impulsive aggression alone. *What would change the answer:* onset before age 10 → childhood-onset type.

Assessment — what to do next. The most appropriate assessment approach is:

- A. Obtain multi-informant history (parents, school) and assess family and peer context and safety
- B. Diagnose from the adolescent's self-report alone
- C. Administer the MoCA
- D. Recommend a boot-camp evaluation

Answer: A. Conduct Disorder assessment requires multi-informant data and attention to family, peer, and safety context. Self-report alone (B) is insufficient; boot camp (D) is a harmful intervention, not an assessment.

Treatment Decision Flow

Conduct and impulse-control items reward family/systemic treatment and punish anything that aggregates antisocial youth.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Adolescent conduct disorder	Family-based evidence treatment (MST or FFT); parent management training	Individual insight therapy, or “scared straight”/boot-camp programs
Oppositional defiant disorder (child)	Parent management training	Child-only individual therapy
Intermittent explosive disorder	CBT for arousal/anger regulation and skills	Cathartic “venting” approaches
Deviant peer affiliation	Build prosocial connections and activities	Group therapy that aggregates antisocial youth

Sequencing & caveats. MST and FFT have the strongest evidence for adolescent conduct disorder; parental supervision and consistency are key, modifiable targets.

Avoid / contraindicated. Scared-straight and boot-camp programs can worsen outcomes; grouping antisocial youth together is iatrogenic.

Scope. Standard counselor scope; coordinate school and family systems.

Chapter 19 — Somatic Symptom and Related Disorders

Overview

Somatic symptom and related disorders involve physical symptoms that cause significant distress or impairment — but the central issue is the *psychological response* to those symptoms (excessive worry, disproportionate fear) rather than whether the symptoms are medically explained. The DSM-5 removed the concept of “medically unexplained symptoms” as a requirement — even a client with a diagnosed medical condition can have a somatic symptom disorder if their response to the symptoms is excessive and disabling.

Diagnosis Reference Table

Diagnosis	ICD-10	Core Feature
Somatic Symptom Disorder	F45.1	≥1 somatic symptom with excessive thoughts, feelings, or behaviors related to the symptom
Illness Anxiety Disorder	F45.21	Fear of having a serious illness with minimal or no somatic symptoms
Conversion Disorder (Functional Neurological Symptom Disorder)	F44.x	Motor or sensory symptoms that cannot be explained neurologically
Factitious Disorder	F68.1x	Falsification of medical/psychological symptoms; motivated by sick role
Psychological Factors Affecting Medical Condition	F54	Psychological factors adversely affect a medical condition

Differentiating Within the Family

Somatic Symptom Disorder vs. Illness Anxiety Disorder: The key distinction: SSD has significant somatic symptoms with excessive psychological response to them. IAD has minimal or absent somatic symptoms with intense fear of having a serious illness (previously called hypochondriasis in DSM-IV).

Conversion Disorder vs. Malingering vs. Factitious Disorder:

- Conversion: Symptoms are real to the client; not consciously produced; no external incentive - Factitious: Symptoms are consciously produced; motivated by the sick role (internal incentive) - Malingering: Symptoms are consciously produced; motivated by external incentive (avoiding legal consequences, getting disability payments)

What the Exam Rewards / What It Punishes

Rewards: - Medical evaluation first (even in somatic disorders, a medical workup establishes the clinical picture) - Avoiding dismissive language (“it’s all in your head” — never appropriate) - CBT targeting catastrophic health beliefs and hypervigilance to bodily sensations - Maintaining the alliance with clients who have been dismissed by medical providers

Punishes: - Diagnosing somatic symptom disorder before completing a medical evaluation - Treating somatic disorders without addressing the psychological response patterns - Colluding with excessive medical testing as a reassurance behavior

Practice Question

Question 52. A client presents reporting persistent chest pain, dizziness, and shortness of breath that have been evaluated by three different cardiologists with no medical findings. She spends 4-5 hours daily researching her symptoms online and visits the ER every two weeks. She is increasingly unable to work. The MOST appropriate clinical response is:

- A. Refer to a fourth specialist, as the medical evaluation may have been incomplete
- B. Diagnose conversion disorder and begin CBT

C. Acknowledge the distress caused by her symptoms, affirm the medical clearances, and begin CBT targeting health anxiety behaviors

D. Refer to a psychiatrist for medication to address anxiety symptoms

Correct: C. This presentation is consistent with Somatic Symptom Disorder or Illness Anxiety Disorder — the physical symptoms are real to her; the response (5 hours of online research, frequent ER visits) is disproportionate and maintaining the anxiety. CBT targeting reassurance-seeking and health anxiety is the evidence-based treatment. A reinforces excessive help-seeking. B misidentifies the diagnosis (conversion involves neurological symptoms). D is not first-line.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 40-year-old has eight months of excessive worry about, and time consumed by, multiple physical symptoms. Repeated medical workups are negative, yet she remains highly distressed and disproportionately focused on the symptoms. The most likely diagnosis is:

A. Illness Anxiety Disorder

B. Somatic Symptom Disorder

C. Conversion Disorder (Functional Neurological Symptom Disorder)

D. Factitious Disorder

Answer: B. Distressing somatic symptoms accompanied by disproportionate thoughts, anxiety, and time devoted to them, persisting six months or more, define Somatic Symptom Disorder. Illness Anxiety (A) features preoccupation with *having* a disease with minimal actual symptoms; Conversion (C) is a neurological deficit; Factitious (D) is intentional falsification. *What would change the answer:* few or no somatic symptoms but strong disease conviction → Illness Anxiety Disorder.

Assessment — what to do next. The most appropriate approach is:

A. Order additional diagnostic tests to reassure her

B. Coordinate with the medical provider to avoid redundant testing,

assess symptom-related distress and functioning, and schedule regular (not symptom-contingent) contact

C. Tell her the symptoms are not real

D. Administer the Y-BOCS

Answer: B. Care is coordinated to prevent iatrogenic over-testing while distress and functioning are assessed and visits are scheduled. More tests (A) and dismissal (C) each worsen the cycle.

Treatment Decision Flow

Somatic-symptom items reward validation plus consistent, non-contingent care — and punish both dismissal and reassurance.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Somatic symptom disorder	CBT with regular, scheduled (not symptom-contingent) visits; validate distress	Ordering more tests, or dismissing symptoms as “not real”
Illness anxiety disorder	CBT that reduces reassurance-seeking and body checking	Providing repeated reassurance
Functional neurological (conversion) disorder	Psychoeducation, physical therapy, CBT	Confronting the client as “faking”

Sequencing & caveats. Scheduled visits decouple care from symptom escalation. Validate the reality of the client’s distress while shifting the focus from investigation to functioning.

Avoid / contraindicated. Excessive testing and repeated reassurance both reinforce the symptom cycle; telling a client it is “all in your head” ruptures the alliance.

Scope. Coordinate with medical providers to prevent over-testing.

Chapter 20 — Dissociative Disorders

Overview

Dissociative disorders involve disruptions in consciousness, memory, identity, or perception. They are almost always rooted in trauma and are among the most misunderstood and stigmatized diagnoses in mental health. The exam tests recognition and appropriate treatment planning — primarily the three-phase trauma treatment model and the stabilization-before-processing principle (which is especially critical in DID).

Diagnosis Reference Table

Diagnosis	ICD-10	Core Feature
Dissociative Identity Disorder (DID)	F44.81	≥2 distinct personality states/identities; amnesia between states
Dissociative Amnesia	F44.0	Inability to recall important autobiographical information (not ordinary forgetting)
Depersonalization/Derealization Disorder	F48.1	Persistent experiences of unreality or detachment from self (depersonalization) or surroundings (derealization)

The Clinical Picture

DID in a case sim: A client reports “losing time,” being told they did or said things they don’t remember, finding objects or writings they don’t recall creating, hearing voices “from inside” (not hallucinations in the schizophrenic sense — internal voices belonging to other identity states), and extreme identity confusion. The voices are often experienced as distinctly different from the client’s own thoughts.

Depersonalization/derealization: The client describes feeling like they are watching themselves from outside their body (depersonalization) or that the world looks unreal, foggy, or dreamlike (derealization). These experiences are disturbing and persistent. Common in panic disorder, PTSD, and trauma histories, but when they are the persistent, primary complaint, the disorder is DP/DR disorder.

What the Exam Rewards / What It Punishes

Rewards: - The three-phase trauma model (stabilization → processing → integration) — applied even more carefully in DID because processing without stabilization can cause rapid destabilization between identity states - Compassionate, non-reactive acceptance of DID presentations (not treating the diagnosis as inherently dramatic or invalid) - Assessment of safety for *all* identity states in DID (some states may be suicidal even if the presenting state is not)

Punishes: - Attempting direct trauma processing with DID clients who lack sufficient stabilization and communication between states - Dismissing dissociative symptoms as manipulation - Failing to recognize that reported “hearing voices” in DID clients is internal (identity-based) rather than external hallucination

Practice Question

Question 53. A client with Dissociative Identity Disorder has been in treatment for four months. She is now requesting to begin EMDR to process childhood abuse memories. The counselor should:

- A. Begin EMDR immediately, as the client has been in treatment long enough
- B. Assess the current level of stabilization: communication between states, capacity to stay grounded, availability of coping skills; proceed to EMDR only if stabilization criteria are met
- C. Refuse EMDR and maintain supportive treatment only
- D. Increase session frequency to support the client through the EMDR process

Correct: B. Phase 1 (stabilization) criteria must be met before Phase 2 (processing) begins — this is even more critical in DID where destabilization can occur rapidly and different identity states

may process trauma differently. A ignores phase criteria. C withholds an evidence-based treatment unnecessarily. D adds sessions without addressing the phase readiness question.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 30-year-old reports gaps in memory for hours at a time, finds unfamiliar items she does not remember buying, and has been told she sometimes behaves like “a different person” who uses a different name. She has an extensive childhood trauma history and no psychotic symptoms. The most likely diagnosis is:

- A. Schizophrenia
- B. Dissociative Identity Disorder
- C. Borderline Personality Disorder
- D. Dissociative Amnesia

Answer: B. Two or more distinct identity states with recurrent amnesia, in the context of severe trauma, define DID; the experiences are internal, not the external hallucinations of psychosis (A). *What would change the answer:* amnesia without distinct identity states → Dissociative Amnesia.

Assessment — what to do next. The most appropriate assessment step is:

- A. Use hypnosis to recover the missing memories
- B. Screen for dissociation with a validated measure (e.g., the Dissociative Experiences Scale) and prioritize current safety and stabilization
- C. Confront the client about the inconsistencies
- D. Administer the MMSE

Answer: B. Validated dissociation screening with a safety-and-stabilization focus is appropriate. Hypnotic memory recovery (A) risks memory contamination and destabilization.

Treatment Decision Flow

Dissociative items reward phase-oriented pacing and punish premature or suggestive memory work.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Dissociative identity disorder	Phase 1: safety and stabilization; build internal communication before any trauma processing	Moving directly to trauma processing
Recovering traumatic memories	Stabilize; let material emerge without pressure	Hypnosis or suggestive techniques to “recover” memories (contamination risk)
Hostile “protector” part	Acknowledge its protective function; build trust across the system	Trying to eliminate or forcibly integrate parts early

Sequencing & caveats. Treatment is phase-oriented: stabilization precedes trauma work; premature processing destabilizes and retraumatizes.

Avoid / contraindicated. Avoid suggestive memory-recovery techniques and forced integration of parts.

Scope. Standard scope; DID benefits from specialized, relationally stable treatment.

Chapter 21 — Sleep-Wake Disorders

Overview

Sleep disorders appear on the NCMHCE primarily as co-occurring conditions that complicate other diagnoses. A counselor who misses that poor sleep is driving cognitive impairment, irritability, or mood instability will misattribute symptoms to a mood or anxiety disorder. The exam expects you to assess sleep as a routine component of the intake.

Key Sleep Disorders for the NCMHCE

Disorder	Core Feature	Counselor Role
Insomnia Disorder	Difficulty initiating/maintaining sleep ≥ 3 nights/week, ≥ 3 months	CBT-I (first-line over medication)
Hypersomnolence Disorder	Excessive sleepiness despite adequate sleep	Medical evaluation, rule out medical causes
Narcolepsy	Recurrent lapses into sleep + cataplexy	Medical management; counselor provides psychoeducation/support
Obstructive Sleep Apnea	Repeated airway obstruction during sleep; snoring, daytime fatigue	Medical referral for sleep study, CPAP
Nightmare Disorder	Repeated, disturbing dreams causing distress; awakening with recall	IRT (Imagery Rehearsal Therapy), especially if trauma-related
REM Sleep Behavior Disorder	Acting out dreams physically (punching, kicking)	Medical evaluation; often prodromal to neurodegenerative disease

CBT-I (Cognitive Behavioral Therapy for Insomnia)

CBT-I is the gold-standard, first-line treatment for chronic insomnia — superior to medication for long-term outcomes. Components:

1. **Sleep restriction therapy** — limit time in bed to actual sleep time to build sleep drive
2. **Stimulus control** — the bed is for sleep and sex only; get up if not asleep within 20 minutes
3. **Sleep hygiene education** — consistent wake time, no screens before bed, cool dark room
4. **Cognitive restructuring** — address catastrophic beliefs about sleep (“If I don’t sleep, tomorrow is ruined”)
5. **Relaxation training** — progressive muscle relaxation, diaphragmatic breathing

Practice Question

Question 54. A client with insomnia reports she has been taking a sleep aid for 6 months and wants to reduce the medication. She often lies awake in bed for 2-3 hours and has begun watching television in bed. The MOST appropriate evidence-based intervention is:

- A. Refer to a sleep specialist for a sleep study
- B. Support the gradual medication taper and add CBT-I: sleep restriction and stimulus control
- C. Recommend that she use the bedroom for sleep only and introduce a consistent wake time without medication changes
- D. Provide relaxation training only, as that addresses the hyperarousal component

Correct: B. CBT-I (specifically sleep restriction and stimulus control, which addresses the bed-watching behavior) combined with a supported medication taper is the evidence-based approach. C is partially correct (stimulus control, wake time) but the medication taper is an important concurrent goal. D addresses only one component of CBT-I.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 45-year-old reports six months of difficulty falling and staying asleep at least four nights a week, with daytime fatigue, despite adequate opportunity to sleep. He reports

no loud snoring or witnessed breathing pauses, and his mood is otherwise stable. The most likely diagnosis is:

- A. Obstructive Sleep Apnea
- B. Insomnia Disorder
- C. Circadian Rhythm Sleep-Wake Disorder
- D. Major Depressive Disorder

Answer: B. Difficulty initiating and maintaining sleep at least three nights a week for three months or more, with daytime impairment and adequate opportunity, defines Insomnia Disorder. The absence of snoring or apneas argues against OSA (A); stable mood argues against MDD (D). *What would change the answer:* loud snoring, witnessed apneas, or obesity → evaluate for OSA; a shifted but otherwise normal sleep schedule → circadian disorder.

Assessment — what to do next. The most appropriate assessment step is:

- A. Have the client keep a one-to-two-week sleep diary, screen for OSA risk (e.g., STOP-BANG), and refer for polysomnography if indicated
- B. Begin a hypnotic immediately
- C. Start sleep restriction without screening
- D. Administer the PHQ-9 only

Answer: A. A sleep diary plus OSA screening guide the diagnosis and the need for a sleep study or referral. Note that sleep restriction (C) is screened first for bipolar and seizure contraindications.

Treatment Decision Flow

Sleep items reward CBT-I over hypnotics and include a genuine medical contraindication to screen for.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Chronic insomnia	CBT-I (first-line; superior to medication long-term)	Long-term hypnotics as the primary treatment
Sleep consolidation needed	Sleep restriction (a CBT-I component)	Advising the client to spend <i>more</i> time in bed
Conditioned arousal (bed associated with wakefulness)	Stimulus control (bed for sleep and sex only)	Allowing work or TV in bed

Sequencing & caveats. CBT-I is first-line and durable. Sleep restriction and stimulus control are counterintuitive but are the active ingredients; pair them with psychoeducation to sustain adherence.

Avoid / contraindicated. Sleep restriction should be used with caution or avoided in bipolar I disorder (sleep loss can precipitate mania) and in seizure disorders — screen before prescribing the technique.

Scope. Short-term hypnotics, if used, are accessed by referral; CBT-I remains the primary treatment.

Chapter 22 — Sexual Dysfunctions and Gender Dysphoria

Overview

This chapter tests two distinct topic areas: (1) sexual dysfunctions — problems with desire, arousal, or orgasm that cause significant distress — and (2) gender dysphoria — distress arising from incongruence between gender identity and assigned sex. The exam’s approach to gender dysphoria reflects an affirming model: the counselor’s role is to support the client’s self-identified gender, explore the distress, and facilitate access to care — not to question, challenge, or attempt to change the client’s gender identity.

Sexual Dysfunctions Reference Table

Diagnosis	ICD-10	Core Feature
Female Sexual Interest/Arousal Disorder	F52.22	Reduced or absent interest in/arousal to sexual activity
Female Orgasmic Disorder	F52.31	Difficulty or inability to achieve orgasm
Genito-Pelvic Pain/Penetration Disorder	F52.6	Difficulty with penetration; pelvic floor tension/pain; fear
Male Hypoactive Sexual Desire Disorder	F52.0	Persistently reduced/absent sexual desire
Erectile Disorder	F52.21	Difficulty achieving/maintaining erection
Delayed/Absent Ejaculation	F52.32	Marked delay or absence of ejaculation
Premature Ejaculation	F52.4	Ejaculation within ~1 minute of penetration, consistently

All sexual dysfunctions require: persistent, recurring, causes significant distress to the individual, and is not better explained by a non-sexual mental disorder, substance, medication, or medical condition.

Medical rule-out is essential: Many sexual dysfunctions have organic causes (hormonal, vascular, neurological, medication side effects). The counselor should always coordinate with a medical provider.

Gender Dysphoria

DSM-5 criteria: Marked incongruence between the person's experienced/expressed gender and assigned gender, lasting ≥ 6 months, causing significant distress or functional impairment.

Affirming stance: The NCMHCE expects counselors to use the client's preferred pronouns and name, validate gender identity as real and valid, assess and support the client's goals (which may include social transition, medical transition, or neither), and address the distress caused by social rejection, discrimination, or dysphoria — not attempt to align the client's gender identity with their assigned sex.

Counselor role: Psychoeducation, support, advocacy, coordination with medical providers for clients seeking hormonal or surgical intervention (informed consent model or WPATH Standards of Care model), and mental health assessment — not gatekeeper, but collaborative partner.

What the Exam Rewards / What It Punishes

Rewards: - Medical evaluation for sexual dysfunctions before initiating psychological treatment - Affirming stance with gender-diverse clients (not questioning the validity of their identity) - Assessment of distress source in gender dysphoria (is it the incongruence itself, or the social/family rejection around it?)

Punishes: - Attempting conversion practices or attempting to change gender identity (clinically and ethically harmful) - Diagnosing sexual dysfunction without ruling out medical and pharmacological causes - Pathologizing gender diversity (non-conforming gender expression without distress is not a disorder)

Practice Question

Question 55. A 17-year-old assigned female at birth presents with significant distress and says, “I have never felt like a girl. I feel like I’m in the wrong body, and my parents don’t believe me.” The counselor’s MOST appropriate initial response is:

- A. Recommend waiting until age 18 before discussing gender identity concerns
- B. Explore the client’s experience with an affirming stance, assess distress, and provide a safe space for self-expression
- C. Refer to a psychiatrist to assess for an underlying psychotic disorder before engaging with gender concerns
- D. Contact the parents to discuss the importance of their support in the client’s mental health treatment

Correct: B. An affirming, exploratory, compassionate response is the appropriate starting point. The counselor validates the client’s experience and assesses the distress. A is not a clinical or ethical basis for deferring care. C pathologizes gender dysphoria (it is not a psychotic disorder). D may eventually involve the parents, but the immediate response to the client’s disclosure is affirming engagement — not immediate parental contact.

Diagnosis & Assessment Selection

Diagnosis — rule in / rule out. A 55-year-old man reports new difficulty achieving erections over the past four months. He recently began a new antihypertensive medication and has poorly controlled diabetes. The most likely first consideration is:

- A. Erectile disorder due to psychological factors
- B. A medical or medication-related contributor requiring rule-out before a primary psychiatric diagnosis
- C. Male Hypoactive Sexual Desire Disorder
- D. Gender Dysphoria

Answer: B. New onset in the context of vascular risk (diabetes) and a new antihypertensive points to a medical or medication etiology, which is ruled out before diagnosing a primary sexual dysfunction. *What would change the answer:* a normal medical workup with a

clear situational or psychological pattern → a psychologically based dysfunction.

Assessment — what to do next. The most appropriate step is:

- A. Begin sensate-focus sex therapy immediately
- B. Refer for medical evaluation of the medication and vascular/endocrine contributors, then assess psychological factors
- C. Administer a personality inventory
- D. Reassure him that it is purely stress-related

Answer: B. Medical and pharmacologic causes are ruled out first, then psychological factors are assessed. Beginning therapy (A) or offering reassurance (D) before the rule-out is premature.

Treatment Decision Flow

These items reward ruling out medical causes first and providing affirmative, ethical care.

Clinical Scenario	Correct First Action	Wrong Answer (The Trap)
Sexual dysfunction	Rule out medical and medication causes first; then sex therapy (e.g., sensate focus) and psychoeducation	Beginning psychological treatment before the medical rule-out
Suspected medication-induced dysfunction (e.g., SSRI)	Coordinate a referral to review the medication	Assuming the cause is purely psychological
Gender dysphoria	Affirmative, informed-consent care; support exploration; refer per WPATH standards for medical steps	Conversion or “reparative” approaches; a purely gatekeeping stance

Sequencing & caveats. For sexual dysfunctions, medical and pharmacologic causes are ruled out before psychological treatment. For gender dysphoria, affirmative care is the standard.

Avoid / contraindicated. Conversion or reparative therapy is unethical and harmful and is never an appropriate option.

Scope. The counselor provides psychological support and refers for medical or hormonal care per accepted (WPATH) standards.

Chapter 23 — Ethical and Legal Decision Points

Overview

Ethics is not a separate chapter on the NCMHCE — it is woven through every simulation. This chapter synthesizes the ethical principles, legal obligations, and decision-making frameworks that the exam tests in context. The exam never asks you to recite an ethical code; it tests ethical reasoning under realistic clinical pressure.

The Five Core Ethical Principles (Beauchamp and Childress):

Principle	Meaning	Clinical Translation
Autonomy	The client’s right to self-determination	Informed consent, voluntary participation, respecting client decisions
Beneficence	Act in the client’s best interest	Choose interventions with the best evidence; prioritize client welfare
Nonmaleficence	Do no harm	Avoid harmful practices; don’t exploit; manage dual relationships carefully
Justice	Fair and equal treatment	Nondiscrimination; equitable access to services
Fidelity	Honoring commitments; being trustworthy	Confidentiality, follow through on agreements, honest communication

ACA Code of Ethics — Most-Tested Areas:

Confidentiality and its limits: - Confidentiality is the rule; exceptions are the exam’s focus - Exceptions: imminent danger to self or others (duty to warn/protect), abuse of a child/elder/dependent adult (mandated reporting), court order, client waives confidentiality, consultation (limited disclosure), group/family (special considerations)

Informed consent: Before treatment begins, clients must be informed of: nature and purpose of treatment, risks and benefits, alternatives, counselor’s credentials, fees, confidentiality and its lim-

its, and how to file a complaint. Informed consent is ongoing (not a one-time form).

Dual relationships: A dual relationship exists when the counselor has a professional role AND another role with the same client (social, familial, financial, romantic). The ACA prohibits relationships that harm the client's welfare or impair the counselor's objectivity. The exam tests the most common violations: sexual contact with clients (always prohibited, including after termination for a specified period), bartering when not in the client's interest, social media contact, self-disclosure that serves the counselor rather than the client.

Competence: Counselors practice only within their areas of competence. When a client presents with needs outside the counselor's competence, the obligation is to refer, consult, or seek additional training. The exam tests whether you recognize the limits of competence and act accordingly.

Record-keeping and privacy (HIPAA): Mental health records are protected health information. Release requires written authorization (with specific exceptions). Notes of a personal nature ("psychotherapy notes" under HIPAA) have heightened protection.

Termination ethics: Counselors may not abandon clients. Termination must be planned, with referrals provided and adequate notice given. Abandonment — stopping treatment without planning — is an ethical violation. The counselor may terminate when the client is no longer benefiting, the relationship is harmful, or the client poses a risk to the counselor — but must do so with appropriate referral and planning.

The Ethical Decision-Making Model

When an ethical dilemma arises on the NCMHCE, apply this sequence:

1. **Identify the dilemma:** What are the competing obligations?
2. **Consult the code:** What does the ACA Code say?
3. **Apply ethical principles:** Which principle is most relevant?
4. **Consider consequences:** What are the likely outcomes of each option?
5. **Consult:** With a supervisor or colleague (this is almost always the right answer when uncertain)

6. **Document:** The reasoning, the decision, and the outcome

What the Exam Rewards / What It Punishes

Rewards: - Consulting with a supervisor when uncertain about ethical decisions - Taking a conservative approach to confidentiality (default to protect) - Recognizing dual relationship risks and acting proactively to maintain boundaries - Reporting abuse based on reasonable suspicion, not certainty - Providing referrals when terminating

Punishes: - Acting on a duty-to-warn breach without first attempting to address the risk within the therapeutic relationship (where clinically possible) - Assuming HIPAA releases cover psychotherapy notes (they have separate protection) - Providing only a “no-harm contract” without a risk assessment as an ethical response to suicidality - Maintaining a relationship the counselor knows has become exploitative because “the client wants to continue”

Practice Questions

Question 56. A client discloses that she has been hitting her 6-year-old son “more than I should” because “I just lose it sometimes.” She asks the counselor to keep this confidential. The counselor should:

- A. Honor the request for confidentiality, as the disclosure was made in a therapeutic context
- B. Assess the severity of the physical discipline and report if it meets the threshold for abuse
- C. Immediately report the disclosure to child protective services based on reasonable suspicion
- D. Discuss the disclosure with a supervisor before deciding whether to report

Correct: C. Physical discipline that rises to “more than I should” and “losing it” constitutes reasonable suspicion of physical abuse. Mandated reporting does not require certainty, and it is not delayed by the request for confidentiality (confidentiality does not protect abuse). B implies the counselor determines the abuse threshold — the role is to report the suspicion, not to investigate. D may be a concurrent step but does not delay reporting.

Question 57. A counselor is offered baked goods by a client as a thank-you gift at the end of their last session. The MOST appropriate response is:

- A. Decline the gift, as all gifts create dual relationship concerns
- B. Accept the gift with gratitude, as this is the final session and the gesture is appropriate
- C. Consider the value of the gift, the cultural context, and the clinical significance before deciding how to respond
- D. Accept the gift only if the client consents in writing

Correct: C. The ACA Code does not prohibit all gifts — it requires considering the clinical and cultural context. A small gift at termination may have significant cultural meaning. A rigid refusal could harm the alliance and cultural relationship at a significant clinical moment. B accepts without any ethical reflection. D is not an actual ACA requirement.

Question 58. A counselor has been seeing a client for individual therapy for two years. The client's spouse contacts the counselor and requests a phone appointment "to discuss my husband's treatment." The counselor should:

- A. Schedule the call, as family involvement is clinically valuable
- B. Decline the contact without the client's written authorization for this specific communication
- C. Contact the client and let him decide whether the spouse may speak with the counselor
- D. Provide general information about the client's progress without disclosing specific session content

Correct: B. The counselor cannot speak with the spouse about the client's treatment without the client's explicit, written authorization. The client's name alone being known to the spouse does not constitute a waiver. C is better than A or D but is technically insufficient — authorization must be documented in writing before any contact.

Question 59. A client in distress calls after hours and says she needs to speak to the counselor immediately. The counselor has an established after-hours protocol that includes a crisis line number

and a clear policy that after-hours contact is for emergencies only. The client says, “This isn’t an emergency — I just feel really alone.” The counselor should:

- A. Speak with the client as long as necessary, as her distress is real
- B. Affirm the client’s feelings, remind her of the after-hours protocol and crisis resources, and confirm the next appointment
- C. Decline to speak and direct her immediately to the crisis line
- D. End the call and document the contact in the record

Correct: B. The counselor acknowledges the distress (affirming), maintains the frame (after-hours policy), provides safety resources (crisis line), and bridges to the next session. A dissolves the therapeutic frame. C is abrupt and clinically inadequate. D is incomplete (no acknowledgment of the client’s state).

Question 60. A client on the counselor’s caseload becomes a friend of the counselor’s spouse. They begin attending the same social events. The counselor should:

- A. Immediately terminate the therapeutic relationship and refer
- B. Discuss the situation with the client, assess the impact on the therapeutic relationship, consult with a supervisor, and document the process
- C. Continue treatment without disclosure, as social proximity does not constitute a dual relationship
- D. Request that the client not attend social events the counselor attends

Correct: B. The emergence of a social proximity dual relationship requires deliberate navigation: disclosure to the client, assessment of impact, consultation, and documentation. The outcome (continue vs. refer) depends on the clinical context and the client’s wishes. Immediate termination (A) is not automatically required. Ignoring it (C) is an ethical failure. D is inappropriate and intrusive of the client’s social autonomy.

End of Part Two. Continue in Part Three and Back Matter.

PART THREE: CLINICAL QUICK REFERENCE

Defense Mechanisms Quick Reference

Defense mechanisms are unconscious psychological processes that protect the ego from anxiety. The NCMHCE tests your ability to identify them in case vignettes — often as part of a psychodynamic conceptualization question.

Mature Defenses (*adaptive; common in healthy adults*)

Mechanism	Definition	Example on Exam
Sublimation	Channeling unacceptable impulses into socially acceptable outlets	A client with aggressive urges becomes a competitive athlete
Humor	Using comedy to manage uncomfortable feelings without denial	A client jokes about their diagnosis in a way that allows perspective-taking
Altruism	Relieving one’s own distress by helping others	A client who experienced childhood poverty becomes a social worker
Suppression	Consciously setting aside a distressing thought to deal with later	A client decides not to think about the diagnosis until after the presentation

How it’s tested: A vignette describes a client who “channels his anger into competitive sports” — this is sublimation. The exam may ask you to identify it or distinguish it from displacement.

Neurotic (Intermediate) Defenses (*less adaptive; common in neurotic range disorders*)

Mechanism	Definition	Example on Exam
Repression	Unconscious exclusion of distressing material from awareness	Client has no memory of traumatic childhood events
Reaction Formation	Expressing the opposite of an unacceptable feeling	A client who feels rage toward a parent becomes excessively devoted
Displacement	Redirecting an impulse from the true target to a safer one	Client yells at her children after a bad day with her boss
Rationalization	Creating logical justifications for unacceptable feelings or behaviors	Client who overspends: "I work hard; I deserve nice things"
Intellectualization	Using abstract thinking to distance from emotional content	A client discusses grief using clinical terminology without showing affect
Undoing	Performing an action to "undo" a transgression	Client feels guilty about a thought and then completes a ritual to cancel it
Isolation of Affect	Separating thoughts from their associated emotions	Client recounts a traumatic event in complete detail with no emotional response

How it's tested: Intellectualization frequently appears in cases where a client describes a loss using clinical or distant language. Reaction formation appears in cases where the stated feeling and the behavioral evidence are directly opposite.

Immature Defenses (*maladaptive; common in personality disorders, severe pathology*)

Mechanism	Definition	Example on Exam
Denial	Refusing to acknowledge a painful reality	Client with a serious medical diagnosis says "I don't believe there's really anything wrong"
Projection	Attributing one's own unacceptable feelings to others	Client says "everyone here is judging me" when she is the one judging herself

Mechanism	Definition	Example on Exam
Splitting	Seeing people as all-good or all-bad; inability to tolerate ambivalence	BPD client idealizes the counselor one week, devalues the next
Regression	Returning to an earlier developmental stage under stress	An adult client begins speaking in a childlike voice when discussing abuse
Somatization	Converting psychological distress into physical symptoms	Client develops unexplained stomach pain before difficult family events
Projective Identification	Causing others to actually feel what you are projecting	BPD client subtly behaves in ways that cause the counselor to feel rage — then attributes the rage to the counselor
Passive Aggression	Expressing hostility indirectly	Client “forgets” appointments consistently when angry at the counselor
Acting Out	Expressing internal conflict through action rather than words	Client gets into a bar fight after a session where trauma was discussed

How it's tested: Splitting is almost always tested in the context of BPD. Projection and projective identification may appear together in personality disorder questions. Acting out appears in cases where the client's behavior outside sessions reflects internal session material.

Cognitive Distortions Quick Reference

Cognitive distortions are errors in thinking identified in CBT (Beck) that maintain depression, anxiety, and other conditions. The exam tests your ability to identify them in client statements.

Distortion	Definition	Client Statement Example
All-or-nothing thinking (black-and-white)	Seeing things in extreme categories	"If I'm not perfect at this, I'm a complete failure."
Catastrophizing	Predicting the worst possible outcome	"If I fail this test, my life is over."
Mind reading	Assuming knowledge of others' thoughts	"I know she thinks I'm stupid."
Fortune telling	Predicting a negative future as fact	"I'll never be able to manage on my own."
Emotional reasoning	Assuming feelings reflect reality	"I feel worthless, so I must be worthless."
Personalization	Blaming oneself for events beyond control	"My parents divorced because of me."
Overgeneralization	Drawing broad conclusions from single events	"I made one mistake — I always mess everything up."
Mental filter	Focusing exclusively on one negative detail	The client hears universally positive feedback and fixates on one mild critique
Discounting the positive	Dismissing positive experiences as not counting	"The promotion doesn't count — they just felt sorry for me."
Should statements	Rigid rules about self/others	"I should be able to handle this without help."
Labeling	Attaching a global negative label to self or others	"I'm just a loser." "She's completely toxic."
Magnification/Minimization	Amplifying negatives; shrinking positives	"My one strength doesn't matter. My one failure defines me."

Restructuring Context

The goal of cognitive restructuring is not to replace negative thoughts with positive ones — it is to replace distorted thoughts with more accurate, evidence-based ones. The Socratic questioning sequence:

1. **What is the evidence for and against this thought?**
2. **What would you tell a friend who had this thought?**
3. **Is this thought helping you or hurting you?**
4. **What is a more accurate way to describe this situation?**

Exam tip: The exam will present a client statement and ask you to identify the distortion, or present a counselor response and ask whether it correctly targets the distortion. Know the distortions by both definition and example.

Assessment Tools Compendium

All instruments organized by domain for cross-reference with clinical chapters.

Depression

Tool	Type	Items	Cutoffs / Notes
PHQ-9	Self-report	9	0–4 minimal; 5–9 mild; 10–14 moderate; 15–19 mod-severe; 20+ severe
BDI-II	Self-report	21	0–13 minimal; 14–19 mild; 20–28 moderate; 29+ severe
HAM-D (HDRS)	Clinician-rated	17 or 21	<7: normal; 7–17: mild; 18–24: moderate; >24: severe
Edinburgh Postnatal Depression Scale	Self-report	10	Perinatal depression screening; score ≥13 requires follow-up
MADRS	Clinician-rated	10	Montgomery-Åsberg; widely used in medication trials

Anxiety

Tool	Type	Items	Cutoffs / Notes
GAD-7	Self-report	7	5 mild; 10 moderate; 15 severe
BAI	Self-report	21	0–21 low; 22–35 moderate; 36+ potentially concerning level
PDSS	Clinician-rated	7	Panic disorder severity; higher score = greater severity
SUDS	Client-rated	Single item	0–100 scale; used within exposure sessions
Social Phobia Inventory (SPIN)	Self-report	17	≥19 suggests social anxiety disorder

Trauma

Tool	Type	Items	Cutoffs / Notes
PCL-5	Self-report	20	Provisional PTSD: ≥ 31 –33; maps to DSM-5 criteria
CAPS-5	Clinician-administered	Structured	Gold standard for PTSD diagnosis and severity
TSI-2	Self-report	136	Comprehensive trauma-related symptoms; complex trauma
IES-R	Self-report	22	Impact of Event Scale — Revised; intrusion, avoidance, hyperarousal subscales
ACE Score	Self-report	10	Adverse childhood experiences; not diagnostic — cumulative risk indicator

Substance Use

Tool	Type	Items	Cutoffs / Notes
AUDIT	Self-report	10	8–15: brief advice; 16–19: counseling; ≥ 20 : referral
AUDIT-C	Self-report	3	≥ 4 men; ≥ 3 women: likely AUD
CAGE	Clinical interview	4	≥ 2 positive: likely AUD
DAST-10	Self-report	10	≥ 3 : need for assessment; ≥ 6 : substantial level of problems
ASI	Structured interview	Multi-domain	Addiction Severity Index; 7 domains for treatment planning
MAST	Self-report	25	Michigan Alcoholism Screening Test; > 5 : alcoholism probable

Suicidality

Tool	Type	Key Features
C-SSRS	Clinician-administered	Ideation (5 levels) + behavior; gold standard for risk research
Columbia Protocol	Clinical decision aid	Guides action based on C-SSRS responses
SBQ-R	Self-report	4

Personality and Comprehensive Assessment

Tool	Type	Notes
MMPI-2 / MMPI-3	Self-report	567/335 items; comprehensive personality and psychopathology; requires licensed psychologist for interpretation
MCMI-IV	Self-report	195 items; Millon Clinical Multiaxial Inventory; personality disorders + clinical syndromes
MSI-BPD	Self-report	10 items; BPD screening; ≥ 7 : warrants further evaluation
PAI	Self-report	344 items; Personality Assessment Inventory

Cognitive and Neurodevelopmental

Tool	Type	Notes
MMSE	Clinician-administered	Mini-Mental State Exam; brief cognitive screen; 0-30; ≤ 23 : cognitive impairment
MoCA	Clinician-administered	Montreal Cognitive Assessment; more sensitive than MMSE; ≤ 25 : possible impairment
WAIS-IV / WISC-V	Clinician-administered	Full intelligence assessment; requires licensed psychologist
Conners-3	Rating scales	Parent, teacher, self; ADHD assessment across settings
ADOS-2	Clinician-administered	Gold standard ASD observational assessment
CARS-2	Rating scale	Childhood Autism Rating Scale; clinical observation-based

Eating Disorders

Tool	Type	Notes
EDE-Q	Self-report	Eating Disorder Examination Questionnaire; 4 subscales; higher = more severe
EAT-26	Self-report	26 items; ≥ 20 : warrants clinical follow-up
SCOFF	Self-report	5 items; rapid screen; ≥ 2 positive: referral indicated

Child and Adolescent Specific

Tool	Type	Notes
CBCL	Parent/teacher rating	Child Behavior Checklist; broad symptom assessment
CDI-2	Self-report (ages 7–17)	Children’s Depression Inventory
RCADS	Self-report	Revised Children’s Anxiety and Depression Scale
SCARED	Self-report	Screen for Child Anxiety Related Disorders; parent and child forms
CPSS-5	Self-report	Child PTSD Symptom Scale — DSM-5 version

BACK MATTER

Complete Answer Key with Expanded Rationales

Questions 1–20

Q1: B — Client signals rupture through disengagement; first response is to explore the rupture, not defend treatment or assess symptoms. Moving to the relationship is the priority over any technique.

Q2: B — Cultural goals rupture; autonomy-focused treatment conflicts with collectivist values. Disengagement is communication about misaligned goals.

Q3: B — UPR validates experience without evaluating the decision. A compares to norms (evaluative). C seeks a “better option” (evaluative). D names concern (conditional).

Q4: C — Pre-termination regression is expected; naming the pattern is the first clinical response. Extension and consultation are possible later steps, not first responses.

Q5: B — Scaling questions with “what would make it a 7” is signature SFBT. MI uses scaling but not this specific extension pattern.

Q6: B — Enactment (structural family therapy): therapist observes family members interacting in session.

Q7: C — Externalization (“the depression has been trying to convince you”) is signature narrative therapy.

Q8: C — REBT targets the irrational belief (B), not the activating event (A) or the consequences (C).

Q9: C — Immediacy response connects in-the-room behavior (apology) to an interpersonal pattern.

Q10: C — Empathic reflection holds ambivalence; alliance is still forming, so influencing skills (B, D) are premature.

Q11: C — Confrontation of an incongruity: today’s nonverbal presentation vs. the usual presentation.

Q12: B — Brief, genuine, client-focused disclosure. A overclaims understanding. C breaches confidentiality. D is counselor-centered.

Q13: C — Storming stage conflict is therapeutic material; name and facilitate, don't deflect.

Q14: B — Brief empathic reflection + redirect to the group; balances warmth and responsibility to the group.

Q15: C — Instillation of hope: observing another member's recovery provides evidence that change is possible.

Q16: B — Accurately distinguishes counselor's guaranteed confidentiality from members' expected (not legally guaranteed) confidentiality.

Q17: C — Score of 9 (mild, not remitted); anhedonia and loss of meaning warrant behavioral activation + meaning-focused additions.

Q18: B — Somatic symptom cluster (fatigue, cognitive slowing, hair loss, weight gain) requires thyroid/medical rule-out before any psychiatric diagnosis.

Q19: C — Prior period of elevated mood, decreased sleep, and impulsive spending = possible hypomanic/manic episode → screen for bipolar.

Q20: C — Grief-triggered MDD with passive SI requires MDD diagnosis and treatment, including safety assessment.

Questions 21–40

Q21: B — Relaxation addresses somatic symptoms; CBT for GAD also requires cognitive work on worry beliefs, intolerance of uncertainty, and avoidance.

Q22: D — Persistence + significant distress + functional impairment = phobia diagnosis. Disproportionate fear and avoidance alone are insufficient.

Q23: B — Blood-injection-injury phobia produces vasovagal response (not sympathetic arousal); applied tension is the evidence-based adaptation.

Q24: C — Three years of pervasive, uncontrollable multi-domain worry with insomnia and hyperarousal = GAD.

Q25: B — Stabilization precedes trauma processing; integrated treatment (not sequential) for co-occurring PTSD and SUD.

Q26: B — Criterion A (specific traumatic stressor) + specific symptom clusters (re-experiencing) = PTSD. Adjustment disorder has no Criterion A requirement.

Q27: B — Dissociation during processing = insufficient stabilization; return to Phase 1 before continuing Phase 2.

Q28: B — Indiscriminate approach to strangers following early caregiving disruption = DSED. RAD presents with inhibited, withdrawn attachment.

Q29: B — MI explores ambivalence from the client's own perspective; doesn't impose the counselor's agenda.

Q30: B — Open question evokes change talk; MI amplifies client's own motivation.

Q31: B or C — Moderate AUD, stable, motivated: IOP (2.1) or outpatient (1). Residential is not indicated.

Q32: B — Delirium tremens (visual hallucinations, disorientation, autonomic instability, 48-72 hours after last drink) = medical emergency.

Q33: B — Follow the pre-established crisis plan; reinforcing between-session contact with immediate callback would reinforce crisis behavior.

Q34: B — DBT goal: skills for emotion regulation, distress tolerance, interpersonal effectiveness, mindfulness → "life worth living."

Q35: B — Explore the meaning of the experience without defensiveness; the clinical content is the client's felt experience of the slight.

Q36: C — Avoidant PD: wants connection, fears rejection. Schizoid PD: genuinely indifferent to connection.

Q37: B — Must screen for prior elevated mood episodes before diagnosing MDD; missing bipolar history = wrong diagnosis + potentially harmful treatment.

Q38: B — Explore concerns + psychoeducation about relapse risk; collaborative conversation with prescriber.

Q39: B — Psychoeducation + comprehensive assessment (comorbidities) + multimodal planning is the first step for adult ADHD.

Q40: B — Stereotyped motor behavior + social communication deficit + echolalia = core ASD features.

Questions 41–60

Q41: B — Acute-onset confusion in a previously oriented elder = delirium until proven otherwise = medical emergency.

Q42: B — ERP breaks the obsession-compulsion link by eliminating the ritual response, allowing anxiety to naturally reduce.

Q43: B — Excoriation disorder = Habit Reversal Training (HRT), not ERP.

Q44: A — Explore the medication concern non-confrontationally + coordinate with prescriber. The counselor does not argue with the delusional content.

Q45: B — Medical safety check (electrolytes) warranted with acute escalation in purging; then address the stressor in CBT.

Q46: C — BMI 14.5, HR 44, syncope = medical instability requiring hospitalization before outpatient therapy.

Q47: B — Specific plan + purchased means + lives alone + “soon” = high risk → full assessment + emergency psychiatric evaluation.

Q48: C — Reasonable suspicion = immediate mandatory report; counselor does not investigate; confidentiality does not protect abuse.

Q49: B — Prior suicide attempt is the single strongest predictor of future attempt.

Q50: B — Every suicidal expression in BPD requires current risk assessment; chronic suicidality does not eliminate the obligation.

Q51: C — Aggression + fire-setting + theft over 12+ months = Conduct Disorder (serious violation of others’ rights).

Q52: C — Affirm medical clearances, acknowledge distress, begin CBT for health anxiety behaviors; additional specialist referral reinforces reassurance-seeking.

Q53: B — Phase 1 (stabilization) criteria must be assessed and met before Phase 2 (EMDR processing) begins in DID.

Q54: B — CBT-I (sleep restriction + stimulus control) with supported medication taper is the evidence-based approach.

Q55: B — Affirming, exploratory response is the appropriate first response to gender dysphoria disclosure; the counselor validates and assesses.

Q56: C — Reasonable suspicion of physical child abuse = immediate mandatory report. Client's request for confidentiality does not override reporting obligation.

Q57: C — Gifts: consider clinical significance, cultural context, and value; small termination gifts may be appropriate.

Q58: B — No communication with third parties without the client's written, specific authorization.

Q59: B — Affirm distress + maintain after-hours frame + provide crisis resources + bridge to next appointment.

Q60: B — Disclose to client + assess impact + consult + document; immediate termination not automatically required.

Alphabetical Diagnosis Index

(ICD-10 codes and diagnostic family for each of the 92 diagnoses in the reference section.)

Diagnosis	ICD-10	Family (Chapter)
Acute Stress Disorder	F43.0	Trauma & Stressor-Related (Ch 8)
Agoraphobia	F40.00	Anxiety (Ch 7)
Alcohol Use Disorder — mild	F10.10	Substance & Addictive (Ch 9)
Alcohol Use Disorder — moderate/severe	F10.20	Substance & Addictive (Ch 9)
Anorexia Nervosa — Binge/Purge	F50.02	Feeding & Eating (Ch 16)
Anorexia Nervosa — Restricting	F50.01	Feeding & Eating (Ch 16)
Antisocial Personality Disorder	F60.2	Personality (Ch 10)
Autism Spectrum Disorder	F84.0	Neurodevelopmental (Ch 12)
Avoidant Personality Disorder	F60.6	Personality (Ch 10)
Avoidant/Restrictive Food Intake Disorder	F50.82	Feeding & Eating (Ch 16)
Binge Eating Disorder	F50.81	Feeding & Eating (Ch 16)
Bipolar I Disorder	F31.x	Bipolar & Related (Ch 11)
Bipolar II Disorder	F31.81	Bipolar & Related (Ch 11)
Body Dysmorphic Disorder	F45.22	OCD & Related (Ch 14)
Borderline Personality Disorder	F60.3	Personality (Ch 10)
Brief Psychotic Disorder	F23	Schizophrenia Spectrum (Ch 15)
Bulimia Nervosa	F50.2	Feeding & Eating (Ch 16)
Cannabis Use Disorder	F12.10	Substance & Addictive (Ch 9)
Conduct Disorder	F91.x	Disruptive/Impulse-Control (Ch 18)
Conversion Disorder	F44.x	Somatic Symptom (Ch 19)
Cyclothymic Disorder	F34.0	Bipolar & Related (Ch 11)
Delayed Ejaculation	F52.32	Sexual Dysfunctions (Ch 22)
Delirium	F05	Neurocognitive (Ch 13)
Delusional Disorder	F22	Schizophrenia Spectrum (Ch 15)
Dependent Personality Disorder	F60.7	Personality (Ch 10)
Depersonalization/Derealization Disorder	F40.1	Dissociative (Ch 20)
Developmental Coordination Disorder	F82	Neurodevelopmental (Ch 12)

Diagnosis	ICD-10	Family (Chapter)
Disinhibited Social Engagement Disorder	F94.2	Trauma & Stressor-Related (Ch 8)
Dissociative Amnesia	F44.0	Dissociative (Ch 20)
Dissociative Identity Disorder	F44.81	Dissociative (Ch 20)
Disruptive Mood Dysregulation Disorder	F34.8	Depressive (Ch 6)
Erectile Disorder	F52.21	Sexual Dysfunctions (Ch 22)
Excoriation (Skin-Picking) Disorder	L98.1	OCD & Related (Ch 14)
Factitious Disorder	F68.1x	Somatic Symptom (Ch 19)
Female Orgasmic Disorder	F52.31	Sexual Dysfunctions (Ch 22)
Female Sexual Interest/Arousal Disorder	F52.22	Sexual Dysfunctions (Ch 22)
Gambling Disorder	F63.0	Substance & Addictive (Ch 9)
Gender Dysphoria	F64.0	Sexual Dysfunctions & Gender (Ch 22)
Generalized Anxiety Disorder	F41.1	Anxiety (Ch 7)
Genito-Pelvic Pain/Penetration Disorder	F52.6	Sexual Dysfunctions (Ch 22)
Histrionic Personality Disorder	F60.4	Personality (Ch 10)
Hoarding Disorder	F42.3	OCD & Related (Ch 14)
Hypersomnolence Disorder	G47.10	Sleep-Wake (Ch 21)
Illness Anxiety Disorder	F45.21	Somatic Symptom (Ch 19)
Insomnia Disorder	G47.00	Sleep-Wake (Ch 21)
Intellectual Disability — mild	F70	Neurodevelopmental (Ch 12)
Intellectual Disability — moderate	F71	Neurodevelopmental (Ch 12)
Intellectual Disability — severe	F72	Neurodevelopmental (Ch 12)
Intermittent Explosive Disorder	F63.81	Disruptive/Impulse-Control (Ch 18)
Kleptomania	F63.2	Disruptive/Impulse-Control (Ch 18)
Major Depressive Disorder — single episode	F32.x	Depressive (Ch 6)
Major Depressive Disorder — recurrent	F33.x	Depressive (Ch 6)
Major Neurocognitive Disorder	F02.xx	Neurocognitive (Ch 13)
Male Hypoactive Sexual Desire Disorder	F52.0	Sexual Dysfunctions (Ch 22)
Mild Neurocognitive Disorder	G31.84	Neurocognitive (Ch 13)

Diagnosis	ICD-10	Family (Chapter)
Narcissistic Personality Disorder	F60.81	Personality (Ch 10)
Narcolepsy	G47.419	Sleep-Wake (Ch 21)
Nightmare Disorder	F51.5	Sleep-Wake (Ch 21)
Obsessive-Compulsive Disorder	F42.2	OCD & Related (Ch 14)
Obsessive-Compulsive Personality Disorder	F60.5	Personality (Ch 10)
Obstructive Sleep Apnea	G47.33	Sleep-Wake (Ch 21)
Opioid Use Disorder	F11.10	Substance & Addictive (Ch 9)
Oppositional Defiant Disorder	F91.3	Disruptive/Impulse-Control (Ch 18)
Panic Disorder	F41.0	Anxiety (Ch 7)
Paranoid Personality Disorder	F60.0	Personality (Ch 10)
Persistent Depressive Disorder	F34.1	Depressive (Ch 6)
Pica	F98.3	Feeding & Eating (Ch 16)
Premature Ejaculation	F52.4	Sexual Dysfunctions (Ch 22)
Premenstrual Dysphoric Disorder	F32.81	Depressive (Ch 6)
Pyromania	F63.1	Disruptive/Impulse-Control (Ch 18)
Reactive Attachment Disorder	F94.1	Trauma & Stressor-Related (Ch 8)
REM Sleep Behavior Disorder	G47.52	Sleep-Wake (Ch 21)
Rumination Disorder	F98.21	Feeding & Eating (Ch 16)
Schizoaffective Disorder	F25.x	Schizophrenia Spectrum (Ch 15)
Schizoid Personality Disorder	F60.1	Personality (Ch 10)
Schizophrenia	F20.9	Schizophrenia Spectrum (Ch 15)
Schizophreniform Disorder	F20.81	Schizophrenia Spectrum (Ch 15)
Schizotypal Personality Disorder	F21	Personality (Ch 10)
Sedative/Hypnotic/Anxiolytic Use Disorder	F13.10	Substance & Addictive (Ch 9)
Selective Mutism	F94.0	Anxiety (Ch 7)
Separation Anxiety Disorder	F93.0	Anxiety (Ch 7)
Social Anxiety Disorder	F40.10	Anxiety (Ch 7)
Somatic Symptom Disorder	F45.1	Somatic Symptom (Ch 19)
Specific Learning Disorder — reading	F81.0	Neurodevelopmental (Ch 12)
Specific Learning Disorder — written expression	F81.81	Neurodevelopmental (Ch 12)

Diagnosis	ICD-10	Family (Chapter)
Specific Learning Disorder — mathematics	F81.2	Neurodevelopmental (Ch 12)
Specific Phobia	F40.2xx	Anxiety (Ch 7)
Stimulant Use Disorder	F15.10	Substance & Addictive (Ch 9)
Tourette's Disorder	F95.2	Neurodevelopmental (Ch 12)
Trichotillomania	F63.3	OCD & Related (Ch 14)

Select References (APA 7th Edition)

American Counseling Association. (2014). *ACA code of ethics*. Author.

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders* (5th ed., text rev.). <https://doi.org/10.1176/appi.books.9780890425787>

Beauchamp, T. L., & Childress, J. F. (2019). *Principles of biomedical ethics* (8th ed.). Oxford University Press.

Beck, J. S. (2021). *Cognitive behavior therapy: Basics and beyond* (3rd ed.). Guilford Press.

Bordin, E. S. (1979). The generalizability of the psychoanalytic concept of the working alliance. *Psychotherapy: Theory, Research & Practice*, 16(3), 252–260. <https://doi.org/10.1037/h0085885>

Corey, G. (2021). *Theory and practice of counseling and psychotherapy* (10th ed.). Cengage Learning.

Ivey, A. E., Ivey, M. B., & Zalaquett, C. P. (2018). *Intentional interviewing and counseling: Facilitating client development in a multi-cultural society* (9th ed.). Cengage Learning.

Linehan, M. M. (2015). *DBT skills training manual* (2nd ed.). Guilford Press.

Miller, W. R., & Rollnick, S. (2013). *Motivational interviewing: Helping people change* (3rd ed.). Guilford Press.

Norcross, J. C., & Wampold, B. E. (2019). Evidence-based psychotherapy responsiveness: The third task force report. *Journal of Clinical Psychology*, 75(11), 1899–1906. <https://doi.org/10.1002/jclp.22843>

Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. *Journal of Consulting Psychology*, 21(2), 95–103. <https://doi.org/10.1037/h0045357>

Tuckman, B. W. (1965). Developmental sequence in small groups. *Psychological Bulletin*, 63(6), 384–399. <https://doi.org/10.1037/h0022100>

Wampold, B. E., & Imel, Z. E. (2015). *The great psychotherapy debate: The evidence for what makes psychotherapy work* (2nd ed.). Routledge.

Yalom, I. D., & Leszcz, M. (2020). *The theory and practice of group psychotherapy* (6th ed.). Basic Books.

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Part Three — Practice Exams

Three full-length NCMHCE-format practice exams. Each presents eleven clinical cases with scored questions that mirror the format, difficulty distribution, and decision-making demands of the actual examination. Work each exam in full before consulting the answer key.

Complete all questions in an exam before turning to the answer key at its end. The keys identify the best answer, explain why each distractor is wrong, and name the clinical error each wrong option represents — the same analysis the NCMHCE rewards.

Practice Exam One

11 clinical cases · 143 scored questions · 255 minutes recommended

Work through all cases before consulting the answer key at the end of this exam.

Case 1 of 11

Low mood and lost interest after a demotion

Diagnosis: Major Depressive Disorder, Moderate

INTAKE

Daniel Okafor, a 34-year-old warehouse supervisor, presents reporting six weeks of low mood, loss of interest in coaching his daughter's soccer team, poor sleep, and difficulty concentrating after a demotion.

SESSION TWO

He describes waking at 4 a.m. most days, eating little, and feeling worthless and slowed down. He denies any past period of elevated, expansive mood or decreased need for sleep with increased energy.

SESSION THREE

He reports passive thoughts that his family would be better off without him but no plan, intent, or prior attempts, and he is frightened by these thoughts and willing to work on them collaboratively.

Question 1. What should the counselor confirm first to anchor the major depressive episode diagnosis at intake?

- A. That symptoms have persisted nearly every day for at least two weeks and impair his daily functioning
- B. That he can identify the single external stressor he believes is responsible for everything he is feeling
- C. That his family history includes at least one first-degree relative who was treated for a depressive disorder
- D. That his current depressive symptoms are clearly more severe today than they were at the very first onset

Question 2. Which finding best distinguishes his presentation as major depression rather than an adjustment disorder?

- A. The presence of pervasive anhedonia with neurovegetative signs that meet the full symptom count and threshold
- B. The fact that his mood problems began only after an identifiable and clearly upsetting workplace stressor occurred
- C. The observation that his distress seems somewhat out of proportion to the actual severity of the demotion event
- D. The likelihood that his symptoms will fully resolve on their own within roughly six months of the stressor ending

Question 3. Before settling on a unipolar depression diagnosis, what must the counselor specifically rule out?

- A. Any past episode of elevated or irritable mood with decreased need for sleep, marking a hypomanic or manic history
- B. Whether he has ever experienced a panic attack with racing heart and a fear of losing control in public places
- C. Whether his concentration problems are better explained by a long-standing attention-deficit pattern since childhood
- D. Whether his beliefs about being worthless have reached a fixed, unshakeable, clearly delusional level of conviction

Question 4. Given his passive thoughts of being better off dead, what is the most appropriate assessment step?

- A. Use a structured suicide-risk screen to clarify ideation severity, any intent, plan, and prior behavior
- B. Reassure him that thoughts like these are extremely common in depression and will almost certainly fade with time
- C. Ask him to promise in writing that he will not act on any of these thoughts before the next scheduled appointment
- D. Defer any discussion of the thoughts until rapport is stronger so that he does not feel judged or pathologized today

Question 5. What additional history is most important to gather to avoid misattributing the depressive picture?

- A. A review of his medications, substance use, and any thyroid or other medical conditions that mimic depression
- B. A detailed genogram tracing relationship patterns and emotional cutoffs across at least three prior family generations
- C. A full account of every job he has held so the counselor can understand his complete occupational and earnings history
- D. A list of the specific coping strategies that have failed him so the counselor can rule each of those approaches out

Question 6. What is the most appropriate first-line, within-scope treatment recommendation?

- A. Offer evidence-based psychotherapy such as cognitive behavioral therapy or behavioral activation for depression
- B. Recommend that he begin a daily antidepressant immediately and titrate the dose upward over the next several weeks
- C. Suggest he wait and monitor his mood on his own for a couple of months before committing to any formal treatment
- D. Encourage him to focus mainly on vigorous daily exercise and a structured sleep routine in place of formal therapy

Question 7. How should the counselor track whether the treatment is actually working over time?

- A. Re-administer a validated symptom measure such as the PHQ-9 at regular intervals to guide measurement-based care
- B. Ask him at each session whether he subjectively feels that things are generally going better than they were before
- C. Wait until the agreed course of therapy has fully ended and then evaluate his overall response in a single review
- D. Track only whether he is consistently attending his scheduled sessions and completing the assigned homework each week

Question 8. Daniel asks about medication. What is the most appropriate counselor action?

- A. Provide general information and coordinate a referral to a prescriber for a medication evaluation with his consent
- B. Tell him which specific antidepressant tends to work best for people who present with his particular symptom pattern
- C. Advise him that medication is generally unnecessary as long as he stays fully committed to attending his therapy
- D. Suggest that he research antidepressant options online and bring his own preferred choice to a future appointment

Question 9. Which initial behavioral activation step best fits Daniel's presentation?

- A. Collaboratively schedule small, valued activities such as briefly attending his daughter's practice to rebuild reward
- B. Direct him to resume the full coaching role and every other prior responsibility right away to prove he can still cope
- C. Have him keep a detailed daily log of every negative automatic thought before introducing any change to his routine
- D. Encourage him to wait to re-engage in any activities until he genuinely feels his interest and energy have returned

Question 10. How can the counselor best address Daniel's belief that his depression makes him a failure?

- A. Offer psychoeducation framing depression as a common, treatable condition while validating how heavy it currently feels
- B. Firmly challenge the belief as irrational and present him with logical evidence that he is objectively not a failure
- C. Agree that the demotion was a genuine setback and focus the work on helping him accept that he underperformed at work
- D. Redirect him away from the topic entirely and refocus the conversation on practical goal-setting for the coming week

Question 11. Daniel says he doubts anything will help. What is the most therapeutic response?

- A. Acknowledge the hopelessness as a symptom and collaboratively set one small, achievable goal to test it against
- B. Promise him with confidence that therapy is guaranteed to work as long as he fully commits to attending every session
- C. Accept his prediction that treatment is unlikely to help and shift the focus toward simply managing his daily symptoms
- D. Explain in detail the neurobiology of depression to convince him on scientific grounds that recovery is fully possible

Question 12. When reviewing informed consent, what must the counselor be sure Daniel understands?

- A. The limits of confidentiality, including when imminent risk of serious harm to self or others must be acted upon
- B. That everything he chooses to share in their sessions will be held in complete confidence under all circumstances
- C. That the counselor will personally decide which specific treatment goals are appropriate for him to pursue in therapy
- D. That he is required to complete the entire recommended course of therapy once he has agreed to begin the work today

Question 13. Daniel declines a medication referral but wants to continue therapy. What is the appropriate response?

- A. Respect his autonomous decision, document it, and continue evidence-based therapy while keeping the option open
- B. Tell him you cannot ethically continue to work with him at all unless he first agrees to be evaluated for medication
- C. Repeatedly revisit the medication question in each upcoming session until he eventually changes his mind and agrees
- D. Document that he refused recommended care and quietly note that he is now solely responsible for any poor outcome

*Case 2 of 11***Persistent Worry Across Many Areas of Life***Diagnosis: Generalized Anxiety Disorder*

INTAKE

Hassan, a 41-year-old civil engineer, reports about eight months of constant worry he cannot switch off. He worries about work deadlines, his children, finances, and his health, and describes feeling keyed up, tired, and tense, with poor sleep and trouble concentrating at the office.

SESSION TWO

Hassan says the worry jumps from one topic to the next and that he cannot control it once it starts. He reports muscle tension, irritability, and lying awake replaying conversations. He denies discrete panic attacks and denies that the worry is about having a specific undiagnosed illness.

SESSION THREE

Hassan frequently asks for reassurance, double-checks his work, and avoids opening bills until the last moment. He says caffeine helps him push through fatigue, and he drinks several cups daily. He is motivated to feel calmer but doubts that anything will quiet his mind.

Question 1. What is the most appropriate way to begin assessing Hassan's presentation?

- A. Administer the GAD-7 and clarify the breadth, duration, and controllability of his worry
- B. Begin teaching relaxation breathing in the first minutes before gathering any history
- C. Refer him to a physician for medication first and defer the clinical interview
- D. Reassure him that everyone worries and that his symptoms are nothing to address

Question 2. Which question best establishes the duration criterion for this disorder?

- A. Whether the excessive worry has been present more days than not for six months or longer
- B. Whether his worry became noticeably worse during the past one or two weeks
- C. Whether he can identify the single life event that first triggered his worry
- D. Whether his family members have also struggled with similar anxious feelings

Question 3. Hassan drinks several cups of coffee daily. How should the counselor address possible medical contributors?

- A. Ask about caffeine, medications, and medical conditions, and refer for evaluation if indicated
- B. Order thyroid and metabolic laboratory tests directly to rule out a medical cause
- C. Assume caffeine is the entire cause and advise him to simply stop drinking coffee
- D. Disregard physical factors entirely because his worry is clearly psychological

Question 4. What best distinguishes Hassan's presentation from Panic Disorder?

- A. His anxiety is persistent worry across many domains rather than recurrent unexpected panic attacks
- B. The fact that he experiences ongoing muscle tension, restlessness, and difficulty falling asleep at night
- C. The presence of any worry at all about his finances and his job security
- D. That his symptoms tend to feel worse on busy and demanding days at his work

Question 5. How does Hassan's presentation differ from Illness Anxiety Disorder?

- A. His worry is spread diffusely across many areas rather than fixed on having a serious disease
- B. The fact that he sometimes worries about his physical health among other topics
- C. The observation that he feels tired and run down by the end of the workday
- D. The way his worry seems to ease somewhat when he is busy and distracted

Question 6. What is the most appropriate first-line psychotherapy for Hassan's moderate GAD?

- A. Cognitive behavioral therapy with cognitive restructuring and worry exposure as core components
- B. Open-ended supportive listening with no structured techniques or between-session tasks
- C. Long-term exploration of early childhood as the primary treatment for his worry
- D. Immediate referral for medication as the only intervention he should receive

Question 7. Which target is most central to treating Hassan's worry?

- A. Building his tolerance of uncertainty instead of chasing certainty through checking and reassurance
- B. Helping him gather more information and double-check each concern so he can finally feel certain about every worry
- C. Encouraging him to avoid the situations that tend to set off his worry
- D. Focusing only on relaxing his body without addressing his worry process

Question 8. How should the counselor address Hassan's frequent reassurance-seeking and avoidance?

- A. Use worry exposure while gradually reducing reassurance-seeking and the avoidance that maintains it
- B. Provide the reassurance he requests each and every time he asks so that he feels fully supported in session
- C. Allow him to keep avoiding stressful tasks until he feels completely ready
- D. Tell him firmly to stop worrying and to simply think more positively each day

Question 9. How should measurement-based care be built into Hassan's plan?

- A. Re-administer the GAD-7 at intervals to track response and step up care if he does not improve
- B. Judge progress only from his general sense of how the week felt to him
- C. Wait until the final session to find out whether the treatment helped him
- D. Have him track only his daily coffee intake and use that single number as the main indicator of his anxiety

Question 10. In session, Hassan repeatedly asks, "But what if something goes wrong?" seeking reassurance. What is the best response?

- A. Name the reassurance pattern with warmth and help him practice sitting with the uncertainty
- B. Provide a confident answer each time so that he leaves the session feeling calm
- C. Tell him his worries are irrational and that he should already know better
- D. Change the subject quickly so that the worry does not take over the session

Question 11. Hassan says he doubts that anything will quiet his mind. Which response best strengthens engagement?

- A. Explore what he has already tried, affirm his strengths, and evoke his own reasons to keep going
- B. Insist that the treatment will definitely work as long as he follows every instruction
- C. Warn him that without therapy his anxiety will only continue to get worse
- D. Reassure him broadly that many people feel this way and that it will pass

Question 12. Hassan's employer calls and asks the counselor to confirm he is in treatment. How should the counselor respond?

- A. Decline to confirm or deny anything and explain that disclosure requires his written consent
- B. Confirm only that he is attending sessions, since that single detail seems harmless
- C. Share a brief progress summary so the employer can support him at work
- D. Refer the employer to Hassan directly and avoid documenting that the call occurred

Question 13. Hassan also describes possible obsessive-compulsive symptoms the counselor has limited training to treat. What is the most ethical action?

- A. Seek supervision or consultation, and refer for the components that exceed his demonstrated competence
- B. Continue treating all of it alone to maintain continuity of the relationship
- C. Tell him those symptoms are not really a problem worth addressing in counseling
- D. Transfer him immediately to another provider without any explanation, consultation, or coordinated referral planning

*Case 3 of 11***Unrelenting yearning more than a year after a loss***Diagnosis: Prolonged Grief Disorder*

INTAKE

Mei Lin, a 58-year-old librarian, presents fourteen months after her husband's death with intense daily yearning for him, preoccupation with memories, and a sense that part of herself has died; she avoids their shared bedroom and his belongings.

SESSION TWO

She describes disbelief that he is gone, emotional pain that has not eased, difficulty re-engaging with friends, and a feeling that life is meaningless without him. She has kept his voicemail to listen to and cannot bring herself to give away his clothes.

SESSION THREE

She reports guilt over a missed early symptom and occasional thoughts that she would rather be with him, without plan or intent. Her grief remains as raw as the first month and now keeps her from returning fully to work and her community.

Question 1. What is most important to confirm to support a prolonged grief disorder diagnosis?

- A. That the death occurred at least twelve months ago and that intense yearning and preoccupation persist nearly every day
- B. That she can describe in detail the specific circumstances and the exact sequence of events surrounding her husband's death
- C. That her low mood and loss of interest have been present nearly every day for at least the past two consecutive weeks
- D. That she experiences intrusive memories and a heightened startle response whenever she is reminded of his final days

Question 2. Which feature best distinguishes her presentation from major depressive disorder?

- A. Her distress centers on yearning and preoccupation with her husband rather than pervasive low mood across all areas of life
- B. The fact that she has felt persistently sad and has cried frequently during the many months since her husband passed away
- C. The presence of disrupted sleep and reduced appetite that she has been experiencing across most days for a long while now
- D. The way her distress tends to intensify noticeably around anniversaries, holidays, and other reminders of her late husband

Question 3. How does prolonged grief disorder differ from an expected, uncomplicated grief reaction?

- A. Its symptoms are persistent, pervasive, and impairing well beyond the timeframe and intensity expected within her cultural context
- B. The fact that she still feels sad and misses her husband deeply more than a year after he died and left her on her own
- C. That she has chosen to keep some of her late husband's belongings and personal mementos rather than giving them all away
- D. That her grief tends to come in waves, with some days feeling notably more difficult for her to manage than other days do

Question 4. Given her thought that she would rather be with her husband, the most appropriate step is to:

- A. Conduct a structured suicide-risk assessment to clarify ideation, intent, and plan, recognizing grief elevates risk
- B. Treat the thought as an entirely understandable and ultimately harmless expression of how much she misses her late husband
- C. Move ahead with grief-processing work first, assuming that easing the grief itself will naturally take care of the thoughts
- D. Ask her to sign a brief written promise that she will stay safe and call the office if the thoughts get worse this week

Question 5. How does her presentation differ from posttraumatic stress disorder?

- A. Her core experience is yearning and preoccupation with the deceased rather than re-experiencing a threatening traumatic event
- B. The fact that she goes out of her way to avoid certain reminders, such as their shared bedroom and his clothing and effects
- C. The intense emotional pain and distress that she feels welling up whenever something unexpectedly brings her husband to mind
- D. The trouble she has had concentrating and staying engaged at her job during the long months since her husband's death

Question 6. What is the most appropriate first-line treatment approach?

- A. A grief-focused psychotherapy that helps her process the loss and gradually approach the reminders she has been avoiding
- B. Open-ended supportive counseling that mainly offers her a warm space to reminisce about her husband whenever she wishes to
- C. Encouraging her to keep avoiding the most painful reminders for now so that she does not have to feel the distress they bring
- D. A primarily insight-oriented exploration of her early attachments as the main route to relieving her current grief symptoms

Question 7. Her avoidance of his belongings and their bedroom should be addressed by:

- A. Gradually and collaboratively approaching the avoided reminders so the associated pain can be tolerated and processed
- B. Insisting that she clear out and give away all of her late husband's belongings right away in order to help her finally move on
- C. Reassuring her that it is perfectly fine to keep avoiding the bedroom and his things for as long as that feels easier for her
- D. Waiting until she no longer feels any pain about the reminders before she begins to approach the bedroom or sort his clothes

Question 8. Mei Lin shows possible co-occurring depression. The most appropriate within-scope action is to:

- A. Screen for a co-occurring depressive disorder and coordinate a prescriber referral if criteria are met, with her consent
- B. Start her on an antidepressant regimen yourself and adjust the dose over the coming weeks based on how she responds to it
- C. Assume the low mood is simply part of normal grieving and therefore does not require any further assessment at this point
- D. Tell her that grief and depression are essentially the same thing and that there is no real need to distinguish between them

Question 9. Mei Lin says she should have caught her husband's illness sooner. The most therapeutic response is to:

- A. Validate her pain and gently explore the guilt, examining what she could realistically have known at the time
- B. Agree that she probably could have done more at the time so that she is able to learn something useful from the experience
- C. Tell her quickly that none of it was her fault at all and that she really needs to stop blaming herself and move forward
- D. Steer the conversation away from the guilt so that the session does not become too painful or overwhelming for her today

Question 10. Mei Lin worries that processing the loss means she is being asked to “forget” her husband. Best response?

- A. Clarify that the goal is to carry a continuing bond with him while re-engaging with life, not to forget or let go of him
- B. Explain that the central purpose of the work really is to help her gradually let go of her husband and detach from his memory
- C. Reassure her that she will be able to keep every single memory exactly as vivid and intense as it is for her right now forever
- D. Avoid the concern and redirect her toward setting concrete behavioral goals for re-engaging with her work and her community

Question 11. Mei Lin's mourning includes specific cultural and religious practices the counselor is unfamiliar with. Best response?

- A. Approach her practices with cultural humility, learn their meaning to her, and incorporate them respectfully into the work
- B. Gently encourage her to set aside those particular practices so that they do not end up slowing down her grief recovery
- C. Apply the standard expected grief timeline to her case and judge her progress strictly against those usual benchmarks
- D. Refer her elsewhere immediately on the assumption that you cannot possibly work with someone from her cultural background

Question 12. Mei Lin asks the counselor to attend her husband's memorial anniversary gathering as her guest. Best response?

- A. Warmly explore the meaning of the request and maintain the professional boundary, explaining its protective purpose
- B. Agree to attend the gathering as her guest, since being there would clearly mean a great deal to her during a painful time
- C. Decline quickly and move on without acknowledging the feelings or the significance behind her invitation to the gathering
- D. Offer to attend just briefly and only to quietly pay your respects, as long as you do not stay for the rest of the gathering

Question 13. After several months Mei Lin's grief has not shifted and may need a specialized grief protocol. Best step?

- A. Reassess progress and seek consultation or refer to a clinician trained in a specialized grief protocol if indicated
- B. Continue the same approach indefinitely because she is already comfortable and well established in the work with you
- C. Tell her that grief simply takes as long as it takes and that there is no real need to change anything about the treatment
- D. Terminate the counseling relationship at once and instruct her to find a grief specialist entirely on her own without help

*Case 4 of 11***Escalating evening drinking after the kids leave home***Diagnosis: Alcohol Use Disorder, Moderate*

INTAKE

Marcus Delgado, a 50-year-old accountant, self-refers to outpatient counseling reporting that his evening drinking has crept up steadily over the past year since both of his adult children moved out, leaving long, unstructured evenings at home.

SESSION TWO

He describes intending to have one or two beers most nights but often ending up with four or five to feel any relaxing effect, and needing noticeably more alcohol than a year ago for the same effect; he denies morning shakiness, sweating, nausea, or any legal or medical problems tied to drinking.

SESSION THREE

He reports two attempts in the past six months to cut back to one or two drinks a night, each lasting only a few days, and a strong urge to pour a drink soon after dinner; his drinking now fills two to three hours of his evening compared with under one hour a year ago.

Question 1. What is the most important focus for the intake interview to establish Marcus's alcohol use pattern?

- A. Systematically review the past 12 months for Criterion A symptoms: amount, tolerance, cut-down attempts, and time spent
- B. Ask mainly about any past legal consequences such as arrests or citations connected to his drinking history
- C. Focus mainly on his extended family's history of alcohol problems across several prior generations
- D. Rely mainly on a short report from his wife about his drinking rather than interviewing Marcus about his own use

Question 2. Given his reported drinking, what routine intake screening step should the counselor take?

- A. Screen for withdrawal risk and medical complications, referring for a medical evaluation if any signs emerge
- B. Begin managing any withdrawal symptoms directly in session using relaxation and grounding techniques
- C. Skip any withdrawal screening entirely since he denies morning shakiness, sweating, or nausea at intake
- D. Require inpatient medical detoxification before any outpatient counseling sessions can begin with him

Question 3. What else should the counselor gather at intake to guide a stage-matched treatment approach?

- A. His current readiness and motivation to change his drinking, including how he views the pros and cons
- B. A full inventory of every beverage brand and price point he typically purchases during his weekly shopping
- C. A detailed account of his daily schedule for the past ten years to map how his routine has evolved
- D. Whether he would agree right now to commit permanently to total abstinence as the only acceptable goal

Question 4. Which finding set most clearly supports a moderate, rather than mild, severity specifier?

- A. Five endorsed symptoms: drinking more than intended, time spent, two failed cut-down attempts, tolerance, and craving
- B. The single fact that his drinking increased sometime after his children moved out of the family home
- C. His own subjective sense that his drinking feels somewhat heavier than it did when he was younger
- D. The observation that he now drinks beer most evenings instead of only on weekends as he once did

Question 5. What distinguishes Marcus's presentation from Adjustment Disorder, with Anxiety?

- A. He meets full Criterion A symptom criteria for a substance use disorder rather than only a stress reaction
- B. The fact that his drinking pattern changed only after an identifiable life stressor took place
- C. His distress appears somewhat out of proportion to how significant the actual life change really was
- D. The likelihood that his symptoms will resolve on their own within about six months without any treatment

Question 6. Given Marcus's ambivalence, what is the most appropriate treatment-planning approach?

- A. Use a motivational, stage-matched approach that meets him at his current level of readiness to change
- B. Insist that he commit to complete abstinence starting immediately as a condition of continuing treatment
- C. Tell him to simply try harder using the same approach that already failed him on two prior occasions
- D. Wait passively without any active planning until he independently decides on his own that change is necessary

Question 7. Which level-of-care determination is most appropriate for Marcus at this point?

- A. Outpatient counseling, since he has no withdrawal risk, medical complications, or safety concerns present
- B. Medically managed inpatient withdrawal, since any regular drinking pattern always requires hospital admission
- C. A residential program lasting several months so he is fully removed from his home drinking environment
- D. No formal level-of-care placement at all, since he can simply be told to monitor his own drinking informally

Question 8. How should the counselor best set an initial drinking-related goal with Marcus?

- A. Collaboratively explore his goals and readiness together, rather than the counselor selecting the goal alone
- B. Assign lifelong total abstinence as the only treatment goal without first discussing his own preferences
- C. Postpone any goal discussion indefinitely until he raises the topic of drinking again entirely on his own
- D. Set a goal based only on what worked well for a different client the counselor treated in the past

Question 9. What is the best way to monitor Marcus's progress once treatment begins?

- A. Have him track daily drinks consumed and urges using a simple log reviewed together at each session
- B. Ask him only once, at the very end of treatment, whether he generally feels his drinking has improved
- C. Track only whether he attends each scheduled session, without discussing his drinking pattern directly
- D. Have his wife independently report on his drinking each week instead of asking Marcus directly himself

Question 10. Marcus says, "I don't know if I really have a problem, I just drink to relax." Best response?

- A. Reflect his ambivalence and gently explore the gap between his goals and his current drinking pattern
- B. Tell him directly that he is clearly wrong and that his drinking is obviously a serious problem
- C. Agree with him that relaxing after a long day with a drink is a completely normal and harmless habit
- D. Change the subject to something lighter so the conversation does not feel uncomfortable for either person

Question 11. Marcus becomes defensive when asked about his two failed attempts to cut back. Best counselor move?

- A. Roll with the resistance, affirm his effort in trying, and invite him to explore what got in the way
- B. Point out firmly that failing twice already proves he cannot manage this without more intensive treatment
- C. Drop the subject immediately and avoid ever bringing up his prior cut-down attempts again in future sessions
- D. Minimize the importance of the topic by saying it does not really matter and is not worth discussing further

Question 12. What must the counselor ensure Marcus understands during informed consent at intake?

- A. The limits of confidentiality and the scope of counseling, including referral for any needed medical evaluation
- B. That everything he discusses in every session will be kept completely private under any circumstance
- C. That the counselor alone will decide which specific drinking-related goals he is required to pursue
- D. That once he agrees to begin, he is obligated to complete the entire recommended course of treatment

Question 13. Marcus asks whether he needs to see a physician about his drinking. Best counselor response?

- A. Explain that a brief medical evaluation is a reasonable step and offer to coordinate a referral with his consent
- B. Tell him confidently that a physician visit is completely unnecessary since he denies any withdrawal symptoms
- C. Suggest he research his own symptoms online and independently decide for himself whether a visit is warranted
- D. Avoid answering his question directly and instead redirect the conversation back to his drinking goals

*Case 5 of 11***Intense relationships and recurrent self-harm in a graduate student***Diagnosis: Borderline Personality Disorder*

INTAKE

Aisha Thompson, a 26-year-old graduate student, presents with a long-standing pattern of intense, unstable relationships, fear of abandonment, rapidly shifting self-image, and recurrent self-harm by cutting during periods of emotional distress.

SESSION TWO

She describes relationships that swing between idealizing and devaluing others, chronic feelings of emptiness, and intense anger she struggles to control; her mood shifts within hours in response to perceived rejection rather than across weeks.

SESSION THREE

She reports cutting her arms to relieve unbearable tension and has had two past low-lethality overdoses during breakups; she denies current intent but says she urges return whenever she fears someone close to her is pulling away from her.

Question 1. What is most important for the counselor to establish to support a borderline personality disorder diagnosis?

- A. A pervasive, long-standing pattern of at least five of the nine criteria across many situations and relationships
- B. A clearly identifiable single traumatic event in her childhood that can be pinpointed as the original cause of the pattern
- C. A series of distinct mood episodes that have each clearly lasted for at least several consecutive days or weeks at a time
- D. A consistent and well-documented pattern of clear psychotic symptoms that reliably emerge during her most stressful periods

Question 2. Which feature best differentiates her presentation from bipolar II disorder?

- A. That her mood shifts within hours in reaction to interpersonal events rather than lasting as sustained discrete episodes
- B. That she clearly experiences strong feelings of irritability and anger during many of her most emotionally intense moments
- C. That she goes through clear stretches of noticeably elevated energy and reduced need for sleep lasting several days at a time
- D. That her difficulties tend to become a great deal more intense and harder to manage during the late evening and nighttime hours

Question 3. Given her recurrent self-harm and past overdoses, the most appropriate assessment step is to:

- A. Complete a structured suicide-risk assessment that distinguishes her self-harm urges from suicidal intent and plan
- B. Assume that because her self-harm appears to be chronic and long-standing it does not require any formal ongoing assessment
- C. Ask her to promise that she will not cut or harm herself again at any point before her next scheduled counseling session
- D. Focus the assessment narrowly on the specific physical wounds themselves rather than on her thoughts, intent, and triggers

Question 4. Distinguishing her self-harm from a suicide attempt most importantly requires assessing:

- A. Her intent and the function of the behavior, since cutting to relieve tension differs from acting with intent to die
- B. The precise depth and overall medical seriousness of the physical wounds that she has produced during her cutting episodes
- C. Exactly how frequently the self-harming behavior tends to occur across a typical week or month of her ongoing daily life
- D. Whether the people closest to her are fully aware that she has been engaging in this self-harming behavior for some time

Question 5. What co-occurring condition is most important to screen for given her history?

- A. Co-occurring posttraumatic stress and depressive symptoms, which commonly accompany borderline personality disorder
- B. A previously undiagnosed primary psychotic disorder that could fully account for the brief paranoid moments she sometimes has
- C. An underlying neurodevelopmental disorder that could explain the impulsivity she has shown since she was a young child
- D. A primary eating disorder that might be the true underlying driver of both her impulsivity and her emotional instability

Question 6. What is the most appropriate evidence-based treatment recommendation?

- A. Referral to a structured, evidence-based psychotherapy for borderline personality disorder such as dialectical behavior therapy
- B. Open-ended supportive counseling without any particular structure, focused mostly on listening to whatever arises each week
- C. Brief solution-focused work over a small number of sessions aimed at quickly resolving the presenting interpersonal conflicts
- D. A primarily insight-oriented exploration of her early attachments as the single main route to relieving her current symptoms

Question 7. For managing her recurrent suicidal and self-harm urges, the plan should center on:

- A. A collaborative safety plan identifying warning signs, coping strategies, and supports she can use when urges escalate
- B. A signed no-suicide contract in which she formally agrees in writing never to harm herself for the duration of treatment
- C. An arrangement for her to be psychiatrically hospitalized each and every time that she experiences any urge to harm herself
- D. A strict set of rules forbidding her from contacting the counselor whenever the self-harm urges happen to become stronger

Question 8. Between sessions Aisha frequently calls in crisis. The most appropriate plan element is to:

- A. Establish clear, consistent between-session contact guidelines and coach her in using skills during crises within that frame
- B. Make yourself personally available to her by phone at any hour of the day or night so that she never has to face a crisis alone
- C. Tell her plainly that she may not contact you at all between sessions and that every concern must wait until the next appointment
- D. Refer her out of your practice entirely so that an on-call crisis line can take over all of the between-session contact instead

Question 9. When Aisha alternates between idealizing and devaluing the counselor, the most therapeutic response is to:

- A. Maintain a steady, nonreactive stance and a consistent frame while validating the feelings underneath the shifts
- B. Point out to her directly that she is splitting and explain in detail how this same pattern is damaging all of her relationships
- C. Respond warmly when she idealizes you and noticeably more distantly when she devalues you so she learns about consequences
- D. Take the devaluation as accurate feedback about your work and substantially change your whole approach each time it happens

Question 10. Aisha becomes intensely angry when the counselor mentions an upcoming vacation. The best response is to:

- A. Validate the abandonment fear, prepare for the break in advance, and arrange clear coverage and supports during the absence
- B. Reassure her firmly that her anger is unwarranted because the vacation is brief and she will be completely fine without you
- C. Offer to cancel the planned vacation entirely so that she does not have to experience any distress about the coming separation
- D. Avoid the topic of the vacation as much as possible and reveal the details only at the very last session before you leave

Question 11. Aisha asks the counselor to be her friend outside of sessions because she trusts no one else. Best response?

- A. Warmly maintain the professional boundary, explain its protective purpose, and explore the longing for connection in therapy
- B. Agree to occasional friendly contact outside of sessions because the therapeutic relationship is clearly so important to her
- C. Decline curtly and move on quickly without acknowledging the feelings or the meaning behind her request for closeness
- D. Promise her that once the formal therapy has fully ended the two of you can certainly become close personal friends afterward

Question 12. The counselor notices unusually strong frustration with Aisha's frequent crises. The most appropriate step is to:

- A. Seek clinical supervision or consultation to manage the countertransference and protect the quality of her care
- B. Address the frustration by setting noticeably firmer and more rigid limits with her in order to reduce the demands on yourself
- C. Disclose your frustration openly to her in session so that she can fully understand the real impact her behavior has on you
- D. Begin the process of transferring her to another provider right away, since your frustration clearly means you can no longer help her

Question 13. Aisha discloses an urge to overdose tonight after a breakup but pleads with the counselor to keep it secret. Best response?

- A. Explain the limits of confidentiality, assess imminent risk fully, and act to protect her safety as the situation requires
- B. Agree to keep the disclosure completely confidential so that she continues to feel safe being fully honest with you in the future
- C. Reassure her that since this is most likely just her chronic pattern of urges there is no real need to take any further action tonight
- D. Wait until the next scheduled session to revisit the disclosure once she has had some time to fully calm down on her own first

*Case 6 of 11***A week of soaring energy, big plans, and little sleep***Diagnosis: Bipolar I Disorder, Current Episode Manic*

INTAKE

Darnell Carter, a 30-year-old entrepreneur, is brought in by his sister, who reports that for the past week he has barely slept, talks rapidly, and has launched several grand business schemes, maxing out credit cards and approaching strangers with big plans.

SESSION TWO

Darnell is energized and irritable when questioned, insisting he feels “the best ever” and needs little sleep. His speech is hard to interrupt, his thoughts race, and his sister notes a similar but milder episode last year followed by months of deep depression.

SESSION THREE

He denies drug use, and a recent screen was negative; he has no fixed delusions but is grandiose about his abilities. His risky spending and driving worry his family, and he minimizes any concern, saying everyone else simply cannot keep up with him.

Question 1. What is most important to confirm to establish a manic episode?

- A. A distinct week or more of elevated or irritable mood with increased energy plus several associated manic symptoms
- B. That he can identify the specific stressful event that he believes is responsible for how he has been acting this week
- C. That his elevated mood has clearly been more intense over the past day or two than it was earlier in the week
- D. That he currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Which finding most strongly supports Bipolar I rather than a major depressive disorder?

- A. The current manic episode itself, since a single lifetime manic episode establishes a Bipolar I diagnosis
- B. The fact that his sister recalls a period of deep depression that lasted for several months after last year's episode
- C. The irritability and short temper that he displays whenever his family members try to question his recent behavior
- D. The trouble he has concentrating and staying on one topic during the conversation in the counselor's office today

Question 3. Given his presentation, what is the most important condition to rule out?

- A. A substance-induced manic picture, which his negative drug screen and denial of use make less likely but worth confirming
- B. A primary anxiety disorder that could be producing his restlessness and his difficulty sitting still during the session
- C. A long-standing personality pattern that could account for why he tends to be so confident and ambitious in his business
- D. A neurocognitive decline that might explain the poor judgment he has shown in his recent spending and driving decisions

Question 4. How should the counselor weigh his grandiosity in the differential?

- A. As mood-congruent grandiosity within the manic episode rather than the fixed delusions seen in a psychotic disorder
- B. As clear evidence of a primary psychotic disorder, since any grandiosity at all means that psychosis is the main problem
- C. As an ordinary, healthy level of entrepreneurial confidence that does not require any particular clinical attention at all
- D. As a defensive personality style he uses to cope whenever his family members start to criticize the choices he has made

Question 5. Given his risky behavior and irritability, what safety assessment is most important now?

- A. Assess danger to self and others, including suicide risk and the impulsive, high-risk behaviors he is currently engaging in
- B. Focus only on whether he currently has a specific, detailed plan to end his own life and disregard the other risks for now
- C. Defer any safety assessment until his mood has settled, since he is clearly far too elevated to answer the questions reliably
- D. Rely on his own reassurance that he feels great and is in full control, since he knows his own situation best of all

Question 6. What is the most appropriate immediate treatment-planning priority?

- A. Coordinate an urgent psychiatric evaluation for pharmacologic management of the acute mania and to address safety
- B. Begin weekly insight-oriented psychotherapy to help him understand the deeper roots of his recent grand business schemes
- C. Start him on a mood-stabilizing medication yourself and arrange to monitor his blood levels over the coming weeks
- D. Reassure the family that the episode will most likely pass on its own and simply schedule a routine follow-up in a month

Question 7. Once he is stabilized, which approach best supports relapse prevention?

- A. Interpersonal and social rhythm therapy to regularize his sleep-wake and daily routines and detect early warning signs
- B. A primarily cathartic approach focused on fully expressing the strong emotions that build up during his elevated periods
- C. Encouraging him to push himself to stay highly productive and busy so that he does not slip back into a depressive low
- D. A long course of exposure therapy aimed at reducing his anxiety about having another manic episode in the future

Question 8. Darnell asks the counselor what mood stabilizer dose he should request. The most appropriate action is to:

- A. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate his questions and progress with them
- B. Suggest a typical starting dose that tends to work for most people who are in the middle of an acute manic episode like his
- C. Tell him to ask for the lowest possible dose so that the medication will not slow him down or dampen his current productivity
- D. Say that medication dosing has nothing to do with counseling and decline to help him prepare for the conversation in any way

Question 9. Darnell insists nothing is wrong and that his family is overreacting. The most therapeutic stance is to:

- A. Engage him respectfully, find shared goals such as protecting his business, and link safety steps to what he values
- B. Confront him directly with all the evidence that he is manic until he is finally forced to admit that something is wrong
- C. Agree with him that his family is overreacting so that he feels understood and stays willing to keep working with you
- D. Avoid the disagreement entirely and redirect the conversation to neutral topics until he becomes more open to discussing it

Question 10. How can the counselor best involve Darnell's family during this acute period?

- A. With his consent, provide psychoeducation about warning signs and engage the family in supporting the safety plan
- B. Share the full details of his sessions with the family right away so they can take charge of managing all of his behavior
- C. Decline any family involvement at all and tell them this is entirely a private matter between you and the client alone
- D. Encourage the family to take over his finances and his major business decisions for him without first discussing it with him

Question 11. Darnell's grandiose driving and spending raise safety concerns. When does confidentiality most clearly yield?

- A. When there is a serious, foreseeable risk of harm to him or others, disclosure may be required to protect safety
- B. Whenever a family member simply asks for information, since concerned relatives are entitled to know how he is doing
- C. Never, because what a client shares in counseling must always remain completely confidential under all possible circumstances
- D. Whenever the counselor personally feels that disclosing the information would probably be in the client's own best interest

Question 12. Given his current state, what is the most appropriate way to handle informed consent and decision-making?

- A. Assess his capacity to make decisions, involve him as fully as possible, and document consent and any capacity concerns
- B. Assume he completely lacks capacity and make all of the treatment decisions for him without involving him in the process
- C. Assume he has full capacity and proceed exactly as usual without giving his current manic state any particular consideration
- D. Postpone the entire consent discussion until the episode fully resolves, even though treatment needs to begin right away

Question 13. Darnell wants the counselor to coordinate with the psychiatrist and his sister. What must come first?

- A. Obtain his specific, informed consent for each party before sharing any of his protected health information
- B. Begin coordinating with both parties immediately, since he has already said in session that he wants you to work with them
- C. Share freely with the psychiatrist without a release because both of you are treating providers working on the same case
- D. Coordinate only with his sister for now and decide about the psychiatrist later without discussing the matter with him

*Case 7 of 11***Voices and growing suspicion in a college student***Diagnosis: Schizophrenia*

INTAKE

Aaliyah Brooks, a 23-year-old college student, is referred after eight months of declining function. She reports hearing voices commenting on her actions, believes classmates are conspiring against her, and her speech at times becomes hard to follow.

SESSION TWO

Her grades have collapsed and she has withdrawn from friends, stopped attending most classes, and shows flattened affect and little motivation. The hallucinations and suspicion have been present nearly continuously for months rather than only briefly.

SESSION THREE

She denies sustained periods of elevated or depressed mood driving the symptoms, and a recent toxicology screen was negative. She is frightened by the voices, has limited insight that she is ill, and at times wonders whether life is worth continuing.

Question 1. What is most important to confirm to support a schizophrenia diagnosis?

- A. Two or more active-phase symptoms with continuous signs of disturbance and functional decline lasting at least six months
- B. That she can name the specific stressful event on campus that she believes first triggered all of these experiences
- C. That her hallucinations have become noticeably more intense over the past few days than they were a week ago
- D. That she currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Which feature best distinguishes her presentation from schizoaffective disorder?

- A. Her psychotic symptoms are present without a sustained mood episode driving them across most of the illness course
- B. The fact that she experiences auditory hallucinations of voices that comment on what she is doing throughout the day
- C. The persecutory belief that her classmates have been conspiring and working against her over the past several months
- D. The way her symptoms seem somewhat worse during the high-stress weeks of midterm and final examinations on campus

Question 3. How does her presentation differ from brief psychotic disorder?

- A. Her symptoms have persisted nearly continuously for months rather than resolving within the one-month brief-disorder window
- B. The fact that she has clearly experienced delusions and hallucinations during the course of her current illness
- C. The significant distress and the fear that the frightening voices have clearly been causing her over recent months
- D. The trouble she has had keeping up with her coursework and her assignments since these experiences first began

Question 4. What is the most important condition to rule out before confirming the diagnosis?

- A. A substance- or medication-induced psychosis, which her negative toxicology screen makes less likely but worth confirming
- B. A primary anxiety disorder that could be producing her suspicion that classmates are working against her on campus
- C. A long-standing personality pattern that could account for why she has become so withdrawn and distrustful of others
- D. An eating disorder that might explain both her social withdrawal and the decline in her academic performance this year

Question 5. Given her statement that she wonders whether life is worth continuing, the most appropriate step is to:

- A. Conduct a structured suicide-risk assessment, recognizing that suicide risk is elevated in early psychosis
- B. Treat the comment as a fleeting and ultimately harmless part of the distress that her frightening voices are causing her
- C. Move ahead with planning psychiatric referral first, assuming that treating the psychosis will resolve the thoughts on its own
- D. Ask her to sign a brief written promise that she will stay safe and call the clinic if the thoughts get worse this week

Question 6. What is the most appropriate first-line treatment recommendation?

- A. Coordinate an urgent psychiatric evaluation for antipsychotic medication alongside psychosocial treatment and support
- B. Begin intensive insight-oriented psychotherapy to help her uncover the deeper meaning behind the voices she has been hearing
- C. Start her on an antipsychotic medication yourself and arrange to monitor her response and side effects over the coming weeks
- D. Recommend a course of weekly supportive counseling on its own as the complete treatment for her psychotic symptoms

Question 7. Which psychosocial component is most appropriate to include in her plan?

- A. Cognitive behavioral therapy for psychosis and family psychoeducation to support coping and reduce relapse risk
- B. A primarily confrontational approach that repeatedly challenges her delusional beliefs until she finally rejects them as false
- C. An immediate, intensive exposure protocol aimed at the situations on campus that she has increasingly been avoiding lately
- D. A long course of psychodynamic exploration of her early childhood as the primary route to resolving her current psychosis

Question 8. What additional support is most appropriate to coordinate given her functional decline?

- A. Case management and supported education or employment services to help her re-engage with her studies and daily roles
- B. A recommendation that she withdraw from school indefinitely and simply rest at home until all of her symptoms fully resolve
- C. Detailed academic tutoring focused only on raising her grades, without addressing the symptoms driving the decline
- D. A strict daily schedule of demanding tasks designed to push her to return to her full prior workload as quickly as possible

Question 9. During sessions Aaliyah is guarded and suspicious of the counselor. The most therapeutic stance is to:

- A. Build a steady, trustworthy alliance with consistency and transparency, going at a pace she can tolerate
- B. Press her to explain her suspicions in full detail right away so that you can immediately correct the inaccurate beliefs
- C. Reassure her repeatedly that her classmates would never conspire against her and that her fears are completely unfounded
- D. Go along with her belief that the classmates are conspiring so that she feels understood and keeps coming to her sessions

Question 10. When Aaliyah describes the voices, the most therapeutic response is to:

- A. Acknowledge how distressing and real they feel to her without confirming or arguing about their objective accuracy
- B. Tell her plainly and repeatedly that the voices are simply not real and that she needs to stop paying any attention to them
- C. Ask her detailed questions about exactly what the voices are instructing her to do so you can follow all of their commands
- D. Agree with her that the voices are probably other real people somehow speaking to her from somewhere else nearby on campus

Question 11. A voice reportedly tells Aaliyah to harm a specific classmate she names. The most appropriate action is to:

- A. Assess the risk carefully and follow duty-to-protect obligations, which may require warning or protective steps
- B. Keep the disclosure entirely confidential, since anything she shares in counseling must always remain private no matter what
- C. Wait to see whether she mentions the same threat again at the next session before taking any action or assessing the risk
- D. Immediately notify the classmate and the police without first assessing the seriousness or imminence of the actual risk

Question 12. Aaliyah's parents call asking for full details of her treatment. She is 23. Best response?

- A. Decline to disclose without her consent and, with her permission, explore appropriate, helpful family involvement
- B. Share the full details with her parents, since family members are clearly going to be central to supporting her recovery
- C. Refuse any contact with the parents at all and tell them you cannot even acknowledge whether she is a client of yours
- D. Give the parents a general update on how she is doing while leaving out the most sensitive specific clinical details of her care

Question 13. The counselor has limited training in psychosis care. What is the most ethical course of action?

- A. Seek consultation and coordinate with a treatment team, referring for the components beyond his demonstrated competence
- B. Continue to manage all aspects of her care alone in order to preserve the continuity of the relationship he has built with her
- C. Tell her the symptoms are not serious enough to need a psychiatric referral and keep seeing her for supportive talks only
- D. Transfer her immediately to another provider with no explanation, consultation, or coordinated plan for the transition of care

*Case 8 of 11***Lifelong distractibility and restlessness derailing work***Diagnosis: Attention-Deficit/Hyperactivity Disorder, Combined*

INTAKE

Diego Morales, a 27-year-old graphic designer, presents with lifelong distractibility, disorganization, and restlessness that are now jeopardizing his job. He misses deadlines, loses track of tasks, interrupts others, and struggles to sit through meetings.

SESSION TWO

He recalls similar problems since grade school—careless errors, fidgeting, and trouble finishing work—and report cards noting he was “bright but unfocused.” The difficulties show up both at work and at home and are not limited to one setting or one period.

SESSION THREE

He denies discrete episodes of elevated mood or decreased need for sleep, and his concentration problems are chronic rather than episodic. He feels demoralized and calls himself “lazy,” though he works long hours trying to compensate for the disorganization.

Question 1. What is most important to establish to support an adult ADHD diagnosis?

- A. That several symptoms were present before age 12 and that impairment occurs across more than one setting
- B. That his attention problems first clearly began only after the recent pressures at his current job started building up
- C. That he can identify the single stressful life event that he believes is responsible for his current difficulties focusing
- D. That his symptoms are noticeably more severe right now than they were at any earlier point in his life so far

Question 2. Which feature best differentiates his presentation from generalized anxiety disorder?

- A. His attention problems are a lifelong, pervasive pattern rather than concentration difficulty arising from chronic worry
- B. The fact that he feels restless and has trouble sitting still during long meetings and lengthy work tasks at his job
- C. The trouble he has falling asleep at night and the fatigue that he reports across most of his ordinary workdays
- D. The way his difficulties tend to feel worse during the busiest and most demanding weeks of his design projects at work

Question 3. How does his presentation differ from bipolar II disorder?

- A. His symptoms are a chronic, continuous pattern rather than occurring in discrete episodes of elevated mood and energy
- B. The fact that he is often impulsive and tends to interrupt others during conversations and meetings throughout the day
- C. The trouble he has organizing his tasks and following through on his work projects from start to finish at his job
- D. The high energy he puts into working long hours as he tries to keep up with and compensate for his disorganization

Question 4. What assessment method most strengthens the diagnostic evaluation?

- A. Validated ADHD rating scales together with developmental history and collateral information from those who know him
- B. Asking him to keep an open-ended journal describing his feelings each day without any structured measure or collateral input
- C. Observing only how he behaves during the session itself and basing the diagnosis entirely on that single observation
- D. Ordering neuropsychological and neuroimaging tests yourself to confirm the diagnosis before beginning any treatment planning

Question 5. What is most important to rule out before confirming the diagnosis?

- A. Medical and substance contributors and other conditions, such as a sleep disorder or depression, that can impair attention
- B. A previously undetected primary psychotic disorder that could account for his disorganization and trouble finishing tasks
- C. A long-standing eating disorder that might be contributing to his low energy and his trouble concentrating at his job
- D. An emerging neurocognitive decline that could explain the careless errors and forgetfulness he has shown since childhood

Question 6. What is the most appropriate within-scope treatment recommendation?

- A. Provide skills-based behavioral and organizational intervention and coordinate a referral for a medication evaluation
- B. Recommend that he start a stimulant medication right away and adjust the dose himself depending on how focused he feels
- C. Focus mainly on long-term exploration of his early childhood as the central route to resolving his attention problems
- D. Suggest he simply try harder to focus and reassure him that more willpower and discipline will resolve the difficulties

Question 7. Which skills-based target is most appropriate for his presentation?

- A. Externalizing organization through planners, reminders, task breakdown, and environmental structure to support follow-through
- B. Encouraging him to keep all of his deadlines and task lists in his head so that he learns to strengthen his memory over time
- C. Teaching him deep relaxation techniques to use whenever he feels restless, without addressing his organization or planning
- D. Advising him to take on even more projects at once so that he stays busy enough to avoid getting bored and distracted

Question 8. Diego asks which ADHD medication and dose he should request. The most appropriate action is to:

- A. Explain that medication selection and dosing are the prescriber's role and offer, with consent, to coordinate his questions
- B. Recommend the specific stimulant and dose that tends to work best for adults who present with his particular symptom pattern
- C. Tell him to ask for the highest available dose so the medication is sure to keep him focused throughout his entire workday
- D. Say medication is purely a medical matter and decline to help him prepare any questions at all for his upcoming appointment

Question 9. Diego calls himself “lazy” and feels demoralized about his difficulties. The most therapeutic response is to:

- A. Reframe the difficulties as a treatable neurodevelopmental condition and highlight his real efforts to compensate
- B. Agree that he does seem to lack discipline and encourage him to simply hold himself to a much higher standard going forward
- C. Move past the comment quickly and refocus the session on reviewing the organizational tools he has started to put in place
- D. Provide detailed statistics on how common adult ADHD is in the population to logically prove that he is not actually lazy

Question 10. How can the counselor best support Diego’s follow-through between sessions?

- A. Collaboratively set small, specific, achievable between-session steps and review them with a measurement-based check-in
- B. Assign him a large, ambitious overhaul of his entire work system to complete on his own before the next appointment
- C. Leave the between-session work entirely open-ended and trust that he will figure out the details on his own as he goes
- D. Track only whether he attends his sessions and treat consistent attendance as the main sign that he is making progress

Question 11. Diego worries that an ADHD diagnosis just gives him an “excuse” to fail. Best response?

- A. Validate the worry and reframe the diagnosis as an explanation that guides effective strategies, not an excuse to give up
- B. Agree that a diagnosis can indeed become a convenient excuse and warn him not to rely on it to justify any future mistakes
- C. Tell him not to worry about labels at all and move directly on to reviewing his organizational strategies for the week
- D. Explain the detailed neuroscience of ADHD at length to convince him on technical grounds that it is a legitimate condition

Question 12. Diego requests a letter for workplace accommodations. What is the most appropriate counselor action?

- A. Provide accurate documentation within competence, with his consent, describing functional impacts relevant to accommodations
- B. Write whatever will most help him secure the accommodations he wants, even if it overstates how impaired he actually is
- C. Refuse to provide any documentation at all and tell him that workplace accommodations are entirely outside the counseling role
- D. Send the letter directly to his employer yourself before discussing with him exactly what it will say or obtaining his consent

Question 13. Diego’s employer calls asking whether he has “a mental condition affecting his work.” Best response?

- A. Decline to confirm or deny without a valid release and explain that disclosure requires his written consent
- B. Confirm that he has ADHD, since the employer clearly already suspects it and is only trying to support him at work
- C. Share a general summary of his condition so that the employer can decide what accommodations might be reasonable for him
- D. Refuse to speak with the employer at all and hang up without explaining anything about confidentiality or the proper process

*Case 9 of 11***Intrusive Thoughts and Endless Rituals***Diagnosis: Obsessive-Compulsive Disorder*

INTAKE

Aaron, a 30-year-old teacher, reports intrusive thoughts that his hands are contaminated and that he might accidentally harm a student. To relieve the distress he washes his hands until they crack and checks his classroom repeatedly before leaving. The rituals now take more than two hours a day and make him late.

SESSION TWO

Aaron knows the thoughts are excessive and does not want them, but he feels he must perform the rituals to prevent something terrible. The harm thoughts horrify him and are completely against his values. He has started avoiding being alone with students and feels ashamed and exhausted.

SESSION THREE

Aaron asks repeatedly for reassurance that he is not a dangerous person. His partner has begun helping him check the apartment to save time, which has made the checking worse. He is motivated to get better and wonders whether medication or therapy would help him most.

Question 1. What is the most appropriate way to begin assessing Aaron's symptoms?

- A. Assess his obsessions, compulsions, time spent, distress, and degree of insight
- B. Begin response prevention exercises right away before completing the assessment
- C. Focus mainly on his childhood to find the origin of his intrusive thoughts
- D. Reassure him that the thoughts are normal and that nothing needs attention

Question 2. Which finding best supports the time/impairment criterion for OCD?

- A. His rituals consume more than an hour a day and are making him late for work
- B. He sometimes feels anxious when he thinks about germs or contamination
- C. He prefers to keep his classroom tidy and well organized for his students
- D. He occasionally double-checks that he has locked his classroom door

Question 3. Why is assessing Aaron's level of insight important?

- A. Insight is a diagnostic specifier and helps guide how treatment is framed for him
- B. Good insight means that he does not actually meet full criteria for the disorder at all
- C. Poor insight always means the person has a psychotic disorder instead
- D. Insight determines whether the obsessions count as worries instead

Question 4. What distinguishes Aaron's OCD from Generalized Anxiety Disorder?

- A. He has specific intrusive obsessions with ritualized compulsions, not diffuse worry
- B. He feels anxious and tense during much of his typical day
- C. He worries about more than one area of his life at times
- D. He sometimes has trouble sleeping at night because his mind feels far too busy to settle

Question 5. How does Aaron's OCD differ from Obsessive-Compulsive Personality Disorder?

- A. His obsessions and compulsions are unwanted and distressing, not a valued life style
- B. He values order and high standards in the work that he does
- C. He can be rather rigid about how certain tasks should be done
- D. He tends to be quite conscientious, careful, and thorough in nearly all of his daily life

Question 6. What is the first-line psychotherapy for Aaron's OCD?

- A. Exposure and response prevention, in which he faces triggers without doing the ritual
- B. Supportive talk therapy that helps him feel heard about his distress
- C. Repeated reassurance from the counselor that his fears will not come true
- D. Analysis of the hidden meaning behind each of his intrusive thoughts

Question 7. How should the counselor explain the rationale for response prevention to Aaron?

- A. Facing the fear without the ritual lets his anxiety fall on its own through habituation
- B. Performing the ritual a few more times will gradually wear out the urge
- C. Avoiding the triggers entirely will keep his anxiety at a low level
- D. Thinking positive thoughts whenever an obsession appears will cancel it out

Question 8. Aaron's partner helps him check the apartment. How should the counselor address this?

- A. Work to reduce the family accommodation and reassurance that keep the OCD going
- B. Encourage the partner to help with the checking to ease his distress
- C. Leave the partner out of treatment since OCD is an individual problem
- D. Tell the partner to immediately stop all support without any plan

Question 9. How should the counselor monitor Aaron's progress?

- A. Track his symptoms with a validated OCD measure and adjust the exposure hierarchy
- B. Ask whether he feels the rituals are a little better each week
- C. Count only how often he washes and ignore the other rituals entirely
- D. Wait until the end of treatment to see whether the rituals changed

Question 10. In session, Aaron repeatedly asks the counselor to confirm he is not dangerous. What is the best response?

- A. Kindly limit the reassurance and support him in tolerating the uncertainty instead
- B. Firmly reassure him each and every time that he is definitely not a dangerous person at all
- C. Tell him his fear is irrational and that he should already know better
- D. Change the topic quickly so the reassurance request does not continue

Question 11. Aaron is frightened that exposure work will be unbearable. What is the best response?

- A. Give a clear rationale and build the exposure hierarchy together at a manageable pace
- B. Start immediately with the most frightening exposure on the list so he sees that he can survive it
- C. Agree to skip exposure and rely on relaxation methods instead
- D. Tell him the fear is unfounded and that exposure is never difficult

Question 12. Aaron's intrusive thoughts include fears of harming a student. How should the counselor handle duty-to-protect considerations?

- A. Recognize the harm thoughts as ego-dystonic OCD symptoms, not intent, and avoid needless disclosure
- B. Report him to the authorities right away simply because he happened to mention having some thoughts of harm
- C. Warn the school about him just to be cautious about the students
- D. Stop treatment until he can prove that he is not a danger to anyone

Question 13. Aaron asks whether medication might help alongside therapy. What is the most appropriate action?

- A. Explain that an SSRI is evidence-based for OCD and refer for evaluation with his consent
- B. Recommend a specific medication along with a dose for him to begin taking on his own at home
- C. Tell him medication never helps OCD and that therapy alone is enough
- D. Contact a prescriber about his case before he has consented to it

*Case 10 of 11***Acute suicidal crisis after a breakup and job loss***Diagnosis: Suicidal Behavior / Acute Risk*

INTAKE

Jordan Ellis, a 28-year-old veteran, presents in acute distress one week after a breakup and a job loss, saying he sees no reason to keep going. He reports current thoughts of suicide, a plan involving a firearm at home, and a prior attempt two years ago.

SESSION TWO

He describes intense hopelessness, poor sleep, and withdrawal from friends; he has been drinking more heavily this week. He says the firearm is accessible and that he has thought about using it, though part of him is frightened by how strong the thoughts are.

SESSION THREE

He is ambivalent—he mentions his younger sister and his dog as reasons he has not acted—and he is willing to talk through options. He has not set a specific time, and he agrees to work collaboratively on staying safe over the coming days.

Question 1. What is the most appropriate first step in assessing Jordan's presentation?

- A. Conduct a structured suicide-risk assessment clarifying ideation severity, intent, plan, access to means, and prior attempts
- B. Begin teaching him calming and relaxation techniques right away before gathering the details of his suicidal thoughts
- C. Reassure him that the breakup and job loss are temporary setbacks and that he is certain to feel better before long
- D. Refer him to a prescriber for medication first and postpone any direct discussion of the suicidal thoughts until later

Question 2. How should the counselor distinguish Jordan's presentation from nonsuicidal self-injury?

- A. By assessing his intent, since acting with intent to die differs fundamentally from self-injury aimed at relieving tension
- B. By focusing on whether he has visible physical injuries on his body from harming himself in the recent past
- C. By determining how frequently he has had thoughts about harming himself across a typical week or month recently
- D. By asking whether the people in his life are already aware that he has been having these kinds of thoughts lately

Question 3. Given the firearm at home, what assessment is most critical to complete?

- A. A careful assessment of his access to lethal means, since available, lethal methods sharply elevate acute suicide risk
- B. A detailed history of every firearm he has ever owned so the counselor can document his complete background with weapons
- C. A thorough exploration of the childhood experiences that he believes first shaped how he copes with painful emotions
- D. A precise calculation of how much alcohol he has consumed each day so the counselor can quantify his drinking pattern

Question 4. What is most important to assess to form a balanced risk formulation?

- A. Both his risk factors and his protective factors, such as his ambivalence and the relationships he names as reasons to live
- B. Only the single most severe risk factor he reports, since the most serious item alone determines his overall level of risk
- C. Whether his current distress is objectively proportionate to the severity of the breakup and the job loss he experienced
- D. How well he is able to describe, in detail, the specific sequence of events that led up to this current crisis

Question 5. What co-occurring factor is most important to screen for that also raises his acute risk?

- A. His increased alcohol use, which can heighten impulsivity and acute suicide risk and warrants direct assessment
- B. A previously undetected primary psychotic disorder that could be the underlying explanation for his current hopelessness
- C. A long-standing eating disorder that might be contributing to both his poor sleep and his social withdrawal this week
- D. An emerging neurocognitive decline that could explain why he is having so much trouble concentrating during this crisis

Question 6. What is the most appropriate cornerstone of the safety plan?

- A. A collaborative safety plan identifying warning signs, coping strategies, supports, and crisis contacts he can use
- B. A signed no-suicide contract in which he formally promises in writing not to harm himself before the next appointment
- C. A detailed schedule of pleasant activities for the week ahead intended to lift his mood enough that the thoughts simply fade
- D. A firm instruction that he must call the counselor's personal phone the very moment any suicidal thought enters his mind

Question 7. How should the plan address the firearm at home?

- A. Counsel collaboratively on means restriction, such as temporarily storing the firearm away from the home with a trusted person
- B. Leave the firearm question alone entirely, since raising it might offend him and damage the therapeutic relationship you are building
- C. Advise him to simply unload the firearm and keep the ammunition stored in a separate drawer in the same bedroom at home
- D. Tell him that as a responsible adult it is entirely his own decision whether to keep the firearm accessible during this crisis

Question 8. On what basis should the counselor decide whether a higher level of care, such as hospitalization, is needed?

- A. On a clinical judgment weighing intent, plan, means, and his ability to maintain safety with the current plan and supports
- B. On whether he explicitly asks to be hospitalized, since the decision should ultimately be left entirely up to the client himself
- C. On hospitalizing him automatically, since anyone who mentions suicide at all should always be admitted to an inpatient unit
- D. On avoiding hospitalization in all cases, since inpatient admission would only disrupt the therapeutic relationship you have built

Question 9. What follow-up element most strengthens his safety after this session?

- A. A warm handoff and a scheduled follow-up with caring contact rather than simply sending him out with a phone number
- B. A reminder that he can look up crisis resources online on his own if he happens to feel worse over the coming weekend
- C. A plan to wait and see how he is doing at his next routine appointment in several weeks without any contact before then
- D. A suggestion that his ex-partner check in on him regularly, since reconnecting with her would clearly help him feel better

Question 10. Jordan says he is not sure he wants to live but is not sure he wants to die either. Best response?

- A. Explore his ambivalence with empathy and build on his stated reasons for living to strengthen his motivation to stay safe
- B. Tell him firmly that he has so much to live for and that he should not be thinking this way after only one difficult week
- C. Accept his uncertainty about living and shift the focus to practical matters like his job search and his finances for now
- D. Explain the statistics on how most people who feel suicidal go on to recover so that he sees that his odds are quite good

Question 11. How can the counselor best instill hope without dismissing Jordan's pain?

- A. Validate how unbearable things feel right now while helping him see the crisis and his options as changeable over time
- B. Promise him confidently that everything is definitely going to work out fine for him as long as he sticks with the treatment
- C. Agree with him that his situation does look genuinely hopeless right now and focus only on getting through the next few hours
- D. Quickly change the subject to lighter topics whenever his hopelessness comes up so that the session does not feel so heavy

Question 12. Jordan asks the counselor to promise that nothing he says about suicide will ever leave the room. Best response?

- A. Explain the limits of confidentiality, including that serious, foreseeable risk of harm may require disclosure to protect him
- B. Promise him that absolutely nothing he discloses about suicide will ever be shared with anyone under any circumstances at all
- C. Tell him that because he mentioned suicide you must immediately call the police right now regardless of how the risk assessment turns out
- D. Avoid answering the question about confidentiality at all so that he keeps talking openly about his suicidal thoughts with you

Question 13. After assessment, Jordan can maintain safety with a plan and supports. How should the counselor document this?

- A. Document the risk assessment, the formulation, the safety plan, means-restriction steps, and the clinical rationale for the disposition
- B. Record only that he agreed to a no-suicide contract and promised that he would not harm himself before the next appointment
- C. Write a brief note simply stating that the client is fine now and that there is no further need for any ongoing concern
- D. Keep the details of the suicide assessment out of the chart entirely to protect his privacy and avoid stigmatizing him

*Case 11 of 11***Recurrent binges without compensatory behavior***Diagnosis: Binge-Eating Disorder*

INTAKE

Renee Walters, a 38-year-old nurse, presents with several months of recurrent episodes of eating unusually large amounts of food with a sense of loss of control, at least weekly. The episodes leave her feeling disgusted, guilty, and distressed, and she eats alone out of shame.

SESSION TWO

She eats rapidly and past fullness even when not physically hungry, and she does not vomit, fast, over-exercise, or use laxatives afterward. There is no regular compensatory behavior following the binges, which distinguishes her pattern from a binge-purge cycle.

SESSION THREE

She is at a higher body weight and reports low mood and shame about her eating, and she fears being judged or told simply to diet. She is motivated to get help and to understand why the binges happen and how to interrupt them rather than to pursue another restrictive diet.

Question 1. What is most important to confirm to support a binge-eating disorder diagnosis?

- A. Recurrent binges with loss of control at least weekly for three months with marked distress and no regular compensatory behavior
- B. That she can identify the single stressful event she believes first triggered her binge-eating episodes
- C. That her episodes have become more frequent over the past week than they were the week before that
- D. That she currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Which feature best differentiates her presentation from bulimia nervosa?

- A. She does not engage in the regular compensatory behaviors—such as vomiting or fasting—that define bulimia nervosa
- B. The fact that she eats large amounts of food with a clear sense of loss of control during her binge episodes
- C. The shame and the distress that she experiences after each one of her binge-eating episodes
- D. The way her episodes tend to be worse during the most stressful weeks of her hospital work schedule

Question 3. How does her presentation differ from ordinary overeating?

- A. Her episodes involve a sense of loss of control and marked distress at a clinically significant frequency, not just occasional overindulgence
- B. The fact that she sometimes eats somewhat more than she had originally planned to during ordinary meals and on the special celebratory occasions she happens to share with family and friends
- C. The enjoyment that she sometimes takes in eating larger portions of the foods that she particularly likes
- D. The way she tends to eat a bit more during the holidays and other celebratory times of the year

Question 4. What co-occurring condition is most important to screen for given her presentation?

- A. Co-occurring depression and suicide risk, which commonly accompany binge-eating disorder and shape the plan
- B. A previously undetected primary psychotic disorder that might account for both her low mood and her shame about eating
- C. A long-standing anxiety disorder that could be the sole and complete explanation for all of her binge episodes
- D. An emerging neurocognitive decline that could explain why she keeps forgetting how much she has eaten lately

Question 5. What is the most appropriate counselor action regarding the medical aspects of her care?

- A. Coordinate a medical evaluation and ongoing monitoring as part of a team while she engages in psychotherapy
- B. Order laboratory tests and a metabolic panel yourself so that you can personally evaluate her medical status before treatment
- C. Recommend that she begin a strict weight-loss diet right away as the primary intervention for her binge-eating disorder
- D. Avoid any involvement of medical providers so that her counseling remains entirely separate from her medical care

Question 6. What is the most appropriate first-line treatment framework for Renee?

- A. Eating-disorder-focused cognitive behavioral therapy, within a team that includes medical involvement as needed
- B. A structured, restrictive weight-loss diet as the primary treatment to reduce her eating and her body weight
- C. Open-ended supportive counseling that mainly gives her space to talk about how guilty she feels after she binges
- D. A primarily insight-oriented exploration of her childhood as the central route to resolving her binge-eating behavior

Question 7. A central target in eating-disorder-focused CBT for her would be:

- A. Establishing regular eating and reducing the dietary restraint and triggers that drive the binge episodes
- B. Helping her design a stricter daily diet with tighter food rules so that she feels more in control of her eating
- C. Encouraging her to skip meals during the day so that she is able to save up her calories for later in the evening
- D. Focusing mainly on relaxation skills so she feels calmer without addressing her eating patterns or beliefs

Question 8. Renee asks whether medication could help. The most appropriate counselor action is to:

- A. Provide general information and coordinate a referral to a prescriber for evaluation with her consent, within scope
- B. Recommend the specific medication and dose that tends to work best for people with binge-eating disorder like hers
- C. Recommend an over-the-counter appetite suppressant so that she can reduce how much she eats during her binge episodes
- D. Discourage any medication and tell her she should overcome the binge eating through willpower and discipline alone

Question 9. Renee is deeply ashamed and eats alone to hide her binges. The most therapeutic response is to:

- A. Respond with a nonjudgmental, validating stance that reduces shame and supports her engaging openly in treatment
- B. Tell her plainly that she simply needs more self-control and that the solution is to eat less and exercise more
- C. Reassure her quickly that the binge eating is not really a big deal and that she should simply stop doing it on her own
- D. Provide detailed statistics on how common binge eating is to logically prove to her that she should not feel ashamed

Question 10. Renee fears the counselor will just put her on a diet. The most therapeutic response is to:

- A. Clarify that the focus is on interrupting the binge cycle and the distress, not on prescribing another restrictive diet
- B. Reassure her that the very first step will be to put her on a strict calorie-controlled diet to get her eating under control
- C. Tell her that her weight is the real problem and that everything in treatment should be aimed at reducing it as fast as possible
- D. Avoid answering her concern and steer the conversation toward reviewing her eating log for the week instead

Question 11. Renee, distressed, mentions she sometimes feels life is not worth living because of her eating and her weight. Best step?

- A. Conduct a structured suicide-risk assessment and respond to the risk, recognizing eating disorders can carry elevated risk
- B. Reassure her that things will improve with treatment and that there is no real need to discuss those thoughts further
- C. Continue with the planned CBT agenda, assuming the comment was just a passing expression of her frustration today
- D. Ask her to sign a brief promise to stay safe and to reach out to you if those thoughts get any worse this week

Question 12. How should the counselor coordinate with Renee's primary care provider about her care?

- A. Obtain her informed consent and release before communicating with the provider, then collaborate on a shared plan
- B. Contact the provider right away to align the plan, since coordinating her care is clearly in her own best interest anyway
- C. Avoid any contact with the provider so that her counseling remains entirely separate from her medical care
- D. Share her full counseling record with the provider so that they have complete information about all of her treatment

Question 13. Renee asks the counselor to keep her eating disorder out of any records her insurer might see. Best response?

- A. Explain that accurate clinical documentation is required and discuss how records are protected and who may access them
- B. Agree to leave the diagnosis out of the record entirely so that she feels more comfortable continuing in treatment with you
- C. Tell her she has no say at all in what goes into her record and move on without addressing her privacy concern further
- D. Offer to keep a private, separate set of unofficial notes about her eating disorder outside of her formal clinical record

Answer Key — Exam One

Case 1: Low mood and lost interest after a demotion

Diagnosis: Major Depressive Disorder, Moderate

Q1. A. That symptoms have persisted nearly every day for at least two weeks and impair his daily functioning

Duration and impairment anchor the episode

Common error: Diagnosing from a single low mood report

B. Cause does not establish the diagnosis — Treating a trigger as the diagnostic test

C. Family history supports but does not confirm — Requiring family history to diagnose

D. Worsening is not required for the diagnosis — Demanding progression to diagnose

Q2. A. The presence of pervasive anhedonia with neurovegetative signs that meet the full symptom count and threshold

Full criteria exceed an adjustment reaction

Common error: Calling any stress reaction adjustment

B. A trigger does not rule MDD out — Equating any precipitant with adjustment

C. Proportionality is an adjustment feature — Using proportionality to confirm MDD

D. Expected course does not define the episode — Diagnosing from anticipated course

Q3. A. Any past episode of elevated or irritable mood with decreased need for sleep, marking a hypomanic or manic history

Past hypomania reframes the disorder

Common error: Skipping the bipolar screen

B. Panic is a different rule-out — Confusing comorbidity with differential

C. ADHD is a low-priority rule-out here — Chasing ADHD before mood history

D. Psychosis screen matters but is secondary — Over-reading ordinary guilt as psychosis

Q4. A. Use a structured suicide-risk screen to clarify ideation severity, any intent, plan, and prior behavior

Structured screening clarifies risk

Common error: Asking vaguely or avoiding the topic

B. Premature reassurance closes assessment — Reassuring before assessing

C. No-suicide contracts are not evidence-based — Substituting a contract for assessment

D. Deferring risk assessment is unsafe — Postponing a safety assessment

Q5. A. A review of his medications, substance use, and any thyroid or other medical conditions that mimic depression

Rule out medical and substance causes

Common error: Skipping the organic rule-out

B. Useful later, not the priority now — Prioritizing depth history over safety rule-outs

C. Exhaustive work history is low yield — Collecting low-yield detail

D. Coping review is secondary here — Confusing planning data with rule-outs

Q6. A. Offer evidence-based psychotherapy such as cognitive behavioral therapy or behavioral activation for depression

First-line psychotherapy fits scope

Common error: Defaulting to medication advice

B. Prescribing is outside counselor scope — Acting outside scope of practice

C. Watchful waiting under-treats moderate MDD — Delaying indicated care

D. Lifestyle alone is insufficient first-line — Offering adjuncts as the primary plan

Q7. A. Re-administer a validated symptom measure such as the PHQ-9 at regular intervals to guide measurement-based care

Measurement-based care guides decisions

Common error: Relying on impressions alone

B. Global impressions are unreliable — Trusting unstandardized self-report

C. End-only review misses early non-response — Skipping interim monitoring

D. Engagement is necessary but not sufficient — Mistaking adherence for outcome

Q8. A. Provide general information and coordinate a referral to a prescriber for a medication evaluation with his consent

Refer and coordinate within scope

Common error: Giving specific medication advice

B. Naming a drug exceeds scope — Overstepping professional scope

C. Discouraging meds overreaches — Steering a medical decision

D. Outsourcing to self-research is poor care — Replacing referral with self-directed search

Q9. A. Collaboratively schedule small, valued activities such as briefly attending his daughter's practice to rebuild reward

Graded valued activity rebuilds reward

Common error: Waiting for motivation before activity

B. Too much too soon invites failure — Ignoring graded pacing

C. Thought logs fit CBT, not BA first step — Confusing BA with cognitive work

D. Waiting on mood reverses BA logic — Letting mood gate behavior

Q10. A. Offer psychoeducation framing depression as a common, treatable condition while validating how heavy it currently feels

Psychoeducation reframes self-blame

Common error: Either lecturing or only sympathizing

B. Premature disputation feels invalidating — Disputing before validating

C. Endorsing the self-blame deepens it — Colluding with hopeless thinking

D. Avoidance leaves the belief intact — Avoiding emotionally central material

Q11. A. Acknowledge the hopelessness as a symptom and collaboratively set one small, achievable goal to test it against

Name hopelessness and test it behaviorally

Common error: Arguing him out of hopelessness

B. Overpromising risks credibility — Making promises you cannot keep

C. Agreeing confirms the hopelessness — Adopting the client's hopeless stance

D. Lecturing rarely shifts hopelessness — Substituting facts for experience

Q12. A. The limits of confidentiality, including when imminent risk of serious harm to self or others must be acted upon

Disclose confidentiality limits up front

Common error: Omitting the limits of confidentiality

B. Absolute confidentiality is inaccurate — Guaranteeing unconditional secrecy

C. Goals are collaborative, not imposed — Removing the client from planning

D. Clients may withdraw consent — Treating consent as binding

Q13. A. Respect his autonomous decision, document it, and continue evidence-based therapy while keeping the option open

Honor autonomy and document

Common error: Coercing a medical decision

B. Conditioning care is coercive — Making therapy contingent on compliance

C. Persistent pressure undermines autonomy — Pressuring after a clear decision

D. Blame-shifting documentation is improper — Using the chart to assign blame

Case 2: Persistent Worry Across Many Areas of Life

Diagnosis: Generalized Anxiety Disorder

Q1. A. Administer the GAD-7 and clarify the breadth, duration, and controllability of his worry

A validated measure plus core criteria anchors assessment.

Common error: Skipping a standardized baseline measure.

B. Intervening before assessing is premature. — Jumping to intervention before assessment.

C. Deferring assessment abandons the counselor task. — Outsourcing assessment by default.

D. Minimizing impairing symptoms is dismissive. — Normalizing clinically significant anxiety.

Q2. A. Whether the excessive worry has been present more days than not for six months or longer

GAD requires 6+ months of worry more days than not.

Common error: Diagnosing GAD from a brief recent stretch.

B. A short window does not establish chronicity. — Confusing a recent flare with chronic course.

C. A precipitant is not the duration criterion. — Substituting a precipitant for course.

D. Family history does not establish duration. — Using family history to specify course.

Q3. A. Ask about caffeine, medications, and medical conditions, and refer for evaluation if indicated

Rule out mimics within scope, refer when indicated.

Common error: Diagnosing anxiety without screening mimics.

B. Ordering labs is outside counselor scope. — Acting outside scope by ordering tests.

C. Attributing all symptoms to caffeine is premature. — Reducing a syndrome to one habit.

D. Ignoring medical mimics is unsafe. — Treating anxiety as purely psychological by default.

Q4. A. His anxiety is persistent worry across many domains rather than recurrent unexpected panic attacks

GAD is chronic worry; panic centers on attacks.

Common error: Labeling chronic worry as panic disorder.

B. Somatic tension appears in both conditions. — Using tension to separate the two.

C. Worry content does not differentiate the disorders. — Treating worry topics as diagnostic.

D. Stress reactivity is common to many conditions. — Using stress sensitivity as a differentiator.

Q5. A. His worry is spread diffusely across many areas rather than fixed on having a serious disease

Illness anxiety fixes on disease; GAD is diffuse.

Common error: Calling broad worry illness anxiety.

B. Some health worry does not equal illness anxiety. — Equating any health worry with illness anxiety.

C. Fatigue is nonspecific to both. — Using fatigue as a differentiator.

D. Distraction effects are nonspecific. — Reading meaning into distraction relief.

Q6. A. Cognitive behavioral therapy with cognitive restructuring and worry exposure as core components

CBT is first-line for GAD.

Common error: Choosing a model without GAD evidence.

B. Support alone is less effective than CBT. — Relying on support alone for GAD.

C. Not first-line for acute GAD relief. — Prioritizing insight over evidence-based care.

D. Medication is an option, not the only path. — Framing medication as the sole option.

Q7. A. Building his tolerance of uncertainty instead of chasing certainty through checking and reassurance

Intolerance of uncertainty maintains GAD.

Common error: Trying to answer every worry instead of building tolerance.

B. Chasing certainty reinforces the worry cycle. — Solving each worry to provide certainty.

C. Avoidance maintains anxiety. — Recommending avoidance as coping.

D. Relaxation alone leaves the core untreated. — Treating arousal while ignoring worry.

Q8. A. Use worry exposure while gradually reducing reassurance-seeking and the avoidance that maintains it

Exposure plus dropping safety behaviors treats GAD.

Common error: Leaving safety behaviors in place.

B. Giving reassurance reinforces the behavior. — Reassuring to comfort in the moment.

C. Permitting avoidance prevents new learning. — Waiting for readiness before facing tasks.

D. Thought suppression is ineffective. — Prescribing positive thinking as a fix.

Q9. A. Re-administer the GAD-7 at intervals to track response and step up care if he does not improve

Serial measures guide stepped care.

Common error: Relying on impression instead of data.

B. Global impression is less reliable. — Substituting impression for measurement.

C. End-only measurement cannot guide care. — Deferring measurement to termination.

D. A single habit is not a symptom measure. — Tracking a habit instead of symptoms.

Q10. A. Name the reassurance pattern with warmth and help him practice sitting with the uncertainty

Address the pattern rather than feed it.

Common error: Answering the worry to soothe it.

B. Answering reinforces reassurance-seeking. — Comforting by answering every worry.

C. Dismissiveness harms the alliance. — Invalidating instead of guiding.

D. Avoiding the moment misses a chance to intervene. — Avoiding the worry rather than using it.

Q11. A. Explore what he has already tried, affirm his strengths, and evoke his own reasons to keep going

Evoking strengths and change talk builds engagement.

Common error: Lecturing instead of evoking motivation.

B. Overpromising can backfire. — Guaranteeing results to motivate.

C. Fear appeals tend to raise resistance. — Using fear to drive change.

D. Generic reassurance does little to engage. — Offering generic comfort.

Q12. A. Decline to confirm or deny anything and explain that disclosure requires his written consent

Disclosure to an employer needs written consent.

Common error: Confirming treatment to an employer without consent.

B. Confirming attendance is still a disclosure. — Sharing attendance as if it were not protected.

C. Sharing progress without consent breaches confidentiality. — Disclosing to be helpful.

D. The contact should be documented. — Failing to document a third-party call.

Q13. A. Seek supervision or consultation, and refer for the components that exceed his demonstrated competence

Practice within competence; consult or refer.

Common error: Treating beyond demonstrated competence.

B. Treating beyond competence is unethical. — Prioritizing continuity over competence.

C. Minimizing the concern fails the client. — Downplaying issues outside your skill set.

D. Abrupt transfer mishandles continuity of care. — Dropping the client without coordination.

Case 3: Unrelenting yearning more than a year after a loss

Diagnosis: Prolonged Grief Disorder

Q1. A. That the death occurred at least twelve months ago and that intense yearning and preoccupation persist nearly every day

Duration plus persistent yearning anchor it

Common error: Diagnosing PGD before the time threshold

B. Event detail is not the criterion — Confusing the death narrative with the criteria

C. That threshold describes MDD, not PGD — Applying MDD criteria to grief

D. That pattern points toward PTSD — Reading grief as a trauma disorder

Q2. A. Her distress centers on yearning and preoccupation with her husband rather than pervasive low mood across all areas of life

Yearning focus separates PGD from MDD

Common error: Collapsing grief into depression

B. Sadness occurs in both — Using sadness to differentiate

C. Neurovegetative signs occur in both — Reading somatic signs as decisive

D. Reminder-linked surges are nonspecific — Using reminder reactivity to differentiate

Q3. A. Its symptoms are persistent, pervasive, and impairing well beyond the timeframe and intensity expected within her cultural context

Persistence and impairment beyond norms

Common error: Pathologizing normal grief

B. Ongoing missing is normal grief — Treating normal longing as the disorder

C. Keeping mementos is normal — Reading ordinary keepsakes as pathology

D. Wave-like grief is normal — Misreading normal waves as disorder

Q4. A. Conduct a structured suicide-risk assessment to clarify ideation, intent, and plan, recognizing grief elevates risk

Assess elevated risk with structure

Common error: Overlooking risk within grief

B. Normalizing away ideation is unsafe — Dismissing ideation as longing

C. Risk must be assessed before this — Sequencing treatment ahead of safety

D. No-suicide contracts are not evidence-based — Substituting a contract for assessment

Q5. A. Her core experience is yearning and preoccupation with the deceased rather than re-experiencing a threatening traumatic event

Yearning versus threat re-experiencing

Common error: Forcing grief into a trauma frame

B. Avoidance appears in both — Using avoidance alone to differentiate

C. Distress on reminders occurs in both — Reading distress as decisive

D. Concentration problems occur in both — Using concentration to differentiate

Q6. A. A grief-focused psychotherapy that helps her process the loss and gradually approach the reminders she has been avoiding

Grief-focused processing is indicated

Common error: Offering generic support without grief focus

B. Unstructured support underperforms grief work — Defaulting to nondirective support

C. Sanctioning avoidance maintains PGD — Reinforcing avoidance as coping

D. Insight-only is not first-line here — Prioritizing origins over grief work

Q7. A. Gradually and collaboratively approaching the avoided reminders so the associated pain can be tolerated and processed

Graded approach to avoided reminders

Common error: Leaving the avoidance untouched

B. Forcing disposal can deepen distress — Pushing premature, forced action

C. Endorsing avoidance maintains the grief — Reinforcing the maintaining behavior

D. Waiting for no pain reverses the logic — Letting distress gate the work

Q8. A. Screen for a co-occurring depressive disorder and coordinate a prescriber referral if criteria are met, with her consent

Assess comorbidity and refer if indicated

Common error: Ignoring a treatable comorbidity

B. Prescribing is outside counselor scope — Acting outside scope of practice

C. Dismissing possible MDD misses it — Attributing everything to grief

D. Conflating the two is inaccurate — Erasing a clinically meaningful distinction

Q9. A. Validate her pain and gently explore the guilt, examining what she could realistically have known at the time

Validate, then examine the guilt

Common error: Leaving the guilt cognition unexamined

B. Endorsing the self-blame deepens guilt — Colluding with the self-blame

C. Premature reassurance can invalidate — Reassuring without processing

D. Avoiding the guilt leaves it intact — Sidestepping emotionally central material

Q10. A. Clarify that the goal is to carry a continuing bond with him while re-engaging with life, not to forget or let go of him

Reframe the goal as a continuing bond

Common error: Implying she must sever the bond

B. Severing the bond is not the goal — Framing grief work as forgetting

C. Overpromising about memory is unrealistic — Making promises you cannot keep

D. Bypassing the fear misses a key target — Avoiding emotionally salient material

Q11. A. Approach her practices with cultural humility, learn their meaning to her, and incorporate them respectfully into the work

Practice cultural humility

Common error: Imposing a single timeline or norm

B. Dismissing practices is culturally insensitive — Treating cultural norms as obstacles

C. Norms vary across cultures — Ignoring cultural context in assessment

D. Reflexive referral is not required — Equating unfamiliarity with incompetence

Q12. A. Warmly explore the meaning of the request and maintain the professional boundary, explaining its protective purpose

Hold the boundary, explore the meaning

Common error: Either attending or coldly refusing

B. Attending creates a boundary crossing — Trading the clinical role for a social one

C. A cold refusal can feel like rejection — Enforcing limits without warmth

D. A brief visit is still a boundary crossing — Rationalizing a limited crossing

Q13. A. Reassess progress and seek consultation or refer to a clinician trained in a specialized grief protocol if indicated

Reassess and match competence to need

Common error: Continuing a stalled plan without review

B. Ignoring non-response is an error — Failing to revisit a non-responding plan

C. Using normal-grief framing to avoid action — Excusing non-response as normal

D. Abrupt abandonment is unethical — Abandoning rather than coordinating

Case 4: Escalating evening drinking after the kids leave home

Diagnosis: Alcohol Use Disorder, Moderate

Q1. A. Systematically review the past 12 months for Criterion A symptoms: amount, tolerance, cut-down attempts, and time spent

Structured criterion review anchors the diagnosis

Common error: Diagnosing from a general impression instead of counting symptoms

B. Legal history is not present and not central — Assuming legal trouble is required for AUD

C. Family history informs risk, not the diagnosis itself — Requiring family history to diagnose AUD

D. Bypassing the client's own report undermines assessment — Substituting collateral report for direct interview

Q2. A. Screen for withdrawal risk and medical complications, referring for a medical evaluation if any signs emerge

Routine withdrawal and medical screening is standard practice

Common error: Skipping the routine screen because none are expected

B. Managing withdrawal is outside counselor scope — Treating a medical concern as a counseling task

C. Screening should still be documented routinely — Omitting a routine screen based on assumption

D. Inpatient detox is not indicated without withdrawal risk — Over-placing a client into a higher level of care

Q3. A. His current readiness and motivation to change his drinking, including how he views the pros and cons

Readiness data guides stage-matched intervention

Common error: Assuming every client is ready for action-stage goals

B. Purchase detail is low clinical yield — Collecting detail unrelated to treatment planning

C. A decade-long schedule review is excessive — Over-collecting historical detail

D. Demanding abstinence upfront ignores his readiness — Insisting on abstinence before assessing readiness

Q4. A. Five endorsed symptoms: drinking more than intended, time spent, two failed cut-down attempts, tolerance, and craving

Five symptoms fall in the moderate range

Common error: Estimating severity without counting symptoms

B. A precipitant does not establish severity — Using a life event to estimate severity

C. Subjective comparison is not a diagnostic count — Substituting impression for symptom count

D. Frequency alone does not set the specifier — Equating frequency change with severity level

Q5. A. He meets full Criterion A symptom criteria for a substance use disorder rather than only a stress reaction

Meeting AUD criteria rules out adjustment disorder

Common error: Defaulting to adjustment disorder whenever a stressor is present

B. A stressor can precede either diagnosis — Assuming any stressor points to adjustment disorder

C. Proportionality points toward adjustment, not away from it — Using proportionality to support AUD

D. Anticipated course does not define the current diagnosis — Diagnosing from anticipated outcome

Q6. A. Use a motivational, stage-matched approach that meets him at his current level of readiness to change

Stage-matched motivational work fits ambivalence

Common error: Prescribing an action-stage plan to an ambivalent client

B. Mandating abstinence conflicts with his readiness level — Making treatment contingent on premature commitment

C. Repeating a failed strategy is not indicated — Recycling an approach that already failed

D. Passive waiting under-serves a moderate use disorder — Withholding active intervention for a diagnosable disorder

Q7. A. Outpatient counseling, since he has no withdrawal risk, medical complications, or safety concerns present

Outpatient care matches his low-risk profile

Common error: Defaulting to a higher level of care without indication

B. Inpatient withdrawal care is not indicated here — Over-placing a client without withdrawal risk

C. Residential care exceeds what his profile requires — Choosing a more restrictive setting than needed

D. An unplanned, informal approach under-serves a diagnosable disorder — Leaving a diagnosed disorder without a structured plan

Q8. A. Collaboratively explore his goals and readiness together, rather than the counselor selecting the goal alone

Collaborative goal-setting matches his current stage

Common error: Selecting the goal for the client unilaterally

B. A unilateral abstinence mandate disregards readiness — Dictating the goal instead of collaborating

C. Indefinite postponement stalls treatment planning — Avoiding goal-setting altogether

D. Borrowing another client's goal ignores individual readiness — Copying a plan instead of individualizing it

Q9. A. Have him track daily drinks consumed and urges using a simple log reviewed together at each session

A structured drinking log gives objective tracking

Common error: Relying only on vague verbal check-ins

B. A single end-point check misses interim data — Skipping interim progress checks

C. Attendance alone does not measure the target symptom — Mistaking attendance for symptom improvement

D. Bypassing the client's own report is inappropriate — Replacing client self-monitoring with a third party

Q10. A. Reflect his ambivalence and gently explore the gap between his goals and his current drinking pattern

Reflective listening develops discrepancy

Common error: Arguing that he does have a problem

B. Direct confrontation increases resistance — Arguing the client into agreement

C. Agreeing forecloses further exploration — Colluding with the client's minimization

D. Avoiding the topic leaves ambivalence unaddressed — Redirecting away from ambivalence

Q11. A. Roll with the resistance, affirm his effort in trying, and invite him to explore what got in the way

Rolling with resistance keeps him engaged

Common error: Pushing harder against the defensiveness

B. Framing failure this way increases defensiveness and shame — Using past attempts as leverage or blame

C. Avoiding the topic forfeits useful clinical material — Abandoning a clinically useful topic after one defensive moment

D. Dismissing the topic devalues his disclosure — Brushing past a meaningful client disclosure

Q12. A. The limits of confidentiality and the scope of counseling, including referral for any needed medical evaluation

Consent must cover confidentiality limits and scope

Common error: Omitting scope-of-practice limits from consent

B. Absolute confidentiality is an inaccurate promise — Guaranteeing unconditional privacy

C. Goal-setting is collaborative, not counselor-imposed — Removing the client from goal-setting

D. Consent is ongoing and may be withdrawn — Treating consent as a binding, irrevocable contract

Q13. A. Explain that a brief medical evaluation is a reasonable step and offer to coordinate a referral with his consent

Referral and coordination stay within scope

Common error: Assessing or ruling out medical issues personally

B. A definitive medical ruling exceeds counselor scope — Making a medical determination as a counselor

C. Outsourcing to self-research is poor coordination of care — Replacing professional coordination with self-directed searching

D. Deflecting a direct question undermines informed care — Avoiding a straightforward clinical question

Case 5: Intense relationships and recurrent self-harm in a graduate student

Diagnosis: Borderline Personality Disorder

Q1. A. A pervasive, long-standing pattern of at least five of the nine criteria across many situations and relationships

Pervasive, enduring pattern of 5/9

Common error: Diagnosing from a single trait or episode

B. A specific trauma is not required — Requiring a discrete cause to diagnose

C. Sustained episodes point to bipolar — Confusing reactive shifts with mood episodes

D. Frank psychosis is not the criterion — Over-reading stress reactions as psychosis

Q2. A. That her mood shifts within hours in reaction to interpersonal events rather than lasting as sustained discrete episodes

Reactive, brief shifts versus episodes

Common error: Conflating reactivity with mood episodes

B. Irritability occurs in both — Using a shared symptom to differentiate

C. Sustained elevation points to bipolar — Reading hypomania into reactive shifts

D. Diurnal pattern is not diagnostic — Over-reading a circadian pattern

Q3. A. Complete a structured suicide-risk assessment that distinguishes her self-harm urges from suicidal intent and plan

Structured assessment of chronic risk

Common error: Treating chronic risk as unassessable

B. Chronic risk still needs assessment — Becoming desensitized to chronic risk

C. No-harm contracts are not evidence-based — Substituting a contract for assessment

D. Wound-only focus misses risk data — Assessing injury without context

Q4. A. Her intent and the function of the behavior, since cutting to relieve tension differs from acting with intent to die

Intent and function separate the two

Common error: Assuming all self-harm is a suicide attempt

B. Lethality alone does not reveal intent — Inferring intent from wound severity

C. Frequency does not clarify intent — Equating frequency with risk type

D. Others' awareness is not the test — Using disclosure as the criterion

Q5. A. Co-occurring posttraumatic stress and depressive symptoms, which commonly accompany borderline personality disorder

Screen common BPD comorbidities

Common error: Assessing the personality pattern in isolation

B. Transient paranoia is not psychosis — Over-reading transient symptoms as psychosis

C. Neurodevelopmental framing is low yield — Chasing an improbable comorbidity

D. Eating pathology is not indicated — Inventing an unsupported comorbidity

Q6. A. Referral to a structured, evidence-based psychotherapy for borderline personality disorder such as dialectical behavior therapy

Structured EBP such as DBT is indicated

Common error: Offering unstructured supportive care only

B. Unstructured support underperforms — Defaulting to unstructured care

C. Brief therapy under-treats the pattern — Under-dosing a chronic condition

D. Insight-only is not first-line — Prioritizing insight over skills

Q7. A. A collaborative safety plan identifying warning signs, coping strategies, and supports she can use when urges escalate

Collaborative safety planning

Common error: Relying on a no-suicide contract

B. No-suicide contracts are not evidence-based — Using a contract instead of a plan

C. Reflexive hospitalization is not the standard — Defaulting to hospitalization for every urge

D. Cutting off crisis contact is unsafe — Removing crisis resources

Q8. A. Establish clear, consistent between-session contact guidelines and coach her in using skills during crises within that frame

Consistent frame plus skills coaching

Common error: Either unlimited access or none

B. Unlimited access fosters dependence — Overextending availability

C. No access ignores her risk — Leaving a high-risk client without supports

D. Offloading all contact is premature — Abandoning the therapeutic relationship

Q9. A. Maintain a steady, nonreactive stance and a consistent frame while validating the feelings underneath the shifts

Consistency plus validation

Common error: Reacting personally to the shifts

B. Labeling can feel invalidating — Interpreting before validating

C. Reinforcing swings worsens instability — Letting the client's shifts drive your stance

D. Overcorrecting destabilizes the frame — Abandoning the frame under pressure

Q10. A. Validate the abandonment fear, prepare for the break in advance, and arrange clear coverage and supports during the absence

Validate and plan for the break

Common error: Dismissing or avoiding the reaction

B. Dismissing the fear invalidates her — Telling her how she should feel

C. Accommodating undermines the frame — Abandoning appropriate limits

D. Withholding prevents needed preparation — Springing separations without planning

Q11. A. Warmly maintain the professional boundary, explain its protective purpose, and explore the longing for connection in therapy

Hold the boundary, explore the need

Common error: Either crossing the line or rejecting coldly

B. A dual relationship harms the work — Trading the clinical role for friendship

C. A cold refusal can feel like rejection — Enforcing limits without warmth

D. Future-friendship promises are improper — Dangling a personal relationship

Q12. A. Seek clinical supervision or consultation to manage the countertransference and protect the quality of her care

Use consultation for countertransference

Common error: Letting reactions silently shape care

B. Reactive limits serve the clinician, not the client — Letting reactions drive boundary changes

C. Burdening the client is inappropriate — Shifting your reaction onto the client

D. Premature transfer abandons the client — Transferring instead of addressing the reaction

Q13. A. Explain the limits of confidentiality, assess imminent risk fully, and act to protect her safety as the situation requires

Confidentiality yields to imminent risk

Common error: Promising secrecy when risk is imminent

B. Secrecy at imminent risk is unsafe — Prioritizing secrecy over safety

C. Dismissing imminent risk is dangerous — Assuming chronic means non-acute

D. Delaying response to imminent risk is unsafe — Postponing an urgent safety response

Case 6: A week of soaring energy, big plans, and little sleep

Diagnosis: Bipolar I Disorder, Current Episode Manic

Q1. A. A distinct week or more of elevated or irritable mood with increased energy plus several associated manic symptoms

Duration, mood, energy, and symptoms anchor mania

Common error: Diagnosing mania from a single day of high energy

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent intensity is not the criterion — Confusing a brief peak with the episode

D. Depressive count does not define mania — Applying depressive criteria to mania

Q2. A. The current manic episode itself, since a single lifetime manic episode establishes a Bipolar I diagnosis

One manic episode defines Bipolar I

Common error: Calling it depression because of past low periods

B. Past depression alone does not decide it — Letting the depressive history drive the diagnosis

C. Irritability occurs across disorders — Using irritability to differentiate

D. Poor focus is nonspecific — Reading inattention as decisive

Q3. A. A substance-induced manic picture, which his negative drug screen and denial of use make less likely but worth confirming

Exclude a substance-induced cause

Common error: Skipping the substance rule-out

B. Anxiety does not explain the syndrome — Reducing mania to anxiety

C. Trait confidence is not the rule-out — Attributing an episode to personality

D. Cognitive decline is improbable here — Pursuing an improbable rule-out

Q4. A. As mood-congruent grandiosity within the manic episode rather than the fixed delusions seen in a psychotic disorder

Mood-congruent grandiosity fits mania

Common error: Reading manic grandiosity as schizophrenia

B. Grandiosity alone is not psychosis — Equating grandiosity with a psychotic disorder

C. Minimizing impairing grandiosity is unsafe — Dismissing a clinically significant symptom

D. It is a manic symptom, not a defense — Reframing a symptom as a coping style

Q5. A. Assess danger to self and others, including suicide risk and the impulsive, high-risk behaviors he is currently engaging in

Assess risk across mood states

Common error: Overlooking risk during an elevated mood

B. Narrow suicide-only focus misses risk — Assessing only one channel of risk

C. Deferring risk assessment is unsafe — Postponing an urgent safety assessment

D. Manic self-report alone is unreliable here — Accepting reassurance without assessment

Q6. A. Coordinate an urgent psychiatric evaluation for pharmacologic management of the acute mania and to address safety

Urgent psychiatric/medication evaluation

Common error: Offering outpatient psychotherapy as the primary acute treatment

B. Insight work is not the acute priority — Prioritizing insight during acute mania

C. Prescribing is outside counselor scope — Acting outside scope of practice

D. Watchful waiting is unsafe in acute mania — Delaying urgent care

Q7. A. Interpersonal and social rhythm therapy to regularize his sleep-wake and daily routines and detect early warning signs

IPSRT stabilizes rhythms in maintenance

Common error: Ignoring sleep and routine in maintenance

B. Catharsis is not the maintenance target — Choosing an approach without bipolar evidence

C. Overactivity can trigger relapse — Promoting overactivity in bipolar care

D. Exposure is not the indicated approach — Applying an unmatched protocol

Q8. A. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate his questions and progress with them

Defer dosing; coordinate with consent

Common error: Advising on a specific medication dose

B. Naming a dose exceeds scope — Overstepping into medication management

C. Directing dosing is unsafe and out of scope — Giving specific pharmacologic direction

D. Refusing to coordinate abandons the need — Declining to support communication with the prescriber

Q9. A. Engage him respectfully, find shared goals such as protecting his business, and link safety steps to what he values

Engage via shared goals despite low insight

Common error: Arguing with him to force insight

B. Confrontation raises resistance — Using confrontation to force acceptance

C. Colluding ignores real risk — Validating the denial to keep rapport

D. Avoiding the issue forfeits engagement — Sidestepping the central concern

Q10. A. With his consent, provide psychoeducation about warning signs and engage the family in supporting the safety plan

Consented family psychoeducation and support

Common error: Excluding family or sharing without consent

B. Disclosing without consent breaches confidentiality — Sharing clinical details without consent

C. Excluding family forgoes useful support — Refusing collateral that could help

D. Acting without his input oversteps — Arranging control behind the client's back

Q11. A. When there is a serious, foreseeable risk of harm to him or others, disclosure may be required to protect safety

Danger threshold can require disclosure

Common error: Treating confidentiality as absolute when risk is serious

B. Family requests do not override confidentiality — Disclosing on a relative's request alone

C. Absolute confidentiality is inaccurate — Denying any limit to confidentiality

D. A vague best-interest test is too loose — Using personal judgment instead of the standard

Q12. A. Assess his capacity to make decisions, involve him as fully as possible, and document consent and any capacity concerns

Assess capacity; maximize his involvement

Common error: Either ignoring capacity or dismissing his voice entirely

B. Presuming total incapacity overreaches — Stripping the client of all input

C. Ignoring the manic state is unsafe — Overlooking capacity concerns

D. Delaying consent stalls urgent care — Postponing consent when care is urgent

Q13. A. Obtain his specific, informed consent for each party before sharing any of his protected health information

Get a valid release for each party

Common error: Coordinating before obtaining consent

B. Verbal interest is not a valid release — Disclosing before consent is documented

C. Coordination still requires consent — Assuming shared care waives consent

D. Each disclosure is his informed choice — Making release decisions for the client

Case 7: Voices and growing suspicion in a college student

Diagnosis: Schizophrenia

Q1. A. Two or more active-phase symptoms with continuous signs of disturbance and functional decline lasting at least six months

Symptom count, duration, and decline anchor it

Common error: Diagnosing schizophrenia from brief symptoms

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent intensity is not the criterion — Confusing a brief change with the criteria

D. Depressive count does not define it — Applying depressive criteria here

Q2. A. Her psychotic symptoms are present without a sustained mood episode driving them across most of the illness course

Psychosis without sustained mood episodes

Common error: Calling it schizoaffective without mood episodes

B. Hallucinations occur in both — Using hallucinations to differentiate

C. Delusions occur in both — Reading the delusion as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. Her symptoms have persisted nearly continuously for months rather than resolving within the one-month brief-disorder window

Duration beyond one month rules out brief

Common error: Diagnosing brief psychotic disorder outside its window

B. Both can involve these symptoms — Using symptom type to differentiate

C. Distress occurs in both — Reading distress as decisive

D. Functional decline occurs in both — Using decline alone to differentiate

Q4. A. A substance- or medication-induced psychosis, which her negative toxicology screen makes less likely but worth confirming

Exclude a substance-induced cause

Common error: Skipping the substance and medical rule-out

B. Anxiety does not explain the psychosis — Reducing psychosis to anxiety

C. Trait withdrawal is not the rule-out — Attributing psychosis to personality

D. Eating pathology is not indicated — Inventing an unsupported rule-out

Q5. A. Conduct a structured suicide-risk assessment, recognizing that suicide risk is elevated in early psychosis

Assess elevated risk in early psychosis

Common error: Overlooking risk amid psychotic symptoms

B. Minimizing the ideation is unsafe — Dismissing the ideation

C. Risk must be assessed before this — Sequencing referral ahead of safety

D. No-suicide contracts are not evidence-based — Substituting a contract for assessment

Q6. A. Coordinate an urgent psychiatric evaluation for antipsychotic medication alongside psychosocial treatment and support

Antipsychotic eval plus psychosocial care

Common error: Offering counseling alone for schizophrenia

B. Insight-only is not first-line — Prioritizing insight over indicated care

C. Prescribing is outside counselor scope — Acting outside scope of practice

D. Counseling alone is insufficient — Omitting medication coordination

Q7. A. Cognitive behavioral therapy for psychosis and family psychoeducation to support coping and reduce relapse risk

CBT for psychosis plus family work

Common error: Relying on medication with no psychosocial care

B. Confronting delusions can rupture alliance — Arguing the client out of delusions

C. Exposure is not the indicated approach — Applying an unmatched protocol

D. Depth-only work is not first-line — Prioritizing origins over indicated care

Q8. A. Case management and supported education or employment services to help her re-engage with her studies and daily roles

Coordinate functional supports

Common error: Ignoring functional and role recovery

B. Reflexive withdrawal can worsen function — Defaulting to complete disengagement

C. Tutoring alone misses the cause — Treating the grades without the illness

D. Overload can overwhelm her — Demanding too much too soon

Q9. A. Build a steady, trustworthy alliance with consistency and transparency, going at a pace she can tolerate

Prioritize a trustworthy, paced alliance

Common error: Pushing for disclosure faster than she can tolerate

B. Pressing too fast can deepen mistrust — Moving faster than the alliance allows

C. Flat reassurance rarely shifts delusions — Arguing against the belief directly

D. Colluding with delusions is harmful — Validating a delusion to keep rapport

Q10. A. Acknowledge how distressing and real they feel to her without confirming or arguing about their objective accuracy

Validate the distress, stay neutral on content

Common error: Either confirming or flatly disputing the voices

B. Bluntly disputing can rupture trust — Confronting the symptom dismissively

C. Treating commands as instructions is unsafe — Colluding with hallucination content

D. Endorsing the explanation reinforces the belief — Validating a delusional explanation

Q11. A. Assess the risk carefully and follow duty-to-protect obligations, which may require warning or protective steps

Assess and act on duty to protect

Common error: Treating an identifiable threat as fully confidential

B. Confidentiality yields to a serious threat — Treating confidentiality as absolute when there is danger

C. Delaying assessment of a threat is unsafe — Postponing an urgent risk assessment

D. Act on assessed risk, not reflexively — Disclosing before assessing the risk

Q12. A. Decline to disclose without her consent and, with her permission, explore appropriate, helpful family involvement

Protect confidentiality; pursue consented involvement

Common error: Disclosing to parents without consent

B. Disclosing without consent breaches confidentiality — Sharing clinical details without authorization

C. Total refusal forgoes consented collaboration — Refusing collaboration that could help

D. A general update is still a disclosure — Leaking information under the guise of an update

Q13. A. Seek consultation and coordinate with a treatment team, referring for the components beyond his demonstrated competence

Consult, coordinate, and refer within competence

Common error: Managing complex psychosis alone without competence

B. Going it alone exceeds competence — Prioritizing continuity over competence

C. Minimizing the illness fails the client — Underestimating the need for specialist care

D. Abrupt transfer mishandles continuity — Dropping the client without coordination

Case 8: Lifelong distractibility and restlessness derailing work

Diagnosis: Attention-Deficit/Hyperactivity Disorder, Combined

Q1. A. That several symptoms were present before age 12 and that impairment occurs across more than one setting

Childhood onset and cross-setting impairment

Common error: Diagnosing adult ADHD without childhood onset

B. Adult-onset-only argues against ADHD — Accepting a purely adult onset

C. A precipitant is not the criterion — Using a trigger as the diagnostic test

D. Current severity is not the criterion — Requiring progression to diagnose

Q2. A. His attention problems are a lifelong, pervasive pattern rather than concentration difficulty arising from chronic worry

Lifelong trait pattern versus worry-driven

Common error: Mistaking anxiety-driven distraction for ADHD

B. Restlessness occurs in both — Using restlessness to differentiate

C. Sleep and fatigue occur in both — Reading sleep issues as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. His symptoms are a chronic, continuous pattern rather than occurring in discrete episodes of elevated mood and energy

Chronic pattern versus discrete episodes

Common error: Confusing chronic ADHD with episodic hypomania

B. Impulsivity occurs in both — Using impulsivity to differentiate

C. Disorganization can occur in both — Reading disorganization as decisive

D. Effortful overwork is not hypomania — Misreading hard work as hypomania

Q4. A. Validated ADHD rating scales together with developmental history and collateral information from those who know him

Rating scales plus collateral history

Common error: Relying on a single unstructured interview

B. An open journal is not structured assessment — Using narrative in place of measurement

C. One observation is insufficient — Diagnosing from one in-session sample

D. Ordering such tests exceeds scope — Acting outside scope of practice

Q5. A. Medical and substance contributors and other conditions, such as a sleep disorder or depression, that can impair attention

Rule out mimics of inattention

Common error: Diagnosing ADHD without screening mimics

B. Psychosis is not indicated — Pursuing an unsupported rule-out

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Lifelong symptoms argue against decline — Pursuing an improbable rule-out

Q6. A. Provide skills-based behavioral and organizational intervention and coordinate a referral for a medication evaluation

Skills plus coordinated med referral

Common error: Treating ADHD with insight-only therapy

B. Prescribing is outside counselor scope — Acting outside scope of practice

C. Insight-only is not first-line — Prioritizing insight over skills

D. Willpower framing is inaccurate and harmful — Moralizing about a neurodevelopmental condition

Q7. A. Externalizing organization through planners, reminders, task breakdown, and environmental structure to support follow-through

Externalize organization and structure

Common error: Relying on internal willpower alone

B. Relying on memory worsens follow-through — Removing external supports

C. Relaxation alone misses the core — Treating restlessness while ignoring planning

D. Overloading worsens follow-through — Increasing demands instead of structuring them

Q8. A. Explain that medication selection and dosing are the prescriber's role and offer, with consent, to coordinate his questions

Defer prescribing; coordinate with consent

Common error: Recommending a specific medication or dose

B. Naming a drug and dose exceeds scope — Overstepping into medication management

C. Directing dosing is unsafe and out of scope — Giving specific pharmacologic direction

D. Refusing to coordinate abandons the need — Declining to support communication with the prescriber

Q9. A. Reframe the difficulties as a treatable neurodevelopmental condition and highlight his real efforts to compensate

Reframe shame; affirm effort

Common error: Either reinforcing or ignoring the self-blame

B. Endorsing the self-blame deepens shame — Colluding with the “lazy” self-narrative

C. Bypassing the shame misses a target — Avoiding emotionally salient material

D. Facts alone rarely shift shame — Answering emotion with numbers

Q10. A. Collaboratively set small, specific, achievable between-session steps and review them with a measurement-based check-in

Small specific steps plus measured review

Common error: Assigning vague, oversized goals

B. A huge task invites failure — Ignoring the need to grade tasks

C. Open-ended tasks reduce follow-through — Omitting structure the client needs

D. Attendance is not an outcome measure — Mistaking attendance for progress

Q11. A. Validate the worry and reframe the diagnosis as an explanation that guides effective strategies, not an excuse to give up

Reframe diagnosis as a guide, not an excuse

Common error: Either dismissing or reinforcing the belief

B. Endorsing the “excuse” framing harms engagement — Reinforcing the self-judgment

C. Bypassing the concern misses a target — Avoiding emotionally salient material

D. Lecturing rarely shifts the worry — Substituting facts for engagement

Q12. A. Provide accurate documentation within competence, with his consent, describing functional impacts relevant to accommodations

Document accurately within competence

Common error: Misrepresenting or overstating the findings

B. Overstating impairment is dishonest — Falsifying clinical documentation

C. A blanket refusal overstates the limit — Declining appropriate documentation

D. Disclosing without consent breaches confidentiality — Sharing records without authorization

Q13. A. Decline to confirm or deny without a valid release and explain that disclosure requires his written consent

Protect confidentiality without a release

Common error: Disclosing to an employer without consent

B. Confirming breaches confidentiality — Disclosing without authorization

C. Disclosing for accommodations still needs consent — Leaking details under the guise of helping

D. Abrupt refusal misses a teachable moment — Being needlessly obstructive

Case 9: Intrusive Thoughts and Endless Rituals

Diagnosis: Obsessive-Compulsive Disorder

Q1. A. Assess his obsessions, compulsions, time spent, distress, and degree of insight

Map obsessions, compulsions, time, and insight.

Common error: Assessing only the visible rituals.

B. Intervening before assessing is premature. — Beginning treatment before assessment.

C. Origin hunting is not the priority. — Searching for causes instead of assessing symptoms.

D. Minimizing impairing OCD is dismissive. — Normalizing clinically significant OCD.

Q2. A. His rituals consume more than an hour a day and are making him late for work

Over an hour daily or marked impairment qualifies.

Common error: Diagnosing OCD without the time or impairment threshold.

B. Occasional anxiety does not meet the criterion. — Treating any contamination worry as OCD.

C. A preference for order is not OCD. — Confusing orderliness with OCD.

D. Occasional checking is common and subthreshold. — Pathologizing ordinary checking.

Q3. A. Insight is a diagnostic specifier and helps guide how treatment is framed for him

Insight is a specifier and shapes treatment.

Common error: Overlooking the insight specifier.

B. Good insight does not rule out OCD. — Excluding OCD because insight is intact.

C. Poor insight is a specifier, not psychosis. — Equating poor insight with psychosis.

D. Insight does not reclassify obsessions as worries. — Reclassifying obsessions by insight.

Q4. A. He has specific intrusive obsessions with ritualized compulsions, not diffuse worry

OCD has obsessions plus rituals; GAD does not.

Common error: Calling ritualized OCD a worry disorder.

B. Anxiety is common to both. — Using anxiety to differentiate.

C. Multiple worries do not define OCD. — Using worry breadth to call it OCD.

D. Sleep disturbance is nonspecific. — Using sleep to decide.

Q5. A. His obsessions and compulsions are unwanted and distressing, not a valued life style

OCD is ego-dystonic; OCPD is ego-syntonic.

Common error: Confusing OCD with OCPD.

B. Valuing order points toward OCPD, not OCD. — Using valued order to define OCD.

C. Rigidity leans toward OCPD traits. — Reading rigidity as OCD.

D. Conscientiousness is nonspecific. — Using conscientiousness to decide.

Q6. A. Exposure and response prevention, in which he faces triggers without doing the ritual

ERP is first-line for OCD.

Common error: Choosing a non-ERP approach as primary.

B. Support alone does not reduce OCD. — Relying on support alone.

C. Reassurance reinforces OCD. — Using reassurance as the intervention.

D. Analyzing obsessions is not first-line. — Treating obsession content as meaningful.

Q7. A. Facing the fear without the ritual lets his anxiety fall on its own through habituation

Habituation occurs when rituals are blocked.

Common error: Failing to give a clear ERP rationale.

B. Rituals strengthen the cycle, not weaken it. — Suggesting rituals will extinguish themselves.

C. Avoidance maintains OCD. — Recommending avoidance.

D. Thought neutralizing is itself a compulsion. — Teaching thought suppression or neutralizing.

Q8. A. Work to reduce the family accommodation and reassurance that keep the OCD going

Reducing accommodation is part of treatment.

Common error: Leaving accommodation in place.

B. Accommodation worsens OCD. — Recruiting family into rituals.

C. Family involvement is often important. — Ignoring the family role.

D. Abrupt withdrawal without a plan can backfire. — Cutting accommodation abruptly.

Q9. A. Track his symptoms with a validated OCD measure and adjust the exposure hierarchy

Validated tracking guides the hierarchy.

Common error: Relying on impression instead of a measure.

B. Global self-report is less reliable. — Substituting impression for measurement.

C. One ritual is not the full picture. — Monitoring one ritual only.

D. End-only measurement cannot guide care. — Deferring measurement to the end.

Q10. A. Kindly limit the reassurance and support him in tolerating the uncertainty instead

Limiting reassurance treats the compulsion.

Common error: Reassuring to relieve his distress.

B. Reassurance reinforces the OCD loop. — Answering the reassurance request.

C. Dismissiveness harms the alliance. — Invalidating instead of guiding.

D. Avoiding the moment misses the intervention. — Avoiding the reassurance pattern.

Q11. A. Give a clear rationale and build the exposure hierarchy together at a manageable pace

Rationale and collaborative pacing build readiness.

Common error: Forcing exposure without preparation.

B. Flooding without grading can overwhelm. — Starting with the hardest item.

C. Skipping ERP omits the active ingredient. — Replacing ERP with relaxation.

D. Dismissing the fear is inaccurate. — Overpromising that ERP is easy.

Q12. A. Recognize the harm thoughts as ego-dystonic OCD symptoms, not intent, and avoid needless disclosure

Ego-dystonic obsessions are not intent.

Common error: Treating OCD harm obsessions as homicidal intent.

B. Reporting obsessions as intent is wrong. — Confusing obsessions with a threat.

C. Unwarranted disclosure breaches confidentiality. — Disclosing on obsession content alone.

D. Conditioning care on disproving obsessions is harmful. — Withholding care over ego-dystonic fears.

Q13. A. Explain that an SSRI is evidence-based for OCD and refer for evaluation with his consent

SSRIs help OCD; refer to a prescriber.

Common error: Recommending a specific drug or dose.

B. Specifying a drug and dose exceeds scope. — Making pharmacologic recommendations.

C. Dismissing medication contradicts the evidence. — Denying a role for medication.

D. Disclosing without consent breaches confidentiality. — Coordinating care without consent.

Case 10: Acute suicidal crisis after a breakup and job loss

Diagnosis: Suicidal Behavior / Acute Risk

Q1. A. Conduct a structured suicide-risk assessment clarifying ideation severity, intent, plan, access to means, and prior attempts

Structured risk assessment comes first

Common error: Assessing mood while skipping a structured risk screen

B. Intervening before assessing is premature — Jumping to coping skills before risk assessment

C. Premature reassurance forecloses assessment — Reassuring instead of assessing acute risk

D. Deferring risk assessment is unsafe — Outsourcing an acute safety assessment

Q2. A. By assessing his intent, since acting with intent to die differs fundamentally from self-injury aimed at relieving tension

Intent to die separates the two

Common error: Treating all self-harm as equivalent

B. Visible injury does not establish intent — Inferring intent from wounds

C. Frequency does not clarify intent — Equating frequency with risk type

D. Others' awareness is not the test — Using disclosure as the criterion

Q3. A. A careful assessment of his access to lethal means, since available, lethal methods sharply elevate acute suicide risk

Means access is a key risk driver

Common error: Overlooking means access in the assessment

B. Exhaustive gun history is low yield — Collecting low-yield detail

C. Depth history is not the priority now — Prioritizing depth over acute safety

D. Quantifying intake is not the priority — Confusing intake detail with means assessment

Q4. A. Both his risk factors and his protective factors, such as his ambivalence and the relationships he names as reasons to live

Weigh risk and protective factors

Common error: Cataloguing only risk factors

B. One factor does not define overall risk — Reducing risk to one variable

C. Proportionality is not a risk metric — Judging risk by proportionality

D. Narrative detail is not the risk metric — Equating a good history with low risk

Q5. A. His increased alcohol use, which can heighten impulsivity and acute suicide risk and warrants direct assessment

Acute substance use raises risk

Common error: Ignoring substance use in a suicide crisis

B. Psychosis is not indicated here — Pursuing an unsupported rule-out

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Cognitive decline is improbable here — Over-attributing to cognitive decline

Q6. A. A collaborative safety plan identifying warning signs, coping strategies, supports, and crisis contacts he can use

Collaborative safety planning

Common error: Relying on a no-suicide contract

B. No-suicide contracts are not evidence-based — Using a contract instead of a plan

C. Activity scheduling alone is insufficient — Substituting mood work for safety planning

D. A single contact rule is not a plan — Reducing safety planning to one instruction

Q7. A. Counsel collaboratively on means restriction, such as temporarily storing the firearm away from the home with a trusted person

Means restriction is central

Common error: Leaving the lethal means unaddressed

B. Avoiding means restriction is unsafe — Omitting means-restriction counseling

C. In-home storage still leaves access — Settling for an inadequate means measure

D. Deferring abandons means counseling — Using autonomy to avoid the conversation

Q8. A. On a clinical judgment weighing intent, plan, means, and his ability to maintain safety with the current plan and supports

Stratified judgment drives level of care

Common error: Using a single factor to decide disposition

B. Disposition is a clinical judgment — Deferring disposition to the client alone

C. Reflexive admission is not the standard — Admitting reflexively without assessment

D. Refusing admission ignores acute danger — Ruling out admission regardless of risk

Q9. A. A warm handoff and a scheduled follow-up with caring contact rather than simply sending him out with a phone number

Warm handoff and follow-up

Common error: Ending contact without a follow-up plan

B. Self-directed lookup is too passive — Relying on the client to self-refer in crisis

C. A long gap in an acute crisis is unsafe — Leaving a long gap after a crisis

D. Outsourcing to the ex is inappropriate — Delegating safety to an unsuitable contact

Q10. A. Explore his ambivalence with empathy and build on his stated reasons for living to strengthen his motivation to stay safe

Work with ambivalence and reasons to live

Common error: Treating ambivalence as either fully safe or hopeless

B. Dismissing the pain invalidates him — Telling him how he should feel

C. Pivoting away abandons the crisis — Redirecting away from suicidal ambivalence

D. Statistics rarely shift acute despair — Answering despair with numbers

Q11. A. Validate how unbearable things feel right now while helping him see the crisis and his options as changeable over time

Validate pain, frame the crisis as time-limited

Common error: Either only sympathizing or only reassuring

B. Overpromising can ring hollow — Making promises you cannot keep

C. Endorsing hopelessness is dangerous — Adopting the client's hopeless stance

D. Avoiding the despair leaves it intact — Sidestepping the crisis to ease discomfort

Q12. A. Explain the limits of confidentiality, including that serious, foreseeable risk of harm may require disclosure to protect him

Confidentiality has safety-based limits

Common error: Promising unconditional confidentiality

B. Absolute confidentiality is unethical here — Guaranteeing unconditional secrecy

C. Act on assessed risk, not reflexively — Escalating before assessing the risk

D. Dodging the question undermines consent — Concealing the limits of confidentiality

Q13. A. Document the risk assessment, the formulation, the safety plan, means-restriction steps, and the clinical rationale for the disposition

Document assessment, plan, and rationale

Common error: Documenting only that he denied a plan

B. A contract note is inadequate and misleading — Substituting a contract note for real documentation

C. Minimizing the record is unsafe and inaccurate — Understating risk in the record

D. Omitting the assessment is improper — Omitting essential clinical information

Case 11: Recurrent binges without compensatory behavior

Diagnosis: Binge-Eating Disorder

Q1. A. Recurrent binges with loss of control at least weekly for three months with marked distress and no regular compensatory behavior

Binges, loss of control, distress, no compensation

Common error: Diagnosing BED from occasional overeating

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent change is not the criterion — Confusing a recent uptick with the criteria

D. Depressive count does not define BED — Applying depressive criteria here

Q2. A. She does not engage in the regular compensatory behaviors—such as vomiting or fasting—that define bulimia nervosa

Absence of regular compensatory behavior

Common error: Overlooking the absence of compensation

B. Binges occur in both — Using binges to differentiate

C. Post-binge distress occurs in both — Reading distress as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. Her episodes involve a sense of loss of control and marked distress at a clinically significant frequency, not just occasional overindulgence

Loss of control and distress at threshold frequency

Common error: Calling clinically significant binges ordinary overeating

B. Occasional overeating is not the disorder — Equating any overeating with BED

C. Enjoyment is not the criterion — Reading ordinary enjoyment as BED

D. Seasonal overeating is nonspecific — Over-reading seasonal overeating

Q4. A. Co-occurring depression and suicide risk, which commonly accompany binge-eating disorder and shape the plan

Screen depression and suicide risk

Common error: Treating the eating pattern without screening mood and risk

B. Psychosis is not indicated — Pursuing an unsupported rule-out

C. Anxiety alone is not the explanation — Over-attributing to anxiety alone

D. Cognitive decline is improbable here — Pursuing an improbable rule-out

Q5. A. Coordinate a medical evaluation and ongoing monitoring as part of a team while she engages in psychotherapy

Coordinate medical evaluation and monitoring

Common error: Providing solo therapy without medical coordination

B. Ordering tests exceeds counselor scope — Acting outside scope of practice

C. Dieting can worsen the binge cycle — Prescribing a diet as treatment

D. No coordination undercuts team care — Refusing helpful coordination

Q6. A. Eating-disorder-focused cognitive behavioral therapy, within a team that includes medical involvement as needed

CBT-ED within a coordinated team

Common error: Defaulting to a weight-loss diet as the treatment

B. Restrictive dieting fuels binges — Treating BED with a restrictive diet

C. Unstructured support underperforms CBT-ED — Defaulting to nondirective support

D. Insight-only is not first-line here — Prioritizing origins over CBT-ED

Q7. A. Establishing regular eating and reducing the dietary restraint and triggers that drive the binge episodes

Regular eating to interrupt the binge cycle

Common error: Adding restriction that fuels binges

B. More restriction fuels binges — Reinforcing the restraint that drives binges

C. Meal-skipping drives binges — Promoting restriction that triggers binges

D. Relaxation alone misses the core — Treating arousal while ignoring eating

Q8. A. Provide general information and coordinate a referral to a prescriber for evaluation with her consent, within scope

Refer and coordinate within scope

Common error: Giving specific medication advice

B. Naming a drug and dose exceeds scope — Overstepping professional limits

C. Recommending a suppressant is inappropriate and out of scope — Advising an unsafe, out-of-scope product

D. Willpower framing is inaccurate — Moralizing about a clinical disorder

Q9. A. Respond with a nonjudgmental, validating stance that reduces shame and supports her engaging openly in treatment

Reduce shame with a nonjudgmental stance

Common error: Responding with judgment or diet advice that deepens shame

B. Moralizing deepens shame — Reducing the disorder to willpower

C. Minimizing the disorder is inaccurate — Downplaying a treatable condition

D. Facts alone rarely shift shame — Answering shame with numbers

Q10. A. Clarify that the focus is on interrupting the binge cycle and the distress, not on prescribing another restrictive diet

Reframe the aim away from dieting

Common error: Confirming her fear by centering the plan on dieting

B. Centering a diet drives binges and her fear — Confirming the feared, counterproductive approach

C. Weight-centric framing increases shame — Centering the work on weight loss

D. Bypassing the concern harms engagement — Avoiding a key alliance issue

Q11. A. Conduct a structured suicide-risk assessment and respond to the risk, recognizing eating disorders can carry elevated risk

Assess risk directly

Common error: Overlooking risk amid the eating focus

B. Minimizing the ideation is unsafe — Dismissing the ideation

C. Risk must be assessed before continuing — Proceeding past a risk disclosure

D. No-suicide contracts are not evidence-based — Substituting a contract for assessment

Q12. A. Obtain her informed consent and release before communicating with the provider, then collaborate on a shared plan

Consent and release before coordinating

Common error: Contacting the provider before obtaining consent

B. Coordinating without consent breaches confidentiality — Disclosing before consent is documented

C. No coordination undercuts collaborative care — Refusing helpful, consented coordination

D. Blanket sharing over-discloses — Releasing more than is appropriate or consented

Q13. A. Explain that accurate clinical documentation is required and discuss how records are protected and who may access them

Keep honest records; explain protections

Common error: Falsifying or omitting clinical information

B. Omitting the diagnosis falsifies the record — Falsifying records to reassure

C. Dismissing her concern harms rapport — Ignoring the client's concern

D. Shadow records are improper — Keeping hidden duplicate notes

Practice Exam Two

11 clinical cases · 143 scored questions · 255 minutes recommended

Work through all cases before consulting the answer key at the end of this exam.

*Case 1 of 11***Twenty years of feeling "flat" in a small hardware-store town***Diagnosis: Persistent Depressive Disorder*

INTAKE

Walt Higby, a 41-year-old man who has worked at Pruitt's Hardware and Supply in his small rural town for eighteen years, presents reporting that he has felt "down" or "flat" for as long as he can remember, dating back to his early twenties, with no stretch of feeling fully like himself lasting more than about six weeks.

SESSION TWO

He describes chronically low energy, a poor appetite most days, trouble falling asleep, and a nagging sense that he is not good enough, though he still opens the store most mornings and rarely misses a shift. Friends have called him the "quiet, steady one" for two decades, and he assumed this flatness was simply his personality.

SESSION THREE

He denies any lifetime period of elevated or irritable mood, decreased need for sleep, or reckless spending, and he denies any current thoughts of suicide, a plan, or intent, saying he has "never gotten that low." His wife recently urged him to seek help after noticing him withdraw further from their weekend routines.

Question 1. What intake detail is most essential to document to support the working diagnosis of Persistent Depressive Disorder?

- A. That his low mood has been present most of the day, more days than not, for at least two years, without a symptom-free stretch longer than two months
- B. That he can name the exact week his low mood first began after finishing high school
- C. That a first-degree relative, such as a parent or sibling, was also treated at some point in the past for a chronic low-mood condition resembling his own
- D. That his mood tends to feel somewhat heavier on workdays at the store than it does on the weekends when he is off

Question 2. What additional history is most important to gather before finalizing the diagnosis?

- A. A review of his medications, substance use, and any thyroid or other medical conditions that could mimic chronic low mood
- B. A full inventory of every hobby he has ever tried so the counselor understands his complete leisure history
- C. A detailed account of his coworkers' opinions about his mood and work performance over the years
- D. A complete list of every store inventory task he currently manages so his daily workload can be mapped

Question 3. Which line of questioning is most important to rule out a bipolar-spectrum course before confirming Persistent Depressive Disorder?

- A. Asking whether he has ever had a period of elevated or irritable mood with decreased need for sleep or increased energy
- B. Asking whether he has ever experienced a panic attack with a racing heart in the store's crowded checkout line
- C. Asking whether his concentration problems have been present since childhood in a pattern consistent with attention-deficit disorder
- D. Asking whether he has ever held beliefs about himself that reached a fixed, delusional level of conviction

Question 4. Which combination of findings best satisfies the associated-symptom criterion required alongside his chronic low mood?

- A. His chronically low energy, poor appetite, and sleep-onset difficulty occurring alongside the low mood most days
- B. His steady attendance record and his reputation among coworkers as dependable and even-tempered
- C. His preference for quiet evenings at home rather than socializing at the town's weekend events
- D. His tendency to double-check the store's cash drawer twice before closing each night

Question 5. Given his denial of any current suicidal thoughts, what is the most appropriate assessment step?

- A. Use a structured suicide-risk screen to confirm and document the absence of ideation, intent, plan, or prior behavior
- B. Skip any risk questions entirely since he denies current thoughts and appears calm during the session
- C. Ask him to sign a written agreement promising not to harm himself before the next scheduled appointment
- D. Wait until several sessions of rapport-building have passed before ever raising the topic of safety

Question 6. What is the most appropriate first-line, within-scope treatment recommendation for his chronic presentation?

- A. Offer evidence-based psychotherapy such as cognitive behavioral therapy or behavioral activation adapted for chronic depression
- B. Tell him to begin a daily antidepressant right away and adjust the dose himself based on how he feels each week
- C. Suggest he simply wait another year or two to see whether the flat mood eventually lifts on its own
- D. Recommend he focus mainly on a stricter sleep schedule and daily exercise instead of starting formal counseling

Question 7. How should the counselor track whether treatment is producing meaningful change over the coming months?

- A. Re-administer a validated symptom measure such as the PHQ-9 at regular intervals to guide measurement-based care
- B. Simply ask him at each visit whether he generally feels a little better than he did the week before
- C. Wait until an entire year of sessions has passed and then evaluate his overall response in one single review
- D. Track only whether he keeps showing up to sessions on time and completes the homework assigned each week

Question 8. Walt asks whether he should also be evaluated for medication. What is the most appropriate counselor action?

- A. Provide general information and, with his consent, coordinate a referral to a prescriber for a medication evaluation
- B. Tell him exactly which antidepressant tends to work best for people with a chronic, low-grade symptom pattern like his
- C. Advise him that medication is generally unnecessary as long as he keeps attending his counseling sessions
- D. Suggest he research antidepressants online himself and bring back his preferred choice at a later appointment

Question 9. Which initial treatment-planning goal best fits Walt's chronic, low-grade presentation?

- A. Collaboratively schedule one or two small, valued activities each week, such as a fishing trip he used to enjoy
- B. Set a goal for him to resume every hobby and social role he has ever held, all within the first two weeks of treatment
- C. Have him complete a detailed daily log of every automatic negative thought before any other treatment goal is set
- D. Encourage him to wait to re-engage in any valued activities until his energy and motivation naturally return on their own

Question 10. How can the counselor best respond to Walt's belief that his flat mood is simply his personality rather than something treatable?

- A. Offer psychoeducation that chronic low mood is a treatable condition, while validating how normal it felt to him for years
- B. Firmly tell him his belief is wrong and present a list of facts proving that his personality is not actually flat
- C. Agree that this is simply how he has always been and focus sessions only on coping strategies rather than change
- D. Change the subject and move the conversation toward scheduling next week's appointment time instead

Question 11. Walt says he doubts anything can really change after twenty years of feeling this way. What is the most therapeutic response?

- A. Acknowledge his doubt as understandable and collaboratively set one small, achievable goal to test it against
- B. Promise him confidently that his mood is guaranteed to improve within a set number of weeks of treatment
- C. Agree that twenty years is likely too long for real change and shift the focus to simply accepting the mood
- D. Spend the session explaining the neurobiology of chronic depression in detail to convince him change is possible

Question 12. When reviewing informed consent at the outset of treatment, what must the counselor be sure Walt understands?

- A. The limits of confidentiality, including the circumstances under which information must be disclosed to others
- B. That everything he says in session will remain completely confidential under every possible circumstance
- C. That the counselor alone will decide which treatment goals he is allowed to pursue during their work together
- D. That once he agrees to begin, he is obligated to complete the full recommended course of treatment

Question 13. The counselor realizes Walt is a regular customer-facing employee at the only hardware store in their shared small town. What is the most appropriate way to manage this?

- A. Discuss the likelihood of unavoidable public encounters with him in advance and agree on how to handle them respectfully
- B. Decline to continue working with him and refer him elsewhere simply because he works at a public-facing local business
- C. Say nothing about it and simply hope that the two of them never happen to cross paths anywhere in town
- D. Ask him to stop working his usual shifts at the store so that the two of them are less likely to ever encounter each other

*Case 2 of 11***Sudden surges of fear with a racing heart***Diagnosis: Panic Disorder*

INTAKE

Renée Dubois, a 33-year-old nurse, presents after several sudden, unexpected surges of intense fear with a pounding heart, chest tightness, dizziness, and a sense she might die or lose control; two emergency-department visits found nothing wrong.

SESSION TWO

She describes attacks that peak within minutes and arise without warning, even at rest, followed by a persistent dread of the next one. She has started leaving work early and avoiding the gym for fear of triggering the frightening sensations.

SESSION THREE

She fears the racing heart signals a heart attack despite a normal cardiac workup, and she now carries water and sits near exits. She denies stimulant use beyond one morning coffee and wants to understand what is happening to her body.

Question 1. What is most important to confirm to establish panic disorder rather than isolated panic attacks?

- A. That the unexpected attacks are recurrent and followed by a month or more of worry or avoidance about further attacks
- B. That she can name the specific external situation that reliably brings on each one of her frightening panic episodes
- C. That she has experienced a continuous, hard-to-control background of worry across many domains for the past six months
- D. That her physical sensations during the attacks are severe enough that they have twice prompted an emergency visit

Question 2. Which feature best distinguishes her presentation from generalized anxiety disorder?

- A. Discrete attacks that peak within minutes rather than a persistent, free-floating background of multi-domain worry
- B. The presence of physical symptoms such as a racing heart, dizziness, and tension that she notices during the episodes
- C. Her worry about her job performance and whether her colleagues have noticed that she keeps leaving work early lately
- D. The way her symptoms tend to feel noticeably worse on days that are especially busy and demanding at the hospital

Question 3. Given her cardiac fears and normal workup, what is the most appropriate counselor action?

- A. Confirm that a medical evaluation has ruled out cardiac causes and coordinate further workup if any new symptoms arise
- B. Order an electrocardiogram and thyroid panel yourself so that you can personally rule out any underlying medical condition
- C. Tell her with confidence that her heart is completely fine and that she should stop worrying about it from now on
- D. Disregard the physical symptoms entirely because the emergency department already determined that nothing was wrong

Question 4. How does her presentation differ from a situational specific phobia?

- A. Her attacks arise unexpectedly and at rest rather than only when she confronts one specific feared situation or object
- B. The fact that she has now begun to avoid certain places and situations that she associates with having had an attack
- C. The intense physical fear response that she experiences, complete with a pounding heart and a strong urge to escape
- D. Her belief during the episodes that something catastrophic, such as losing control, is about to happen to her right then

Question 5. What developing pattern is most important to assess to complete the clinical picture?

- A. Whether agoraphobic avoidance is emerging as she increasingly avoids places where escape or help might be difficult
- B. Whether she has a fixed and persistent preoccupation with the idea that she has a serious undiagnosed medical disease
- C. Whether she experiences clear stretches of elevated mood and reduced need for sleep lasting several consecutive days
- D. Whether her symptoms are generally more intense in the evenings than they tend to be earlier during the daytime hours

Question 6. What is the most appropriate first-line psychotherapy recommendation?

- A. Cognitive behavioral therapy with interoceptive exposure to the feared bodily sensations that drive her panic
- B. Long-term supportive counseling that gives her a calm space to talk through how frightening the attacks have been for her
- C. Teaching her breathing and relaxation techniques to use the instant she notices any of the early signs of a panic attack
- D. A primarily insight-oriented exploration of her childhood to uncover the deeper origins of her current anxiety symptoms

Question 7. What is the central rationale for interoceptive exposure in her treatment?

- A. Repeatedly and deliberately bringing on the feared sensations so she learns they are not dangerous and her fear extinguishes
- B. Helping her become skilled at distracting herself away from the sensations as quickly as possible whenever they appear
- C. Teaching her to monitor her pulse and other bodily signs closely so she can catch the very earliest warning of an attack
- D. Encouraging her to steer clear of any exertion that raises her heart rate until she feels much calmer about the symptoms

Question 8. Renée carries water and sits near exits during outings. How should the plan address these behaviors?

- A. Identify them as safety behaviors and gradually drop them so she can learn the feared catastrophes do not occur
- B. Encourage her to keep using whatever small rituals help her feel calmer so that she is able to keep going out in public
- C. Treat these behaviors as harmless personal habits that have nothing to do with the maintenance of her panic symptoms
- D. Replace them with a different and more elaborate set of preventive routines she can carry out before she leaves the house

Question 9. Renée asks whether medication might help. The most appropriate counselor action is to:

- A. Coordinate a referral to a prescriber for evaluation while continuing exposure-based therapy within her own scope
- B. Recommend the specific anti-anxiety medication that tends to calm panic the fastest for patients with her symptom profile
- C. Advise her that as-needed sedatives are the best option to keep on hand for whenever she feels an attack beginning to build
- D. Discourage medication entirely and tell her she should be able to manage the panic with the therapy techniques alone

Question 10. What is the most useful psychoeducation to offer Renée early in treatment?

- A. Explain the panic cycle: how catastrophic misreading of normal sensations escalates arousal into a full-blown attack
- B. Reassure her in detail and repeatedly that panic attacks have never actually caused real physical harm to anyone at all
- C. Give her a thorough overview of the autonomic nervous system so she can fully understand every physiological detail involved
- D. Tell her that the most important thing is simply to stay calm and to try her best not to let the attacks frighten her so much

Question 11. Renée begins having a panic attack during a session. What is the most therapeutic response?

- A. Calmly coach her to stay with the sensations and notice them passing rather than fleeing or fighting the attack
- B. End the session early and let her step outside immediately so that she can get away from whatever set the attack off
- C. Direct her to breathe deeply and immediately use every relaxation technique she knows to make the attack stop right away
- D. Reassure her over and over that she is completely safe and that the attack is guaranteed to be over in just a moment

Question 12. Renée asks the counselor to tell her physician to stop her beta blocker because she dislikes it. Best response?

- A. Clarify that medication decisions rest with her prescriber and offer, with consent, to coordinate her concerns with them
- B. Advise her to go ahead and stop the beta blocker on her own, since it may well be contributing to how she has been feeling
- C. Call her physician yourself and recommend that the beta blocker be discontinued based on what she has told you in session
- D. Tell her this is entirely a medical matter that has nothing to do with counseling and decline to discuss it any further

Question 13. As exposure-based work is planned, what does informed consent most importantly require here?

- A. Explaining the rationale, the temporary discomfort exposure can provoke, and that participation remains voluntary
- B. Assuring her in advance that the exposure exercises will be completely comfortable and will never provoke any real anxiety
- C. Having her agree in advance that she will complete every exposure exercise assigned no matter how distressing it becomes
- D. Postponing any explanation of the method until after the first exercise so that her anticipatory anxiety does not build up

*Case 3 of 11***Dazed and hypervigilant three weeks after a car crash***Diagnosis: Acute Stress Disorder*

INTAKE

Emily Navarro, a 24-year-old dental hygienist, presents for outpatient counseling two and a half weeks after being T-boned at an intersection near her apartment while stopped at a red light. She was referred by the emergency department after treatment for minor injuries and reports intrusive memories of the crash, nightmares, and a dazed, 'unreal' feeling that comes and goes.

SESSION TWO

She describes driving the long way to work every day to avoid the intersection where the crash occurred, scanning cross-traffic obsessively at every light, and startling badly at horns or screeching brakes. She has trouble concentrating on patient charts at the dental office and reports two or three nightmares about the collision each week.

SESSION THREE

Emily denies any current suicidal ideation, self-harm, or substance use, and her emergency department workup showed no ongoing medical injury requiring further treatment. She is unsettled by how 'unlike herself' she feels during the dazed episodes but is motivated, has supportive family nearby, and is willing to begin trauma-focused counseling right away.

Question 1. What should the counselor confirm first at intake to anchor a working diagnosis of Acute Stress Disorder?

- A. That she directly experienced the threatened injury of the crash and that it has been under one month since it happened
- B. That she can recall the exact speed of both vehicles and the precise sequence of events leading up to the collision itself
- C. That a family member also witnessed the crash and can independently corroborate every detail of what actually happened
- D. That her auto insurance claim regarding the collision has already been fully processed and formally resolved

Question 2. Emily mentions feeling 'unreal' and dazed at times. What should the counselor specifically assess about this?

- A. Whether this reflects a dissociative symptom such as derealization, one of the symptom clusters counted toward ASD
- B. Whether this dazed quality means she likely sustained an undiagnosed traumatic brain injury that the ED overlooked entirely
- C. Whether this feeling means she is exaggerating her distress to strengthen a personal injury claim from the crash
- D. Whether she has ever felt this exact way before, at any point earlier across her entire life, unrelated to any stressor

Question 3. Given her presentation, what safety screening is appropriate at this intake?

- A. A structured suicide risk screen covering ideation, intent, plan, and behavior, done routinely regardless of apparent risk
- B. No formal risk screening is needed since she denies distress about the dazed episodes and appears to be coping well overall
- C. A screening focused only on whether she has ever previously been hospitalized psychiatrically for any reason at all
- D. A screening deferred until the third or fourth session once the counselor has built more rapport with her over time

Question 4. What single factor would most clearly shift this diagnosis from Acute Stress Disorder to Posttraumatic Stress Disorder?

- A. The qualifying symptom cluster persisting for one month or longer after the crash rather than resolving before then
- B. The nightmares becoming somewhat more vivid and disturbing in their content than they have been up to this point
- C. Her decision to permanently stop driving altogether rather than only avoiding the specific intersection where the crash occurred
- D. The other driver being found legally at fault for causing the collision once the police investigation concludes

Question 5. Why is Adjustment Disorder not the better fit for Emily's presentation?

- A. Her reaction meets Criterion A trauma exposure and the specific ASD symptom cluster that Adjustment Disorder does not require
- B. Her distress began very soon after a single identifiable stressor rather than building up gradually over several months
- C. Her level of distress appears somewhat greater than what most people would experience after a similar car accident
- D. She is expected to return to her usual functioning within about six months once the stressor itself has fully passed

Question 6. What is the most appropriate first-line, within-scope treatment recommendation for Emily?

- A. Begin trauma-focused psychotherapy incorporating psychoeducation, grounding skills, and gradual exposure to driving cues
- B. Recommend she begin a daily anti-anxiety medication right away and follow up on the dosage in a few weeks
- C. Suggest she wait several more weeks to see whether the symptoms resolve naturally before starting any formal treatment
- D. Encourage her to avoid discussing the crash directly for now and instead focus sessions on unrelated relaxation topics

Question 7. How should the counselor monitor Emily's progress over the coming weeks?

- A. Track her symptom cluster and functioning at each session to note whether it resolves or persists past one month
- B. Ask her only once, at the very final session of the whole course of treatment, how she generally feels she has done
- C. Track only how many sessions she attends and whether she arrives on time, without reviewing her symptoms directly
- D. Rely on her employer's occasional comments about her work performance as the main indicator of her clinical progress

Question 8. Emily asks whether she should also see a prescriber about medication for her sleep and anxiety. What is the most appropriate counselor action?

- A. Provide general information and, with her consent, coordinate a referral to a prescriber for a full evaluation
- B. Tell her which specific sleep medication tends to work best for people going through a stressful period like hers
- C. Advise her that medication is unnecessary at this early stage as long as she keeps attending therapy sessions consistently
- D. Suggest she look into over-the-counter sleep aids on her own and report back on how well they seem to be working

Question 9. Which initial intervention best targets Emily's dazed, derealized episodes?

- A. Teach brief grounding techniques, such as naming sensory details in the room, to use during dazed or unreal moments
- B. Begin detailed, repeated exposure to the full narrative of the crash before teaching her any coping skills first
- C. Explain to her at length that derealization is not a real medical concern and that she should simply try to ignore it
- D. Recommend she avoid driving altogether until the dazed feelings stop occurring completely on their own over time

Question 10. Emily says she feels weak for still avoiding the intersection three weeks later. How should the counselor respond?

- A. Normalize the avoidance as a common, treatable stress response rather than a personal weakness, and validate her frustration
- B. Agree that avoiding one intersection for three weeks does seem like an overreaction to a routine traffic accident
- C. Firmly challenge the belief as irrational and present a logical argument for why she should not feel this way
- D. Redirect the conversation away from the topic and refocus entirely on scheduling next week's session logistics

Question 11. What is the most therapeutic way to respond when Emily describes a nightmare in vivid detail during session?

- A. Listen empathically, reflect her distress, and gently pace the discussion so she does not feel overwhelmed or flooded
- B. Quickly change the subject to something lighter so the session does not become too heavy or emotionally difficult
- C. Ask her to describe the nightmare again in even more graphic detail so the counselor can fully picture every element
- D. Tell her that nightmares like this are meaningless and will stop mattering once enough time has simply passed

Question 12. When reviewing informed consent with Emily, what must the counselor be sure she understands?

- A. The limits of confidentiality, including when serious foreseeable harm to self or others would require disclosure
- B. That everything she discusses in every session will be held in absolute confidence under all circumstances whatsoever
- C. That the counselor alone will decide which specific treatment goals she is permitted to pursue during their work together
- D. That once she agrees to begin, she is required to complete the entire recommended course of therapy without exception

Question 13. Emily mentions she has retained an attorney regarding the crash and asks the counselor to send her full session notes directly to the attorney's office. What is the appropriate response?

- A. Explain the release-of-information process, obtain her signed written authorization, and coordinate the disclosure appropriately
- B. Send the complete session notes to the attorney's office right away since she is the one who requested it verbally
- C. Refuse to release any information at all and tell her that session notes can never be shared with an attorney
- D. Summarize her progress from memory over the phone to the attorney rather than sharing any written documentation

*Case 4 of 11***From prescribed pain pills to daily opioid use***Diagnosis: Opioid Use Disorder, Severe*

INTAKE

Tyrone Jackson, a 38-year-old construction worker, presents after a back injury led to prescribed opioids and then to buying pills and, recently, using heroin. He reports strong cravings, tolerance, failed attempts to stop, and use despite job loss.

SESSION TWO

He identifies at least seven of the eleven criteria over the past year, including cravings, tolerance, withdrawal, and giving up activities he valued. He describes flu-like sickness, aching, and sweating within a day of his last use when he tries to quit.

SESSION THREE

He is exhausted by the cycle and afraid of overdose after a friend died last month; he is unsure whether medication could help. He denies current suicidal intent but feels hopeless at times, and he has used alone, which raises his overdose risk.

Question 1. What is most important to establish to confirm a severe opioid use disorder?

- A. The number of DSM-5-TR criteria he meets over twelve months, since six or more place severity in the severe range
- B. The exact quantity of opioids he uses each day so that you can calculate precisely how physically dependent he has become
- C. Whether he first became addicted to opioids through a legitimate prescription rather than through buying them illegally
- D. Whether members of his immediate family have also struggled with opioids or with another substance-related condition

Question 2. How should the counselor understand the sickness he feels within a day of stopping?

- A. As opioid withdrawal—a physiologic cessation syndrome that is one criterion within his use disorder, not a separate problem
- B. As evidence that he has simply been using opioids for too many years in a row without ever taking a meaningful break
- C. As proof that he has developed a separate primary anxiety disorder that happens to flare whenever he stops using opioids
- D. As an indication that he must immediately be admitted for inpatient detoxification before anything else can be considered

Question 3. Given that he has used alone and a friend recently died of overdose, what is the priority assessment and response?

- A. Assess overdose risk and withdrawal safety, and provide overdose-prevention education including naloxone access
- B. Focus the session on identifying the underlying emotional pain that you believe is the true root cause of all of his using
- C. Advise him to taper his opioid use down on his own at home over the next week so that he can avoid the worst of the sickness
- D. Reassure him that as long as he is motivated to quit, the risk of an accidental overdose is really quite low for him now

Question 4. What distinguishes his severe opioid use disorder from a moderate one?

- A. That he meets six or more of the eleven criteria, whereas four to five would indicate moderate severity instead
- B. That he has now progressed from using prescribed pills to using heroin that he buys from someone on the street
- C. That his opioid use has already cost him his construction job and created serious financial strain for his household
- D. That he experiences strong cravings to use opioids during much of the day when he is trying his hardest to abstain

Question 5. What co-occurring concern is most important to screen for to inform the plan?

- A. Suicide risk and co-occurring depression, screened with a structured tool given his hopelessness and recent loss
- B. A previously undetected primary psychotic disorder that could better explain the difficulties he has had holding a job
- C. A long-standing eating disorder that might be contributing to both his low energy and his disrupted daily routine
- D. An emerging neurocognitive decline that could account for why he keeps forgetting commitments and appointments lately

Question 6. What is the most appropriate first-line treatment recommendation for severe opioid use disorder?

- A. Coordinate a referral for medication for opioid use disorder, such as buprenorphine or methadone, alongside counseling
- B. Recommend an abstinence-only, medication-free program because relying on any medication merely substitutes one drug for another
- C. Suggest he first prove his commitment by staying completely abstinent on his own for thirty days before any referral is made
- D. Recommend a course of weekly supportive counseling and self-help meetings as the complete treatment for his opioid use

Question 7. How should the counselor determine the appropriate level of care?

- A. Use a multidimensional assessment of withdrawal risk, environment, and supports to match him to a safe level of care
- B. Automatically place him in the longest available residential program so that he is kept away from drugs for as long as possible
- C. Recommend only a weekly outpatient counseling slot on the assumption that talking through it regularly will be sufficient
- D. Let him choose whichever program has the most convenient schedule for him regardless of his clinical needs or safety risks

Question 8. What harm-reduction element is most important to build into the plan now?

- A. Naloxone access and education plus never-use-alone guidance to reduce his risk of a fatal overdose during treatment
- B. A firm rule that he will be discharged from treatment immediately if he uses opioids again at any point during the program
- C. A detailed plan for which over-the-counter remedies he can take at home to manage the worst of his withdrawal symptoms
- D. A requirement that he hand over any remaining pills to a family member who can then dole them out to him a little at a time

Question 9. Tyrone says part of him wants to quit and part of him is terrified of life without opioids. The MI-consistent response is to:

- A. Reflect both sides of his ambivalence and gently explore the change talk he has already begun to voice
- B. Point out firmly that his fear is just an excuse and that he clearly needs to commit to stopping opioids completely today
- C. Agree with the fearful side and suggest that perhaps this is simply not the right moment for him to try to make a change
- D. Lay out a detailed day-by-day quit schedule with firm rules and deadlines for him to begin following starting tomorrow morning

Question 10. Tyrone returns to use after ten days abstinent and is ashamed. The most therapeutic response is to:

- A. Frame the return to use as common in recovery, review overdose risk after abstinence, and revisit the plan without shaming him
- B. Warn him sternly that another slip like this one will prove that he is simply not serious about getting clean this time
- C. Reassure him that the slip really does not matter at all and encourage him to simply put it behind him and move forward
- D. Tell him that he clearly needs to be discharged to a higher level of care since the current plan has obviously failed him

Question 11. Tyrone's former employer calls asking whether he is being treated for a drug problem. The best response is to:

- A. Decline to confirm or deny without a valid release, explaining the heightened confidentiality of substance-use information
- B. Confirm that he is in treatment, since the employer already let him go and is now only trying to look out for his wellbeing
- C. Share only that he is attending sessions, since merely confirming attendance seems like a fairly harmless detail to give out
- D. Refuse to speak with the caller at all and hang up without explaining anything about confidentiality or the proper process

Question 12. Tyrone asks the counselor what dose of buprenorphine he should request from the prescriber. Best response?

- A. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate his questions and progress with them
- B. Suggest a starting dose that usually works well for someone using the amount of opioids that he has described to you today
- C. Tell him to ask for the highest dose available so that the medication will be sure to fully block his cravings right away
- D. Say that medication dosing is purely a medical matter and decline to help him prepare for the conversation in any way at all

Question 13. Tyrone wants the counselor to coordinate with his prescriber and his probation officer. What must come first?

- A. Obtain his specific, informed written consent for each party before sharing any of his protected information
- B. Begin coordinating with both parties right away, since he has clearly already told you that he wants you to work with them
- C. Share information freely with the probation officer because the criminal-justice context overrides his usual confidentiality
- D. Coordinate only with the prescriber for now and decide about the probation officer later without discussing it further with him

*Case 5 of 11***Feeling detached and unreal, as if watching life from outside***Diagnosis: Depersonalization/Derealization Disorder*

INTAKE

Owen Park, a 24-year-old graduate student, presents with months of persistent, distressing feelings of detachment—he feels like an observer of his own thoughts and actions, and the world around him often seems foggy, dreamlike, and unreal.

SESSION TWO

He describes feeling robotic and emotionally numb and says his surroundings sometimes look flat or two-dimensional. Throughout, he knows these are subjective experiences and that things are not actually unreal; his reality testing remains intact between and during episodes.

SESSION THREE

He is frightened he is “going crazy” and has been monitoring the sensations closely and avoiding stressful situations that seem to worsen them. He denies cannabis or other substance use as a cause and denies hallucinations or fixed delusional beliefs.

Question 1. What is most important to confirm to support a depersonalization/derealization disorder diagnosis?

- A. Persistent or recurrent detachment or unreality with intact reality testing, causing significant distress or impairment
- B. That he firmly and genuinely believes that he and the world around him have actually become physically unreal
- C. That he can identify the single stressful event he believes first triggered these feelings of detachment and unreality
- D. That his detachment has become more intense over the past day or two than it was earlier during the same week

Question 2. Which feature best distinguishes his presentation from a primary psychotic disorder?

- A. His reality testing is intact—he knows the detachment and unreality are subjective experiences, not actual changes in the world
- B. The fact that he feels emotionally numb and robotic during many of his interactions with other people each day
- C. The way his surroundings sometimes look flat or two-dimensional when his symptoms are most intense
- D. The distress and fear he experiences about the symptoms whenever they happen to become especially strong

Question 3. How does his presentation differ from panic disorder?

- A. His detachment is persistent rather than occurring only as brief, discrete attacks of intense fear that peak within minutes
- B. The fact that he feels frightened and worried about the unsettling sensations he keeps experiencing in his body and mind
- C. The way his symptoms tend to worsen during the most stressful and demanding weeks of his graduate program
- D. The fact that he tends to avoid the situations that seem to bring on or worsen his feelings of detachment

Question 4. What is the most important condition to rule out before confirming the diagnosis?

- A. A substance-induced cause, such as cannabis or another drug, which can produce depersonalization and derealization
- B. A long-standing personality disorder that could account for why he feels so detached from himself and from other people
- C. A previously undetected eating disorder that might explain both his emotional numbness and his social withdrawal recently
- D. An emerging neurocognitive decline that could explain why his surroundings sometimes look foggy and unreal to him

Question 5. What co-occurring concern is most important to assess to complete the picture?

- A. Co-occurring anxiety, depression, and trauma history, along with suicide risk, which commonly accompany the disorder
- B. A previously undetected primary psychotic disorder that has been stable since well before his symptoms first began
- C. A long-standing eating disorder that might be the underlying driver of both his numbness and his perceptual changes
- D. An emerging neurocognitive decline that could explain his sense that he is observing himself from outside his own body

Question 6. What is the most appropriate treatment approach for his presentation?

- A. Cognitive behavioral therapy with psychoeducation and grounding, addressing catastrophic appraisals of the symptoms
- B. An antipsychotic-focused referral on the assumption that his detachment reflects an underlying psychotic process
- C. Encouraging him to keep avoiding the situations that worsen the detachment until the symptoms resolve on their own
- D. A primarily insight-oriented exploration of his early childhood as the central route to resolving his current symptoms

Question 7. How should the plan address his close monitoring of the detachment sensations?

- A. Help him reduce hypervigilant self-monitoring, since constantly checking the sensations tends to intensify and maintain them
- B. Encourage him to monitor the sensations even more closely so that he can detect the earliest sign of each episode coming on
- C. Tell him to keep a detailed minute-by-minute log of exactly when the detachment is present throughout each and every day
- D. Advise him to avoid any activity that might bring on the sensations so that he never has to notice the detachment at all

Question 8. Owen asks whether medication could help. The most appropriate counselor action is to:

- A. Coordinate a referral to a prescriber for evaluation, especially of comorbid anxiety or depression, while continuing therapy
- B. Recommend the specific medication and dose that tends to work best for people with depersonalization symptoms like his
- C. Tell him that medication is unnecessary and that the detachment will resolve entirely through the grounding techniques alone
- D. Suggest he research medications for dissociation himself and bring whichever one he prefers to a future appointment with you

Question 9. Owen says he is terrified the detachment means he is “going crazy.” The most therapeutic response is to:

- A. Validate the fear and provide psychoeducation that these symptoms, while frightening, are a recognized and treatable condition
- B. Agree that these symptoms are indeed a worrying sign and suggest that he may well be developing a serious mental illness
- C. Tell him quickly that there is absolutely nothing to worry about and that he should simply try to stop thinking about it
- D. Explain in technical detail the neuroscience of dissociation to convince him on scientific grounds that he is not going crazy

Question 10. Which technique is most useful to help Owen during an episode of detachment in session?

- A. Guide him through grounding that reconnects him with his senses and surroundings to reduce the intensity of the episode
- B. Encourage him to focus intently on exactly how detached and unreal he feels so he can fully understand the experience
- C. End the session immediately and send him home so that he can rest until the detachment passes on its own at home
- D. Wait quietly without intervening until the episode resolves entirely by itself before continuing the conversation

Question 11. Owen worries the diagnosis will appear in records and affect his academic standing. Best response?

- A. Explain how his records are protected and disclosed only with his consent or as required, supporting his informed decisions
- B. Reassure him by promising that you will simply leave the diagnosis out of his clinical record entirely to protect his standing
- C. Tell him his concern is unfounded and that no one would ever have any interest in his counseling records in the first place
- D. Offer to keep a private, separate set of unofficial notes about his diagnosis outside of his formal clinical record

Question 12. As CBT with interoceptive and grounding exercises is planned, informed consent most importantly requires:

- A. Explaining the rationale, that some exercises may briefly heighten the sensations, and that participation remains voluntary
- B. Assuring him in advance that the exercises will be completely comfortable and will never bring on any detachment at all
- C. Having him agree up front that he will complete every exercise assigned no matter how distressing it turns out to be for him
- D. Postponing any explanation of the exercises until after the first one so that his anticipatory anxiety does not build up first

Question 13. Owen reveals heavy cannabis use that began around when the symptoms started, and asks the counselor to ignore it. Best response?

- A. Explain that the cannabis use is clinically relevant to his symptoms and address it collaboratively rather than ignoring it
- B. Agree to leave the cannabis use out of the assessment and the plan entirely so that he feels more comfortable with you
- C. Tell him he must stop all cannabis use immediately or you will not be able to continue working with him in counseling at all
- D. Document the cannabis use but avoid ever discussing it with him so that the topic does not create tension between you

*Case 6 of 11***Recurrent depression with overlooked periods of elevated energy***Diagnosis: Bipolar II Disorder*

INTAKE

Elena Ortiz, a 34-year-old nurse, presents seeking help for recurrent depression. She describes several depressive episodes over the years and currently low mood, anhedonia, and poor sleep, and she came in mainly to ask about restarting an antidepressant.

SESSION TWO

On careful history she recalls distinct multi-day periods of unusually elevated, energized mood with decreased need for sleep, rapid thoughts, and heightened productivity that others noticed, without psychosis or hospitalization and not severe enough to be full mania.

SESSION THREE

She had not connected those “great” stretches to her mood problems and worries something is wrong with her. She denies sustained symptom-free intervals being only mild, and she denies any past episode requiring hospitalization or causing severe functional breakdown.

Question 1. What is most important to establish to support a bipolar II diagnosis?

- A. A history of at least one hypomanic episode in addition to her major depressive episodes, with no full manic episode
- B. That she can identify the single stressful event she believes first triggered her recurring problems with her mood
- C. That her current depressive symptoms are clearly more severe today than they were earlier in this same week
- D. That she currently meets the full symptom count required for a discrete manic episode at this point in time

Question 2. Which feature most clearly distinguishes her presentation from unipolar major depression?

- A. Her history of distinct hypomanic episodes, which unipolar depression does not include
- B. The fact that she has experienced several separate episodes of depressed mood and anhedonia over the course of years
- C. The poor sleep and the low energy that she is experiencing during her current depressive episode right now
- D. The way her mood seems to dip somewhat more in the darker winter months than during the brighter summer months

Question 3. How does her presentation differ from bipolar I disorder?

- A. Her elevated periods are hypomanic—without psychosis, hospitalization, or the severe impairment of a full manic episode
- B. The fact that during her elevated periods she felt more energetic and needed noticeably less sleep than she usually does
- C. The recurrent depressive episodes that she has experienced over the years between her elevated periods
- D. The way her symptoms tend to be a bit worse during the busiest and most stressful stretches at the hospital

Question 4. How does her presentation differ from cyclothymic disorder?

- A. She has had full major depressive episodes rather than only the milder, fluctuating mood symptoms of cyclothymia
- B. The fact that her mood has shifted up and down at various points over the course of several years of her life
- C. The elevated, energized periods that she has experienced at several points across the past few years
- D. The way her difficulties tend to feel worse during the most stressful and demanding weeks at her job

Question 5. Given her current depressive episode, what risk assessment is most important?

- A. A structured suicide-risk assessment, recognizing that suicide risk is elevated in bipolar depression
- B. A detailed reconstruction of every job she has held so the counselor can fully map her complete occupational history
- C. A precise log of how many hours she sleeps each night so the counselor can quantify the full extent of her insomnia
- D. A complete list of every antidepressant she has ever tried so the counselor can document her full medication history

Question 6. Elena asks the counselor to support her starting an antidepressant on its own. The most appropriate action is to:

- A. Coordinate a psychiatric evaluation and share that antidepressant monotherapy is a concern in bipolar II, given the diagnosis
- B. Tell her which specific antidepressant and dose she should ask her physician to start for her depressive symptoms
- C. Reassure her that an antidepressant by itself is the clearly correct treatment and encourage her to start one right away
- D. Tell her medication is entirely outside the counseling role and decline to discuss her question about it any further

Question 7. Once she is stabilized, which psychotherapy best supports relapse prevention?

- A. Interpersonal and social rhythm therapy to regularize her sleep-wake routines and help her detect early warning signs
- B. A primarily cathartic approach focused on fully venting the strong emotions that build up during her low periods
- C. Encouraging her to push herself to stay extremely productive and busy so that she does not slip back into a depressive low
- D. A long course of exposure therapy aimed at reducing her anxiety about possibly having another mood episode in the future

Question 8. What is most important to build into her ongoing plan?

- A. Mood and sleep monitoring with early-warning-sign tracking so emerging hypomania or depression can be caught and managed
- B. Tracking only her depressive symptoms while disregarding any signs of elevated or energized mood going forward
- C. Waiting until her next routine visit in several months to reassess her mood rather than monitoring it in the meantime
- D. Tracking mainly whether she attends her sessions and treating attendance as the main sign that she is doing well

Question 9. Elena is surprised and worried to learn her “great” periods may have been hypomania. Best response?

- A. Validate her reaction and provide psychoeducation about hypomania and bipolar II as understandable and manageable with treatment
- B. Tell her that this means she has a severe, lifelong mental illness that will almost certainly only get worse over time
- C. Reassure her quickly that the hypomania is nothing to be concerned about and that it does not really change anything at all
- D. Explain the detailed neurobiology of bipolar disorder at length to convince her on technical grounds not to worry about it

Question 10. How can the counselor best help Elena recognize early signs of a hypomanic episode?

- A. Collaboratively identify her personal early warning signs, such as reduced sleep need and racing thoughts, and a plan to respond
- B. Tell her not to worry about watching for any warning signs and to simply wait until a full episode is clearly underway
- C. Advise her to treat any happy or productive day as a warning sign of hypomania so that she stays especially vigilant
- D. Have her rely entirely on her family to notice any warning signs for her so that she does not have to monitor herself at all

Question 11. Elena's primary care provider calls to coordinate her care. What must come first?

- A. Obtain her informed consent and release before sharing any clinical information with the primary care provider
- B. Share the relevant clinical information right away, since coordinating with her physician is clearly in her own best interest
- C. Refuse to coordinate with the physician at all so that her counseling remains entirely separate from her medical care
- D. Share only her diagnosis with the physician without a release, since a diagnosis alone seems like a harmless detail to provide

Question 12. Elena asks the counselor what mood-stabilizer dose she should request. The most appropriate response is to:

- A. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate her questions and progress with them
- B. Suggest a typical starting dose that tends to work for most people who have bipolar II disorder like hers does
- C. Tell her to ask for the highest dose available so that her mood is sure to stay completely stable going forward
- D. Say medication dosing has nothing to do with counseling and decline to help her prepare for the conversation in any way

Question 13. The counselor has limited training in bipolar disorder. The most ethical course of action is to:

- A. Seek consultation and coordinate with a prescriber and team, or refer for the components beyond his demonstrated competence
- B. Continue to manage all of her care alone to preserve the continuity of the relationship he has built with her
- C. Tell her that her bipolar disorder is not serious enough to require any psychiatric involvement and continue supportive talks only
- D. Transfer her at once to another provider with no explanation, consultation, or coordinated plan for the transition of care

*Case 7 of 11***A fixed persecutory belief in an otherwise functioning man***Diagnosis: Delusional Disorder*

INTAKE

Harold Means, a 52-year-old accountant, is referred by his physician. For over four months he has been convinced that a neighbor is conspiring to poison his water supply, and he has installed locks and filters and confronted the neighbor, though there is no evidence.

SESSION TWO

Apart from this fixed belief he functions well at work and self-care, his speech is organized, and he shows no hallucinations, disorganized behavior, or negative symptoms. The belief is non-bizarre—plausible on its face—but held with unshakable conviction.

SESSION THREE

A recent medical and toxicology workup was unremarkable, and he denies substance use. He is angry that no one believes him and has hinted that he may "handle it himself" if the authorities will not act, which raises a concern about risk to the neighbor.

Question 1. What is most important to confirm to support a delusional disorder diagnosis?

- A. One or more delusions present for at least a month without other prominent active-phase psychotic symptoms
- B. That he can identify the single stressful event he believes first triggered his concerns about his neighbor
- C. That his belief has become noticeably more intense over the past few days than it was the week before that
- D. That he currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Which feature best distinguishes his presentation from schizophrenia?

- A. His functioning is otherwise preserved and he lacks the hallucinations, disorganization, and negative symptoms of schizophrenia
- B. The fact that he holds his belief about the neighbor with a very firm and unshakable degree of conviction over time
- C. The suspicion and the mistrust that he feels toward the neighbor he believes is conspiring against him
- D. The way his distress about the situation tends to be worse during the most stressful weeks at his accounting job

Question 3. How does his presentation differ from paranoid personality disorder?

- A. He holds a specific fixed delusion, rather than a pervasive lifelong trait of generalized suspiciousness without a fixed false belief
- B. The fact that he is mistrustful of the people around him and tends to interpret some of their actions as threatening to him
- C. The anger and the frustration that he expresses when other people do not believe his account of what is happening
- D. The way his concerns tend to feel more intense in the evenings than they do earlier during the daytime hours

Question 4. What is the most important condition to rule out before confirming the diagnosis?

- A. A substance- or medical-induced cause, which his unremarkable workup and denial of substance use make less likely but worth confirming
- B. A previously undetected primary anxiety disorder that might better account for his vigilance about his neighbor and his home
- C. A long-standing eating disorder that could be contributing to both his distress and his recent preoccupation with his water
- D. An emerging neurodevelopmental disorder that could explain why he has become so focused on the actions of his neighbor

Question 5. Given his hint that he may "handle it himself," what is the most important assessment step?

- A. Assess risk to the neighbor and to himself, including intent and means, and prepare to act on duty-to-protect obligations
- B. Focus only on whether he has a specific plan to harm himself and disregard any risk to the neighbor for the time being
- C. Defer any risk assessment until his delusion has resolved, since he is too distressed to answer the questions reliably now
- D. Rely on his assurance that he would never actually do anything, since he has not harmed anyone up to this point in time

Question 6. What is the most appropriate first-line treatment recommendation?

- A. Coordinate an urgent psychiatric evaluation for antipsychotic medication alongside supportive psychological care
- B. Begin intensive insight-oriented therapy to help him uncover the deeper meaning behind his belief about the neighbor
- C. Start him on an antipsychotic medication yourself and arrange to monitor his response and side effects over the coming weeks
- D. Recommend a course of weekly supportive counseling on its own as the complete treatment for his fixed delusional belief

Question 7. How should the counselor approach his delusional belief in the work?

- A. Neither directly confirm nor argue against the belief, focusing on the alliance and the distress and goals he can engage with
- B. Repeatedly and firmly challenge the belief with evidence until he is finally forced to admit that it is not actually true
- C. Agree with him that the neighbor is most likely conspiring against him so that he feels understood and stays in treatment
- D. Avoid the topic of the neighbor entirely so that the sessions never touch on anything that might upset or provoke him

Question 8. Given the risk concern, what should the plan prioritize regarding the neighbor?

- A. Ongoing risk monitoring and a plan that addresses safety, including protective steps if the risk to the neighbor escalates
- B. No particular attention to the neighbor, since the client has not actually harmed anyone at any point so far
- C. Immediate involuntary hospitalization based solely on the presence of the delusion, regardless of the assessed level of risk
- D. Encouraging him to confront the neighbor directly to resolve the conflict so that his distress about it finally settles down

Question 9. How should the counselor handle coordination with the psychiatrist and physician?

- A. Obtain his informed consent and release before sharing information, then collaborate as part of a coordinated team
- B. Share information with the other providers right away, since coordinating his care is clearly in his own best interest anyway
- C. Avoid contacting the other providers at all so that his counseling remains entirely separate from his medical care
- D. Share his full record with every provider so that they all have complete information about every aspect of his treatment

Question 10. Harold becomes angry that the counselor has not confirmed his belief. The most therapeutic response is to:

- A. Empathize with how distressing and isolating it feels to not be believed, without endorsing or attacking the belief itself
- B. Tell him plainly that the belief is false and that he simply needs to accept that the neighbor is not doing anything to him
- C. Agree with him that he is right about the neighbor so that he calms down and is willing to continue working with you
- D. End the session early to give him time to cool down rather than addressing his anger in the room with him at all

Question 11. How can the counselor best engage Harold given his limited insight into the belief?

- A. Build trust and work on shared, concrete goals such as reducing his stress and distress that he is willing to engage with
- B. Insist that he first acknowledge that the belief is false before any other work in counseling can begin with him
- C. Focus the sessions mainly on gathering more and more detail about the neighbor and the alleged conspiracy each week
- D. Tell him that there is nothing counseling can offer him unless he is first willing to take antipsychotic medication

Question 12. Harold makes a specific, credible threat to harm the named neighbor during a session. The most appropriate action is to:

- A. Assess the risk and follow duty-to-protect obligations, which may require warning or other protective steps
- B. Keep the threat entirely confidential, since anything he shares in counseling must always remain completely private
- C. Wait to see whether he repeats the threat at the next session before taking any action or assessing the risk
- D. Notify the neighbor and the police at once without first assessing the seriousness or following the proper protective process

Question 13. The counselor has limited training in psychosis care. The most ethical course of action is to:

- A. Seek consultation and coordinate with a treatment team, referring for the components beyond his demonstrated competence
- B. Continue managing the entire case alone to preserve the continuity of the relationship he has built with the client
- C. Tell the client his symptoms are not serious enough to need psychiatric care and keep seeing him for supportive talks only
- D. Transfer the client at once to another provider with no explanation, consultation, or coordinated plan for the transition

*Case 8 of 11***A bright child whose reading falls far below expectation***Diagnosis: Specific Learning Disorder*

INTAKE

Sofia Alvarez, a 9-year-old, is referred by her parents and teacher for persistent reading difficulties. Despite targeted help, her word reading and comprehension remain well below what is expected for her age, and she is increasingly avoidant and discouraged about school.

SESSION TWO

Her difficulties are specific to reading and have persisted for more than six months despite intervention. Her general reasoning and problem-solving are age-appropriate, she sustains attention in non-reading tasks, and there is no vision, hearing, or schooling deficit to explain it.

SESSION THREE

She is articulate and curious in conversation, which contrasts sharply with her struggle to decode text. She has begun saying she is "dumb" and dreads being asked to read aloud, and her parents worry the gap is starting to affect her confidence and her mood.

Question 1. What is most important to confirm to support a specific learning disorder diagnosis?

- A. Reading skill well below age expectations that has persisted over six months despite targeted intervention
- B. That her parents can name the single event they believe first caused her to fall behind in her reading
- C. That her reading has slipped a little further over the past few days than it had the week just before that
- D. That she currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Which feature best differentiates her presentation from inattentive ADHD?

- A. Her difficulty is specific to reading skill, not a broad inattention that shows up across many different tasks
- B. The fact that she finds schoolwork frustrating and would often rather avoid the tasks that she finds the hardest
- C. The discouragement and the low confidence that she shows about her performance at school these days
- D. The way her focus tends to be a little better on the calmer and less noisy days in her classroom

Question 3. How does her presentation differ from mild intellectual disability?

- A. Her general reasoning and problem-solving are age-appropriate, unlike the broad cognitive limits of intellectual disability
- B. The fact that she is currently performing below grade level in her reading at school this year
- C. The frustration she shows and the help that she clearly needs to keep up with the rest of her class
- D. The way her work tends to be a little better on the days when she has had a good night of sleep beforehand

Question 4. What is most useful to coordinate to support the assessment?

- A. A psychoeducational evaluation and school records that document the skill discrepancy and the response to intervention
- B. A detailed family pedigree going back several generations to find relatives who also disliked reading as children
- C. Whether her reading trouble can be traced to one specific bad school year that fully explains all of it
- D. Her favorite books and hobbies so that the sessions can be arranged around the topics she most enjoys

Question 5. What co-occurring issue is most important to screen for given her remarks about being "dumb"?

- A. Emerging anxiety and low mood related to the academic struggle, which commonly accompany learning disorders
- B. A previously undetected primary psychotic disorder that might better explain her trouble with reading at school
- C. A long-standing eating disorder that could be the single underlying cause of her reading difficulties
- D. An emerging neurocognitive decline that could account for why her reading is below where it is expected to be

Question 6. What is the most appropriate counselor role in supporting Sofia?

- A. Coordinate educational support and accommodations with the school while addressing the emotional impact in counseling
- B. Personally deliver intensive reading remediation lessons yourself as the central component of her treatment plan
- C. Tell her parents to simply have her read more at home and assume the problem will resolve on its own with practice
- D. Start her on a medication that you will select and adjust to improve her reading over the coming weeks

Question 7. How should the counselor address the emotional impact of her struggles?

- A. Build her confidence and coping, reframing the difficulty as a specific, workable challenge rather than a defect
- B. Tell her she just needs to try harder and that her difficulties will disappear if she simply puts in more effort
- C. Avoid discussing her feelings about school entirely so that the sessions never touch on anything that upsets her
- D. Focus the sessions mainly on testing her reading repeatedly so you can see exactly how far behind she really is

Question 8. How should progress be tracked in the counseling work?

- A. Track confidence, school avoidance, mood, and the use of accommodations over time to guide the support plan
- B. Track only her raw reading scores and base every counseling decision solely on whether those numbers improve
- C. Rely only on whether her parents happen to mention that she seems a little happier about school lately
- D. Wait until the end of the school year to review whether anything has changed at all since the first session

Question 9. How should the counselor coordinate with the school regarding accommodations?

- A. With the parents' consent and a release, collaborate with the school team on appropriate accommodations and supports
- B. Contact the school directly with her clinical details right away, since the school plainly needs all of the information
- C. Avoid contacting the school entirely so that her counseling stays completely separate from anything academic
- D. Send the school her full clinical record so that every staff member has complete information about her counseling

Question 10. Sofia tearfully says she hates reading aloud and wants to give up on school. The most therapeutic response is to:

- A. Validate how hard it feels, normalize the difficulty, and connect her to the supports that can make it manageable
- B. Tell her that reading is simply not that important and that she should not let it bother her so much going forward
- C. Agree that school may not be for her and that giving up could be a reasonable choice given how hard it has become
- D. Change the subject to something more cheerful so that she does not have to sit with the difficult feelings at all

Question 11. How can the counselor best engage Sofia given her age and discouragement?

- A. Use developmentally appropriate, strengths-based methods that highlight what she does well and build on it
- B. Talk with her mainly in abstract, adult terms about the neuropsychology of why her reading is impaired
- C. Focus the sessions mainly on drilling the reading skills she struggles with most until she finally improves
- D. Tell her she must improve her reading before any of the counseling work to help her confidence can begin

Question 12. Sofia's parents ask the counselor to label her as "gifted" in a report to qualify for a program. The most appropriate action is to:

- A. Decline to misrepresent her profile and clarify what the assessment and your role can accurately support
- B. Write the label exactly as they request, since keeping the parents satisfied is what matters most for the work
- C. Take the parents' side and advocate for the label in any report, regardless of what the evaluation actually shows
- D. Refuse to provide any documentation at all and decline to explain what could appropriately be reported about her

Question 13. The counselor lacks training in psychoeducational assessment. The most ethical course of action is to:

- A. Refer the formal evaluation to a qualified professional and stay within your supportive counseling role
- B. Conduct and interpret the psychoeducational testing yourself to keep everything within the counseling relationship
- C. Tell the parents the evaluation is unnecessary and that her difficulties will simply resolve with a bit more time
- D. Transfer the family at once to another provider with no explanation, consultation, or coordinated plan for the transition

*Case 9 of 11***Distressing physical symptoms and constant health worry***Diagnosis: Somatic Symptom Disorder*

INTAKE

Beatriz Alvarez, a 45-year-old office manager, presents with more than a year of distressing abdominal pain, fatigue, and headaches that dominate her daily life. She has seen many physicians and had repeated negative workups but remains highly distressed.

SESSION TWO

She spends hours each day worrying about what the symptoms mean, researching diseases, and seeking reassurance, and she has cut back work and social activities. The symptoms are real to her and disabling, whether or not a medical cause has been found.

SESSION THREE

She feels dismissed when providers say tests are normal and fears something is being missed. She is open to counseling but worried it implies her symptoms are “not real.” She denies that her worry is focused on one specific feared disease above the symptoms themselves.

Question 1. What is most important to confirm to support a somatic symptom disorder diagnosis?

- A. Distressing somatic symptoms for over six months with excessive thoughts, feelings, and behaviors devoted to them
- B. That every one of her physical symptoms has been medically proven to have no underlying organic basis whatsoever
- C. That she can identify the single stressful event she believes first triggered all of her physical symptoms
- D. That her physical symptoms have grown more intense over the past week than they were the week before that

Question 2. Which feature best differentiates her presentation from illness anxiety disorder?

- A. Her distress centers on the burdensome somatic symptoms themselves rather than on a fear of having a specific undiagnosed disease
- B. The fact that she worries about her health and seeks reassurance from her doctors fairly often over time
- C. The way she has cut back on her work and her social activities since the symptoms began to dominate her life
- D. The fact that her symptoms tend to feel somewhat worse during the busiest and most stressful weeks at her job

Question 3. How does her presentation differ from generalized anxiety disorder?

- A. Her excessive thoughts and behaviors are organized around her bodily symptoms rather than diffuse worry across many life domains
- B. The fact that she feels anxious and tense much of the time across most of the ordinary days of her week, whether or not her bodily symptoms happen to be flaring up at that particular moment
- C. The trouble she has sleeping and the fatigue that she reports across most of her ordinary workdays
- D. The way her symptoms seem to ease somewhat when she is busy and distracted at work or with friends

Question 4. How should the counselor approach the medical side of her presentation?

- A. Validate that her symptoms are real and distressing and coordinate with one consistent medical provider rather than dismissing them
- B. Order a fresh round of laboratory and imaging tests yourself so that you can personally rule out any missed medical condition
- C. Encourage her to keep seeking additional specialists and second opinions until a definitive physical diagnosis is finally found
- D. Tell her plainly that since the tests are normal her symptoms are psychological and there is no real medical issue to address

Question 5. What co-occurring condition is most important to screen for to inform her plan?

- A. Co-occurring depression and suicide risk, which commonly accompany chronic somatic distress and shape the plan
- B. A previously undetected primary psychotic disorder that could account for her conviction that something is being missed
- C. A long-standing eating disorder that could be contributing to both her abdominal symptoms and her fatigue over the past year
- D. An emerging neurocognitive decline that could explain why she spends so much time researching her symptoms each day

Question 6. What is the most appropriate first-line psychotherapy approach for her?

- A. Cognitive behavioral therapy targeting symptom-focused thoughts, reassurance-seeking, and avoidance, coordinated with her care
- B. Open-ended supportive counseling that mainly gives her space to describe her physical symptoms in detail each week
- C. A primarily insight-oriented exploration of her early childhood as the central route to relieving her physical symptoms
- D. Encouraging her to rest and to avoid activity whenever her symptoms flare so that she does not make the symptoms any worse

Question 7. How should care with her medical providers best be structured?

- A. Coordinate regular, scheduled visits with one primary provider rather than visits driven by each new symptom or crisis
- B. Encourage her to go to urgent care or the emergency department each time a symptom feels especially severe or frightening to her
- C. Advise her to stop seeing physicians altogether and to rely solely on counseling to manage all of her physical symptoms
- D. Recommend that she rotate among several different specialists so that she always has a fresh set of opinions to consider

Question 8. A central cognitive-behavioral target in her treatment would most appropriately be:

- A. Reducing reassurance-seeking and catastrophic symptom interpretations while gradually restoring valued activity
- B. Helping her develop a more thorough daily symptom-monitoring log so she can track every bodily sensation in fine detail
- C. Encouraging her to research each new symptom carefully so that she can feel more informed and certain about what is happening
- D. Teaching her to rest more and to limit her activity until her physical symptoms have fully and completely resolved

Question 9. Beatriz says counseling must mean the counselor thinks her symptoms are “all in her head.” Best response?

- A. Validate that her symptoms and suffering are real and explain that counseling addresses the distress and impact, not their reality
- B. Explain that the tests are all normal, so the symptoms really are psychological and that is exactly what counseling will fix
- C. Reassure her over and over that there is definitely nothing physically wrong with her so that she can stop worrying about it
- D. Avoid the question entirely and steer the session toward reviewing the activity goals she set for the coming week

Question 10. During a session Beatriz repeatedly asks the counselor to confirm her headaches are not a brain tumor. Best response?

- A. Gently name the reassurance-seeking as part of the cycle and help her practice tolerating uncertainty rather than answering it
- B. Provide her with a clear, confident answer every time she asks so that she can leave the session feeling calm and reassured
- C. Tell her that her worries are irrational and that as an intelligent adult she really should know better than to think this way
- D. Suggest she schedule another brain scan just to be sure so that she can finally put this particular worry to rest for good

Question 11. Beatriz asks the counselor to tell her physician to order a specific scan she read about online. Best response?

- A. Clarify that ordering tests is the physician's decision and offer, with consent, to coordinate her concerns with the provider
- B. Call her physician yourself and recommend that the specific scan she found online be ordered for her as soon as possible
- C. Encourage her to insist that her physician order the scan and to keep pushing until the provider finally agrees to do it
- D. Tell her this is purely a medical matter that has nothing to do with counseling and decline to discuss it any further with her

Question 12. How should the counselor coordinate with Beatriz's medical providers?

- A. Obtain her informed consent and release before communicating with her providers, then collaborate on a shared plan
- B. Contact her physicians right away to align the plan, since coordinating her care is clearly in her own best interest anyway
- C. Avoid any contact with her medical providers at all so that her counseling remains entirely separate from her medical care
- D. Share her full counseling records with every provider she sees so that they all have complete information about her treatment

Question 13. Beatriz becomes tearful and hints she sometimes feels life is not worth living with this suffering. Best step?

- A. Conduct a structured suicide-risk assessment and respond to the risk, recognizing chronic somatic distress can carry suicide risk
- B. Reassure her that her symptoms will eventually improve with treatment and that there is no real need to discuss those thoughts
- C. Continue with the planned cognitive-behavioral agenda, assuming the comment was only a passing expression of her frustration
- D. Ask her to sign a brief promise that she will stay safe and reach out to you if those thoughts get any worse this week

*Case 10 of 11***Individual disclosure of intimate partner violence and a partner's request to join therapy***Diagnosis: Acute Crisis / Decompensation*

INTAKE

Maria Delgado, a 31-year-old marketing coordinator, presents for individual outpatient counseling reporting six weeks of anxiety, poor concentration, and disrupted sleep that she links to increasing tension at home with her live-in partner of fourteen months, Danny.

SESSION TWO

In session she discloses that Danny has pushed her against a wall on two occasions in the past three months and, two weeks ago, choked her with both hands during an argument until her vision blurred; she also reports that he monitors her text messages, checks her location throughout the day, and has pressured her to stop seeing her sister and two close friends.

SESSION THREE

Before Maria's next appointment, Danny calls the front desk and asks to be added to her 'next session, so we can work on this together as a couple'; when told about the call, Maria appears frightened, says Danny does not know she disclosed the choking incident, and reports he has since questioned why she left the house for two hours the previous afternoon.

Question 1. Maria presents with six weeks of anxiety, poor concentration, and disrupted sleep but has not yet mentioned her relationship. What is the most appropriate intake step?

- A. Include direct, private questions about relationship safety as a routine part of the psychosocial intake
- B. Wait until Maria raises any relationship concerns herself before asking anything about her partner or home life
- C. Invite Danny to join part of the intake so the counselor can get a fuller picture of their home environment together
- D. Concentrate the entire intake on her sleep and concentration complaints and postpone psychosocial history to a later date

Question 2. After Maria discloses that Danny choked her two weeks ago, what should the counselor prioritize next in the assessment?

- A. A structured danger assessment covering the frequency, severity, and escalation pattern of the violence, including access to weapons
- B. A detailed account of every disagreement the couple has had since they moved in together fourteen months ago
- C. An immediate call to Danny to hear his account of the argument before proceeding further with Maria
- D. A referral to a couples communication skills class so the two of them can address the conflict pattern together

Question 3. Danny's choking of Maria is a well-established indicator of increased lethality risk in intimate partner violence. What follow-up is most important to include in the intake?

- A. Specifically ask about any prior strangulation, threats with weapons, and whether the violence has increased in frequency or severity
- B. Ask Maria to rate on a scale of one to ten how committed she currently feels to the relationship overall
- C. Focus only on the most recent incident and avoid asking about any earlier acts of violence to reduce her distress
- D. Refer her immediately to couples counseling so that Danny can learn healthier ways of managing his anger toward her

Question 4. Which finding best supports Acute Crisis / Decompensation rather than Generalized Anxiety Disorder in Maria's case?

- A. Her anxiety and hypervigilance emerged and sharply escalated in direct response to specific, ongoing violence and control
- B. Her worry spans multiple areas of daily life, including work performance and finances, in addition to the relationship
- C. Her anxiety has persisted for longer than six months and is accompanied by muscle tension and restlessness
- D. Her symptoms are severe enough to interfere significantly with her concentration and daily functioning at work

Question 5. Beyond the presenting anxiety symptoms, what else is essential to assess given Maria's exposure to recent near-lethal violence?

- A. Her own current risk for suicide or self-harm, using a structured screen, since intimate partner violence significantly raises that risk
- B. Whether her anxiety symptoms meet full criteria for panic disorder with recurrent, unexpected panic attacks
- C. Whether she has a family history of anxiety disorders that could help explain her current presentation
- D. Whether her sleep problems are better explained by an untreated primary sleep disorder such as chronic insomnia disorder unrelated to any relationship stress

Question 6. What should be the cornerstone of the initial treatment plan for Maria at this point?

- A. A collaborative, individualized safety plan covering warning signs, coping steps, safe contacts, and options for leaving or staying
- B. A directive plan requiring her to end the relationship with Danny and move out within the next two weeks
- C. A structured anger-management referral for Danny that Maria can encourage him to attend on his own so the relationship can continue unchanged
- D. A plan focused mainly on improving her sleep hygiene and daily relaxation practice to reduce her anxiety

Question 7. Danny has called asking to be included in Maria's next session 'so we can work on this together as a couple.' What is the most appropriate treatment-planning response?

- A. Decline the joint session and continue individual treatment, since conjoint work during active violence can raise danger and suppress disclosure
- B. Agree to bring Danny into the next session to begin building healthier communication between the two of them
- C. Tell Danny that Maria can decide for herself and let her choose whether he joins without first discussing the risks with her
- D. Decline the joint session but offer to schedule Danny his own separate individual appointment with the same counselor

Question 8. What is the most appropriate way to help Maria access additional support while respecting her autonomy?

- A. Offer information about IPV-specific resources such as a hotline and a local advocate, and let her decide what steps to take and when
- B. Insist that she call a domestic violence hotline before the end of today's session as a condition of continuing treatment
- C. Give her a general community resource list without any information specific to intimate partner violence
- D. Wait for her to ask about resources on her own rather than proactively raising the option of an IPV advocate

Question 9. How should the safety plan specifically address the choking incident and its elevated lethality risk?

- A. Include specific warning signs preceding escalation and an immediate physical-safety plan for recurrence, built collaboratively with Maria
- B. Advise Maria to try to physically restrain Danny herself the next time she senses he is becoming angry
- C. Recommend that Maria simply avoid arguments with Danny so the situation does not escalate to that point again
- D. Focus the plan solely on long-term relationship counseling goals for the couple rather than any immediate physical-safety steps for Maria

Question 10. Maria says, 'He's just really stressed at work, I don't think he means to hurt me.' What is the most therapeutic response?

- A. Validate her feelings toward Danny without judgment while gently naming the behaviors as unsafe, at a pace she can tolerate
- B. Firmly tell her that she is minimizing serious abuse and that she needs to see the relationship for what it really is
- C. Agree that work stress likely explains his behavior and focus the conversation on how he can better manage that stress
- D. Change the subject to her sleep and anxiety symptoms so she does not feel pressured to discuss the relationship further

Question 11. Maria says she sometimes 'pushes his buttons' and wonders if the violence is partly her fault. What is the best counselor response?

- A. Gently normalize that self-blame is common after abuse and clarify that Danny alone is responsible for his violent behavior
- B. Explore what she does that seems to trigger Danny so she can learn to avoid provoking him in the future
- C. Reassure her that the relationship will likely improve once she works out these behavior patterns in therapy
- D. Avoid directly addressing the self-blame and instead redirect the conversation toward practical coping skills

Question 12. When the counselor returns Danny's call, what is the appropriate ethical response regarding confidentiality?

- A. Decline to confirm or discuss whether Maria is a client, or share any session information, without her explicit consent
- B. Confirm that Maria is a client and explain that the counselor recommends individual rather than couples work
- C. Tell Danny that Maria discussed him in session but withhold the specific content of what she said
- D. Avoid returning the call at all and let Maria explain the situation to Danny on her own without further guidance

Question 13. How should the counselor document the session and support Maria's decision-making about her options going forward?

- A. Document the danger assessment and safety plan thoroughly, and offer protective order information while leaving the timing to Maria
- B. Document that Maria must file a protective order and file the paperwork on her behalf to ensure her safety
- C. Omit details of the violence from the record to protect Maria's privacy from any future disclosure
- D. Note only that general relationship stress was discussed in session, without recording any of the specific safety concerns that Maria raised

*Case 11 of 11***Progressive memory loss and lost independence in an older adult***Diagnosis: Major Neurocognitive Disorder due to Alzheimer's Disease*

INTAKE

Margaret Cole, a 78-year-old retired librarian, is brought by her daughter for two years of gradually worsening memory loss. She repeats questions, gets lost on familiar routes, has trouble managing her finances, and increasingly needs help with daily tasks.

SESSION TWO

Her daughter reports a slow, steady decline rather than a sudden change, with clear attention most of the time and no fluctuating confusion. Margaret is aware that something is wrong and is frustrated and at times tearful about losing her independence.

SESSION THREE

A recent medical workup is underway, and there is no evidence of an acute illness, fever, or new medication causing the change. The daughter worries about safety with driving, the stove, and medications, and feels exhausted and unsure how to help her mother.

Question 1. What is most important to confirm to support a major neurocognitive disorder diagnosis?

- A. A significant decline from her prior level in two or more cognitive domains that interferes with her everyday independence
- B. That she can identify the single stressful event she believes first triggered the memory problems she has been having
- C. That her memory has worsened over the past day or two more sharply than it did earlier in the same week
- D. That she currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Which feature best distinguishes her presentation from delirium?

- A. Her decline is gradual with clear attention, rather than an acute, fluctuating disturbance of attention and awareness
- B. The fact that she has clear trouble with her memory and forgets recent conversations and information across most days now
- C. The way she becomes more confused and disoriented during the busiest and most overstimulating times for the family at home
- D. The fact that she is older and has several chronic medical conditions that are common among adults of her particular age

Question 3. How should the counselor weigh a possible depressive contribution to her cognitive problems?

- A. Screen for depression, since depression can impair cognition and may coexist with or mimic a neurocognitive disorder
- B. Assume her sadness is simply a normal and expected reaction to aging that has no bearing on her thinking or memory at all
- C. Conclude that because she is sad her cognitive decline must be entirely due to depression and not to any neurocognitive disorder
- D. Whether her sadness is clearly more intense today than it was earlier in this same week of assessment sessions

Question 4. What is the most appropriate counselor action regarding the diagnostic workup?

- A. Use cognitive screening and collateral history and coordinate a medical and neurological evaluation rather than diagnosing alone
- B. Order brain imaging and laboratory tests yourself so that you can personally confirm the Alzheimer's diagnosis before treatment
- C. Tell the family it is simply normal aging and that there is no real need for any further medical evaluation at this point
- D. Base the diagnosis only on how Margaret presents during the session, without collateral history or any medical evaluation

Question 5. Given the daughter's concerns, what safety assessment is most important?

- A. Assess safety risks such as driving, the stove, wandering, and medication management given her decline and impaired judgment
- B. Focus only on whether she currently has a specific plan to harm herself and disregard the broader safety concerns for now
- C. Defer any safety assessment until the medical workup is fully complete, even though the daughter is worried about it now
- D. Rely on Margaret's own assurance that she is managing fine, since she knows her own daily routine better than anyone else

Question 6. What is the most appropriate treatment-planning priority for Margaret and her family?

- A. Coordinate medical care and build a plan of caregiver support, safety measures, and structured non-pharmacologic approaches
- B. Begin intensive insight-oriented psychotherapy to help her recover the specific memories that she has been losing over time
- C. Start her on a cognitive-enhancing medication yourself and monitor her response and dosing over the coming weeks
- D. Recommend that the family wait and simply monitor her decline for several more months before arranging any support at all

Question 7. Which non-pharmacologic element is most appropriate to include in her care?

- A. Structured routines, environmental cues, and orientation supports that reduce confusion and support her remaining abilities
- B. An intensive daily memory-training program designed to push her to fully restore the abilities she has been losing recently
- C. A recommendation that the family frequently quiz her on dates and facts to keep her mind as sharp as they possibly can
- D. A plan to frequently change her environment and routine so that she is constantly challenged and never becomes too bored

Question 8. How should the counselor best support the exhausted daughter?

- A. Provide caregiver psychoeducation and connect her with respite and support resources to reduce caregiver burden
- B. Advise the daughter to manage every aspect of her mother's care entirely on her own to keep things consistent for her mother
- C. Tell the daughter that caregiver stress is not really within the focus of counseling and redirect to the client's symptoms only
- D. Encourage the daughter to avoid telling her mother anything about the diagnosis so that her mother never feels any distress

Question 9. Margaret becomes tearful about losing her independence. The most therapeutic response is to:

- A. Validate her grief and preserve her dignity and personhood while supporting the abilities and choices she still retains
- B. Reassure her quickly that everything is completely fine and that she has absolutely nothing at all to feel upset about today
- C. Speak mainly to her daughter about Margaret's care while Margaret is present, since her daughter can answer more reliably
- D. Provide her with detailed statistics about the typical progression of Alzheimer's disease to prepare her for what is coming

Question 10. As her memory worsens, how should the counselor adapt the work?

- A. Use simple, concrete communication, shorter sessions, and supportive engagement adapted to her current cognitive abilities
- B. Continue using the same complex, insight-oriented techniques you would use with any cognitively unimpaired adult client
- C. Assign her detailed written homework to complete independently between sessions to keep her actively working on her own
- D. Reduce contact with her and work mainly through her daughter, since Margaret can no longer benefit from any direct engagement

Question 11. Margaret's capacity to make some decisions is now in question. The most appropriate approach is to:

- A. Assess her capacity for specific decisions, involve her to the fullest extent possible, and engage surrogates as appropriate
- B. Assume she completely lacks capacity for all decisions and have her daughter decide everything without involving Margaret at all
- C. Assume she has full capacity for every decision and proceed exactly as usual without considering her cognitive impairment at all
- D. Postpone all decisions indefinitely until her capacity is perfectly clear, even though some decisions need to be made now

Question 12. How should the counselor handle information-sharing among Margaret and her family?

- A. Clarify consent and confidentiality, respecting Margaret's wishes and capacity while engaging family with appropriate authorization
- B. Share all information freely with every family member, since family clearly needs to know everything to be able to help her
- C. Refuse to involve any family at all, treating everything as fully confidential exactly as with an unimpaired, independent adult
- D. Let the daughter decide on her own what Margaret should and should not be told about her own diagnosis and her care

Question 13. The counselor has limited training in geriatric neurocognitive care. The most ethical course of action is to:

- A. Seek consultation and coordinate with a geriatric or medical team, or refer for the components beyond his competence
- B. Continue managing the entire case alone to preserve the continuity of the relationship he has built with the family
- C. Tell the family the situation is not serious enough to involve any medical or geriatric specialists and continue supportive talks
- D. Transfer the family at once to another provider with no explanation, consultation, or coordinated plan for the transition

Answer Key — Exam Two

Case 1: Twenty years of feeling "flat" in a small hardware-store town

Diagnosis: Persistent Depressive Disorder

Q1. A. That his low mood has been present most of the day, more days than not, for at least two years, without a symptom-free stretch longer than two months

Chronic duration is the defining PDD criterion

Common error: Treating chronic low mood as simply a personality trait

B. A precise onset date does not establish chronicity — Requiring an exact onset before assessing duration

C. Family history supports but does not confirm the diagnosis — Requiring family history to confirm a chronic mood disorder

D. Day-to-day variation is not the diagnostic anchor — Focusing on situational triggers instead of overall course

Q2. A. A review of his medications, substance use, and any thyroid or other medical conditions that could mimic chronic low mood

Medical and substance causes must be excluded

Common error: Skipping the organic rule-out for a chronic presentation

B. Exhaustive hobby history is low yield here — Collecting detail unrelated to the differential

C. Third-party opinion is not a diagnostic priority — Prioritizing collateral impressions over rule-outs

D. Workload mapping does not inform the diagnosis — Gathering administrative detail instead of clinical history

Q3. A. Asking whether he has ever had a period of elevated or irritable mood with decreased need for sleep or increased energy

A lifetime manic/hypomanic history changes the diagnosis

Common error: Skipping the lifetime bipolar screen

B. Panic history is a different rule-out — Confusing an anxiety screen with the mood differential

C. ADHD screening is a lower priority here — Chasing a neurodevelopmental explanation before the mood history

D. Psychosis screening matters but is secondary here — Over-reading ordinary low self-esteem as psychosis

Q4. A. His chronically low energy, poor appetite, and sleep-onset difficulty occurring alongside the low mood most days

Two or more associated symptoms meet Criterion B

Common error: Overlooking that low mood alone is not sufficient

B. Reliable functioning is not an associated depressive symptom — Confusing resilience with diagnostic criteria

C. A quiet preference alone is not a listed criterion symptom — Pathologizing temperament rather than symptoms

D. A checking habit is not part of the symptom list — Mistaking an unrelated habit for a diagnostic symptom

Q5. A. Use a structured suicide-risk screen to confirm and document the absence of ideation, intent, plan, or prior behavior

Structured screening confirms and documents low risk

Common error: Skipping a structured screen because risk seems low

B. Skipping the screen leaves risk undocumented — Assuming calm demeanor rules out the need to screen

C. No-suicide contracts are not evidence-based practice — Substituting a written promise for real assessment

D. Delaying a baseline safety check is unnecessary and unsafe practice — Postponing a routine safety baseline

Q6. A. Offer evidence-based psychotherapy such as cognitive behavioral therapy or behavioral activation adapted for chronic depression

First-line psychotherapy fits both the evidence base and scope

Common error: Defaulting to a medication recommendation

B. Prescribing and dose adjustment are outside counselor scope — Acting outside the scope of counseling practice

C. Watchful waiting is inappropriate for a chronic, treatable condition — Delaying indicated psychotherapy

D. Lifestyle changes alone are not a sufficient first-line plan — Offering adjunct steps as the entire treatment plan

Q7. A. Re-administer a validated symptom measure such as the PHQ-9 at regular intervals to guide measurement-based care

Repeated standardized measures guide treatment decisions

Common error: Relying only on subjective impressions of progress

B. An informal check-in lacks a standardized benchmark — Trusting unstandardized self-report alone

C. A single end-point review misses early lack of progress — Skipping interim outcome monitoring

D. Attendance and adherence are necessary but not sufficient — Mistaking engagement for symptom improvement

Q8. A. Provide general information and, with his consent, coordinate a referral to a prescriber for a medication evaluation

Referral and coordination stay within counselor scope

Common error: Offering specific medication advice directly

B. Naming a specific medication exceeds counselor scope — Overstepping into medical decision-making

C. Discouraging a medical evaluation overreaches the counselor's role — Steering a medical decision away from a prescriber

D. Outsourcing to self-research replaces proper coordination of care — Replacing referral with unsupervised self-research

Q9. A. Collaboratively schedule one or two small, valued activities each week, such as a fishing trip he used to enjoy

Graded valued activity is an appropriate first behavioral activation step

Common error: Waiting for motivation to return before scheduling activity

B. Too much change too quickly invites early failure — Ignoring the need for graded pacing

C. Thought logging is useful but is not the first behavioral step here — Confusing cognitive work with the initial behavioral step

D. Waiting for motivation reverses behavioral activation logic — Letting mood gate all behavioral change

Q10. A. Offer psychoeducation that chronic low mood is a treatable condition, while validating how normal it felt to him for years

Psychoeducation reframes an ego-syntonic belief without dismissing his experience

Common error: Either lecturing him or only agreeing that it is just who he is

B. Blunt correction risks feeling invalidating this early — Disputing before validating the client's experience

C. Agreeing that it is unchangeable forecloses treatment — Colluding with the client's mistaken belief

D. Avoiding the belief leaves a central misconception unaddressed — Avoiding material central to engagement in treatment

Q11. A. Acknowledge his doubt as understandable and collaboratively set one small, achievable goal to test it against

Naming the doubt and testing it behaviorally builds realistic hope

Common error: Trying to argue him out of his doubt

B. Guaranteeing an outcome risks credibility if progress is gradual — Making promises that cannot reliably be kept

C. Agreeing with hopelessness abandons a treatable target — Matching the client's hopelessness instead of gently challenging it

D. A lecture rarely shifts longstanding skepticism by itself — Substituting information for a concrete experience of change

Q12. A. The limits of confidentiality, including the circumstances under which information must be disclosed to others

Confidentiality limits must be disclosed as part of informed consent

Common error: Omitting the limits of confidentiality during consent

B. Promising absolute confidentiality misstates the actual limits — Guaranteeing unconditional secrecy to the client

C. Treatment goals are set collaboratively, not unilaterally — Removing the client's voice from goal-setting

D. Consent to treatment remains ongoing and revocable — Treating initial consent as a permanent commitment

Q13. A. Discuss the likelihood of unavoidable public encounters with him in advance and agree on how to handle them respectfully

Proactively addressing unavoidable overlap protects the client and the relationship

Common error: Ignoring foreseeable overlap until it happens unexpectedly

B. Automatic termination is unnecessary and may not be feasible in a rural area — Refusing rural clients over any foreseeable public overlap

C. Avoiding the topic leaves the client unprepared for a foreseeable situation — Failing to plan for a predictable boundary issue

D. Asking the client to change his livelihood is an inappropriate boundary solution — Placing the burden of boundary management on the client's job

Case 2: Sudden surges of fear with a racing heart

Diagnosis: Panic Disorder

Q1. A. That the unexpected attacks are recurrent and followed by a month or more of worry or avoidance about further attacks

Recurrent attacks plus persistent worry define it

Common error: Diagnosing from one attack alone

B. A reliable cue points away from panic disorder — Requiring a consistent trigger

C. Chronic diffuse worry suggests GAD — Confusing panic with generalized worry

D. Severity does not establish the diagnosis — Reading severity as the criterion

Q2. A. Discrete attacks that peak within minutes rather than a persistent, free-floating background of multi-domain worry

Discrete peaks versus chronic worry

Common error: Treating chronic worry as panic

B. Somatic symptoms occur in both — Using shared symptoms to differentiate

C. Performance worry is nonspecific — Reading worry content as diagnostic

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. Confirm that a medical evaluation has ruled out cardiac causes and coordinate further workup if any new symptoms arise

Verify medical rule-out, coordinate as needed

Common error: Assuming anxiety without a medical rule-out

B. Ordering tests exceeds counselor scope — Acting outside scope of practice

C. Flat reassurance feeds the cycle — Reinforcing reassurance-seeking

D. Dismissing symptoms is unsafe and invalidating — Ignoring the medical differential outright

Q4. A. Her attacks arise unexpectedly and at rest rather than only when she confronts one specific feared situation or object

Uncued attacks separate panic from phobia

Common error: Forcing uncued panic into a phobia frame

B. Avoidance appears in both — Using avoidance alone to differentiate

C. The fear response is shared — Reading the fear response as decisive

D. Catastrophic appraisal occurs in both — Using appraisal content to differentiate

Q5. A. Whether agoraphobic avoidance is emerging as she increasingly avoids places where escape or help might be difficult

Screen for emerging agoraphobia

Common error: Missing spreading avoidance

B. Illness preoccupation is a different focus — Reframing panic as illness anxiety

C. Hypomania screen is low yield here — Chasing an unsupported rule-out

D. Diurnal pattern is not central — Over-reading a circadian pattern

Q6. A. Cognitive behavioral therapy with interoceptive exposure to the feared bodily sensations that drive her panic

CBT with interoceptive exposure is first-line

Common error: Choosing an unsupported approach

B. Unstructured support underperforms CBT — Defaulting to nondirective support

C. Breathing as escape becomes a safety behavior — Turning coping skills into avoidance

D. Insight work is not first-line for panic — Prioritizing origins over exposure

Q7. A. Repeatedly and deliberately bringing on the feared sensations so she learns they are not dangerous and her fear extinguishes

Provoke sensations to extinguish the fear

Common error: Avoiding the sensations she fears

B. Distraction prevents new learning — Teaching distraction instead of exposure

C. Hypervigilant monitoring fuels panic — Encouraging body-checking

D. Avoiding exertion entrenches avoidance — Sanctioning avoidance of arousal

Q8. A. Identify them as safety behaviors and gradually drop them so she can learn the feared catastrophes do not occur

Fade safety behaviors during exposure

Common error: Leaving safety behaviors in place

B. Preserving rituals maintains the fear — Reinforcing safety behaviors

C. They are not harmless in panic — Overlooking safety behaviors

D. New rituals are still avoidance — Trading rituals rather than dropping them

Q9. A. Coordinate a referral to a prescriber for evaluation while continuing exposure-based therapy within her own scope

Refer and coordinate within scope

Common error: Giving prescriptive advice

B. Naming a drug exceeds scope — Overstepping professional limits

C. PRN sedatives can become safety behaviors — Recommending a specific PRN strategy

D. Discouraging meds overreaches — Steering a medical decision

Q10. A. Explain the panic cycle: how catastrophic misreading of normal sensations escalates arousal into a full-blown attack

Teach the panic cycle

Common error: Teaching skills without a rationale

B. Repeated reassurance feeds the cycle — Reinforcing the maintaining behavior

C. Excess detail is not the priority — Overloading instead of clarifying

D. “Just relax” is unhelpful — Giving platitudes instead of education

Q11. A. Calmly coach her to stay with the sensations and notice them passing rather than fleeing or fighting the attack

Coach staying with the sensations

Common error: Helping her escape the attack

B. Escape reinforces avoidance — Reinforcing escape behavior

C. Frantic control becomes a safety behavior — Using skills to suppress rather than tolerate

D. Repeated reassurance maintains panic — Reinforcing the maintaining behavior

Q12. A. Clarify that medication decisions rest with her prescriber and offer, with consent, to coordinate her concerns with them

Stay in scope; coordinate with consent

Common error: Directing another provider’s medical care

B. Directing medication changes exceeds scope — Advising the client to stop a prescribed medication

C. Recommending discontinuation oversteps — Inserting yourself into medical management

D. Refusing to coordinate abandons the concern — Declining to help her communicate concerns

Q13. A. Explaining the rationale, the temporary discomfort exposure can provoke, and that participation remains voluntary

Disclose rationale, discomfort, and choice

Common error: Starting exposure without informed consent

B. Misrepresenting exposure is dishonest — Misstating what the treatment involves

C. Consent is voluntary and revocable — Treating consent as a binding commitment

D. Withholding the method violates consent — Intervening before explaining

Case 3: Dazed and hypervigilant three weeks after a car crash

Diagnosis: Acute Stress Disorder

Q1. A. That she directly experienced the threatened injury of the crash and that it has been under one month since it happened

Exposure plus the ASD time window anchors the diagnosis

Common error: Diagnosing from symptom count alone without confirming exposure and timing

B. Precise recall of the crash mechanics is not diagnostic — Treating narrative precision as a diagnostic requirement

C. Third-party corroboration is not required to diagnose — Requiring outside witnesses before accepting the client's report

D. Insurance status is irrelevant to the diagnosis — Confusing administrative closure with clinical readiness

Q2. A. Whether this reflects a dissociative symptom such as derealization, one of the symptom clusters counted toward ASD

Derealization is a counted ASD dissociative symptom

Common error: Dismissing derealization as vague or unimportant

B. Assuming a missed brain injury is not the counselor's call — Making a medical diagnosis outside the counselor's scope

C. Assuming malingering is unwarranted and harmful — Defaulting to suspicion instead of clinical assessment

D. Lifetime history is secondary to characterizing the current episode — Chasing lifetime history before scoping the presenting episode

Q3. A. A structured suicide risk screen covering ideation, intent, plan, and behavior, done routinely regardless of apparent risk

Routine structured screening is standard practice

Common error: Skipping safety screening because risk seems unlikely

B. Skipping screening based on appearance is unsafe practice — Assuming a calm presentation rules out risk

C. Prior hospitalization alone does not assess current risk — Substituting history for a current risk assessment

D. Deferring routine safety screening is inappropriate — Postponing a standard safety screen

Q4. A. The qualifying symptom cluster persisting for one month or longer after the crash rather than resolving before then

Duration past one month is the reclassifying factor

Common error: Believing symptom severity, not duration, drives the switch

B. Content intensity does not reclassify the disorder — Equating worsening symptoms with a diagnostic switch

C. Broader avoidance alone does not reclassify the diagnosis — Using symptom breadth instead of duration to reclassify

D. Fault determination has no diagnostic relevance — Conflating legal outcomes with clinical diagnosis

Q5. A. Her reaction meets Criterion A trauma exposure and the specific ASD symptom cluster that Adjustment Disorder does not require

ASD requires exposure and cluster criteria Adjustment lacks

Common error: Defaulting to Adjustment Disorder for any stressor-linked distress

B. Rapid onset does not rule out Adjustment Disorder by itself — Using onset speed as the sole differential test

C. Proportionality of distress is not the deciding factor here — Judging severity instead of applying the symptom criteria

D. Anticipated recovery time does not establish the diagnosis — Diagnosing from anticipated recovery rather than present criteria

Q6. A. Begin trauma-focused psychotherapy incorporating psychoeducation, grounding skills, and gradual exposure to driving cues

Trauma-focused therapy is the first-line, within-scope approach

Common error: Defaulting to a medication recommendation instead

B. Prescribing is outside counselor scope of practice — Acting outside scope of practice

C. Watchful waiting delays indicated early intervention — Delaying indicated care while symptoms are active

D. Avoiding the trauma content undercuts effective treatment — Reinforcing avoidance instead of addressing it

Q7. A. Track her symptom cluster and functioning at each session to note whether it resolves or persists past one month

Ongoing tracking clarifies whether ASD resolves or persists

Common error: Setting the plan once and never reassessing it

B. A single end-point check misses the ASD-to-PTSD window — Skipping interim monitoring entirely

C. Attendance alone does not measure clinical progress — Mistaking attendance for clinical improvement

D. Third-party workplace comments are an unreliable clinical measure — Substituting outside opinions for direct assessment

Q8. A. Provide general information and, with her consent, coordinate a referral to a prescriber for a full evaluation

Referral and coordination stay within scope

Common error: Offering specific medication recommendations directly

B. Naming a specific medication exceeds scope — Overstepping professional scope of practice

C. Discouraging a medical evaluation overreaches — Steering a client away from a medical decision

D. Self-directed medication research is poor care coordination — Outsourcing a medical question instead of referring

Q9. A. Teach brief grounding techniques, such as naming sensory details in the room, to use during dazed or unreal moments

Grounding directly targets dissociative episodes

Common error: Jumping to exposure before she can ground herself

B. Exposure before stabilization skips a needed step — Skipping stabilization before trauma processing

C. Dismissing the symptom is invalidating and unhelpful — Telling a client to ignore a distressing symptom

D. Blanket avoidance reinforces the symptom pattern — Prescribing more avoidance instead of coping skills

Q10. A. Normalize the avoidance as a common, treatable stress response rather than a personal weakness, and validate her frustration

Psychoeducation reframes self-blame around avoidance

Common error: Either lecturing her or only sympathizing without information

B. Endorsing the self-blame deepens it — Colluding with a client's harsh self-judgment

C. Premature confrontation can feel invalidating — Disputing before validating the client's experience

D. Avoidance of the topic leaves the belief unaddressed — Avoiding material that feels clinically important

Q11. A. Listen empathically, reflect her distress, and gently pace the discussion so she does not feel overwhelmed or flooded

Paced empathic listening supports processing without flooding

Common error: Letting the client become overwhelmed without pacing

B. Avoiding the material leaves it unprocessed — Steering away from distressing content out of discomfort

C. Pushing for more graphic detail risks flooding her — Prioritizing narrative detail over clinical pacing

D. Dismissing the nightmare's significance is invalidating — Telling a client their symptom does not matter

Q12. A. The limits of confidentiality, including when serious foreseeable harm to self or others would require disclosure

Disclosing confidentiality limits is required at consent

Common error: Omitting the limits of confidentiality from the consent discussion

B. Absolute confidentiality is an inaccurate statement — Guaranteeing unconditional secrecy to a client

C. Goals should be set collaboratively, not imposed — Removing the client's voice from goal-setting

D. Consent to treatment is ongoing and revocable — Treating initial consent as binding for the full course

Q13. A. Explain the release-of-information process, obtain her signed written authorization, and coordinate the disclosure appropriately

A signed release properly authorizes any disclosure

Common error: Sending records out without a proper signed authorization

B. Releasing records without written authorization is improper — Skipping the required written authorization step

C. A blanket refusal misstates how disclosures work — Refusing outright instead of following the proper process

D. An informal verbal summary bypasses proper documentation procedure — Substituting an informal verbal account for a proper release

Case 4: From prescribed pain pills to daily opioid use

Diagnosis: Opioid Use Disorder, Severe

Q1. A. The number of DSM-5-TR criteria he meets over twelve months, since six or more place severity in the severe range

Criterion count sets severity

Common error: Estimating severity by drug type alone

B. Quantity alone does not set the diagnosis — Equating dose with severity

C. Route of onset does not set the diagnosis — Using the origin story to diagnose

D. Family history is risk, not diagnosis — Using family history to diagnose

Q2. A. As opioid withdrawal—a physiologic cessation syndrome that is one criterion within his use disorder, not a separate problem

Withdrawal is a criterion within OUD

Common error: Treating withdrawal as a separate diagnosis

B. Duration is not what defines withdrawal — Equating withdrawal with chronicity

C. It is withdrawal, not a primary anxiety disorder — Mislabeling withdrawal as anxiety

D. Level of care follows assessment — Jumping to a level of care prematurely

Q3. A. Assess overdose risk and withdrawal safety, and provide overdose-prevention education including naloxone access

Address overdose risk and harm reduction

Common error: Overlooking overdose risk in the plan

B. Root-cause work is not the safety priority — Prioritizing insight over immediate safety

C. Unsupervised tapering can be dangerous — Recommending an unsupervised taper

D. Minimizing overdose risk is unsafe — Downplaying a lethal risk

Q4. A. That he meets six or more of the eleven criteria, whereas four to five would indicate moderate severity instead

Six or more criteria define severe

Common error: Under-rating a severe presentation

B. Switching substances is not the severity test — Grading severity by what he uses

C. One consequence is not the severity test — Grading severity by a single consequence

D. Craving occurs across severities — Reading one symptom as the severity marker

Q5. A. Suicide risk and co-occurring depression, screened with a structured tool given his hopelessness and recent loss

Screen suicide risk and depression

Common error: Treating the OUD without screening risk

B. Psychosis is not indicated — Pursuing an unsupported rule-out

C. Eating pathology is not indicated — Inventing an unsupported comorbidity

D. Cognitive decline is premature here — Over-attributing to cognitive decline

Q6. A. Coordinate a referral for medication for opioid use disorder, such as buprenorphine or methadone, alongside counseling

MOUD is first-line; refer and coordinate

Common error: Offering counseling alone for severe OUD

B. Withholding MOUD contradicts the evidence — Repeating the substitution myth

C. Gatekeeping delays life-saving care — Conditioning treatment on prior abstinence

D. Psychosocial support alone is insufficient — Omitting medication from the plan

Q7. A. Use a multidimensional assessment of withdrawal risk, environment, and supports to match him to a safe level of care

Use multidimensional placement

Common error: Defaulting to one fixed program

B. Maximal care ignores placement criteria — Over-placing without assessment

C. Under-placement ignores his risk — Under-treating a high-risk presentation

D. Preference cannot override clinical need — Letting convenience drive placement

Q8. A. Naloxone access and education plus never-use-alone guidance to reduce his risk of a fatal overdose during treatment

Naloxone and overdose prevention

Common error: Omitting overdose prevention

B. Punitive discharge raises overdose risk — Punishing relapse with discharge

C. OTC self-management is not the priority — Substituting comfort care for overdose prevention

D. Family-controlled dosing is unsafe and improper — Improvising an unsound control plan

Q9. A. Reflect both sides of his ambivalence and gently explore the change talk he has already begun to voice

Reflect ambivalence, evoke change talk

Common error: Arguing for one side

B. Arguing triggers resistance — Falling into the righting reflex

C. Siding with fear undercuts change talk — Reinforcing the status-quo side

D. Planning precedes resolved ambivalence — Jumping to planning too soon

Q10. A. Frame the return to use as common in recovery, review overdose risk after abstinence, and revisit the plan without shaming him

Treat the lapse as learning and address risk

Common error: Treating a lapse as total failure

B. Shaming deepens the spiral — Amplifying shame after a lapse

C. Dismissing the lapse skips safety — Glossing over a dangerous lapse

D. One lapse does not mandate escalation — Over-escalating after one slip

Q11. A. Decline to confirm or deny without a valid release, explaining the heightened confidentiality of substance-use information

Protect SUD records without a release

Common error: Disclosing SUD treatment without consent

B. Confirming breaches confidentiality — Disclosing without authorization

C. Confirming attendance is still disclosure — Treating attendance as not protected

D. Abrupt refusal misses a teachable moment — Being needlessly obstructive

Q12. A. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate his questions and progress with them

Defer dosing; coordinate with consent

Common error: Advising on a specific medication dose

B. Naming a dose exceeds scope — Overstepping into medication management

C. Directing a dose is unsafe and out of scope — Giving specific pharmacologic direction

D. Refusing to coordinate abandons the need — Declining to support communication with the prescriber

Q13. A. Obtain his specific, informed written consent for each party before sharing any of his protected information

Get a valid release for each party

Common error: Coordinating before obtaining consent

B. Verbal interest is not a valid release — Disclosing before consent is documented

C. Justice involvement does not erase protections — Assuming probation negates confidentiality

D. Each disclosure should be his informed choice — Making release decisions for the client

Case 5: Feeling detached and unreal, as if watching life from outside

Diagnosis: Depersonalization/Derealization Disorder

Q1. A. Persistent or recurrent detachment or unreality with intact reality testing, causing significant distress or impairment

Persistent detachment with intact reality testing

Common error: Diagnosing from a single brief episode

B. A genuine belief in unreality suggests psychosis — Confusing the disorder with psychosis

C. A precipitant is not the criterion — Using a trigger as the diagnostic test

D. Recent change is not the criterion — Confusing a brief change with the criteria

Q2. A. His reality testing is intact—he knows the detachment and unreality are subjective experiences, not actual changes in the world

Intact reality testing separates it from psychosis

Common error: Mislabeling intact-insight detachment as psychosis

B. Numbing is nonspecific — Using numbing to differentiate

C. Perceptual change alone is not decisive — Reading altered perception as psychosis

D. Distress is nonspecific — Using distress to differentiate

Q3. A. His detachment is persistent rather than occurring only as brief, discrete attacks of intense fear that peak within minutes

Persistent detachment versus discrete attacks

Common error: Treating persistent detachment as panic

B. Fear of the sensations occurs in both — Using fear of sensations to differentiate

C. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

D. Avoidance occurs in both — Using avoidance to differentiate

Q4. A. A substance-induced cause, such as cannabis or another drug, which can produce depersonalization and derealization

Exclude a substance-induced cause

Common error: Skipping the substance and medical rule-out

B. A personality pattern is not the rule-out — Attributing the symptoms to personality

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Cognitive decline is improbable here — Pursuing an improbable rule-out

Q5. A. Co-occurring anxiety, depression, and trauma history, along with suicide risk, which commonly accompany the disorder

Screen anxiety, depression, trauma, and risk

Common error: Assessing the dissociation in isolation

B. Psychosis is not indicated — Pursuing an unsupported rule-out

C. Eating pathology is not indicated — Inventing an unsupported comorbidity

D. Cognitive decline is improbable here — Over-attributing to cognitive decline

Q6. A. Cognitive behavioral therapy with psychoeducation and grounding, addressing catastrophic appraisals of the symptoms

CBT, grounding, and reappraisal

Common error: Choosing an approach that ignores the catastrophic appraisal

B. It is not psychosis; antipsychotics are not indicated — Treating dissociation as psychosis

C. Avoidance maintains the disorder — Sanctioning avoidance as coping

D. Insight-only is not first-line here — Prioritizing origins over symptom management

Q7. A. Help him reduce hypervigilant self-monitoring, since constantly checking the sensations tends to intensify and maintain them

Reduce hypervigilant self-monitoring

Common error: Leaving the self-monitoring in place

B. More monitoring worsens symptoms — Encouraging symptom-checking

C. Detailed logging feeds the focus — Assigning a counterproductive monitoring task

D. Avoidance maintains the disorder — Sanctioning avoidance

Q8. A. Coordinate a referral to a prescriber for evaluation, especially of comorbid anxiety or depression, while continuing therapy

Refer and coordinate within scope

Common error: Giving specific medication advice

B. Naming a drug and dose exceeds scope — Overstepping professional limits

C. Discouraging meds overreaches — Steering a medical decision

D. Outsourcing to self-research is poor care — Replacing referral with self-directed search

Q9. A. Validate the fear and provide psychoeducation that these symptoms, while frightening, are a recognized and treatable condition

Validate and psychoeducate to reduce the fear

Common error: Either dismissing the fear or reinforcing it

B. Confirming the catastrophe deepens the fear — Validating the feared interpretation

C. Dismissive reassurance can invalidate — Reassuring without educating

D. Lecturing rarely shifts the fear — Substituting dense facts for engagement

Q10. A. Guide him through grounding that reconnects him with his senses and surroundings to reduce the intensity of the episode

Use grounding to reconnect with the present

Common error: Leaving him to endure the episode without grounding

B. Focusing inward intensifies the episode — Amplifying the symptom focus

C. Abruptly ending models avoidance — Reinforcing escape from the experience

D. Passive waiting prolongs the episode — Failing to intervene with grounding

Q11. A. Explain how his records are protected and disclosed only with his consent or as required, supporting his informed decisions

Clarify confidentiality and his control over disclosure

Common error: Either dismissing the concern or breaching confidentiality

B. Omitting the diagnosis falsifies the record — Falsifying records to reassure

C. Dismissing the concern harms rapport — Brushing off a legitimate worry

D. Shadow records are improper — Keeping hidden duplicate notes

Q12. A. Explaining the rationale, that some exercises may briefly heighten the sensations, and that participation remains voluntary

Disclose rationale, discomfort, and choice

Common error: Starting exposure-based work without informed consent

B. Misrepresenting the method is dishonest — Misstating what the treatment involves

C. Consent is voluntary and revocable — Treating consent as a binding commitment

D. Withholding the method violates consent — Intervening before explaining

Q13. A. Explain that the cannabis use is clinically relevant to his symptoms and address it collaboratively rather than ignoring it

Address the clinically relevant substance use

Common error: Agreeing to leave out clinically relevant information

B. Ignoring relevant use is poor care — Excluding material information at the client's request

C. A coercive ultimatum harms engagement — Issuing an ultimatum that ruptures the alliance

D. Avoiding the topic abandons the issue — Sidestepping a central clinical issue

Case 6: Recurrent depression with overlooked periods of elevated energy

Diagnosis: Bipolar II Disorder

Q1. A. A history of at least one hypomanic episode in addition to her major depressive episodes, with no full manic episode

Hypomania plus depression defines bipolar II

Common error: Diagnosing unipolar depression by missing the hypomania

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Current severity is not the criterion — Requiring worsening to diagnose

D. Full mania would indicate bipolar I — Applying manic-episode criteria to bipolar II

Q2. A. Her history of distinct hypomanic episodes, which unipolar depression does not include

Presence of hypomanic episodes

Common error: Missing the hypomania and calling it unipolar

B. Recurrent depression occurs in both — Using recurrent depression to differentiate

C. These symptoms occur in both — Reading shared symptoms as decisive

D. Seasonal variation is not the differentiator — Using seasonality to differentiate

Q3. A. Her elevated periods are hypomanic—without psychosis, hospitalization, or the severe impairment of a full manic episode

Hypomania versus full mania

Common error: Calling hypomania a full manic episode

B. These features occur in both hypomania and mania — Using shared features to differentiate

C. Depression occurs in both bipolar types — Reading depression as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q4. A. She has had full major depressive episodes rather than only the milder, fluctuating mood symptoms of cyclothymia

Full episodes versus subthreshold fluctuation

Common error: Calling full episodes a cyclothymic pattern

B. Mood variability occurs in both — Using mood variability to differentiate

C. Elevated periods occur in both — Reading elevation as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q5. A. A structured suicide-risk assessment, recognizing that suicide risk is elevated in bipolar depression

Assess elevated bipolar-depression risk

Common error: Overlooking elevated suicide risk in bipolar depression

B. Exhaustive work history is low yield — Collecting low-yield detail

C. Sleep logging is secondary to risk — Confusing monitoring detail with risk assessment

D. Medication history is useful but secondary now — Prioritizing history over the safety assessment

Q6. A. Coordinate a psychiatric evaluation and share that antidepressant monotherapy is a concern in bipolar II, given the diagnosis

Refer; flag antidepressant-monotherapy caution

Common error: Endorsing antidepressant monotherapy or prescribing

B. Naming a drug and dose exceeds scope — Overstepping into medication management

C. Endorsing monotherapy ignores the diagnosis — Endorsing a potentially destabilizing plan

D. Refusing to engage abandons the issue — Declining to coordinate a relevant medical issue

Q7. A. Interpersonal and social rhythm therapy to regularize her sleep-wake routines and help her detect early warning signs

IPSRT for rhythm stability and early detection

Common error: Ignoring sleep and routine in maintenance

B. Catharsis is not the maintenance target — Choosing an approach without bipolar evidence

C. Overdrive can trigger relapse — Promoting overactivity in bipolar care

D. Exposure is not the indicated approach — Applying an unmatched protocol

Q8. A. Mood and sleep monitoring with early-warning-sign tracking so emerging hypomania or depression can be caught and managed

Monitor mood, sleep, and early warning signs

Common error: Treating the current episode without ongoing monitoring

B. Ignoring elevation misses hypomania — Monitoring only one pole

C. A long monitoring gap risks missed episodes — Leaving a long gap without monitoring

D. Attendance is not an outcome measure — Mistaking adherence for outcome

Q9. A. Validate her reaction and provide psychoeducation about hypomania and bipolar II as understandable and manageable with treatment

Validate and psychoeducate about bipolar II

Common error: Either alarming her or dismissing the new information

B. A grim, inaccurate framing deepens fear — Catastrophizing the diagnosis

C. Minimizing misrepresents the diagnosis — Downplaying a meaningful finding

D. Lecturing rarely addresses the worry — Substituting dense facts for engagement

Q10. A. Collaboratively identify her personal early warning signs, such as reduced sleep need and racing thoughts, and a plan to respond

Identify personal early warning signs and a plan

Common error: Leaving her without a way to detect early episodes

B. Waiting for a full episode misses early intervention — Forgoing early warning-sign monitoring

C. Over-pathologizing normal good days is unhelpful — Pathologizing ordinary good days

D. Outsourcing all monitoring is insufficient — Removing the client from her own monitoring

Q11. A. Obtain her informed consent and release before sharing any clinical information with the primary care provider

Get a valid release before coordinating

Common error: Disclosing before obtaining consent

B. Coordinating without consent breaches confidentiality — Disclosing before consent is documented

C. No coordination undercuts integrated care — Refusing helpful, consented coordination

D. A diagnosis is protected information — Disclosing a diagnosis without consent

Q12. A. Explain that dosing is the prescriber's decision and offer, with consent, to coordinate her questions and progress with them

Defer dosing; coordinate with consent

Common error: Advising on a specific medication dose

B. Naming a dose exceeds scope — Overstepping into medication management

C. Directing dosing is unsafe and out of scope — Giving specific pharmacologic direction

D. Refusing to help abandons the need — Declining to support communication with the prescriber

Q13. A. Seek consultation and coordinate with a prescriber and team, or refer for the components beyond his demonstrated competence

Consult, coordinate, or refer within competence

Common error: Managing complex bipolar care alone without competence

B. Going it alone exceeds competence — Prioritizing continuity over competence

C. Minimizing the condition fails the client — Underestimating the need for coordinated care

D. Abrupt transfer mishandles continuity — Dropping the client without coordination

Case 7: A fixed persecutory belief in an otherwise functioning man

Diagnosis: Delusional Disorder

Q1. A. One or more delusions present for at least a month without other prominent active-phase psychotic symptoms

Delusions without other psychotic symptoms

Common error: Diagnosing from a belief without confirming the full pattern

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent change is not the criterion — Confusing a brief change with the criteria

D. Depressive count does not define it — Applying depressive criteria here

Q2. A. His functioning is otherwise preserved and he lacks the hallucinations, disorganization, and negative symptoms of schizophrenia

Preserved function; no other psychotic symptoms

Common error: Calling an isolated delusion schizophrenia

B. Conviction occurs in both — Using conviction to differentiate

C. Suspicion occurs in both — Reading suspicion as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. He holds a specific fixed delusion, rather than a pervasive lifelong trait of generalized suspiciousness without a fixed false belief

Fixed delusion versus pervasive trait suspiciousness

Common error: Calling a fixed delusion a personality trait

B. Mistrust occurs in both — Using mistrust to differentiate

C. Anger is nonspecific — Reading anger as decisive

D. Diurnal pattern is not diagnostic — Over-reading a circadian pattern

Q4. A. A substance- or medical-induced cause, which his unremarkable workup and denial of substance use make less likely but worth confirming

Exclude substance and medical causes

Common error: Skipping the substance and medical rule-out

B. Anxiety does not explain the delusion — Reducing a delusion to anxiety

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Neurodevelopmental framing is implausible here — Pursuing an improbable rule-out

Q5. A. Assess risk to the neighbor and to himself, including intent and means, and prepare to act on duty-to-protect obligations

Assess risk to others and self

Common error: Letting a possible threat pass without assessment

B. Narrow self-only focus misses risk to others — Assessing only one channel of risk

C. Deferring risk assessment is unsafe — Postponing an urgent risk assessment

D. Past nonviolence does not remove the need to assess — Accepting reassurance without assessment

Q6. A. Coordinate an urgent psychiatric evaluation for antipsychotic medication alongside supportive psychological care

Psychiatric eval plus supportive care

Common error: Offering counseling alone for delusional disorder

B. Insight-only is not first-line — Prioritizing insight over indicated care

C. Prescribing is outside counselor scope — Acting outside scope of practice

D. Counseling alone is insufficient — Omitting medication coordination

Q7. A. Neither directly confirm nor argue against the belief, focusing on the alliance and the distress and goals he can engage with

Neither collude nor confront the delusion

Common error: Either agreeing with or directly disputing the delusion

B. Confronting the delusion ruptures the alliance — Arguing the client out of the delusion

C. Colluding reinforces the delusion — Validating a delusion to keep rapport

D. Total avoidance is not the approach — Sidestepping all of the relevant material

Q8. A. Ongoing risk monitoring and a plan that addresses safety, including protective steps if the risk to the neighbor escalates

Monitor and plan for safety

Common error: Treating the delusion while ignoring the risk to others

B. Ignoring the risk is unsafe — Overlooking a potential risk to others

C. Match level of care to assessed risk — Hospitalizing reflexively without assessment

D. Encouraging confrontation increases risk — Advising an action that raises risk

Q9. A. Obtain his informed consent and release before sharing information, then collaborate as part of a coordinated team

Consent and release before coordinating

Common error: Coordinating before obtaining consent

B. Coordinating without consent breaches confidentiality — Disclosing before consent is documented

C. No coordination undercuts team care — Refusing helpful, consented coordination

D. Blanket sharing over-discloses — Releasing more than is appropriate or consented

Q10. A. Empathize with how distressing and isolating it feels to not be believed, without endorsing or attacking the belief itself

Empathize with the distress, stay neutral on content

Common error: Either confirming or attacking the belief to manage his anger

B. Bluntly disputing entrenches the belief — Confronting the delusion to settle the conflict

C. Agreeing reinforces the delusion — Colluding to defuse the anger

D. Avoiding the moment misses an opening — Sidestepping the emotion rather than working it

Q11. A. Build trust and work on shared, concrete goals such as reducing his stress and distress that he is willing to engage with

Engage via shared goals despite low insight

Common error: Requiring him to give up the belief before engaging

B. Requiring insight first stalls engagement — Setting insight as a precondition

C. Dwelling on the delusion can reinforce it — Centering the work on the delusional content

D. Conditioning care is inappropriate — Making counseling contingent on medication

Q12. A. Assess the risk and follow duty-to-protect obligations, which may require warning or other protective steps

Assess and act on duty to protect

Common error: Treating an identifiable threat as fully confidential

B. Confidentiality yields to a serious threat — Treating confidentiality as absolute when there is danger

C. Delaying assessment of a threat is unsafe — Postponing an urgent risk assessment

D. Act on assessed risk, not reflexively — Disclosing before assessing the risk

Q13. A. Seek consultation and coordinate with a treatment team, referring for the components beyond his demonstrated competence

Consult, coordinate, or refer within competence

Common error: Managing complex psychosis alone without competence

B. Going it alone exceeds competence — Prioritizing continuity over competence

C. Minimizing the illness fails the client — Underestimating the need for specialist care

D. Abrupt transfer mishandles continuity — Dropping the client without coordination

Case 8: A bright child whose reading falls far below expectation

Diagnosis: Specific Learning Disorder

Q1. A. Reading skill well below age expectations that has persisted over six months despite targeted intervention

Below-age skill persisting despite help

Common error: Diagnosing SLD without confirming persistence despite intervention

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent change is not the criterion — Confusing a brief change with the criteria

D. Depressive count does not define SLD — Applying depressive criteria here

Q2. A. Her difficulty is specific to reading skill, not a broad inattention that shows up across many different tasks

Reading-specific deficit, not broad inattention

Common error: Reading a specific skill deficit as a global attention problem

B. Task avoidance occurs in both — Using avoidance to differentiate

C. Discouragement occurs in both — Reading discouragement as decisive

D. Environmental effects are nonspecific — Over-reading an environmental effect

Q3. A. Her general reasoning and problem-solving are age-appropriate, unlike the broad cognitive limits of intellectual disability

Intact general reasoning versus broad limits

Common error: Reading an isolated skill deficit as a global intellectual limit

B. Below-grade performance occurs in both — Using performance to differentiate

C. Need for support occurs in both — Reading need for help as decisive

D. Sleep effects are nonspecific — Over-reading a sleep effect

Q4. A. A psychoeducational evaluation and school records that document the skill discrepancy and the response to intervention

Psychoeducational evaluation and school records

Common error: Concluding SLD without coordinating formal evaluation

B. Extended pedigree is not the priority — Gathering data that does not inform the diagnosis

C. A single year is not the target — Hunting for an origin instead of evaluation data

D. Interests are secondary — Prioritizing rapport detail over evaluation

Q5. A. Emerging anxiety and low mood related to the academic struggle, which commonly accompany learning disorders

Screen anxiety and mood related to the struggle

Common error: Addressing the reading skill without screening the emotional impact

B. Psychosis is not indicated — Pursuing an unsupported rule-out

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Cognitive decline is implausible in a child — Pursuing an implausible rule-out

Q6. A. Coordinate educational support and accommodations with the school while addressing the emotional impact in counseling

Coordinate school support; address the emotional impact

Common error: Trying to remediate the reading skill yourself instead of coordinating and supporting

B. Reading remediation is the educator's role — Taking on a role outside counseling scope

C. More practice alone is not the treatment — Dismissing the need for structured support

D. Prescribing is outside counselor scope — Acting outside scope of practice

Q7. A. Build her confidence and coping, reframing the difficulty as a specific, workable challenge rather than a defect

Build confidence and reframe the difficulty

Common error: Focusing only on the skill while ignoring her eroding self-concept

B. Effort-blaming is harmful and inaccurate — Implying the problem is insufficient effort

C. Avoiding her feelings misses the work — Sidestepping the emotional impact

D. Repeated testing is not counseling — Turning sessions into skill testing

Q8. A. Track confidence, school avoidance, mood, and the use of accommodations over time to guide the support plan

Track confidence, avoidance, mood, accommodations

Common error: Proceeding without tracking the counseling-relevant outcomes

B. Reading scores are the educator's metric — Measuring a metric outside the counseling role

C. Anecdote alone is insufficient — Substituting anecdote for measurement

D. End-only review misses steering data — Deferring all measurement to the end

Q9. A. With the parents' consent and a release, collaborate with the school team on appropriate accommodations and supports

Consent and release before school coordination

Common error: Contacting the school without parental consent

B. Disclosing without consent breaches confidentiality — Disclosing to the school without consent

C. No coordination undercuts the support — Refusing helpful, consented coordination

D. Blanket sharing over-discloses — Releasing more than is appropriate or consented

Q10. A. Validate how hard it feels, normalize the difficulty, and connect her to the supports that can make it manageable

Validate, normalize, and connect to support

Common error: Either minimizing the difficulty or agreeing that giving up is reasonable

B. Minimizing the difficulty invalidates her — Minimizing a genuine struggle

C. Endorsing giving up is harmful — Validating the wish to give up

D. Avoiding the feeling misses the work — Sidestepping a meaningful moment

Q11. A. Use developmentally appropriate, strengths-based methods that highlight what she does well and build on it

Developmentally matched, strengths-based engagement

Common error: Using deficit-focused, adult-oriented methods that deepen discouragement

B. Abstract methods do not fit a child — Pitching the work above her developmental level

C. Skill drilling is not the counseling role — Taking on educational drilling in counseling

D. Demanding improvement first stalls engagement — Setting skill improvement as a precondition

Q12. A. Decline to misrepresent her profile and clarify what the assessment and your role can accurately support

Do not misrepresent; document accurately

Common error: Issuing an inaccurate label to satisfy the parents

B. Falsifying a report is unethical — Misrepresenting the child's profile

C. Advocacy over accuracy is improper — Letting advocacy override accuracy

D. Flat refusal forecloses helpful options — Declining without clarifying the appropriate options

Q13. A. Refer the formal evaluation to a qualified professional and stay within your supportive counseling role

Refer evaluation; stay within role

Common error: Conducting a formal evaluation without the required competence

B. Testing without competence is unethical — Acting outside demonstrated competence

C. Minimizing the need misleads care — Discouraging an indicated evaluation

D. Abrupt transfer mishandles continuity — Dropping the family without coordination

Case 9: Distressing physical symptoms and constant health worry

Diagnosis: Somatic Symptom Disorder

Q1. A. Distressing somatic symptoms for over six months with excessive thoughts, feelings, and behaviors devoted to them

Symptoms plus disproportionate response

Common error: Diagnosing from the symptoms alone without the excessive response

B. Medically unexplained is not required — Requiring symptoms to be proven psychogenic

C. A precipitant is not the criterion — Using a trigger as the diagnostic test

D. Recent change is not the criterion — Confusing a recent uptick with the criteria

Q2. A. Her distress centers on the burdensome somatic symptoms themselves rather than on a fear of having a specific undiagnosed disease

Symptom-focused versus disease-fear-focused

Common error: Equating symptom-focused distress with illness anxiety

B. Health worry occurs in both — Using health worry to differentiate

C. Functional impairment occurs in both — Reading impairment as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. Her excessive thoughts and behaviors are organized around her bodily symptoms rather than diffuse worry across many life domains

Symptom-organized versus diffuse worry

Common error: Calling symptom-focused distress generalized worry

B. General anxiety occurs in both — Using general anxiety to differentiate

C. Sleep and fatigue occur in both — Reading somatic signs as decisive

D. Distraction relief is nonspecific — Reading meaning into distraction relief

Q4. A. Validate that her symptoms are real and distressing and coordinate with one consistent medical provider rather than dismissing them

Validate symptoms; coordinate care

Common error: Telling her the symptoms are not real

B. Ordering tests exceeds counselor scope — Acting outside scope of practice

C. Endless workups reinforce the disorder — Reinforcing the reassurance and testing cycle

D. Dismissive framing damages alliance — Invalidating the client's experience

Q5. A. Co-occurring depression and suicide risk, which commonly accompany chronic somatic distress and shape the plan

Screen depression and suicide risk

Common error: Treating the somatic picture without screening mood and risk

B. Psychosis is not indicated — Over-reading health worry as psychosis

C. Eating pathology is not indicated — Inventing an unsupported comorbidity

D. Cognitive decline is improbable here — Pursuing an improbable rule-out

Q6. A. Cognitive behavioral therapy targeting symptom-focused thoughts, reassurance-seeking, and avoidance, coordinated with her care

CBT for the symptom-focused cycle

Common error: Choosing an approach that ignores the maintaining behaviors

B. Symptom-focused venting can reinforce the cycle — Reinforcing the symptom preoccupation

C. Insight-only is not first-line here — Prioritizing origins over maintaining factors

D. Activity avoidance worsens function — Sanctioning avoidance as coping

Q7. A. Coordinate regular, scheduled visits with one primary provider rather than visits driven by each new symptom or crisis

Scheduled visits with one coordinator

Common error: Leaving care fragmented and symptom-driven

B. Crisis-driven care reinforces the cycle — Reinforcing crisis-driven utilization

C. Cutting off medical care is inappropriate — Severing necessary medical involvement

D. Rotating specialists fragments care — Encouraging doctor-shopping

Q8. A. Reducing reassurance-seeking and catastrophic symptom interpretations while gradually restoring valued activity

Target reassurance-seeking and avoidance

Common error: Treating only the symptoms, not the maintaining behaviors

B. Detailed monitoring fuels the focus — Encouraging hypervigilant monitoring

C. Researching feeds reassurance-seeking — Reinforcing the maintaining behavior

D. Activity restriction worsens function — Letting symptoms gate activity

Q9. A. Validate that her symptoms and suffering are real and explain that counseling addresses the distress and impact, not their reality

Affirm the symptoms are real; reframe the aim

Common error: Implying the symptoms are imaginary

B. This invalidates and alienates her — Invalidating the client's experience

C. Repeated reassurance feeds the cycle — Providing reassurance that maintains the cycle

D. Bypassing the concern harms engagement — Avoiding a key alliance issue

Q10. A. Gently name the reassurance-seeking as part of the cycle and help her practice tolerating uncertainty rather than answering it

Name the pattern; build uncertainty tolerance

Common error: Answering to relieve her distress in the moment

B. Answering reinforces the cycle — Reinforcing the maintaining behavior

C. Shaming language harms the alliance — Invalidating instead of guiding

D. Another scan feeds the cycle — Recommending testing to relieve anxiety

Q11. A. Clarify that ordering tests is the physician's decision and offer, with consent, to coordinate her concerns with the provider

Stay in scope; coordinate with consent

Common error: Directing the physician's medical workup

B. Directing the workup exceeds scope — Inserting yourself into medical decision-making

C. Reinforces reassurance-driven testing — Coaching reassurance-driven testing

D. Refusing to coordinate abandons the concern — Declining to help her communicate with the provider

Q12. A. Obtain her informed consent and release before communicating with her providers, then collaborate on a shared plan

Consent and release before coordinating

Common error: Contacting providers before obtaining consent

B. Coordinating without consent breaches confidentiality — Disclosing before consent is documented

C. No coordination undercuts collaborative care — Refusing helpful, consented coordination

D. Blanket sharing over-discloses — Releasing more than is appropriate or consented

Q13. A. Conduct a structured suicide-risk assessment and respond to the risk, recognizing chronic somatic distress can carry suicide risk

Assess risk directly

Common error: Overlooking risk amid somatic focus

B. Minimizing the ideation is unsafe — Dismissing the ideation

C. Risk must be assessed before continuing — Proceeding past a risk disclosure

D. No-suicide contracts are not evidence-based — Substituting a contract for assessment

Case 10: Individual disclosure of intimate partner violence and a partner's request to join therapy

Diagnosis: Acute Crisis / Decompensation

Q1. A. Include direct, private questions about relationship safety as a routine part of the psychosocial intake

Routine private screening surfaces unreported danger

Common error: Waiting for the client to volunteer relationship concerns unprompted

B. Passive waiting can delay needed screening — Assuming silence means there is nothing to screen for

C. Involving the partner early blocks honest disclosure — Assuming a fuller picture requires the partner present

D. Deferring psychosocial history delays context — Treating symptoms in isolation from psychosocial history

Q2. A. A structured danger assessment covering the frequency, severity, and escalation pattern of the violence, including access to weapons

Structured danger assessment anchors risk level

Common error: Moving to intervention before characterizing the danger

B. Exhaustive conflict history is low yield now — Collecting broad history instead of targeted risk data

C. Contacting the partner breaches confidentiality and safety — Verifying a client's disclosure with the abusive partner

D. Couples referral is premature and misdirected — Jumping to a relationship fix before assessing safety

Q3. A. Specifically ask about any prior strangulation, threats with weapons, and whether the violence has increased in frequency or severity

Strangulation history strongly predicts future lethality

Common error: Treating the choking as an isolated event rather than a lethality marker

B. Commitment rating does not measure danger — Substituting a feelings check for a danger assessment

C. Avoiding history-taking hides escalation and worsens risk — Avoiding distressing questions that are clinically necessary

D. Couples referral bypasses the lethality assessment — Reframing lethal violence as a communication problem

Q4. A. Her anxiety and hypervigilance emerged and sharply escalated in direct response to specific, ongoing violence and control

Onset tied to a specific active danger fits a crisis

Common error: Diagnosing GAD from anxiety symptoms without weighing the acute precipitant

B. Diffuse worry across domains fits GAD better — Ignoring how broadly the worry actually spreads

C. A longer chronic course points away from an acute crisis — Overlooking the actual reported duration

D. Impairment alone does not distinguish the two — Using severity of impairment as the differentiator

Q5. A. Her own current risk for suicide or self-harm, using a structured screen, since intimate partner violence significantly raises that risk

IPV exposure independently elevates suicide risk

Common error: Assessing only the partner's danger and skipping the client's own risk

B. Panic disorder is a lower-priority rule-out here — Chasing a comorbidity before the client's own safety

C. Family history does not address acute risk — Prioritizing etiology over an urgent safety screen

D. Sleep disorder rule-out is secondary right now — Pursuing a low-yield rule-out ahead of a suicide-risk screen

Q6. A. A collaborative, individualized safety plan covering warning signs, coping steps, safe contacts, and options for leaving or staying

Collaborative safety planning fits an active-danger case

Common error: Substituting a directive plan for collaborative planning

B. Directing her to leave overrides her autonomy — Telling a client she must leave rather than planning with her

C. Managing the partner's care is not the client's plan — Making the abusive partner's treatment the centerpiece of her plan

D. Symptom-focused planning ignores the active danger — Treating anxiety symptoms while leaving danger unaddressed

Q7. A. Decline the joint session and continue individual treatment, since conjoint work during active violence can raise danger and suppress disclosure

Conjoint therapy is contraindicated during active IPV

Common error: Treating a request for couples work as automatically reasonable

B. Conjoint sessions with an active abuser endanger the client — Mistaking a well-intentioned-sounding request as clinically safe

C. Deciding without clinical input on risk is incomplete — Confusing autonomy with skipping informed discussion of risk

D. Seeing the partner individually creates a conflict of interest — Overlooking the dual-relationship risk of also treating the partner

Q8. A. Offer information about IPV-specific resources such as a hotline and a local advocate, and let her decide what steps to take and when

Offering resources without mandating action respects autonomy

Common error: Withholding IPV-specific resources out of caution

B. Conditioning care on compliance is coercive — Pressuring immediate action instead of offering informed choice

C. Generic resources miss IPV-specific expertise — Treating any resource list as sufficiently specific

D. Passive waiting can leave her without key information — Assuming she will ask if she wants information

Q9. A. Include specific warning signs preceding escalation and an immediate physical-safety plan for recurrence, built collaboratively with Maria

Targeted warning-sign and safety steps address lethality

Common error: Leaving the plan generic rather than tailored to the choking risk

B. Instructing her to restrain him endangers her further — Giving advice that places the client in greater physical danger

C. This places responsibility for his violence on her — Framing avoidance of conflict as a safety plan

D. Long-term goals alone ignore acute lethality risk — Deferring urgent safety steps in favor of long-term goals

Q10. A. Validate her feelings toward Danny without judgment while gently naming the behaviors as unsafe, at a pace she can tolerate

Validating feelings while naming danger keeps her engaged

Common error: Either confronting too hard or colluding with the minimization

B. Confrontation risks alliance rupture and withdrawal — Pushing insight before the alliance can support it

C. Endorsing the excuse reinforces minimization of violence — Colluding with an excuse for dangerous behavior

D. Avoiding the topic leaves the belief unexamined — Sidestepping emotionally difficult but essential material

Q11. A. Gently normalize that self-blame is common after abuse and clarify that Danny alone is responsible for his violent behavior

Naming responsibility counters self-blame directly

Common error: Leaving the responsibility question unaddressed

B. This reinforces the belief that she causes the violence — Treating the victim's behavior as the explanation for abuse

C. This implies her change will resolve his violence — Suggesting the client's change controls the partner's violence

D. Avoiding the belief leaves it unchallenged — Moving to skills before addressing a core distortion

Q12. A. Decline to confirm or discuss whether Maria is a client, or share any session information, without her explicit consent

Confidentiality requires consent before any disclosure

Common error: Confirming a client relationship to a third party without consent

B. Confirming client status without consent breaches confidentiality — Explaining clinical reasoning to someone who is not an authorized party

C. Partial disclosure still breaches confidentiality without consent — Believing a vague disclosure is safe because it lacks detail

D. Leaving it entirely to the client misses a needed boundary — Assuming no response avoids the ethical issue

Q13. A. Document the danger assessment and safety plan thoroughly, and offer protective order information while leaving the timing to Maria

Thorough documentation plus informed autonomy is standard

Common error: Either under-documenting risk or directing her legal choices

B. Acting on her behalf removes her autonomy — Taking over a legal decision that belongs to the client

C. Omitting risk data undermines safe, continuous care — Sacrificing accurate records for perceived privacy protection

D. Vague notes obscure the actual risk present — Under-specifying documentation of a safety-relevant disclosure

Case 11: Progressive memory loss and lost independence in an older adult

Diagnosis: Major Neurocognitive Disorder due to Alzheimer's Disease

Q1. A. A significant decline from her prior level in two or more cognitive domains that interferes with her everyday independence

Significant decline interfering with independence

Common error: Diagnosing major NCD from minor forgetfulness

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. A sudden change suggests delirium — Confusing an acute change with progressive NCD

D. Depressive count does not define NCD — Applying depressive criteria here

Q2. A. Her decline is gradual with clear attention, rather than an acute, fluctuating disturbance of attention and awareness

Gradual, clear attention versus acute fluctuation

Common error: Confusing progressive NCD with delirium

B. Memory problems occur in both — Using memory problems to differentiate

C. Stress-linked confusion is nonspecific — Using stress sensitivity to differentiate

D. Age alone is not the differentiator — Reading age as diagnostic

Q3. A. Screen for depression, since depression can impair cognition and may coexist with or mimic a neurocognitive disorder

Screen depression as a contributor or mimic

Common error: Ignoring a treatable depressive contribution

B. Dismissing depression misses a contributor — Overlooking a treatable contribution

C. Over-attributing to depression is premature — Forcing one explanation prematurely

D. A mood trend is not the question — Reframing the question as a mood trend

Q4. A. Use cognitive screening and collateral history and coordinate a medical and neurological evaluation rather than diagnosing alone

Screen and coordinate the medical workup

Common error: Diagnosing the medical etiology without a medical evaluation

B. Ordering tests exceeds counselor scope — Acting outside scope of practice

C. Dismissing the decline is unsafe — Mistaking major NCD for normal aging

D. One observation is insufficient — Diagnosing from one in-session sample

Q5. A. Assess safety risks such as driving, the stove, wandering, and medication management given her decline and impaired judgment

Assess functional safety risks

Common error: Overlooking everyday safety risks

B. Narrow focus misses key NCD risks — Assessing only one channel of risk

C. Deferring safety review is unsafe — Postponing a needed safety assessment

D. Impaired insight makes self-report insufficient — Accepting reassurance without assessment

Q6. A. Coordinate medical care and build a plan of caregiver support, safety measures, and structured non-pharmacologic approaches

Coordinated care plus caregiver and safety support

Common error: Offering individual insight therapy alone as the plan

B. Insight therapy cannot reverse NCD — Setting an unrealistic restorative goal

C. Prescribing is outside counselor scope — Acting outside scope of practice

D. Passive waiting delays needed support — Delaying indicated support

Q7. A. Structured routines, environmental cues, and orientation supports that reduce confusion and support her remaining abilities

Structure, cues, and orientation supports

Common error: Relying on effortful new learning

B. Restoring lost function is unrealistic — Setting an unrealistic restorative goal

C. Quizzing can increase frustration — Using testing as an intervention

D. Constant change worsens confusion — Introducing destabilizing novelty

Q8. A. Provide caregiver psychoeducation and connect her with respite and support resources to reduce caregiver burden

Caregiver education and respite support

Common error: Focusing only on the client and ignoring the caregiver

B. Sole caregiving fuels burnout — Discouraging needed support and respite

C. Dismissing caregiver needs is a missed target — Excluding the caregiver from the plan

D. Concealment undermines autonomy — Defaulting to concealment

Q9. A. Validate her grief and preserve her dignity and personhood while supporting the abilities and choices she still retains

Validate grief; preserve dignity and choice

Common error: Talking past her or treating her as incapable

B. Dismissive reassurance invalidates her grief — Minimizing her experience

C. Talking past her undermines dignity — Excluding the client from her own care conversation

D. Heavy prognosis data is not what she needs now — Answering distress with statistics

Q10. A. Use simple, concrete communication, shorter sessions, and supportive engagement adapted to her current cognitive abilities

Adapt communication to her abilities

Common error: Using complex, insight-heavy methods she cannot use

B. Unadapted insight work does not fit — Failing to adapt the approach

C. Independent homework exceeds her capacity — Assigning tasks beyond her current ability

D. Writing her off disregards personhood — Prematurely excluding the client

Q11. A. Assess her capacity for specific decisions, involve her to the fullest extent possible, and engage surrogates as appropriate

Decision-specific capacity; involve her and surrogates

Common error: Either ignoring capacity or removing her voice entirely

B. Global incapacity assumption overreaches — Stripping the client of all input

C. Ignoring impairment is unsafe — Overlooking capacity concerns

D. Indefinite delay is impractical and unsafe — Stalling necessary decisions

Q12. A. Clarify consent and confidentiality, respecting Margaret's wishes and capacity while engaging family with appropriate authorization

Clarify consent within capacity and authorization

Common error: Either freely sharing or excluding family without considering consent and capacity

B. Free sharing ignores consent — Disclosing without proper authorization

C. Blanket exclusion is impractical here — Refusing appropriate, authorized involvement

D. The client's own rights still matter — Ceding the client's rights to a family member

Q13. A. Seek consultation and coordinate with a geriatric or medical team, or refer for the components beyond his competence

Consult, coordinate, or refer within competence

Common error: Managing complex NCD alone without competence

B. Going it alone exceeds competence — Prioritizing continuity over competence

C. Minimizing the case fails the family — Underestimating the need for specialist care

D. Abrupt transfer mishandles continuity — Dropping the family without coordination

Practice Exam Three

11 clinical cases · 143 scored questions · 255 minutes recommended

Work through all cases before consulting the answer key at the end of this exam.

Case 1 of 11

Tearful and doubting her competence six weeks after childbirth

Diagnosis: Major Depressive Disorder, with Peripartum Onset

INTAKE

Danielle Marsh, a 27-year-old woman who is 6 weeks postpartum with her first child, joins for an intake session by telehealth from her living room while her daughter naps nearby. She reports that low mood, tearfulness, and exhaustion began about ten days after delivery and have steadily worsened, leaving her convinced she is failing as a new mother.

SESSION TWO

She describes crying multiple times most days, little interest in things she used to enjoy such as cooking or calling friends, waking well before the baby stirs and being unable to fall back asleep, a poor appetite, and trouble concentrating even on simple tasks like reading a bottle label. She denies any past period of elevated mood, decreased need for sleep, or unusually high energy.

SESSION THREE

When asked directly, she denies any thoughts of harming her daughter or herself, any plan or intent, and any voices, visions, or beliefs that feel unreal; she says she feels warmly bonded to her daughter and is frightened by how incapable she feels, wanting help to feel like herself again.

Question 1. What should the counselor confirm first at intake to anchor the diagnosis of a major depressive episode with peripartum onset?

- A. That her depressive symptoms began within four weeks of delivery and have persisted nearly daily for over two weeks with impairment
- B. That she can identify a single specific event during the pregnancy or delivery that she believes triggered all of her symptoms
- C. That a family member has previously been diagnosed with a mood disorder at some earlier point in their own life
- D. That her symptoms are noticeably worse this week than when she first noticed them roughly ten days after delivery

Question 2. What additional history is most important to gather to avoid misattributing Danielle's presentation to another cause?

- A. A review of her thyroid function, medications, and substance use, since postpartum medical factors can mimic depression
- B. A complete account of every prenatal appointment she attended so the counselor can verify her obstetric care was adequate
- C. A detailed genogram tracing mood and relationship patterns across at least three generations of her extended family
- D. A full list of every parenting book and resource she has read since delivery so the counselor can assess her preparation

Question 3. As part of routine peripartum risk assessment, what must the counselor screen for directly, regardless of how mild Danielle's presentation appears?

- A. Any thoughts of harming her daughter or herself, and any hallucinations, delusions, or disorganized thinking
- B. Whether she feels confident she can breastfeed exclusively for the recommended duration without ever supplementing
- C. Whether her partner agrees that hiring outside childcare help would be an appropriate next step for the family
- D. Whether she has already scheduled her daughter's upcoming pediatric wellness visits for the next several months

Question 4. Danielle denies any hallucinations, delusions, or disorganized behavior. How does this shape the differential from postpartum psychosis?

- A. It rules out postpartum psychosis, since that diagnosis requires psychotic features or grossly disorganized behavior
- B. It confirms she could not possibly develop psychotic features later, since the two conditions never occur together
- C. It suggests her guilt and self-doubt are less clinically significant than they would be if psychotic features were present
- D. It means her case can be managed identically whether or not psychotic features ever emerge in the future

Question 5. Before finalizing a unipolar depression diagnosis, what history must the counselor rule out?

- A. Any past episode of elevated or irritable mood with decreased need for sleep, indicating a hypomanic or manic history
- B. Any history of gestational diabetes or other pregnancy complications documented in her obstetric records
- C. Any prior experience of mild worry before medical appointments that resolved without any formal treatment
- D. Any family history of postpartum depression reported by her mother or older sisters during their own pregnancies

Question 6. What is the most appropriate first-line, within-scope treatment recommendation for Danielle?

- A. Offer evidence-based psychotherapy such as cognitive behavioral therapy or interpersonal therapy for postpartum depression
- B. Recommend she begin a specific antidepressant today and describe exactly how to adjust the dose over coming weeks
- C. Suggest she wait several months to see if her mood improves naturally before starting any formal treatment
- D. Encourage her to rely mainly on extended family support and postpone therapy until the baby sleeps through the night

Question 7. How should the counselor track whether treatment is actually working over the coming weeks?

- A. Re-administer a validated perinatal measure such as the Edinburgh Postnatal Depression Scale at regular intervals
- B. Ask her casually at each session whether she generally feels a bit better than she did the previous week
- C. Wait until the full course of therapy has ended before evaluating whether her symptoms have changed at all
- D. Track only whether she is attending sessions consistently and completing any assigned between-session tasks

Question 8. Danielle asks whether she should also be evaluated for medication. What is the most appropriate counselor action?

- A. Provide general information and, with her consent, coordinate a referral to a prescriber for a medication evaluation
- B. Tell her which specific antidepressant is typically best for new mothers with her particular symptom pattern
- C. Advise her that medication is unnecessary as long as she remains fully committed to attending therapy
- D. Suggest she research antidepressant options herself online and bring a preferred choice to a future appointment

Question 9. Which initial step best supports Danielle's re-engagement with valued activities while accounting for her new role as a mother?

- A. Collaboratively schedule small, manageable activities such as a short walk with the baby or a brief call to a friend
- B. Direct her to resume her full pre-baby schedule immediately to prove to herself that she can still manage everything
- C. Have her keep a detailed log of every negative thought about her parenting before introducing any change to her routine
- D. Encourage her to wait until she genuinely feels motivated again before attempting to resume any prior activities

Question 10. How can the counselor best address Danielle's belief that her depression makes her a bad mother?

- A. Offer psychoeducation framing postpartum depression as common and treatable while validating how exhausting this feels
- B. Firmly challenge the belief as irrational and present objective evidence that she is clearly a capable mother
- C. Agree that her exhaustion and low mood are affecting her daughter and focus on helping her accept this reality
- D. Redirect the conversation away from the topic and refocus entirely on practical scheduling for the coming week

Question 11. Danielle says she doubts therapy will help her feel like a good mother again. What is the most therapeutic response?

- A. Acknowledge the doubt as understandable and collaboratively set one small, achievable goal to test it against
- B. Promise her confidently that therapy is guaranteed to work as long as she attends every scheduled session
- C. Accept her prediction that treatment is unlikely to help and shift the focus to simply managing daily tasks
- D. Explain the neurobiology of postpartum hormonal shifts in detail to convince her that recovery is fully possible

Question 12. When reviewing informed consent, what must the counselor ensure Danielle understands?

- A. The limits of confidentiality, including circumstances that would require the counselor to act to prevent serious harm
- B. That everything she shares in telehealth sessions will remain completely confidential under all circumstances
- C. That the counselor will decide which specific treatment goals are appropriate for her to pursue in therapy
- D. That she is required to complete the entire recommended course of therapy once she agrees to begin today

Question 13. Danielle asks whether it is appropriate to keep having sessions by telehealth while caring for a newborn at home. What is the appropriate response?

- A. Confirm she has a private space and stable connection, review telehealth consent and confidentiality limits, and proceed
- B. Tell her telehealth is never appropriate for a new mother and she must switch to in-person sessions immediately
- C. Proceed with sessions without addressing privacy or confidentiality since she already agreed to telehealth at intake
- D. Suggest she pause therapy altogether until her daughter is older and telehealth sessions are less likely to be interrupted

Case 2 of 11

A 16-year-old who avoids the cafeteria and dreads speaking in class

Diagnosis: Social Anxiety Disorder

INTAKE

Maya Torres, a 16-year-old high school sophomore, is referred to the school-based counseling program by her English teacher, who noticed Maya eats lunch alone in a bathroom stall and asked to be excused from an upcoming class presentation.

SESSION TWO

Maya reports that for over a year she has felt intense fear of being judged or embarrassed in front of peers, worrying her voice will shake or her face will turn red when called on in class. She avoids the cafeteria, eating lunch in a quiet stairwell instead, and has skipped every school dance and pep rally this year.

SESSION THREE

She says she recognizes the fear is stronger than the situation calls for, and that it has cost her a chance to join the yearbook club because meetings involve introducing herself to new people. In familiar one-on-one settings, like talking with her older cousin or her school counselor, she reports feeling completely at ease.

Question 1. What should the counselor confirm first to anchor a Social Anxiety Disorder diagnosis at intake?

- A. That the fear of scrutiny occurs across multiple social situations, has lasted six months or more, and impairs functioning
- B. That she can name the one specific classmate she believes is most likely to judge or embarrass her
- C. That a parent or guardian has also independently observed and reported the same pattern of social avoidance at home
- D. That her fear has grown noticeably more intense this month compared to when it first began over a year ago

Question 2. What additional history is most important to gather to avoid misattributing Maya's presentation?

- A. Whether her avoidance is limited to social/performance situations or also includes fear of separating from caregivers
- B. A complete account of her academic grades in every subject over the past three full years of school
- C. A detailed list of every extracurricular activity she has ever considered joining since starting high school
- D. Whether she has ever completely stopped speaking in specific settings while speaking normally in others

Question 3. Given that Maya is a 16-year-old seen through the school, what is an essential intake step?

- A. Review informed consent with Maya and coordinate appropriate parent/guardian notification per school and state policy
- B. Proceed with counseling exactly as with an adult client, since Maya is old enough to consent to her own care
- C. Wait to begin any counseling contact until a full written consent form has been mailed home and returned by post
- D. Ask Maya to keep the counseling contact private from her parents so she feels free to speak openly at school

Question 4. Which finding best distinguishes Maya's presentation as Social Anxiety Disorder rather than Generalized Anxiety Disorder?

- A. Her fear and avoidance are specifically tied to social and performance situations rather than a broad range of everyday worries
- B. The fact that she reports feeling anxious on most days rather than only occasionally during the school week
- C. The observation that her anxiety is somewhat out of proportion to the actual likelihood that peers are judging her
- D. The likelihood that her anxiety symptoms will improve naturally once she graduates and leaves the high school environment

Question 5. What safety-related step is still appropriate even though Maya's presentation shows no indication of risk?

- A. Briefly and routinely screen for mood symptoms and any thoughts of self-harm as part of a complete intake assessment
- B. Skip any mention of mood or self-harm entirely since her presentation is clearly limited to social anxiety alone
- C. Immediately involve crisis services as a precaution even though nothing in the interview suggests any current risk
- D. Postpone any safety-related questions until several sessions of rapport-building have occurred with Maya

Question 6. What is the most appropriate first-line, within-scope treatment recommendation for Maya?

- A. Offer cognitive behavioral therapy with graded exposure to feared social situations such as the cafeteria and class speaking
- B. Recommend that she begin taking an anti-anxiety medication daily and coordinate the specific dosage herself
- C. Suggest she simply avoid the situations that make her anxious until she naturally feels ready to face them on her own
- D. Wait several months to see whether her anxiety symptoms resolve naturally before considering any formal treatment plan

Question 7. How should the counselor structure the exposure component of treatment for Maya?

- A. Build a graded hierarchy of feared situations, starting with a lower-anxiety step and progressing collaboratively to harder ones
- B. Have her give a full class presentation in front of the entire school as the very first exposure exercise
- C. Focus first on lengthy discussion of her childhood experiences before introducing any exposure exercises
- D. Encourage her to wait until her anxiety naturally decreases before attempting any of the feared situations

Question 8. Maya's mother asks the counselor how she can help at home. What is the most appropriate counselor action?

- A. With Maya's consent, coach the mother to encourage gradual approach rather than accommodating avoidance of feared situations
- B. Advise the mother to write excuse notes so Maya can be exempted from any class presentations for the rest of the year
- C. Tell the mother that her involvement is unnecessary and that this is entirely a matter for Maya to work through alone
- D. Suggest the mother closely monitor and report back on every social interaction Maya has throughout each school day

Question 9. How should the counselor track whether Maya's treatment is actually working over time?

- A. Use a validated anxiety measure at regular intervals alongside tracking her completion of hierarchy steps
- B. Ask her at each session whether she generally feels things are going better than before without further detail
- C. Wait until the full course of therapy has ended and then evaluate her overall response in a single final review
- D. Track only whether Maya attends her scheduled sessions on time each week without assessing symptom change

Question 10. How can the counselor best respond when Maya says she will "never" be able to speak in class without embarrassing herself?

- A. Validate how difficult it feels right now while gently exploring the evidence for and against that all-or-nothing prediction
- B. Firmly tell her that her prediction is completely irrational and list several clear reasons why she is simply wrong
- C. Agree that speaking in class probably will go badly for her and suggest she keep requesting to be excused from it
- D. Change the subject to something lighter so the session does not dwell on an uncomfortable topic for too long

Question 11. Maya hesitates and says she is not sure counseling will actually change how people see her. What is the most therapeutic response?

- A. Normalize her doubt as common early in treatment and collaboratively set one small, achievable exposure goal to test it
- B. Promise her with certainty that treatment will completely change how her classmates see her within a few weeks
- C. Accept her doubt as likely accurate and shift the focus toward helping her simply cope with ongoing avoidance instead
- D. Explain in detail the neuroscience research behind exposure therapy to convince her scientifically that it works

Question 12. When reviewing informed consent and confidentiality with Maya and her mother, what must the counselor be sure they both understand?

- A. The general limits of confidentiality in the school setting, including when safety concerns would require disclosure
- B. That everything Maya shares in session will always remain completely private from her mother under every circumstance
- C. That the school counselor alone will decide all treatment goals without needing input from Maya or her mother
- D. That Maya must complete an entire semester of sessions once she agrees to begin, regardless of how she later feels

Question 13. Maya asks the counselor not to tell her mother any specifics about what they discuss in their school sessions. What is the most appropriate response?

- A. Explain that general updates may go to her mother per the consent agreement, while routine session content stays private absent safety concerns
- B. Agree to keep every detail of their sessions completely secret from her mother for the rest of the current school year without exception
- C. Tell Maya that her mother will automatically receive a full written transcript of everything discussed in each individual session
- D. Avoid answering her question directly and change the subject so that today's session can continue without addressing it

*Case 3 of 11***Sad and discouraged three weeks after a layoff***Diagnosis: Adjustment Disorder, with Depressed Mood*

INTAKE

Marcus Whitfield, a 39-year-old father of two, presents to outpatient counseling through his former employer's EAP three weeks after being laid off from his assembly-line supervisor role at an auto-parts plant. He reports feeling sad, discouraged, and 'stuck,' with trouble falling asleep and little motivation to do much beyond searching for work.

SESSION TWO

He describes lying awake most nights running through bills and job applications, waking unrefreshed, and losing interest in his usual pickup basketball games, though he still gets his kids ready for school, cooks dinner, and has submitted applications to six employers this week. He denies any change in appetite and says his mood lifts somewhat when he is with his children.

SESSION THREE

He denies any nightmares, intrusive memories, or flashbacks about the layoff, any avoidance of his old workplace or former coworkers, and any hypervigilance or exaggerated startle response. He denies current thoughts of self-harm or suicide, says he feels embarrassed but not hopeless, and states he believes 'this will get better once I land something new.'

Question 1. What is the most important timeline detail to establish during Marcus's intake?

- A. That his sadness, sleep trouble, and low motivation began within three months of the layoff and track closely with it
- B. Whether any of his first-degree relatives have ever been formally diagnosed with a depressive or anxiety disorder
- C. A full chronological inventory of every position he has held across his entire manufacturing career to date
- D. How he would numerically rate the layoff's severity compared with other stressful events earlier in his life

Question 2. Given Marcus denies feeling hopeless, what is still the appropriate intake safety step?

- A. Complete a structured suicide-risk screen as a routine part of intake regardless of how mild his presentation seems
- B. Skip any direct risk questions today since he appears to be coping well and clearly denies feeling hopeless
- C. Wait until a later session, once more rapport has been built, before asking him any direct safety-related questions
- D. Have him complete a written pledge stating he will contact the office before acting on any distressing thoughts

Question 3. What functional information is most useful to gather to gauge the impact of Marcus's symptoms?

- A. How his sleep, mood, and motivation are affecting his job search and his ability to care for his two children
- B. A detailed accounting of his monthly household budget so the counselor can calculate his exact severance runway
- C. A description of the exact machinery and daily tasks he performed in his prior assembly-line supervisor position
- D. Whether he believes his former employer treated him fairly when the decision to lay him off was made

Question 4. Which finding best supports Adjustment Disorder over Major Depressive Disorder in Marcus's case?

- A. His symptom count and duration fall short of the five concurrent symptoms nearly daily for two weeks that MDD requires
- B. The fact that his distress began only after a clearly identifiable and upsetting workplace event occurred recently
- C. His report that he still enjoys spending time with his children and feels somewhat better when he is with them
- D. The likelihood that his current symptoms will resolve fully within about six months once he secures new work

Question 5. Which finding best rules out PTSD or Acute Stress Disorder for Marcus?

- A. He denies any qualifying trauma exposure and shows no intrusion, avoidance, or hyperarousal symptoms of any kind
- B. He was laid off only three weeks ago, which is too recent a timeframe for any trauma-related disorder to appear
- C. He is male, and trauma-related disorders are understood to occur far less often in men than in women overall
- D. He continues to function fairly well, still applying for jobs and caring for his children day to day right now

Question 6. What is the most appropriate first-line treatment approach for Marcus?

- A. Brief, present-focused, solution-focused or problem-solving counseling centered on coping with the job loss
- B. An extended trauma-processing protocol involving repeated, detailed exposure to memories of the layoff itself
- C. A recommendation that he begin taking a daily antidepressant right away before any counseling sessions start
- D. Advise him to wait and monitor his mood on his own for a couple of months before starting any formal counseling

Question 7. How should the counselor and Marcus set initial treatment goals?

- A. Collaboratively identify concrete, achievable goals tied to coping with the job loss, such as structuring his days
- B. Have the counselor independently decide the full treatment plan and simply inform Marcus of it at the next visit
- C. Focus the entire treatment plan on exploring his childhood relationship with his own father's work history
- D. Postpone any goal-setting until his mood has fully returned to baseline on its own without any structured plan

Question 8. Marcus asks for help finding concrete job-search and financial resources. What is the best response?

- A. Incorporate problem-solving around concrete resources such as job-placement and financial-counseling referrals
- B. Tell him that discussing practical job-search resources is not appropriate work for a counseling session
- C. Personally review his resume in detail and coach him directly on interview technique during each session
- D. Redirect the conversation away from finances entirely and keep every session focused only on his sleep pattern

Question 9. How should the counselor monitor Marcus's course over the coming weeks?

- A. Track his symptoms at each session and reassess if they persist or worsen well beyond the stressor resolving
- B. Assume the diagnosis will not change and stop asking about his mood once the initial treatment plan is set
- C. Wait until the full course of sessions has ended and then evaluate his overall progress in one final review
- D. Track only whether he attends sessions consistently, without asking directly about his mood or sleep pattern

Question 10. Marcus says he feels embarrassed that a layoff has hit him this hard. What is the most therapeutic response?

- A. Validate that job loss is a significant stressor and normalize his reaction as a common, understandable response
- B. Agree that most people would have handled a layoff like this with far more composure and resilience than he has
- C. Quickly change the subject to his job-search progress so the conversation does not dwell on his feelings
- D. Tell him firmly that his embarrassment is irrational and that he has no real reason to feel that way at all

Question 11. How can the counselor best support Marcus's motivation to re-engage in activities he has pulled back from?

- A. Collaboratively schedule a small, valued activity, like one pickup basketball game, to rebuild a sense of momentum
- B. Tell him to resume his full prior schedule of activities all at once to prove to himself that he can still cope
- C. Advise him to set activities aside completely until his mood and motivation have fully returned to normal
- D. Spend the session having him list, in detail, every reason his motivation has declined since the layoff happened

Question 12. What must the counselor address during informed consent given the EAP referral?

- A. The limits of confidentiality, including exactly what information, if any, will be shared back with the EAP or employer
- B. An assurance that everything he discusses in session will be reported back to the EAP as part of routine follow-up
- C. A promise that absolutely nothing about his case will ever be disclosed to anyone under any circumstance at all
- D. A requirement that he complete the entire recommended course of sessions once he agrees to begin treatment today

Question 13. The EAP case manager emails asking for an update on Marcus's progress. What is the appropriate action?

- A. Decline to share any information unless Marcus has signed a specific, informed release authorizing that disclosure
- B. Reply immediately with a full summary of Marcus's symptoms and diagnosis since the EAP made the referral
- C. Ignore the email entirely and never respond to the EAP case manager about the request at any point going forward
- D. Ask Marcus informally in passing whether it is fine to talk to the EAP, without documenting his answer anywhere

*Case 4 of 11***Cocaine use, crashes, and mounting consequences***Diagnosis: Stimulant Use Disorder (Cocaine)*

INTAKE

Tara Brooks, a 34-year-old bartender, presents after a friend urged her to get help for her cocaine use. She reports strong cravings, using more than she intends, failed attempts to cut down, and continued use despite conflicts at work and with her partner.

SESSION TWO

She describes needing more cocaine to get the same effect and severe “crashes” of fatigue and low mood for a day or two after binges. She identifies about four to five criteria over the past year and has missed shifts and money because of her use.

SESSION THREE

She is ambivalent—relieved someone noticed yet defensive about being judged—and her crashes bring brief but intense low mood. She denies sustained elevated mood apart from intoxication and is frightened by occasional chest pain during heavy use.

Question 1. What is most important to establish to confirm a moderate stimulant use disorder?

- A. The number of DSM-5-TR criteria she meets over twelve months, since four to five place severity in the moderate range
- B. The exact quantity of cocaine she uses each week so that you can calculate precisely how dependent she has become on it
- C. Whether she first started using cocaine recreationally at parties rather than to cope with stress or with difficult emotions
- D. Whether anyone else in her immediate family has previously struggled with cocaine or with another substance-related condition

Question 2. How should the counselor understand the severe fatigue and low mood after her binges?

- A. As stimulant withdrawal—a post-use “crash” that is part of her use disorder rather than a separate primary mood disorder
- B. As clear evidence that she has a separate, independent major depressive disorder that happens to flare up after she uses
- C. As a sign that she has simply been using cocaine for too many years in a row without ever taking any meaningful break from it
- D. As an indication that she should immediately be admitted for inpatient detoxification before anything else can be considered

Question 3. Given her chest pain during heavy use and her post-binge crashes, what assessment is most important?

- A. Assess medical risk and suicide risk, since stimulant use carries cardiac danger and crashes can bring acute low mood
- B. Determine the specific brand or source of the cocaine she uses so that you can document exactly what she has been taking
- C. Explore in depth the early family experiences that she believes first shaped her current relationship with substances
- D. Identify the particular social situations and people that tend to prompt her heaviest cocaine use during the week

Question 4. How should the counselor differentiate her presentation from a primary manic episode?

- A. Her elevated, energized states occur only during intoxication rather than as sustained episodes independent of cocaine use
- B. The fact that she becomes very energetic and talkative and needs little sleep while she is actively using cocaine
- C. The irritability and conflict with her partner that have been happening more often over the past several months at home
- D. The low mood and fatigue that she experiences in the day or two right after one of her heavier cocaine binges

Question 5. What co-occurring issue is most important to screen for to inform her plan?

- A. A co-occurring depressive disorder distinct from her crashes, since mood symptoms shape sequencing and safety planning
- B. A previously undetected primary psychotic disorder that might better account for her conflicts at work and with her partner
- C. A long-standing eating disorder that could be contributing to both her low energy and her disrupted daily routine lately
- D. An emerging neurocognitive decline that could explain why she has been forgetting shifts and commitments at work recently

Question 6. What is the most appropriate first-line treatment recommendation for her stimulant use disorder?

- A. Evidence-based behavioral treatment such as contingency management and CBT, since no medication is approved for stimulant use disorder
- B. A referral to start anti-craving medication for cocaine right away, framed as the single most important step in her treatment
- C. A firm, confrontational intervention about the damage her cocaine use is doing so that she finally admits how serious it is
- D. A recommendation that she simply use willpower to stay away from cocaine, since behavioral treatment is rarely truly necessary

Question 7. How should the counselor determine her appropriate level of care?

- A. Use a multidimensional assessment of her medical risk, environment, and readiness to match her to the least intensive safe setting
- B. Automatically refer her to the most intensive residential program available so that she is kept away from cocaine entirely
- C. Recommend only a single weekly mutual-help meeting, assuming that peer support alone will be enough to address her cocaine use
- D. Let her choose whichever program has the most convenient schedule regardless of her medical risks or her clinical needs

Question 8. Given her ambivalence, what is the most appropriate primary counseling approach?

- A. Motivational interviewing to explore her ambivalence and strengthen her own reasons for change at her current stage
- B. A firm, direct confrontation about the serious damage her cocaine use is causing so that she finally faces the full reality of it
- C. A structured relapse-prevention program built entirely around her committing right now to a firm quit date for complete abstinence
- D. Primarily educational lectures detailing the medical harms of cocaine so that the facts alone convince her to stop using it

Question 9. Tara says, “Maybe my using isn’t even that big a deal.”

The MI-consistent response is to:

- A. Reflect her statement and explore both sides of her ambivalence without arguing against her about how serious it is
- B. Point out all of the evidence that proves her cocaine use is in fact a very serious problem that she needs to address now
- C. Agree fully that her cocaine use is not really a problem so that she feels supported and stays comfortable working with you
- D. Quickly change the subject to avoid any disagreement with her about exactly how serious her cocaine use really is right now

Question 10. Tara returns to use after a week of abstinence and is discouraged. The most therapeutic response is to:

- A. Frame the return to use as common in recovery, explore what it taught her, and revisit the plan without shaming her
- B. Warn her seriously that another slip like this one will likely undo all of her progress and prove she cannot really do this
- C. Recommend that she immediately step up to a far more restrictive level of care since the current plan has obviously failed her
- D. Reassure her that the slip really does not matter at all and encourage her to simply forget about it and move on quickly

Question 11. How should the counselor frame Tara's post-binge crashes in their work together?

- A. As high-risk periods for low mood and safety that warrant monitoring and a plan for support during the crash
- B. As proof that she simply needs to use a smaller amount of cocaine so that her crashes will not feel quite so severe afterward
- C. As an unimportant and temporary discomfort that will pass on its own and therefore does not require any particular attention
- D. As clear evidence that she has a separate bipolar disorder that will need to be treated entirely apart from her cocaine use

Question 12. Tara's employer requests her substance-use treatment records after she missed shifts. Best response?

- A. Decline to release records without a valid release and explain that substance-use information has heightened confidentiality protections
- B. Send the records to the employer promptly so that the employer can decide how best to handle her recent missed work shifts
- C. Share only her diagnosis with the employer, since a single diagnostic label seems like a relatively harmless detail to provide
- D. Forward the employer's request to Tara and avoid documenting in the record that the request was ever received at all

Question 13. Tara asks whether there is a medication that will cure her cocaine cravings. The most appropriate response is to:

- A. Explain honestly that no medication is approved for stimulant use disorder and coordinate medical evaluation for related concerns
- B. Recommend a specific medication and dose that she should ask her physician to prescribe to take away her cocaine cravings
- C. Assure her that a reliable anti-cocaine medication exists and that starting it will be the single most important part of her treatment
- D. Tell her that medication questions have nothing to do with counseling and decline to discuss anything related to her health concerns

*Case 5 of 11***Minimized suicidal thoughts and an accessible firearm after job loss and separation***Diagnosis: Suicidal Behavior / Acute Risk*

INTAKE

Marcus Ridgeway, a 45-year-old Army veteran, presents to outpatient counseling two weeks after his wife moved out and five weeks after being laid off from his warehouse management job. He reports feeling numb and having occasional thoughts of not waking up, but denies any specific plan when asked directly.

SESSION TWO

He describes broken sleep, drinking most nights alone, and feeling like a burden to his two teenage children. Asked about safety, he says, "I think about it sometimes, but I'd never actually do it — I'm not that guy." Almost in passing, he mentions he keeps his old service pistol loaded in the nightstand "just in case," and that he took it apart and cleaned it last week to make sure it still worked, out of habit.

SESSION THREE

He continues to minimize, calling the thoughts "just background noise," but reports growing hopelessness about reconciling with his wife and describes rehearsing what he would say in a note to his children "just to get it out of his head." His weekend drinking has increased, and he is reluctant to discuss the firearm directly, saying he has "always been careful with guns."

Question 1. Marcus says, "I think about it sometimes, but I'd never actually do it," then quickly changes the subject. What is the most appropriate first step?

- A. Conduct a structured suicide risk assessment covering ideation, intent, plan, and access to means, without accepting his reassurance at face value
- B. Accept that he would never act on the thoughts since he stated this clearly, and move the conversation toward his current job search and daily routine instead
- C. Begin teaching grounding and relaxation exercises right away so he has tools to manage the thoughts before anything else
- D. Refer him immediately to a psychiatrist for a medication evaluation and postpone talking further about the thoughts until that visit

Question 2. Marcus mentions, almost in passing, that he keeps his sidearm loaded in the nightstand and checked recently that it still worked, then adds that he'd "never actually do it." What should the counselor do?

- A. Explore the disclosure directly, asking follow-up questions about the firearm's accessibility and his recent thoughts about it, rather than accepting his reassurance
- B. Take his reassurance that he would never act on the thoughts at face value and continue gathering his employment and relationship history for the intake
- C. Note the comment for later in the chart and continue with the standard intake questions in the order originally planned for today
- D. Ask his estranged wife by phone to remove the firearm from the home without first discussing the disclosure further with Marcus himself

Question 3. What does Marcus's disclosure about the loaded, recently checked firearm most significantly indicate for the risk assessment?

- A. Immediate access to a highly lethal method, independent of his verbal reassurance, which sharply elevates acute risk
- B. A need for a detailed history of every firearm he has ever owned so the record fully documents his background with weapons
- C. A signal to explore in depth the childhood experiences that shaped how he learned to cope with painful emotions
- D. A prompt to precisely calculate how many drinks he consumes on an average night so his alcohol use can be quantified

Question 4. Marcus mentions his teenage children as a reason he has not acted, but also that he keeps rehearsing a note to them. What is most important for a balanced risk formulation?

- A. Weigh both his risk factors, such as the accessible firearm and hopelessness, against protective factors like his children, rather than reading either alone
- B. Focus only on the fact that he has not yet written or sent an actual note, since no completed note means the risk is currently low
- C. Determine whether his distress is objectively proportionate to the severity of the divorce and job loss he has experienced recently
- D. Ask him to rate his overall mood on a simple numeric scale each session so the trend can be tracked over the coming weeks

Question 5. What co-occurring factor is most important to screen for that also raises Marcus's acute risk this week?

- A. His increased weekend drinking, which can heighten impulsivity and lower the threshold for acting on suicidal thoughts
- B. A previously undetected primary psychotic disorder that could better explain his numbness and withdrawal from others
- C. A long-standing eating disorder that might be contributing to his poor sleep and his withdrawal from his children this month
- D. An emerging neurocognitive decline that could explain why he has trouble concentrating during the current crisis

Question 6. What is the most appropriate cornerstone of Marcus's safety plan?

- A. A collaborative safety plan identifying his warning signs, coping strategies, supports, and crisis contacts, built together with him
- B. A signed no-suicide contract in which Marcus formally promises in writing that he will not harm himself before the next appointment
- C. A detailed weekly schedule of pleasant activities intended to lift his mood enough that the suicidal thoughts eventually fade
- D. A firm instruction that Marcus must call the counselor's personal phone the moment any suicidal thought enters his mind

Question 7. Marcus is willing to build a safety plan but says he'd rather "get into the counseling stuff first" before talking about the gun. How should the counselor proceed?

- A. Address means restriction for the firearm now, alongside starting the broader safety plan, rather than delaying it to a future session
- B. Follow his lead and postpone the firearm conversation until rapport has developed further over the next several sessions
- C. Spend the current session exploring the roots of his depression in depth and revisit the firearm only if it comes up again naturally
- D. Tell him firmly that counseling cannot continue at all until the firearm is completely removed from the home today

Question 8. What is the most appropriate way for the counselor to address the firearm itself?

- A. Counsel collaboratively on means restriction, such as temporarily storing the firearm with a trusted person away from the home or using a lock
- B. Contact local police directly to have the firearm confiscated from his home without first discussing the plan with Marcus
- C. Suggest he simply unload the firearm and keep the ammunition stored in a separate drawer within the very same bedroom
- D. Respect his autonomy and leave the decision about the firearm entirely up to him without raising the topic again

Question 9. On what basis should the counselor decide whether Marcus needs a higher level of care, such as psychiatric hospitalization?

- A. A clinical judgment weighing his intent, access to means, protective factors, and his ability to engage safely with a collaborative plan
- B. Hospitalizing him automatically today simply because he disclosed both suicidal thoughts and access to a firearm in the same session
- C. Leaving the hospitalization decision entirely up to Marcus himself, since he denies any current intent to act on the thoughts
- D. Avoiding any discussion of hospitalization at all so as not to damage the therapeutic relationship being built with him

Question 10. Marcus says the thoughts are "just background noise" but also mentions he's been rehearsing what he'd say in a note to his kids. What is the most therapeutic response?

- A. Gently name the discrepancy between his minimizing language and the note-rehearsal, and explore its meaning and urgency with empathy
- B. Reassure him that rehearsing a note is a common, low-risk habit that does not need much further attention today
- C. Accept his description of background noise and shift the conversation to lighter, less distressing topics for the rest of the session
- D. Firmly tell him that writing a note to his children is dangerous and that he must stop thinking about it immediately

Question 11. Marcus expresses hopelessness that his marriage and career will ever be okay again. How can the counselor best instill hope without dismissing his pain?

- A. Validate how overwhelming the losses feel right now while helping him see the current crisis and his options as changeable over time
- B. Promise him confidently that his wife will likely return and that a new job will come soon if he just stays positive
- C. Agree that his situation does look genuinely hopeless right now and focus only on getting him through the next few hours
- D. Redirect the conversation to practical job-search tasks whenever his hopelessness about the marriage and career comes up

Question 12. Marcus asks whether anything he says about the gun or his thoughts will ever leave the room. What is the most appropriate response?

- A. Explain the limits of confidentiality, including that a serious and foreseeable risk of harm may require disclosure to keep him safe
- B. Promise him that nothing he discloses about the firearm or his thoughts will ever be shared with anyone under any circumstances
- C. Tell him that because he mentioned a firearm, the counselor must call the police right now regardless of how the full risk assessment turns out
- D. Avoid answering his question directly so that he keeps feeling comfortable continuing to disclose further details

Question 13. Marcus agrees to a collaborative safety plan and to temporarily store the firearm with his brother. How should the counselor document this session?

- A. Document the risk assessment findings, the means-restriction steps agreed to, the safety plan, and the clinical rationale for the disposition
- B. Record only that Marcus promised he would not harm himself before the next scheduled appointment with the counselor
- C. Write a brief note stating simply that the client is fine now and that no further risk monitoring is needed going forward
- D. Leave the details of the suicide risk assessment out of the chart entirely to protect his privacy as a veteran

*Case 6 of 11***Years of mood ups and downs that never reach full episodes***Diagnosis: Cyclothymic Disorder*

INTAKE

Priya Nair, a 27-year-old graduate student, describes more than two years of frequent mood swings. She has stretches of elevated energy and confidence and other stretches of low mood and fatigue, neither of which has ever fully met the threshold for a manic or major depressive episode.

SESSION TWO

Over these two years she has not been free of symptoms for more than a month or two at a time. The mood shifts are not tied to a consistent interpersonal trigger and unfold over days to weeks rather than within hours, and her sense of self and her relationships are stable between them.

SESSION THREE

A medical and substance review was unremarkable. The swings interfere with her studies and leave her unsure which version of herself to trust, but she denies discrete episodes of clearly elevated, expansive mood with the impairment that would mark a hypomanic or manic episode.

Question 1. What is most important to confirm to support a cyclothymic disorder diagnosis?

- A. Two or more years of subthreshold mood swings with no symptom-free stretch longer than about two months
- B. That she can identify the single stressful event she believes first set off her pattern of mood swings
- C. That her swings have become a little more frequent over the past few days than they were the week before
- D. That she currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Which feature best differentiates her presentation from bipolar II disorder?

- A. Her highs and lows have stayed subthreshold, never reaching a full hypomanic or major depressive episode
- B. The fact that she goes through stretches of higher energy and confidence at some times and lower mood at others
- C. The way her mood swings interfere with her ability to keep up with her graduate studies and her work
- D. The way her energy tends to be a little higher during the lighter and less demanding weeks of her semester

Question 3. How does her presentation differ from borderline personality disorder?

- A. Her mood shifts unfold over days to weeks with a stable sense of self, not the rapid, interpersonally triggered instability of BPD
- B. The fact that her mood goes up and down and changes quite noticeably from one period of time to another over the course of her weeks and months
- C. The difficulty she has knowing which version of herself she can trust during the different mood states
- D. The way her mood tends to be a little lower in the evenings than it is earlier on during the daytime hours

Question 4. What is the most important condition to exclude before confirming the diagnosis?

- A. A substance- or medical-induced mood disturbance, which her unremarkable review makes less likely but worth confirming
- B. A previously undetected specific phobia that might better account for the swings in her mood and energy
- C. A long-standing eating disorder that could be the single underlying cause of all of her mood swings
- D. An emerging neurodevelopmental disorder that could explain why she feels so confident during her higher periods

Question 5. What is most important to screen for across the course of her mood disorder?

- A. Suicide risk and any escalation toward a full mood episode, monitoring across both the high and low periods
- B. A previously undetected primary psychotic disorder that might better explain her elevated, confident periods
- C. A long-standing eating disorder that could be the single underlying cause of her fluctuating energy levels
- D. An emerging neurocognitive decline that could explain why she feels unsure which version of herself to trust

Question 6. What is the most appropriate first-line treatment framework for Priya?

- A. Psychotherapy that stabilizes daily routines and rhythms, with psychoeducation about her mood pattern and triggers
- B. A brief protocol aimed only at her low periods, ignoring the high periods and the overall cycling pattern entirely
- C. Starting her on a mood-stabilizing medication that you will select and titrate over the coming weeks yourself
- D. Encouraging her to lean into the high periods and stay up working through them since that is when she feels most productive

Question 7. What lifestyle target is most important to emphasize given her presentation?

- A. Consistent sleep, activity, and daily routines, since rhythm disruption can amplify her mood swings
- B. Eliminating all structure and routine so that she is free to follow whatever her mood happens to dictate each day
- C. A demanding new exercise regimen pursued only during her high periods and dropped entirely during her low periods
- D. Focusing only on her diet and leaving sleep, activity, and daily routine entirely out of the plan for now

Question 8. How should progress be tracked over the course of treatment?

- A. Use a daily mood-and-routine chart with periodic measures to monitor cycling, function, and risk over time
- B. Rely only on whether she happens to say in session that her mood feels a little steadier than before
- C. Track only how she feels on the single day of each appointment and ignore the pattern across the weeks between visits
- D. Wait until the end of treatment to review whether her mood pattern has changed at all since intake

Question 9. How should the counselor handle her questions about medication?

- A. Coordinate a referral to a prescriber for evaluation while continuing the rhythm-focused psychotherapy within scope
- B. Recommend a specific mood-stabilizer and dose for her to begin taking before her next set of exams
- C. Tell her that medication would certainly ruin her creativity and that she should refuse it if it is ever offered
- D. Ignore the medication question and simply redirect her back to the routine charting every time she raises it

Question 10. During a high period Priya says she feels great and wants to stop treatment. The most therapeutic response is to:

- A. Explore the wish nonjudgmentally and use psychoeducation to connect the highs to the broader cycling pattern
- B. Agree that she is clearly cured now and end treatment, since she reports feeling so well during this current stretch
- C. Warn her sharply that she will inevitably crash and that stopping now would be a serious and irreversible mistake
- D. Let the topic pass without comment and simply continue the session as though she had not raised stopping at all

Question 11. How can the counselor best support Priya's engagement given the chronic, fluctuating course?

- A. Normalize the long-term nature of the work and anchor it in goals she values across both her high and low periods
- B. Promise her that a few weeks of treatment will permanently end the mood swings for good and resolve everything
- C. Focus the sessions mainly on cataloguing each individual mood shift in exhaustive detail every single week
- D. Tell her counseling cannot help unless she first agrees to take medication exactly as a prescriber directs

Question 12. Priya asks the counselor to leave her diagnosis out of records shared with her university for accommodations. The most appropriate action is to:

- A. Discuss confidentiality and the limits of any release, sharing only what she consents to and what is needed
- B. Send her full clinical record, including the diagnosis, to the university right away since they will need everything anyway
- C. Write whatever the university prefers to see, even if it is not accurate, so that her accommodations are approved quickly
- D. Refuse to provide any documentation at all and decline to discuss what could appropriately be shared with the university

Question 13. The counselor is unsure whether the presentation has crossed into bipolar II. The most ethical course of action is to:

- A. Seek consultation and reassess against the criteria, coordinating referral if the picture exceeds your competence
- B. Keep the original diagnosis fixed and never revisit it, regardless of any new information that emerges over time
- C. Tell her the distinction does not matter at all and that there is no need to clarify which disorder she has
- D. Transfer her at once to another provider with no explanation, consultation, or coordinated plan for the transition

*Case 7 of 11***Psychosis that persists between mood episodes***Diagnosis: Schizoaffective Disorder, Bipolar Type*

INTAKE

Marcus Bell, a 34-year-old former rideshare driver, is referred after a hospitalization. Over the past two years he has had episodes of elevated, expansive mood with little sleep and grandiose plans, as well as periods of persecutory delusions and auditory hallucinations.

SESSION TWO

Records and his account show at least two weeks of delusions and hallucinations occurring while his mood was stable and neither high nor low. Mood episodes—both manic and depressive—have been present for the majority of the illness, and substance and medical causes were excluded.

SESSION THREE

Currently his mood is euthymic but he still hears critical voices and remains guarded. He is taking medication intermittently, has lost housing stability, and at one low point had fleeting thoughts that life was not worth living, without current plan or intent.

Question 1. What is most important to confirm to support a schizoaffective disorder diagnosis?

- A. At least two weeks of psychosis without a mood episode, with mood episodes present for most of the illness
- B. That he can identify the single stressful event he believes first set off his very first hospitalization
- C. That his symptoms have grown a little more intense over the past few days than they were the week before that
- D. That he currently shows the exact number of neurocognitive deficits required to diagnose a major neurocognitive disorder

Question 2. Which feature best differentiates his presentation from schizophrenia?

- A. Mood episodes have been present for the majority of his illness rather than being brief or only incidental
- B. The fact that he has experienced persecutory delusions and hears critical auditory hallucinations during parts of his illness
- C. The guardedness and the mistrust that he shows toward other people during the times when he is feeling unwell
- D. The way his symptoms have tended to worsen around the most stressful and disruptive periods in his living situation

Question 3. How does his presentation differ from bipolar I disorder with psychotic features?

- A. He has had psychosis for two or more weeks while his mood was stable, not only during mood episodes
- B. The fact that he has gone through clear periods of elevated, expansive mood with reduced sleep and grandiose plans
- C. The grandiose thoughts and the ambitious plans that he tends to form when his mood is running high
- D. The way his sleep tends to be more broken during the noisier and more crowded nights at the places where he has stayed

Question 4. What is the most important condition to exclude before confirming the diagnosis?

- A. A substance- or medical-induced cause, which the records indicate were already assessed and excluded
- B. A previously undetected specific phobia that might better account for his guardedness around other people
- C. A long-standing eating disorder that could be the sole and complete explanation for his mood and psychotic symptoms
- D. An emerging neurodevelopmental disorder that could explain why his plans during high periods are so ambitious

Question 5. Given his fleeting thoughts that life was not worth living, what is the most important assessment step?

- A. Conduct a structured suicide-risk assessment of current ideation, intent, plan, and protective factors now
- B. Assume the risk has fully passed because he says the thoughts were only fleeting and he denies any plan right now
- C. Defer any risk assessment until his psychotic symptoms have fully resolved and his mood has been stable for several months
- D. Focus only on his housing situation for now and plan to return to any questions about his safety at a much later date

Question 6. What is the most appropriate first-line treatment recommendation?

- A. Coordinate combined pharmacotherapy and psychosocial support within a treatment team and your counseling role
- B. Begin intensive insight-oriented therapy alone to help him uncover the deeper meaning of his hallucinations before anything else
- C. Adjust his antipsychotic and mood-stabilizer doses yourself and monitor his blood levels over the coming weeks
- D. Recommend a course of weekly supportive talks on their own as the complete treatment for his condition going forward

Question 7. Given his intermittent medication use, what should the plan emphasize?

- A. Collaborative work on engagement and adherence, addressing the barriers that interrupt his medication and care
- B. Tell him that if he will not take his medication exactly as directed there is no point in continuing any counseling at all
- C. Leave all decisions about his medication entirely to him without ever raising adherence as a topic in the counseling work
- D. Advise him to stop the medication during good stretches and only restart it whenever his symptoms happen to come back

Question 8. How should the counselor address his unstable housing within the plan?

- A. Coordinate case management and community resources for housing as part of recovery-oriented, wraparound support
- B. Treat his housing situation as completely unrelated to his mental health and leave it out of the plan entirely
- C. Personally guarantee a lease and advance him the rent yourself so that his housing problem is resolved immediately
- D. Tell him to sort out his own housing entirely before he returns, since counseling cannot start until his life is stable

Question 9. How should the counselor coordinate with his prescriber and the hospital team?

- A. Obtain his informed consent and a release, then collaborate with the team and share what is needed for his care
- B. Share his full history with everyone involved right away, since coordinating his care is plainly in his best interest anyway
- C. Avoid contacting the prescriber or the hospital at all so that his counseling stays completely separate from his other care
- D. Release every detail of his record to all parties so they each hold the complete picture of everything in his treatment

Question 10. During a session Marcus says the voices are mocking him right now. The most therapeutic response is to:

- A. Acknowledge how distressing the voices are and stay supportive, without confirming or arguing about their reality
- B. Insist firmly and repeatedly that the voices are not real until he finally agrees that there is nothing there at all
- C. Agree with him that the voices are real people who are truly speaking to him so that he feels you are on his side
- D. Change the subject immediately so the conversation never touches anything that might upset or provoke him further

Question 11. How can the counselor best build engagement given his guardedness?

- A. Move at his pace, be transparent and consistent, and work toward concrete goals he chooses and values
- B. Press him to disclose all of his most private experiences in the very first sessions to speed the work along quickly
- C. Focus the sessions mainly on collecting ever more detail about the content of the voices he hears each week
- D. Tell him counseling can do nothing for him unless he first agrees to take his medication exactly as prescribed

Question 12. Marcus discloses a specific, credible plan to harm an identifiable person. The most appropriate action is to:

- A. Assess the risk and follow duty-to-protect obligations, which may require warning or other protective steps
- B. Keep the threat entirely confidential, since everything shared in counseling must always stay completely private
- C. Wait to see whether he repeats the threat at the next appointment before assessing the risk or taking any action
- D. Notify the police and the person at once without first assessing the seriousness or following the proper protective process

Question 13. The counselor has limited experience with serious psychotic illness. The most ethical course of action is to:

- A. Seek consultation and work within a treatment team, referring for the components beyond your competence
- B. Continue managing the entire case alone to preserve the continuity of the relationship he has built with you
- C. Tell him his illness is not serious enough to need a team and keep seeing him for supportive talks only
- D. Transfer him at once to another provider with no explanation, consultation, or coordinated plan for the transition

Case 8 of 11

Childhood motor and vocal tics with social and school impact

Diagnosis: Tourette's Disorder

INTAKE

Eli Foster, an 11-year-old, is referred for involuntary movements and sounds. Since age seven he has had multiple motor tics—blinking, head jerks, shoulder shrugs—and at least one vocal tic, such as throat-clearing and grunting, occurring many times a day on most days.

SESSION TWO

Both motor and vocal tics have been present for more than a year, which fits Tourette's rather than a tic disorder limited to one type. The tics are not driven by intrusive obsessions, and they are involuntary movements rather than a functional neurological symptom pattern.

SESSION THREE

He feels embarrassed and is being teased at school, and he has begun avoiding class participation. His parents worry about his self-esteem, and he says he sometimes feels bad about himself, without any current thoughts of self-harm, and wishes the tics would just stop.

Question 1. What is most important to confirm to support a Tourette's disorder diagnosis?

- A. Multiple motor tics and at least one vocal tic present for more than a year with childhood onset
- B. That his parents can name the single event they believe first caused his tics to begin
- C. That his tics have grown a little more frequent over the past few days than they were the week before
- D. That he currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Which feature best differentiates his presentation from a persistent motor or vocal tic disorder?

- A. He has both multiple motor tics and a vocal tic, not tics limited to only one of the two types
- B. The fact that he has involuntary movements and sounds that he repeats many times during most days
- C. The embarrassment and the self-consciousness that he feels about the tics he cannot fully control
- D. The way his tics tend to be a little worse during the busiest and most stressful weeks at his school

Question 3. How does his presentation differ from obsessive-compulsive disorder?

- A. His tics are involuntary movements and sounds, not compulsions performed to neutralize intrusive obsessions
- B. The fact that he repeats certain movements and sounds again and again throughout much of his day
- C. The discomfort and the urge he sometimes feels building before a tic happens during his day
- D. The way his tics tend to be a little worse on the days when he has had less sleep the night before

Question 4. What co-occurring condition is most important to screen for given his presentation?

- A. Co-occurring ADHD, OCD, and anxiety, which commonly accompany Tourette's and shape the plan
- B. A previously undetected primary psychotic disorder that might better account for his involuntary movements
- C. A long-standing eating disorder that could be the single underlying cause of all of his tics
- D. An emerging neurocognitive decline that could explain why his tics have continued over the past few years

Question 5. Given the teasing and his remark that he feels bad about himself, what is most important to assess?

- A. The emotional impact on his self-esteem and mood, with developmentally appropriate screening for safety
- B. A previously undetected primary psychotic disorder that might better account for his low self-esteem
- C. A long-standing eating disorder that could be the single underlying cause of his low mood and tics
- D. An emerging neurocognitive decline that could account for why he is avoiding participating in his class

Question 6. What is the most appropriate first-line treatment framework for Eli?

- A. Behavioral intervention for tics with psychoeducation and school support, coordinated with the family
- B. Telling him to simply suppress all of his tics through willpower whenever he is in class or around others
- C. Starting him on a medication that you will select and adjust to reduce his tics over the coming weeks
- D. Telling the parents the tics are nothing to address and that he will simply grow out of them on his own

Question 7. How should the counselor address the teasing and school impact?

- A. Provide psychoeducation and coordinate school supports and anti-bullying measures to protect his participation
- B. Tell him to just ignore the teasing and that he should not let what other children say bother him at all
- C. Advise his parents to keep him out of school until the tics stop so that the teasing simply cannot happen
- D. Focus only on reducing the tics and leave the teasing and school environment entirely out of the plan

Question 8. How should progress be tracked over the course of treatment?

- A. Track tic frequency, distress, school participation, and self-esteem over time to guide the plan
- B. Rely only on whether his parents happen to mention that he seems a little happier at home lately
- C. Track only the raw number of tics per minute and base every decision solely on that one figure
- D. Wait until the end of treatment to review whether anything has changed at all since the first session

Question 9. How should the counselor coordinate with the school and any medical providers?

- A. With the parents' consent and a release, coordinate school supports and any medical care for the tics
- B. Contact the school and providers directly with his clinical details right away since they will need the information
- C. Avoid contacting the school or any providers at all so that his counseling stays entirely separate from them
- D. Send his full clinical record to the school so that every staff member has complete information about his care

Question 10. Eli says he hates his tics and wishes he were "normal like everyone else." The most therapeutic response is to:

- A. Validate his feelings, provide age-appropriate psychoeducation, and build self-acceptance and coping
- B. Tell him he just needs to try harder to control the tics so that he can be like the other children
- C. Agree that the tics do make him quite different from everyone else so that he learns to accept that reality
- D. Change the subject to something more fun so that he does not have to sit with the difficult feelings at all

Question 11. How can the counselor best engage Eli given his age and embarrassment?

- A. Use developmentally appropriate, strengths-based methods that build coping and confidence at his level
- B. Talk with him mainly in abstract, adult terms about the neurology of why his tics occur each week
- C. Focus the sessions mainly on counting and reviewing each of his tics in exhaustive detail every week
- D. Tell him he must stop his tics before any of the counseling work to help his confidence can begin

Question 12. Eli's parents ask the counselor to write that his tics are "fully under control" for a school activity he wants to join. The most appropriate action is to:

- A. Decline to misrepresent his status and clarify what the assessment and your role can accurately support
- B. Write the statement exactly as they request, since keeping the parents satisfied is what matters most here
- C. Take the parents' side and advocate for the claim in any letter, regardless of what the assessment shows
- D. Refuse to provide any documentation at all and decline to explain what could appropriately be reported about him

Question 13. The counselor has limited training in behavioral tic interventions. The most ethical course of action is to:

- A. Seek consultation and training, and refer or coordinate for the components beyond your competence
- B. Continue managing the case alone to preserve the continuity of the relationship built with the family
- C. Tell the parents the tics are not serious enough to need specialized help and continue with general support only
- D. Transfer the family at once to another provider with no explanation, consultation, or coordinated plan for the transition

*Case 9 of 11***Months of broken sleep and exhausting days***Diagnosis: Insomnia Disorder, Chronic*

INTAKE

Hank Whitfield, a 50-year-old accountant, presents with about five months of difficulty falling and staying asleep at least four nights a week despite a regular schedule and adequate time in bed. He feels exhausted, irritable, and foggy at work during the day.

SESSION TWO

He lies awake for an hour or more, watches the clock, and worries about not sleeping; he then naps and uses caffeine to get through the day. He denies loud snoring, witnessed pauses in breathing, or gasping awake, and his partner has not noticed these signs.

SESSION THREE

He says the sleep problem is his main concern and is not driven by pervasive daytime worry across many areas or by persistent low mood and anhedonia. He has begun spending long stretches in bed trying to force sleep, which seems to make the nights worse.

Question 1. What is most important to confirm to support a chronic insomnia disorder diagnosis?

- A. Difficulty with sleep at least three nights a week for over three months, with daytime impairment despite adequate opportunity
- B. That he can identify the single stressful event he believes first triggered his current difficulty with sleeping at night
- C. That his sleep has become noticeably worse over the past two or three nights compared with how it was the week before
- D. That he has very little opportunity to sleep because of a demanding work schedule that keeps him up late most nights

Question 2. Which finding is most important to screen for before settling on the diagnosis?

- A. Signs of obstructive sleep apnea, such as loud snoring, witnessed breathing pauses, or gasping awake, which warrant referral
- B. The fact that he feels tired, irritable, and foggy during the day after his nights of poor and broken sleep at home
- C. Whether he tends to feel more tired on the busiest and most demanding workdays than on his quieter days at the office
- D. Whether his sleep problems are generally a little worse during the colder and darker months of the winter season

Question 3. How does his presentation differ from generalized anxiety disorder?

- A. Sleep is his primary concern rather than excessive, uncontrollable worry spanning many areas of his daily life
- B. The fact that he lies awake worrying about not being able to fall asleep and about how tired he will be the following day
- C. The irritability and the trouble concentrating that he experiences during many of his ordinary days at the office
- D. The way his sleep tends to be a bit worse during the busiest and most stressful weeks of his accounting calendar

Question 4. How does his presentation differ from a depressive disorder?

- A. His main problem is the sleep disturbance itself rather than pervasive low mood, anhedonia, and the other depressive symptoms
- B. The fact that he feels tired and low in energy during the day after sleeping poorly through most of the night
- C. The irritability and frustration that he expresses about how poorly he has been sleeping over the past several months
- D. The way his mood and energy seem somewhat lower in the early mornings than later in the afternoon and evening hours

Question 5. What assessment tool best informs the evaluation and the treatment plan?

- A. A daily sleep diary tracking bedtime, sleep onset, awakenings, rise time, and daytime functioning over a couple of weeks
- B. A single global question asking him to estimate how well he generally thinks he sleeps on an average sort of night
- C. Ordering an overnight polysomnography sleep study yourself to confirm the diagnosis before beginning any treatment planning
- D. A detailed reconstruction of his childhood sleep habits across his earliest years to find the origin of the problem

Question 6. What is the most appropriate first-line treatment recommendation?

- A. Cognitive behavioral therapy for insomnia, including stimulus control, sleep restriction, and cognitive work on sleep beliefs
- B. A referral to begin a nightly sleeping medication indefinitely, framed as the single most important part of his treatment
- C. General advice to simply relax more and to try harder to fall asleep each night without any structured behavioral protocol
- D. A primarily insight-oriented exploration of his early childhood as the central route to resolving his current sleep problems

Question 7. He spends long stretches in bed trying to force sleep. Which CBT-I component most directly addresses this?

- A. Stimulus control and sleep restriction, which strengthen the bed–sleep association and consolidate his time asleep
- B. Advising him to go to bed much earlier and to stay in bed longer so that he has more total opportunity to fall asleep
- C. Encouraging him to remain in bed and keep trying to fall asleep no matter how long he ends up lying awake during the night
- D. Recommending that he take a long daytime nap whenever he feels tired so that he can make up for the lost nighttime sleep

Question 8. Hank asks whether he should use sleeping pills. The most appropriate counselor action is to:

- A. Recommend CBT-I as first-line and, if he wants to consider medication, coordinate a referral to a prescriber with his consent
- B. Tell him which specific sleeping medication and dose he should ask his physician to prescribe for him at his next appointment
- C. Reassure him that nightly over-the-counter sleep aids are perfectly safe to rely on indefinitely for his chronic insomnia
- D. Discourage any medication entirely and tell him he must manage his insomnia through willpower and discipline alone

Question 9. How should the counselor monitor his response to CBT-I over time?

- A. Review the ongoing sleep diary at intervals to track sleep efficiency and daytime functioning and adjust the plan
- B. Ask him each week whether he subjectively feels that his sleep is generally going better than it was before
- C. Wait until the agreed course of CBT-I is fully over and then evaluate his overall response in one single review
- D. Track mainly whether he attends his sessions and treat consistent attendance as the main sign that he is improving

Question 10. Hank says he is convinced one bad night will ruin his entire next day. The most therapeutic response is to:

- A. Address the catastrophic sleep beliefs with cognitive work, since such beliefs heighten arousal and maintain the insomnia
- B. Agree that a single poor night really will likely ruin his day and help him plan how to clear his schedule for those days
- C. Reassure him repeatedly that he will definitely sleep well tonight so that he can stop worrying about it before bed
- D. Explain in technical detail the neuroscience of sleep architecture to convince him on scientific grounds not to worry

Question 11. What psychoeducation is most useful to offer Hank early in treatment?

- A. Explain how clock-watching, daytime napping, and effortful sleep maintain insomnia and how CBT-I targets that cycle
- B. Tell him the most important thing is simply to relax and not to think about his sleep at all from this point forward
- C. Give him an extensive packet on sleep neuroscience and ask him to study it thoroughly before the next appointment
- D. Reassure him that most people with insomnia eventually sleep fine again, so there is really nothing for him to address now

Question 12. Hank reveals he has been taking a friend's prescription sedative to sleep and asks the counselor to keep it off the record. Best response?

- A. Address the safety risk of using another person's prescription and document accurately rather than omitting it from the record
- B. Agree to leave the sedative use out of the record entirely so that he feels comfortable continuing to be open with you
- C. Tell him this is none of your concern as a counselor and simply move on without addressing the borrowed-medication use at all
- D. Report him immediately to the authorities for the borrowed medication before discussing the safety concern with him at all

Question 13. His employer asks the counselor whether his sleep problems make him unsafe to work. Best response?

- A. Decline to disclose without a valid release and clarify the limits of what a treating counselor can appropriately attest to
- B. Tell the employer he is perfectly safe to work so that he does not get into any trouble at his job over his sleep problems
- C. Share a general summary of his sleep treatment so the employer can decide for themselves whether he is fit to work
- D. Refuse to engage with the employer at all and hang up without explaining anything about confidentiality or the process

*Case 10 of 11***Persistent erectile difficulty straining a relationship***Diagnosis: Erectile Disorder*

INTAKE

Marcus Lee, a 44-year-old accountant, seeks help for about eight months of consistent difficulty obtaining and maintaining an erection during partnered sexual activity. The difficulty causes him significant distress and has strained his relationship with his partner.

SESSION TWO

The problem occurs on most occasions and has persisted well beyond the six-month threshold, causing clinically significant distress. It is not fully explained by another mental disorder, and he denies that the difficulty is attributable to a substance or medication he is taking.

SESSION THREE

He describes performance anxiety and anticipatory worry that worsen the difficulty, and he denies that a primary depressive episode is the driver. A medical evaluation is underway, he is embarrassed but motivated, and he wants to restore intimacy and reduce the anxiety around sex.

Question 1. What is most important to confirm to support an erectile disorder diagnosis?

- A. Difficulty obtaining or maintaining an erection on most occasions for at least six months, causing distress
- B. That he can pinpoint the single event he is convinced first caused his difficulties with erections
- C. That his difficulty has grown a little worse over the past few days than it was the week just before that
- D. That he currently meets the full symptom count required for a discrete major depressive episode at this time

Question 2. Which feature best differentiates his presentation from a primary depressive disorder?

- A. The sexual difficulty is the primary problem and he denies a depressive episode as its driver
- B. The fact that he feels distressed and discouraged about the difficulties he has been having in his relationship
- C. The low confidence and the frustration that he feels about his performance during sexual activity
- D. The way his difficulty tends to feel a little worse during the busiest and most stressful weeks at his job

Question 3. How does his presentation differ from generalized anxiety disorder?

- A. His anxiety centers on sexual performance and the erectile difficulty, not free-floating worry across many life areas
- B. The fact that he feels anxious and worried and that this affects how he functions in some situations
- C. The anticipatory worry that he experiences before situations that he finds stressful or difficult
- D. The way his worry tends to ease a little on the weekends when his overall schedule is a good deal lighter

Question 4. What is the most important thing to exclude before confirming the diagnosis?

- A. A substance-, medication-, or medical-cause for the difficulty, which the ongoing evaluation should help clarify
- B. A previously undetected specific phobia that might better account for his difficulty during sexual activity
- C. A long-standing eating disorder that could be the single underlying cause of all of his sexual difficulties
- D. An emerging neurodevelopmental disorder that could explain why the difficulty began in his forties

Question 5. What is most important to coordinate given his presentation?

- A. Coordinate with his medical provider so the physical evaluation and psychological care proceed together
- B. Recommend a specific erectile-dysfunction medication and dose for him to start yourself before his next appointment
- C. A previously undetected primary psychotic disorder that might better account for his sexual difficulty
- D. An emerging neurocognitive decline that could account for why he has difficulty during sexual activity

Question 6. What is the most appropriate first-line approach for Marcus within counseling?

- A. Psychosexual therapy addressing performance anxiety and the couple's intimacy, coordinated with medical care
- B. Telling him to simply try harder and not think about it, since the difficulty is entirely a matter of willpower
- C. Focusing only on his individual anxiety and excluding his partner and the intimacy from the plan entirely
- D. Starting him on a medication that you will select and adjust over the coming weeks yourself

Question 7. How should the counselor address his performance anxiety?

- A. Use anxiety-reduction and sensate-focus-style approaches that reduce performance pressure and rebuild intimacy
- B. Encourage him to focus intensely on achieving an erection every time so that he proves to himself he still can
- C. Tell him to avoid all sexual activity indefinitely so that he never has to face the difficulty or the anxiety again
- D. Avoid discussing his anxiety about sex entirely so that the sessions never touch on anything uncomfortable

Question 8. How should progress be tracked over the course of treatment?

- A. Track sexual function, performance anxiety, relationship satisfaction, and distress over time to guide the plan
- B. Rely only on whether he happens to mention in session that things feel a little better than they did before
- C. Track only whether he achieves an erection each time and base every decision solely on that single outcome
- D. Wait until the end of treatment to review whether his difficulty has changed at all since the first session

Question 9. How should the counselor involve his partner in the work?

- A. With his consent, consider couple-based work on communication and intimacy with appropriate agreements
- B. Bring his partner into every individual session right away without discussing it or obtaining his consent at all
- C. Tell him the relationship is not relevant and keep the work focused only on his individual performance anxiety
- D. Refuse any contact with the partner under all circumstances, even where he requests and consents to couple sessions

Question 10. Marcus is deeply embarrassed and reluctant to discuss the sexual difficulty. The most therapeutic response is to:

- A. Respond with a calm, nonjudgmental, and normalizing stance that makes it safe for him to talk about it
- B. Tell him there is no reason to be embarrassed and quickly move past the topic so he does not feel uncomfortable
- C. Joke about the difficulty to lighten the mood so that he does not take the whole thing quite so seriously
- D. Insist he describe every detail of his sexual difficulties immediately before the work can move forward at all

Question 11. How can the counselor best support Marcus's engagement and hope?

- A. Frame the difficulty as common and treatable, anchor the work in his goals, and reinforce his motivation
- B. Tell him that this kind of difficulty usually cannot be improved and that he should simply learn to live with it
- C. Focus the sessions mainly on detailing every past failed sexual encounter in exhaustive detail every week
- D. Tell him counseling cannot help him at all unless he first starts an erectile-dysfunction medication right away

Question 12. Marcus asks the counselor to keep the diagnosis out of records his insurer could access. The most appropriate action is to:

- A. Clarify confidentiality and the limits of any release, sharing only what he consents to and what is needed
- B. Send his full clinical record, including the diagnosis, to the insurer right away since they may need it anyway
- C. Falsify the record to remove the diagnosis entirely so that there is no chance the insurer could ever see it
- D. Refuse to discuss the confidentiality question at all and simply change the subject whenever he raises it

Question 13. The counselor has limited training in sexual dysfunctions. The most ethical course of action is to:

- A. Seek consultation and training, and coordinate or refer for the components beyond your competence
- B. Continue managing the case alone to preserve the continuity of the relationship built with him
- C. Tell him the difficulty is not serious enough to need specialized help and continue with general talks only
- D. Transfer him at once to another provider with no explanation, consultation, or coordinated plan for the transition

*Case 11 of 11***A defiant, irritable nine-year-old at home and school***Diagnosis: Oppositional Defiant Disorder*

INTAKE

Jamal Carter, a 9-year-old boy, is brought by his mother after months of escalating conflict. He argues with adults, refuses to follow rules, deliberately annoys his sister, blames others for his mistakes, and is often touchy, angry, and resentful at home and school.

SESSION TWO

His mother reports the pattern has lasted more than six months and occurs with several different adults. He has not been physically aggressive toward people or animals, has not destroyed property or stolen, and has no history of serious rule violations beyond defiance.

SESSION THREE

His teacher confirms similar defiance and irritability in class. Jamal can be cooperative and warm when not in conflict, and there is no indication of abuse. His mother feels exhausted and is caught in escalating power struggles and inconsistent discipline at home.

Question 1. What is most important to confirm to support an oppositional defiant disorder diagnosis?

- A. A 6-month pattern of angry/irritable mood and defiant, vindictive behavior occurring with more than one person or setting
- B. That he can clearly explain the single specific event that he believes caused him to start behaving this way at home
- C. That his defiant behavior has clearly become more severe over the past week than it was the week before that
- D. That his behavior occurs only at home with his mother and not in any other setting or with any other adults

Question 2. Which feature best differentiates his presentation from conduct disorder?

- A. His behavior is defiant and argumentative without the serious violations of others' rights, aggression, or theft seen in conduct disorder
- B. The fact that he frequently argues with the adults in his life and refuses to follow the rules they set for him, which on its own does not establish the more serious conduct-disorder diagnosis
- C. The irritability and the angry, resentful mood that he displays during many of his conflicts with adults
- D. The way his behavior tends to be worse on the busiest and most stressful days for the family at home

Question 3. How should the counselor weigh a possible co-occurring ADHD?

- A. ADHD can co-occur and should be assessed separately, since defiance differs from inattentive and hyperactive-impulsive symptoms
- B. The defiance is almost certainly just a direct expression of ADHD, so no separate assessment of ADHD is really needed
- C. ADHD can be ruled out entirely on the basis that he is able to be cooperative and warm when he is not in conflict
- D. Whether the ADHD symptoms are clearly more severe today than they were earlier in this same school year so far

Question 4. How does his presentation differ from disruptive mood dysregulation disorder (DMDD)?

- A. His core problem is defiant, argumentative behavior rather than the persistent severe irritability with frequent temper outbursts of DMDD
- B. The fact that he is often touchy and easily annoyed during many of his daily interactions with the adults around him, a feature that on its own does not point to one of these diagnoses over the other
- C. The conflict and tension that he has with his mother and his sister at home over the course of most days
- D. The way his behavior tends to be worse during the busiest and most stressful weeks at home and at school

Question 5. What assessment method most strengthens the diagnostic evaluation for Jamal?

- A. Multi-informant assessment using parent and teacher reports and rating scales across settings
- B. Basing the diagnosis only on how Jamal behaves during the counseling session itself without other informants
- C. Relying solely on the mother's account and not seeking any information from his teacher or the school
- D. Ordering neuropsychological and neuroimaging tests yourself to confirm the diagnosis before any treatment planning begins

Question 6. What is the most appropriate first-line treatment recommendation?

- A. Parent management training to build consistent, positive discipline and interrupt the coercive parent-child cycle
- B. Individual insight-oriented play therapy for Jamal alone as the complete treatment, without any parent involvement at all
- C. Recommending that his mother apply much stricter and more punitive consequences until his defiant behavior finally stops
- D. Referring the family directly to medication management as the primary treatment for his oppositional behavior

Question 7. A central target of the parent-focused work would most appropriately be:

- A. Replacing escalating commands and inconsistent follow-through with clear expectations, consistency, and positive reinforcement
- B. Coaching his mother to win each confrontation decisively so that Jamal learns once and for all exactly who is in charge
- C. Encouraging his mother to give in whenever the conflict becomes intense so that the household stays calmer overall
- D. Advising his mother to ignore all of Jamal's behavior entirely until he simply grows out of this difficult phase on his own

Question 8. How should the school be involved in the plan?

- A. With consent, coordinate consistent expectations and supports across home and school given the cross-setting pattern
- B. Keep the school entirely out of the plan so that Jamal's counseling remains a completely private matter for the family
- C. Contact the school directly to share the details of his sessions without first obtaining the family's informed consent
- D. Recommend that the school apply stricter punishments so that the consequences at school are tougher than those at home

Question 9. Jamal's mother says she feels like a failure as a parent. The most therapeutic response is to:

- A. Validate how exhausting the conflict is and reframe parent training as a skill set, not a verdict on her worth as a parent
- B. Agree that her parenting has indeed been the main cause of the problem and that she will need to fix her mistakes
- C. Tell her quickly that she is a wonderful parent and that there is really nothing she needs to change about her approach at all
- D. Provide her with detailed statistics on how common oppositional behavior is so she can see that she should not feel responsible

Question 10. How can the counselor best engage Jamal himself in the work?

- A. Build rapport, find his strengths and goals, and involve him in age-appropriate problem-solving rather than only correcting him
- B. Confront him firmly about every instance of his misbehavior so that he clearly understands just how serious the situation is
- C. Focus the sessions only on listing his misbehaviors so that he is fully aware of everything he has been doing wrong lately
- D. Tell him that if he does not change his behavior he will face increasingly serious consequences from everyone around him

Question 11. Because Jamal is a minor, how should the counselor handle confidentiality with him and his mother?

- A. Clarify at the outset, in age-appropriate terms, what will be shared with his mother and what stays private, within legal limits
- B. Promise Jamal that nothing he ever says will be shared with his mother under any circumstances at all throughout treatment
- C. Share everything Jamal says with his mother automatically, since as a minor he has no confidentiality interests at all to consider
- D. Avoid discussing confidentiality with either of them so that the issue does not complicate the start of the treatment

Question 12. During a session Jamal hints that an adult sometimes hits him hard enough to leave marks. Best step?

- A. Follow mandated-reporting duties, assess safety, and make the required child-abuse report through proper channels
- B. Keep the disclosure confidential because the information came up during a private counseling session with the child
- C. Wait several weeks to see whether Jamal mentions the same thing again before deciding whether any action is needed at all
- D. Confront the mother directly about the marks yourself before making any report through the official channels

Question 13. The counselor has limited training in child and family behavioral treatment. The most ethical course of action is to:

- A. Seek consultation and training, or refer to a clinician competent in parent training and child behavioral treatment
- B. Continue to manage the case entirely alone to preserve the continuity of the relationship he has built with the family
- C. Tell the mother the behavior is just a normal phase that needs no specialized treatment and continue with supportive talks only
- D. Transfer the family at once to another provider with no explanation, consultation, or coordinated plan for the transition

Answer Key — Exam Three

Case 1: Tearful and doubting her competence six weeks after childbirth

Diagnosis: Major Depressive Disorder, with Peripartum Onset

Q1. A. That her depressive symptoms began within four weeks of delivery and have persisted nearly daily for over two weeks with impairment

Onset window plus duration and impairment anchor the episode

Common error: Diagnosing from a single tearful moment without confirming duration

B. A specific trigger does not establish the diagnosis — Treating an identifiable trigger as the diagnostic test

C. Family history supports but does not confirm the diagnosis — Requiring family history before confirming a current episode

D. Worsening trajectory is not required for the diagnosis — Demanding a worsening course before diagnosing

Q2. A. A review of her thyroid function, medications, and substance use, since postpartum medical factors can mimic depression

Rule out medical and substance contributors first

Common error: Skipping the postpartum thyroid and medical rule-out

B. Exhaustive appointment history is low yield here — Collecting low-yield detail instead of a medical rule-out

C. Useful later, but not the immediate priority — Prioritizing depth history over the medical rule-out

D. Reading history is not diagnostically relevant — Confusing preparation review with a medical screen

Q3. A. Any thoughts of harming her daughter or herself, and any hallucinations, delusions, or disorganized thinking

Direct infant-harm and psychosis screening is essential

Common error: Skipping direct safety questions because the presentation seems mild

B. Feeding preference is not a safety screen — Substituting lifestyle questions for a safety screen

C. This bypasses the required safety screen entirely — Replacing a safety screen with a childcare-logistics discussion

D. Scheduling is administrative, not a safety screen — Confusing administrative follow-up with risk assessment

Q4. A. It rules out postpartum psychosis, since that diagnosis requires psychotic features or grossly disorganized behavior

Absence of psychotic features rules out psychosis

Common error: Treating ordinary maternal guilt as a psychotic symptom

B. This overstates certainty and is clinically inaccurate — Assuming a single negative screen rules out future risk permanently

C. This minimizes a genuine depressive symptom — Dismissing non-psychotic symptoms as unimportant

D. Emergent psychosis would change the care pathway — Assuming the initial plan never needs updating

Q5. A. Any past episode of elevated or irritable mood with decreased need for sleep, indicating a hypomanic or manic history

Past hypomania or mania would change the diagnosis

Common error: Skipping the bipolar screen before finalizing unipolar MDD

B. Obstetric complications are a separate medical concern — Confusing pregnancy complications with a psychiatric rule-out

C. Situational worry is unrelated to the bipolar rule-out — Chasing minor anxiety instead of the relevant mood history

D. Family history is informative but not the required rule-out — Substituting family history for a personal mania screen

Q6. A. Offer evidence-based psychotherapy such as cognitive behavioral therapy or interpersonal therapy for postpartum depression

First-line psychotherapy fits counselor scope

Common error: Defaulting to a medication recommendation instead

B. Prescribing is outside counselor scope — Acting outside scope of practice

C. Watchful waiting under-treats an active episode — Delaying indicated psychotherapy

D. Informal support alone is not first-line treatment — Offering an adjunct in place of the primary plan

Q7. A. Re-administer a validated perinatal measure such as the Edinburgh Postnatal Depression Scale at regular intervals

Standardized repeated measurement guides care

Common error: Relying on impressions instead of a validated measure

B. Informal check-ins lack a standardized benchmark — Trusting an unstandardized impression alone

C. End-only review misses early non-response — Skipping interim progress monitoring

D. Engagement alone does not measure symptom change — Mistaking adherence for clinical improvement

Q8. A. Provide general information and, with her consent, coordinate a referral to a prescriber for a medication evaluation

Referral and coordination stay within scope

Common error: Giving specific medication advice directly

B. Naming a specific medication exceeds scope — Overstepping professional scope of practice

C. Discouraging medication overreaches — Steering a medical decision unilaterally

D. Outsourcing to self-research is poor coordination — Replacing referral with unguided self-research

Q9. A. Collaboratively schedule small, manageable activities such as a short walk with the baby or a brief call to a friend

Graded, valued activity rebuilds reward

Common error: Waiting for motivation to return before scheduling activity

B. Too much too soon invites failure — Ignoring graded pacing in favor of an abrupt full schedule

C. Thought logs fit cognitive work, not the first behavioral step — Confusing behavioral activation with cognitive restructuring

D. Waiting on motivation reverses the intervention logic — Letting mood gate behavior instead of scheduling it

Q10. A. Offer psychoeducation framing postpartum depression as common and treatable while validating how exhausting this feels

Psychoeducation reframes self-blame with validation

Common error: Either only lecturing on facts or only sympathizing without information

B. Premature disputation can feel invalidating — Disputing a belief before validating the feeling behind it

C. Endorsing the self-blame deepens the distortion — Colluding with a client's harshest self-judgment

D. Avoidance leaves the belief unaddressed — Steering away from emotionally central material

Q11. A. Acknowledge the doubt as understandable and collaboratively set one small, achievable goal to test it against

Name the doubt and test it behaviorally together

Common error: Trying to argue the client out of her doubt

B. Overpromising risks credibility — Making promises the counselor cannot keep

C. Agreeing confirms the doubt and abandons treatment — Adopting the client's pessimistic stance as fact

D. Lecturing rarely shifts emotional doubt — Substituting facts for an experiential intervention

Q12. A. The limits of confidentiality, including circumstances that would require the counselor to act to prevent serious harm

Confidentiality limits must be disclosed up front

Common error: Omitting the limits of confidentiality from consent

B. Absolute confidentiality is an inaccurate promise — Guaranteeing unconditional secrecy

C. Goals are set collaboratively, not imposed — Removing the client from her own treatment planning

D. Clients may withdraw consent at any time — Treating initial consent as a binding, irrevocable contract

Q13. A. Confirm she has a private space and stable connection, review telehealth consent and confidentiality limits, and proceed

Verify privacy and consent, then continue telehealth

Common error: Assuming telehealth suitability without checking privacy each time

B. A blanket rule against telehealth is unsupported — Ruling out an appropriate modality without justification

C. Skipping the ongoing privacy check is unethical — Failing to reverify privacy conditions on an ongoing basis

D. Pausing indicated treatment is not necessary — Delaying needed treatment for a manageable logistical issue

Case 2: A 16-year-old who avoids the cafeteria and dreads speaking in class

Diagnosis: Social Anxiety Disorder

Q1. A. That the fear of scrutiny occurs across multiple social situations, has lasted six months or more, and impairs functioning

Duration, breadth, and impairment anchor the diagnosis

Common error: Diagnosing from a single avoided situation

B. A specific target person is not required for the diagnosis — Treating one feared peer as the diagnostic test

C. Collateral report is helpful but not required to anchor the diagnosis — Requiring parent confirmation before assessing criteria

D. Worsening trajectory is not a diagnostic requirement — Demanding recent escalation to diagnose

Q2. A. Whether her avoidance is limited to social/performance situations or also includes fear of separating from caregivers

Clarifying the object of fear separates social anxiety from separation anxiety

Common error: Assuming any school avoidance is separation-driven

B. Exhaustive grade history is low yield for this differential — Collecting data that does not inform the differential

C. A full activity inventory is not the priority rule-out — Prioritizing breadth over targeted differential questions

D. Worth screening but secondary since she speaks normally one-on-one — Over-weighting a rule-out already partly answered

Q3. A. Review informed consent with Maya and coordinate appropriate parent/guardian notification per school and state policy

Minors require guardian involvement alongside the client's assent

Common error: Treating the minor's consent as fully sufficient alone

B. Skipping guardian involvement ignores minor consent requirements — Applying adult consent standards to a minor

C. Delaying all contact is unnecessarily rigid for a routine referral — Confusing thoroughness with unnecessary delay

D. Concealing services from guardians conflicts with consent obligations — Prioritizing secrecy over proper consent procedure

Q4. A. Her fear and avoidance are specifically tied to social and performance situations rather than a broad range of everyday worries

A focused social/performance fear points to SAD over diffuse GAD worry

Common error: Labeling any teen anxiety as generalized worry

B. Frequency alone does not distinguish the two disorders — Using frequency instead of content to differentiate

C. Disproportionate fear is a shared SAD/GAD feature, not a distinguishing one — Treating proportionality as diagnosis-specific

D. Anticipated future course does not define the current differential — Diagnosing from anticipated outcome rather than present criteria

Q5. A. Briefly and routinely screen for mood symptoms and any thoughts of self-harm as part of a complete intake assessment

Routine screening is standard even when risk appears low

Common error: Skipping routine screening because risk seems unlikely

B. Omitting a routine safety screen is poor practice regardless of presenting concern — Assuming a clear presentation means no screening is needed

C. Invoking crisis services without indicators is disproportionate — Escalating response beyond what the data supports

D. Delaying a routine screen unnecessarily removes a standard intake element — Postponing routine screening past the initial assessment

Q6. A. Offer cognitive behavioral therapy with graded exposure to feared social situations such as the cafeteria and class speaking

CBT with graded exposure is first-line for social anxiety

Common error: Defaulting to medication as the first recommendation

B. Prescribing and dosing is outside counselor scope — Acting outside scope of practice

C. Encouraging continued avoidance maintains the disorder — Reinforcing the very pattern that needs to change

D. Watchful waiting under-treats a year-long, impairing pattern — Delaying indicated care for a clear presentation

Q7. A. Build a graded hierarchy of feared situations, starting with a lower-anxiety step and progressing collaboratively to harder ones

A graded hierarchy is the standard exposure structure

Common error: Starting with the most feared situation first

B. Starting at the top of the hierarchy risks flooding and dropout — Skipping graded pacing entirely

C. Extended historical exploration delays the indicated exposure work — Substituting history review for active treatment

D. Waiting for anxiety to subside first reverses exposure logic — Letting anxiety level gate behavioral steps

Q8. A. With Maya's consent, coach the mother to encourage gradual approach rather than accommodating avoidance of feared situations

Reducing family accommodation supports exposure-based progress

Common error: Leaving the caregiver with no concrete role

B. Facilitating avoidance undermines treatment — Supporting the exact behavior treatment targets for change

C. Dismissing the caregiver misses a useful support resource — Declining a helpful support resource

D. Excessive monitoring can increase self-consciousness and undermine autonomy — Recommending intrusive oversight instead of graded support

Q9. A. Use a validated anxiety measure at regular intervals alongside tracking her completion of hierarchy steps

Standardized measurement plus behavioral tracking guides care

Common error: Relying on impressions alone

B. A vague global impression lacks a standardized benchmark — Trusting unstandardized impressions alone

C. Reviewing only at the end misses the chance to adjust the plan early — Skipping interim progress checks

D. Attendance is necessary but does not measure symptom change — Mistaking attendance for treatment outcome

Q10. A. Validate how difficult it feels right now while gently exploring the evidence for and against that all-or-nothing prediction

Validation paired with gentle cognitive exploration addresses the distortion collaboratively

Common error: Either only sympathizing or immediately disputing

B. Blunt confrontation without validation can feel invalidating — Disputing before validating the feeling

C. Agreeing with the catastrophic prediction reinforces avoidance — Colluding with the client's catastrophic belief

D. Avoiding the topic leaves the core belief unaddressed — Sidestepping emotionally central material

Q11. A. Normalize her doubt as common early in treatment and collaboratively set one small, achievable exposure goal to test it

Normalizing doubt and testing it behaviorally builds engagement and hope

Common error: Arguing her out of the doubt verbally

B. Overpromising a specific outcome risks damaging credibility later — Making promises that cannot be kept

C. Conceding to the doubt abandons a treatable target — Adopting the client's doubtful stance as fact

D. A lengthy scientific lecture rarely shifts an adolescent's doubt as effectively as direct experience — Substituting facts for experiential evidence

Q12. A. The general limits of confidentiality in the school setting, including when safety concerns would require disclosure

Confidentiality limits must be disclosed to both the minor and guardian up front

Common error: Omitting the limits of confidentiality

B. Promising absolute confidentiality from a guardian misstates the actual limits — Guaranteeing unconditional secrecy from a parent

C. Goals should be collaborative, not unilaterally imposed — Removing the client and family from planning

D. Consent is ongoing and can be withdrawn — Treating initial consent as binding for a fixed duration

Q13. A. Explain that general updates may go to her mother per the consent agreement, while routine session content stays private absent safety concerns

Balancing minor privacy with guardian involvement per the consent agreement is appropriate

Common error: Either promising full secrecy or disclosing everything without limits

B. Promising total secrecy from the consenting guardian conflicts with consent obligations — Making a confidentiality promise the counselor cannot ethically keep

C. Full transcripts exceed the appropriate level of disclosure for routine sessions — Disclosing far more than necessary or appropriate

D. Dodging the question undermines clear informed consent — Failing to give a clear, honest answer about confidentiality

Case 3: Sad and discouraged three weeks after a layoff

Diagnosis: Adjustment Disorder, with Depressed Mood

Q1. A. That his sadness, sleep trouble, and low motivation began within three months of the layoff and track closely with it

Onset tied to the stressor anchors the diagnosis

Common error: Skipping the timeline link between stressor and symptoms

B. Family history is secondary to onset timing — Requiring family history before establishing onset

C. Exhaustive work history is low yield here — Collecting low-yield background instead of the timeline

D. Comparative severity ratings are not diagnostic — Substituting a rating scale for timeline history

Q2. A. Complete a structured suicide-risk screen as a routine part of intake regardless of how mild his presentation seems

Routine structured screening applies to every intake

Common error: Skipping risk screening because the case looks straightforward

B. Omitting screening based on appearance is unsafe practice — Judging risk by demeanor instead of asking directly

C. Deferring routine screening delays needed data — Postponing baseline safety screening

D. A pledge is not a substitute for structured screening — Using a written pledge instead of screening

Q3. A. How his sleep, mood, and motivation are affecting his job search and his ability to care for his two children

Functional impact across domains reflects impairment

Common error: Focusing only on mood words instead of functional impact

B. Budget arithmetic is not a clinical priority — Treating budgeting detail as clinical assessment

C. Job-task detail does not inform current impairment — Gathering job description detail over functional impact

D. Fairness perceptions do not measure impairment — Exploring grievance instead of functional impact

Q4. A. His symptom count and duration fall short of the five concurrent symptoms nearly daily for two weeks that MDD requires

Subthreshold symptoms rule out an MDD episode

Common error: Diagnosing MDD from mood and sleep symptoms alone

B. A trigger does not rule out MDD by itself — Assuming any precipitant automatically means adjustment

C. Preserved pleasure supports it but is not the deciding factor — Treating one preserved pleasure as the whole differential

D. Expected course does not define the current episode — Diagnosing from anticipated outcome rather than current criteria

Q5. A. He denies any qualifying trauma exposure and shows no intrusion, avoidance, or hyperarousal symptoms of any kind

No Criterion A stressor or symptom cluster present

Common error: Treating any distressing life event as qualifying trauma exposure

B. Recency alone does not rule out ASD — Assuming trauma disorders require a longer elapsed time

C. Gender prevalence is not a valid individual rule-out — Using group statistics to rule out an individual diagnosis

D. Preserved functioning alone does not rule out ASD — Equating preserved functioning with an absent trauma disorder

Q6. A. Brief, present-focused, solution-focused or problem-solving counseling centered on coping with the job loss

Brief stressor-focused approaches fit adjustment disorder

Common error: Defaulting to a lengthy trauma-processing protocol

B. Trauma exposure work is not indicated without a trauma diagnosis — Applying trauma-focused therapy to a non-trauma presentation

C. Recommending medication is outside counselor scope — Acting outside scope of practice

D. Watchful waiting delays indicated brief treatment — Delaying indicated brief treatment

Q7. A. Collaboratively identify concrete, achievable goals tied to coping with the job loss, such as structuring his days

Collaborative, concrete goals fit brief stressor-focused work

Common error: Setting vague, counselor-imposed goals

B. Unilateral planning removes client input — Excluding the client from goal-setting

C. Deep historical exploration is not the priority goal — Prioritizing depth work over present-focused goals

D. Delaying goals removes an active treatment structure — Waiting for spontaneous improvement instead of planning

Q8. A. Incorporate problem-solving around concrete resources such as job-placement and financial-counseling referrals

Concrete problem-solving supports the treatment goal

Common error: Treating concrete resource needs as outside the counseling scope

B. Refusing to engage practical needs is overly rigid — Excluding practical problem-solving entirely

C. Direct career coaching exceeds the counselor's role — Substituting counselor time for specialized vocational services

D. Ignoring the stressor narrows treatment too much — Avoiding the client's stated practical concern

Q9. A. Track his symptoms at each session and reassess if they persist or worsen well beyond the stressor resolving

Ongoing monitoring flags a need to reassess the diagnosis

Common error: Assuming the diagnosis is fixed with no need to reassess

B. Failing to monitor risks missing a worsening course — Treating the initial diagnosis as permanently fixed

C. End-only review misses a worsening course early — Skipping interim check-ins

D. Attendance alone does not capture symptom course — Mistaking attendance for symptom monitoring

Q10. A. Validate that job loss is a significant stressor and normalize his reaction as a common, understandable response

Validation and normalization counter self-blame

Common error: Minimizing his feelings or rushing past them

B. Agreeing with the self-critical comparison deepens shame — Colluding with the client's harsh self-judgment

C. Redirecting away avoids the emotional content — Avoiding emotionally significant material

D. Blunt disputation can feel invalidating — Disputing a feeling before validating it

Q11. A. Collaboratively schedule a small, valued activity, like one pickup basketball game, to rebuild a sense of momentum

Graded, valued activity rebuilds engagement

Common error: Waiting for motivation to return before suggesting activity

B. Too much too soon risks setting him up to fail — Skipping graded pacing

C. Waiting on mood reverses the usual sequence — Letting mood gate all behavior

D. Cataloging causes delays actual behavioral change — Substituting analysis for a concrete next step

Q12. A. The limits of confidentiality, including exactly what information, if any, will be shared back with the EAP or employer

EAP referrals require explicit disclosure of what is shared

Common error: Omitting the EAP disclosure limits from informed consent

B. Assuming automatic reporting misstates confidentiality — Treating an EAP referral as removing confidentiality

C. Absolute confidentiality is not accurate — Guaranteeing unconditional secrecy

D. Consent is ongoing and does not bind him to completion — Treating initial consent as irrevocable

Q13. A. Decline to share any information unless Marcus has signed a specific, informed release authorizing that disclosure

Disclosure requires a valid signed release

Common error: Sharing progress information without a signed release

B. Disclosing without authorization breaches confidentiality — Assuming a referral implies open information sharing

C. Ignoring the request avoids proper handling of it — Avoiding the issue instead of clarifying the process

D. Informal, undocumented consent is insufficient — Relying on undocumented verbal permission

Case 4: Cocaine use, crashes, and mounting consequences

Diagnosis: Stimulant Use Disorder (Cocaine)

Q1. A. The number of DSM-5-TR criteria she meets over twelve months, since four to five place severity in the moderate range

Criterion count sets severity

Common error: Estimating severity by amount used alone

B. Quantity alone does not set the diagnosis — Equating quantity with severity

C. Reason for onset is not the criterion — Using the onset story to diagnose

D. Family history is risk, not diagnosis — Using family history to diagnose

Q2. A. As stimulant withdrawal—a post-use “crash” that is part of her use disorder rather than a separate primary mood disorder

The crash is stimulant withdrawal

Common error: Diagnosing a primary mood disorder from the crash

B. The crash is substance-linked, not independent — Calling withdrawal a primary depression

C. Duration is not what defines withdrawal — Equating withdrawal with chronicity

D. Level of care follows assessment — Jumping to a level of care prematurely

Q3. A. Assess medical risk and suicide risk, since stimulant use carries cardiac danger and crashes can bring acute low mood

Screen cardiac and suicide risk

Common error: Overlooking medical and suicide risk

B. Source detail is low yield — Collecting irrelevant detail

C. Depth history is secondary to safety — Prioritizing depth over safety

D. Trigger mapping is for later planning — Skipping the safety assessment

Q4. A. Her elevated, energized states occur only during intoxication rather than as sustained episodes independent of cocaine use

Intoxication-bound versus sustained episode

Common error: Misreading intoxication as a manic episode

B. Intoxication features overlap with mania — Using intoxication effects to differentiate

C. Irritability is nonspecific — Reading conflict as decisive

D. The crash is withdrawal, not mania's opposite pole — Reading the crash as a bipolar depressive episode

Q5. A. A co-occurring depressive disorder distinct from her crashes, since mood symptoms shape sequencing and safety planning

Screen for independent depression

Common error: Treating the use disorder without screening mood

B. Psychosis is not indicated — Pursuing an unsupported rule-out

C. Eating pathology is not indicated — Inventing an unsupported comorbidity

D. Cognitive decline is premature here — Over-attributing to cognitive decline

Q6. A. Evidence-based behavioral treatment such as contingency management and CBT, since no medication is approved for stimulant use disorder

Behavioral treatment is first-line here

Common error: Waiting for or expecting a medication cure

B. No approved medication exists for it — Overstating a pharmacologic option that does not exist

C. Confrontation raises resistance — Using confrontation to force insight

D. Willpower framing is inaccurate — Moralizing instead of treating

Q7. A. Use a multidimensional assessment of her medical risk, environment, and readiness to match her to the least intensive safe setting

Use multidimensional placement

Common error: Defaulting to one fixed program

B. Maximal care ignores placement criteria — Over-placing without assessment

C. Under-placement ignores her needs — Under-treating a moderate presentation

D. Preference cannot override clinical need — Letting convenience drive placement

Q8. A. Motivational interviewing to explore her ambivalence and strengthen her own reasons for change at her current stage

MI fits her ambivalence and stage

Common error: Pushing action before readiness

B. Confrontation raises resistance — Using confrontation to force insight

C. Action-stage tools precede her readiness — Applying action tools too early

D. Education alone rarely resolves ambivalence — Relying on information to motivate

Q9. A. Reflect her statement and explore both sides of her ambivalence without arguing against her about how serious it is

Roll with resistance; explore ambivalence

Common error: Arguing her into agreement

B. Arguing the change side raises resistance — Taking up the change side of the argument

C. Colluding with minimization is unhelpful — Siding with the status quo to please her

D. Avoiding the moment wastes an opening — Avoiding the topic to dodge conflict

Q10. A. Frame the return to use as common in recovery, explore what it taught her, and revisit the plan without shaming her

Treat the lapse as learning

Common error: Treating a lapse as total failure

B. Catastrophizing deepens shame — Amplifying shame after a lapse

C. One lapse does not mandate escalation — Over-escalating after one slip

D. Dismissing the lapse skips learning — Glossing over a teachable moment

Q11. A. As high-risk periods for low mood and safety that warrant monitoring and a plan for support during the crash

Treat crashes as high-risk windows

Common error: Ignoring the crash as a risk window

B. Optimizing use is not the goal — Coaching “safer” use instead of addressing risk

C. Dismissing the crash misses risk — Overlooking a high-risk period

D. The crash is withdrawal, not bipolar — Misattributing the crash to bipolar disorder

Q12. A. Decline to release records without a valid release and explain that substance-use information has heightened confidentiality protections

SUD records get heightened protection

Common error: Releasing SUD records without proper consent

B. Releasing without consent breaches confidentiality — Releasing records on request alone

C. A diagnosis is protected information — Sharing a diagnosis as if harmless

D. The request should be documented — Failing to document a records request

Q13. A. Explain honestly that no medication is approved for stimulant use disorder and coordinate medical evaluation for related concerns

State the evidence honestly; coordinate care

Common error: Implying a medication cure that does not exist

B. Naming a drug and dose exceeds scope — Overstepping into medication management

C. Overstating a nonexistent option is dishonest — Promising a treatment that does not exist

D. Refusing to coordinate abandons the need — Declining to help with a legitimate concern

Case 5: Minimized suicidal thoughts and an accessible firearm after job loss and separation

Diagnosis: Suicidal Behavior / Acute Risk

Q1. A. Conduct a structured suicide risk assessment covering ideation, intent, plan, and access to means, without accepting his reassurance at face value

Structured risk assessment comes first, not the reassurance

Common error: Letting a client's reassurance substitute for a structured screen

B. Taking the reassurance at face value skips assessment — Trusting a minimizing statement instead of assessing further

C. Intervening before assessing is premature — Jumping to coping skills before a risk assessment

D. Deferring the assessment to another provider is unsafe — Outsourcing an acute safety assessment

Q2. A. Explore the disclosure directly, asking follow-up questions about the firearm's accessibility and his recent thoughts about it, rather than accepting his reassurance

The buried disclosure must be followed up, not skipped

Common error: Hearing the reassurance and missing the buried disclosure of means

B. Ignoring the disclosed means after a reassurance is the core trap — Letting reassuring language override a disclosed lethal means

C. Deferring a means disclosure is unsafe — Sticking rigidly to a planned question order

D. Bypassing Marcus without exploring the disclosure first is premature — Acting on a third party before assessing with the client

Q3. A. Immediate access to a highly lethal method, independent of his verbal reassurance, which sharply elevates acute risk

Accessible, functional means is a major acute risk driver

Common error: Weighing his reassurance more heavily than the disclosed means

B. Exhaustive ownership history is low yield right now — Collecting low-yield background detail instead of acting on current access

C. Depth history is not the priority right now — Prioritizing depth work over an acute safety concern

D. Quantifying drinks is not the priority the disclosure raises — Substituting a different data point for the means question

Q4. A. Weigh both his risk factors, such as the accessible firearm and hopelessness, against protective factors like his children, rather than reading either alone

Balanced formulation weighs risk and protective factors together

Common error: Treating the mention of his children as reassuring without weighing the note-rehearsal

B. Absence of a finished note does not lower acute risk — Treating an unfinished note as evidence that risk is low

C. Proportionality is not a risk metric — Judging risk by whether distress seems proportionate

D. A mood scale alone does not formulate acute risk — Substituting a mood scale for a full risk formulation

Q5. A. His increased weekend drinking, which can heighten impulsivity and lower the threshold for acting on suicidal thoughts

Escalating alcohol use raises acute risk

Common error: Ignoring escalating alcohol use in a suicide risk assessment

B. Psychosis is not indicated here — Pursuing an unsupported rule-out instead of the indicated one

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Cognitive decline is an improbable explanation here — Over-attributing acute distress to cognitive decline

Q6. A. A collaborative safety plan identifying his warning signs, coping strategies, supports, and crisis contacts, built together with him

Collaborative safety planning is the evidence-based cornerstone

Common error: Relying on a no-suicide contract instead

B. No-suicide contracts are not evidence-based — Using a contract in place of a real safety plan

C. Activity scheduling alone is insufficient — Substituting mood work for a full safety plan

D. A single contact rule is not a full plan — Reducing safety planning to one instruction

Q7. A. Address means restriction for the firearm now, alongside starting the broader safety plan, rather than delaying it to a future session

Means restriction must happen alongside deeper work, not after it

Common error: Letting the client's preference to defer the topic delay urgent means restriction

B. Deferring means restriction leaves lethal access in place — Sequencing deeper exploration before urgent means restriction

C. Leaving the firearm to chance is unsafe sequencing — Hoping a safety topic reappears instead of raising it

D. An ultimatum is coercive and can rupture engagement — Issuing a rigid ultimatum instead of collaborating

Q8. A. Counsel collaboratively on means restriction, such as temporarily storing the firearm with a trusted person away from the home or using a lock

Collaborative means restriction is the central intervention

Common error: Leaving the disclosed lethal means unaddressed in the plan

B. Unilateral confiscation without collaboration is inappropriate — Escalating to police confiscation before working with the client

C. In-home storage still leaves ready access — Settling for an inadequate in-home storage measure

D. Deferring entirely abandons means-restriction counseling — Using autonomy as a reason to avoid the conversation

Q9. A. A clinical judgment weighing his intent, access to means, protective factors, and his ability to engage safely with a collaborative plan

Stratified clinical judgment drives the disposition decision

Common error: Using a single factor alone to decide the disposition

B. Reflexive admission bypasses individualized judgment — Admitting reflexively without weighing the full picture

C. Disposition is a clinical judgment, not the client's alone — Deferring the disposition decision to the client alone

D. Refusing to consider hospitalization ignores acute danger — Ruling out hospitalization regardless of assessed risk

Q10. A. Gently name the discrepancy between his minimizing language and the note-rehearsal, and explore its meaning and urgency with empathy

Naming the discrepancy with empathy opens the disclosure further

Common error: Letting the minimizing label pass without exploring the note-rehearsal

B. Reassurance here forecloses needed exploration — Minimizing a warning sign the same way the client does

C. Avoiding the disclosure leaves acute risk unexplored — Redirecting away from a safety disclosure to reduce discomfort

D. A firm command can shut down further disclosure — Commanding rather than exploring with empathy

Q11. A. Validate how overwhelming the losses feel right now while helping him see the current crisis and his options as changeable over time

Validating pain while framing the crisis as changeable supports hope

Common error: Either only sympathizing or only reassuring him

B. Overpromising outcomes can ring hollow and backfire — Making promises about outcomes you cannot control

C. Endorsing hopelessness reinforces the despair driving the crisis — Adopting the client's hopeless stance

D. Redirecting away from despair leaves it unaddressed — Avoiding emotionally central material with a task list

Q12. A. Explain the limits of confidentiality, including that a serious and foreseeable risk of harm may require disclosure to keep him safe

Confidentiality has safety-based limits that must be explained

Common error: Promising unconditional confidentiality

B. Absolute confidentiality is inaccurate and unethical here — Guaranteeing unconditional secrecy

C. Disclosure should follow the completed risk assessment, not precede it — Escalating to police before the assessment is complete

D. Dodging the question undermines informed consent — Concealing the limits of confidentiality to keep him talking

Q13. A. Document the risk assessment findings, the means-restriction steps agreed to, the safety plan, and the clinical rationale for the disposition

Thorough documentation captures assessment, plan, and rationale

Common error: Documenting only that he denied a plan

B. A promise-only note is inadequate and misleading — Substituting a promise note for real documentation

C. Minimizing the record is unsafe and inaccurate — Understating risk in the clinical record

D. Omitting the assessment from the record is improper — Omitting essential clinical information from the chart

Case 6: Years of mood ups and downs that never reach full episodes

Diagnosis: Cyclothymic Disorder

Q1. A. Two or more years of subthreshold mood swings with no symptom-free stretch longer than about two months

Two years of subthreshold swings, no long remission

Common error: Diagnosing cyclothymia without confirming the duration and persistence

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent change is not the criterion — Confusing a brief change with the criteria

D. Full-episode count would point elsewhere — Applying full-episode criteria here

Q2. A. Her highs and lows have stayed subthreshold, never reaching a full hypomanic or major depressive episode

Subthreshold, never a full episode

Common error: Calling subthreshold cycling bipolar II

B. Mood cycling occurs in both — Using cycling to differentiate

C. Impairment occurs in both — Reading impairment as decisive

D. Schedule effects are nonspecific — Over-reading a schedule effect

Q3. A. Her mood shifts unfold over days to weeks with a stable sense of self, not the rapid, interpersonally triggered instability of BPD

Slow shifts, stable self versus rapid reactivity

Common error: Confusing slow mood cycling with rapid affective instability

B. Mood variability occurs in both — Using variability to differentiate

C. Subjective uncertainty is nonspecific — Reading uncertainty as decisive

D. Diurnal pattern is not diagnostic — Over-reading a circadian pattern

Q4. A. A substance- or medical-induced mood disturbance, which her unremarkable review makes less likely but worth confirming

Exclude substance and medical causes

Common error: Skipping the substance and medical rule-out

B. Phobia does not explain mood cycling — Reducing mood cycling to a phobia

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Neurodevelopmental framing is implausible — Pursuing an improbable rule-out

Q5. A. Suicide risk and any escalation toward a full mood episode, monitoring across both the high and low periods

Screen risk and watch for escalation

Common error: Tracking mood without screening risk and escalation

B. Psychosis is not indicated — Pursuing an unsupported rule-out

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Cognitive decline is improbable here — Pursuing an improbable rule-out

Q6. A. Psychotherapy that stabilizes daily routines and rhythms, with psychoeducation about her mood pattern and triggers

Rhythm-stabilizing psychotherapy and psychoeducation

Common error: Offering open-ended support without a rhythm-stabilizing structure

B. Depression-only focus misses the cycling — Treating only one side of the cycling

C. Prescribing is outside counselor scope — Acting outside scope of practice

D. Amplifying highs destabilizes rhythms — Advising behavior that destabilizes mood

Q7. A. Consistent sleep, activity, and daily routines, since rhythm disruption can amplify her mood swings

Stabilize sleep and daily rhythms

Common error: Overlooking the role of sleep and routine in mood stability

B. Removing routine destabilizes mood — Advising the opposite of what stabilizes mood

C. Inconsistent, pole-driven routines do not stabilize — Building a routine that tracks the mood swings

D. Diet-only misses the key target — Narrowing to one factor and missing the main target

Q8. A. Use a daily mood-and-routine chart with periodic measures to monitor cycling, function, and risk over time

Mood-and-routine charting plus measures

Common error: Proceeding without systematic mood and routine tracking

B. Anecdote alone is insufficient — Substituting anecdote for measurement

C. Single-day snapshots miss the cycling — Measuring isolated snapshots

D. End-only review misses steering data — Deferring all measurement to the end

Q9. A. Coordinate a referral to a prescriber for evaluation while continuing the rhythm-focused psychotherapy within scope

Refer for prescribing, keep doing therapy

Common error: Advising on specific medication outside scope

B. Recommending a drug and dose exceeds scope — Making a prescribing recommendation

C. Blanket refusal advice overreaches — Directing a medical decision

D. Dismissing the question is unhelpful — Failing to address a legitimate question

Q10. A. Explore the wish nonjudgmentally and use psychoeducation to connect the highs to the broader cycling pattern

Explore and psychoeducate about the pattern

Common error: Either simply agreeing she is done or dismissing how good she feels

B. Ending in a high ignores the cycling — Ending treatment based on a transient high

C. Fear-based pressure undermines the alliance — Using fear rather than collaborative exploration

D. Ignoring it misses key material — Sidestepping a meaningful moment

Q11. A. Normalize the long-term nature of the work and anchor it in goals she values across both her high and low periods

Normalize the course and anchor in values

Common error: Implying a quick fix that sets her up for discouragement

B. Promising a quick cure is misleading — Setting an unrealistic expectation

C. Cataloguing shifts is not the work — Centering sessions on enumerating shifts

D. Conditioning care is inappropriate — Making counseling contingent on medication

Q12. A. Discuss confidentiality and the limits of any release, sharing only what she consents to and what is needed

Honor consent and minimum-necessary disclosure

Common error: Disclosing more than she consents to or than is necessary

B. Sending everything ignores consent — Over-disclosing without consent

C. Falsifying documentation is unethical — Misrepresenting information in records

D. Flat refusal forecloses a workable option — Declining without exploring an appropriate disclosure

Q13. A. Seek consultation and reassess against the criteria, coordinating referral if the picture exceeds your competence

Consult and reassess within competence

Common error: Settling diagnostic uncertainty alone without consultation

B. Refusing to reassess is poor practice — Treating the diagnosis as immutable

C. Dismissing the distinction misleads care — Minimizing a clinically meaningful distinction

D. Abrupt transfer mishandles continuity — Dropping the client without coordination

Case 7: Psychosis that persists between mood episodes

Diagnosis: Schizoaffective Disorder, Bipolar Type

Q1. A. At least two weeks of psychosis without a mood episode, with mood episodes present for most of the illness

Psychosis without mood plus mood majority

Common error: Diagnosing without confirming both the timing criteria

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent change is not the criterion — Confusing a brief change with the criteria

D. Cognitive count does not define it — Applying neurocognitive criteria here

Q2. A. Mood episodes have been present for the majority of his illness rather than being brief or only incidental

Mood episodes predominate the course

Common error: Overlooking the predominance of mood episodes

B. Psychosis occurs in both — Using psychosis to differentiate

C. Guardedness occurs in both — Reading guardedness as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. He has had psychosis for two or more weeks while his mood was stable, not only during mood episodes

Psychosis outside of mood episodes

Common error: Missing the psychosis that occurs outside mood episodes

B. Manic episodes occur in both — Using mania to differentiate

C. Grandiosity occurs in both — Reading grandiosity as decisive

D. Environmental sleep disruption is nonspecific — Over-reading an environmental effect

Q4. A. A substance- or medical-induced cause, which the records indicate were already assessed and excluded

Exclude substance and medical causes

Common error: Skipping the substance and medical rule-out

B. Phobia does not explain psychosis — Reducing psychosis to a phobia

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Neurodevelopmental framing is implausible — Pursuing an improbable rule-out

Q5. A. Conduct a structured suicide-risk assessment of current ideation, intent, plan, and protective factors now

Structured suicide-risk assessment now

Common error: Failing to assess current suicide risk

B. Assuming risk passed is unsafe — Dismissing risk on reassurance

C. Deferring assessment is unsafe — Postponing an urgent assessment

D. Deferring safety to address housing first — Sequencing safety behind other needs

Q6. A. Coordinate combined pharmacotherapy and psychosocial support within a treatment team and your counseling role

Combined meds and psychosocial care

Common error: Offering counseling alone for schizoaffective disorder

B. Insight-only is not first-line — Prioritizing insight over indicated care

C. Prescribing is outside counselor scope — Acting outside scope of practice

D. Talks alone are insufficient — Omitting medication coordination

Q7. A. Collaborative work on engagement and adherence, addressing the barriers that interrupt his medication and care

Engagement and adherence support

Common error: Ignoring the adherence and engagement barriers

B. Ultimatums rupture engagement — Making care contingent on adherence

C. Never raising adherence neglects a key issue — Avoiding an important treatment target

D. Encouraging stop-start worsens course — Recommending an unsafe medication pattern

Q8. A. Coordinate case management and community resources for housing as part of recovery-oriented, wraparound support

Coordinate housing and community resources

Common error: Treating symptoms while ignoring housing instability

B. Housing is part of recovery — Ignoring a key recovery factor

C. Financial entanglement crosses boundaries — Crossing financial boundaries with a client

D. Withholding care is inappropriate — Setting stability as a precondition for treatment

Q9. A. Obtain his informed consent and a release, then collaborate with the team and share what is needed for his care

Consent and release before coordinating

Common error: Coordinating before obtaining consent

B. Coordinating without consent breaches confidentiality — Disclosing before consent is documented

C. No coordination undercuts team care — Refusing helpful, consented coordination

D. Blanket sharing over-discloses — Releasing more than is appropriate or consented

Q10. A. Acknowledge how distressing the voices are and stay supportive, without confirming or arguing about their reality

Validate distress, stay neutral on content

Common error: Either agreeing the voices are real or flatly arguing they are not

B. Confronting the psychosis ruptures the alliance — Disputing the hallucination to manage it

C. Colluding reinforces the symptom — Validating the hallucination to keep rapport

D. Total avoidance misses an opening — Sidestepping the relevant material

Q11. A. Move at his pace, be transparent and consistent, and work toward concrete goals he chooses and values

Pace, transparency, and shared goals

Common error: Pushing for disclosure faster than the alliance allows

B. Pressured disclosure ruptures trust — Demanding disclosure too fast

C. Dwelling on content can reinforce it — Centering the work on psychotic content

D. Conditioning care is inappropriate — Making counseling contingent on medication

Q12. A. Assess the risk and follow duty-to-protect obligations, which may require warning or other protective steps

Assess and act on duty to protect

Common error: Treating an identifiable threat as fully confidential

B. Confidentiality yields to a serious threat — Treating confidentiality as absolute when there is danger

C. Delaying assessment is unsafe — Postponing an urgent risk assessment

D. Act on assessed risk, not reflexively — Disclosing before assessing the risk

Q13. A. Seek consultation and work within a treatment team, referring for the components beyond your competence

Consult, coordinate, or refer within competence

Common error: Managing serious psychotic illness alone without competence

B. Going it alone exceeds competence — Prioritizing continuity over competence

C. Minimizing the illness fails the client — Underestimating the need for team care

D. Abrupt transfer mishandles continuity — Dropping the client without coordination

Case 8: Childhood motor and vocal tics with social and school impact

Diagnosis: Tourette's Disorder

Q1. A. Multiple motor tics and at least one vocal tic present for more than a year with childhood onset

Multiple motor plus a vocal tic over a year

Common error: Diagnosing without confirming both motor and vocal tics over the required duration

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent change is not the criterion — Confusing a brief change with the criteria

D. Depressive count does not define it — Applying depressive criteria here

Q2. A. He has both multiple motor tics and a vocal tic, not tics limited to only one of the two types

Both motor and vocal tics, not one type

Common error: Missing that both tic types are required for Tourette's

B. Tics occur in both — Using the presence of tics to differentiate

C. Distress is nonspecific — Reading distress as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. His tics are involuntary movements and sounds, not compulsions performed to neutralize intrusive obsessions

Involuntary tics, not obsession-driven compulsions

Common error: Reading involuntary tics as OCD compulsions

B. Repetition occurs in both — Using repetition to differentiate

C. A premonitory urge is not an obsession — Equating a premonitory urge with an obsession

D. Sleep effects are nonspecific — Over-reading a sleep effect

Q4. A. Co-occurring ADHD, OCD, and anxiety, which commonly accompany Tourette's and shape the plan

Screen ADHD, OCD, and anxiety

Common error: Treating the tics without screening for the common comorbidities

B. Psychosis is not indicated — Pursuing an unsupported rule-out

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Cognitive decline is implausible in a child — Pursuing an implausible rule-out

Q5. A. The emotional impact on his self-esteem and mood, with developmentally appropriate screening for safety

Assess self-esteem, mood, and safety

Common error: Addressing the tics while ignoring the emotional and self-esteem impact

B. Psychosis is not indicated — Pursuing an unsupported rule-out

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Cognitive decline is implausible in a child — Pursuing an implausible rule-out

Q6. A. Behavioral intervention for tics with psychoeducation and school support, coordinated with the family

Behavioral tic intervention plus school support

Common error: Defaulting to medication or doing nothing instead of behavioral intervention and support

B. Willful suppression alone is not the treatment — Reducing treatment to willful suppression

C. Prescribing is outside counselor scope — Acting outside scope of practice

D. Dismissing the impact misses the need — Dismissing a presentation that warrants support

Q7. A. Provide psychoeducation and coordinate school supports and anti-bullying measures to protect his participation

Psychoeducation and school supports

Common error: Treating the tics while leaving the school environment unaddressed

B. Telling him to ignore it is insufficient — Minimizing the impact of the teasing

C. Withdrawal from school is harmful — Endorsing harmful avoidance

D. Ignoring the environment narrows the plan — Omitting a key part of the plan

Q8. A. Track tic frequency, distress, school participation, and self-esteem over time to guide the plan

Measurement-based tracking of tics and function

Common error: Proceeding without tracking the relevant outcomes

B. Anecdote alone is insufficient — Substituting anecdote for measurement

C. Tic count alone is too narrow — Reducing monitoring to one number

D. End-only review misses steering data — Deferring all measurement to the end

Q9. A. With the parents' consent and a release, coordinate school supports and any medical care for the tics

Consent and release before coordinating

Common error: Contacting the school or providers without parental consent

B. Disclosing without consent breaches confidentiality — Disclosing without consent

C. No coordination undercuts support — Refusing helpful, consented coordination

D. Blanket sharing over-discloses — Releasing more than is appropriate or consented

Q10. A. Validate his feelings, provide age-appropriate psychoeducation, and build self-acceptance and coping

Validate, psychoeducate, and build self-acceptance

Common error: Either minimizing his feelings or reinforcing that the tics make him abnormal

B. Blaming effort is harmful and inaccurate — Implying the tics reflect insufficient effort

C. Reinforcing otherness is unhelpful — Reinforcing a sense of being abnormal

D. Avoiding the feeling misses the work — Sidestepping a meaningful moment

Q11. A. Use developmentally appropriate, strengths-based methods that build coping and confidence at his level

Developmentally matched, strengths-based engagement

Common error: Using abstract, adult-oriented methods that a child cannot engage with

B. Abstract methods do not fit a child — Pitching the work above his developmental level

C. Cataloguing tics is not the work — Centering sessions on enumerating tics

D. Demanding tic cessation first stalls engagement — Setting tic cessation as a precondition

Q12. A. Decline to misrepresent his status and clarify what the assessment and your role can accurately support

Do not misrepresent; document accurately

Common error: Writing an inaccurate statement to satisfy the parents

B. Falsifying a statement is unethical — Misrepresenting the child's status

C. Advocacy over accuracy is improper — Letting advocacy override accuracy

D. Flat refusal forecloses helpful options — Declining without clarifying the appropriate options

Q13. A. Seek consultation and training, and refer or coordinate for the components beyond your competence

Consult, train, or refer within competence

Common error: Delivering a specialized intervention without the required competence

B. Going it alone exceeds competence — Prioritizing continuity over competence

C. Minimizing the need misleads care — Underestimating the need for specialized care

D. Abrupt transfer mishandles continuity — Dropping the family without coordination

Case 9: Months of broken sleep and exhausting days

Diagnosis: Insomnia Disorder, Chronic

Q1. A. Difficulty with sleep at least three nights a week for over three months, with daytime impairment despite adequate opportunity

Frequency, duration, opportunity, and impairment

Common error: Diagnosing chronic insomnia from a brief sleepless stretch

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. A few bad nights is not chronic insomnia — Confusing a brief change with the criteria

D. Inadequate opportunity argues against the diagnosis — Diagnosing insomnia when opportunity is inadequate

Q2. A. Signs of obstructive sleep apnea, such as loud snoring, witnessed breathing pauses, or gasping awake, which warrant referral

Screen for sleep apnea and refer if present

Common error: Skipping the sleep-apnea screen

B. Daytime fatigue is nonspecific — Using daytime fatigue to differentiate

C. Stress reactivity is nonspecific — Reading stress sensitivity as decisive

D. Seasonal variation is not the priority screen — Pursuing a low-yield factor

Q3. A. Sleep is his primary concern rather than excessive, uncontrollable worry spanning many areas of his daily life

Sleep-focused versus diffuse worry

Common error: Calling sleep-focused distress generalized anxiety

B. Sleep-related worry occurs in insomnia itself — Using sleep worry to differentiate

C. Irritability and poor focus occur in both — Reading shared symptoms as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q4. A. His main problem is the sleep disturbance itself rather than pervasive low mood, anhedonia, and the other depressive symptoms

Sleep-centered versus a depressive syndrome

Common error: Diagnosing depression from insomnia alone

B. Low energy occurs in both — Using fatigue to differentiate

C. Irritability is nonspecific — Reading irritability as decisive

D. Diurnal variation is not diagnostic — Over-reading a circadian pattern

Q5. A. A daily sleep diary tracking bedtime, sleep onset, awakenings, rise time, and daytime functioning over a couple of weeks

Use a sleep diary baseline

Common error: Planning treatment without a sleep baseline

B. A global estimate is imprecise — Relying on a vague global rating

C. Ordering a sleep study exceeds scope — Acting outside scope of practice

D. Early-history detail is low yield here — Prioritizing origins over current data

Q6. A. Cognitive behavioral therapy for insomnia, including stimulus control, sleep restriction, and cognitive work on sleep beliefs

CBT-I is first-line for chronic insomnia

Common error: Defaulting to long-term hypnotic medication

B. Long-term hypnotics are not first-line — Defaulting to standing hypnotics

C. Trying harder to sleep worsens insomnia — Advising the client to try to sleep harder

D. Insight-only is not first-line here — Prioritizing origins over CBT-I

Q7. A. Stimulus control and sleep restriction, which strengthen the bed–sleep association and consolidate his time asleep

Stimulus control and sleep restriction

Common error: Leaving extended time-in-bed in place

B. More time in bed worsens insomnia — Increasing time in bed

C. Lying awake in bed reinforces the cycle — Reinforcing the maintaining behavior

D. Long naps undermine night sleep — Encouraging compensatory napping

Q8. A. Recommend CBT-I as first-line and, if he wants to consider medication, coordinate a referral to a prescriber with his consent

Lead with CBT-I; refer for medication questions

Common error: Giving specific medication advice

B. Naming a drug and dose exceeds scope — Overstepping professional limits

C. Endorsing indefinite sleep aids is inappropriate — Advising long-term reliance on sleep aids

D. Discouraging meds overreaches — Steering a medical decision

Q9. A. Review the ongoing sleep diary at intervals to track sleep efficiency and daytime functioning and adjust the plan

Use the sleep diary for measurement-based care

Common error: Relying on impressions alone

B. Global impressions are unreliable — Trusting unstandardized self-report

C. End-only review misses non-response — Skipping interim monitoring

D. Attendance is not an outcome measure — Mistaking adherence for outcome

Q10. A. Address the catastrophic sleep beliefs with cognitive work, since such beliefs heighten arousal and maintain the insomnia

Target catastrophic sleep beliefs

Common error: Leaving the maintaining beliefs unaddressed

B. Endorsing the belief feeds arousal — Confirming the maintaining belief

C. Reassurance feeds the sleep-effort cycle — Providing reassurance that maintains arousal

D. Lecturing rarely shifts the belief — Substituting facts for cognitive work

Q11. A. Explain how clock-watching, daytime napping, and effortful sleep maintain insomnia and how CBT-I targets that cycle

Teach the insomnia-maintaining cycle

Common error: Teaching techniques without the rationale

B. Not-thinking-about-it is unhelpful advice — Giving platitudes instead of education

C. A heavy reading load is not the priority — Overloading instead of explaining

D. Dismissive reassurance forgoes treatment — Reassuring instead of treating

Q12. A. Address the safety risk of using another person's prescription and document accurately rather than omitting it from the record

Address the safety risk; document honestly

Common error: Agreeing to leave a safety-relevant fact out of the record

B. Omitting a safety fact falsifies the record — Falsifying records to reassure

C. Ignoring the risk is poor care — Sidestepping a relevant safety issue

D. Reflexive reporting is disproportionate here — Escalating before addressing it clinically

Q13. A. Decline to disclose without a valid release and clarify the limits of what a treating counselor can appropriately attest to

Protect confidentiality; clarify role limits

Common error: Disclosing or issuing a fitness opinion without consent or competence

B. Disclosing without consent breaches confidentiality — Issuing an unconsented, out-of-role opinion

C. A general summary is still a disclosure — Leaking details under the guise of helping

D. Abrupt refusal misses a teachable moment — Being needlessly obstructive

Case 10: Persistent erectile difficulty straining a relationship

Diagnosis: Erectile Disorder

Q1. A. Difficulty obtaining or maintaining an erection on most occasions for at least six months, causing distress

Persistent difficulty over six months with distress

Common error: Diagnosing without confirming the persistence, frequency, and distress

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent change is not the criterion — Confusing a brief change with the criteria

D. Depressive count does not define it — Applying depressive criteria here

Q2. A. The sexual difficulty is the primary problem and he denies a depressive episode as its driver

Primary sexual dysfunction, not mood-driven

Common error: Assuming sexual difficulty always reflects depression

B. Distress is nonspecific — Using distress to differentiate

C. Low confidence is nonspecific — Reading confidence as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. His anxiety centers on sexual performance and the erectile difficulty, not free-floating worry across many life areas

Performance-focused, not multi-domain worry

Common error: Calling focused performance anxiety a generalized anxiety disorder

B. Anxiety occurs in both — Using anxiety to differentiate

C. Anticipatory worry is nonspecific — Reading anticipatory worry as decisive

D. Schedule effects are nonspecific — Over-reading a schedule effect

Q4. A. A substance-, medication-, or medical-cause for the difficulty, which the ongoing evaluation should help clarify

Exclude substance, medication, and medical causes

Common error: Skipping the substance, medication, and medical rule-out

B. Phobia does not explain the dysfunction — Reducing the dysfunction to a phobia

C. Eating pathology is not indicated — Inventing an unsupported rule-out

D. Neurodevelopmental framing is implausible — Pursuing an improbable rule-out

Q5. A. Coordinate with his medical provider so the physical evaluation and psychological care proceed together

Coordinate medical and psychological care

Common error: Providing psychological care in isolation from the medical evaluation

B. Recommending a drug and dose exceeds scope — Making a prescribing recommendation

C. Psychosis is not indicated — Pursuing an unsupported rule-out

D. Cognitive decline is improbable here — Pursuing an improbable rule-out

Q6. A. Psychosexual therapy addressing performance anxiety and the couple's intimacy, coordinated with medical care

Psychosexual therapy plus medical coordination

Common error: Offering only general talk without addressing the sexual anxiety and coordinating medical care

B. Willpower framing is inaccurate and shaming — Dismissing the disorder as willpower

C. Excluding the couple narrows the plan — Leaving the relational component out

D. Prescribing is outside counselor scope — Acting outside scope of practice

Q7. A. Use anxiety-reduction and sensate-focus-style approaches that reduce performance pressure and rebuild intimacy

Reduce performance pressure and rebuild intimacy

Common error: Increasing the focus on performance, which worsens the anxiety

B. Increasing performance focus worsens it — Amplifying the performance pressure

C. Endorsing avoidance maintains the problem — Reinforcing avoidance

D. Avoiding the anxiety misses the work — Sidestepping the central issue

Q8. A. Track sexual function, performance anxiety, relationship satisfaction, and distress over time to guide the plan

Measurement-based tracking of function and relationship

Common error: Proceeding without tracking the relevant outcomes

B. Anecdote alone is insufficient — Substituting anecdote for measurement

C. A single performance metric is too narrow — Reducing monitoring to one outcome

D. End-only review misses steering data — Deferring all measurement to the end

Q9. A. With his consent, consider couple-based work on communication and intimacy with appropriate agreements

Couple work with consent and clear agreements

Common error: Involving the partner without consent or role clarity

B. Involving the partner without consent breaches confidentiality — Disregarding consent and confidentiality

C. Excluding the relationship narrows the plan — Leaving the relationship out

D. Blanket refusal ignores his wishes — Ruling out a consented, indicated option

Q10. A. Respond with a calm, nonjudgmental, and normalizing stance that makes it safe for him to talk about it

Calm, nonjudgmental, normalizing stance

Common error: Reacting awkwardly or pushing him to disclose before he feels safe

B. Rushing past it misses the work — Rushing past a sensitive topic

C. Joking is invalidating and unprofessional — Undermining a sensitive disclosure with humor

D. Demanding detail before safety is counterproductive — Demanding disclosure too fast

Q11. A. Frame the difficulty as common and treatable, anchor the work in his goals, and reinforce his motivation

Normalize, anchor in goals, reinforce motivation

Common error: Framing the difficulty as hopeless or shameful

B. Endorsing hopelessness is harmful — Communicating hopelessness

C. Cataloguing failures is not the work — Centering sessions on failures

D. Conditioning care is inappropriate — Making counseling contingent on medication

Q12. A. Clarify confidentiality and the limits of any release, sharing only what he consents to and what is needed

Honor consent and minimum-necessary disclosure

Common error: Disclosing more than he consents to or than is necessary

B. Sending everything ignores consent — Over-disclosing without consent

C. Falsifying records is unethical — Falsifying documentation

D. Avoiding the issue is unhelpful — Sidestepping a needed conversation

Q13. A. Seek consultation and training, and coordinate or refer for the components beyond your competence

Consult, train, or refer within competence

Common error: Providing specialized psychosexual care without the required competence

B. Going it alone exceeds competence — Prioritizing continuity over competence

C. Minimizing the problem fails the client — Underestimating the need for specialized care

D. Abrupt transfer mishandles continuity — Dropping the client without coordination

Case 11: A defiant, irritable nine-year-old at home and school

Diagnosis: Oppositional Defiant Disorder

Q1. A. A 6-month pattern of angry/irritable mood and defiant, vindictive behavior occurring with more than one person or setting

Six-month pattern across settings

Common error: Diagnosing from a brief or single-setting conflict

B. A precipitant is not the criterion — Using a trigger as the diagnostic test

C. Recent change is not the criterion — Confusing a recent uptick with the criteria

D. Single-setting-only argues against the diagnosis — Requiring the behavior to be limited to one setting

Q2. A. His behavior is defiant and argumentative without the serious violations of others' rights, aggression, or theft seen in conduct disorder

Defiance without rights violations

Common error: Calling defiance without rights violations conduct disorder

B. Arguing and rule refusal appear in both disorders — Choosing a feature both disorders share instead of one that separates them

C. Irritability occurs in both — Reading irritability as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q3. A. ADHD can co-occur and should be assessed separately, since defiance differs from inattentive and hyperactive-impulsive symptoms

Assess ADHD as a separable comorbidity

Common error: Assuming the defiance is simply ADHD

B. Collapsing ODD into ADHD is an error — Treating defiance as identical to ADHD

C. Calm moments do not rule out ADHD — Ruling out ADHD on insufficient grounds

D. Severity trend is not the question — Reframing the question as a severity trend

Q4. A. His core problem is defiant, argumentative behavior rather than the persistent severe irritability with frequent temper outbursts of DMDD

Defiance-centered versus severe persistent irritability

Common error: Calling defiance-centered behavior DMDD

B. Touchiness occurs in both — Using touchiness to differentiate

C. Family conflict is nonspecific — Reading conflict as decisive

D. Stress reactivity is nonspecific — Using stress sensitivity to differentiate

Q5. A. Multi-informant assessment using parent and teacher reports and rating scales across settings

Parent and teacher multi-informant data

Common error: Relying on a single informant or one setting

B. One session is insufficient — Diagnosing from one in-session sample

C. A single informant is insufficient — Using only one informant

D. Ordering such tests exceeds scope — Acting outside scope of practice

Q6. A. Parent management training to build consistent, positive discipline and interrupt the coercive parent-child cycle

Parent management training is first-line

Common error: Offering individual insight therapy to the child alone as primary

B. Child-only insight work is not first-line — Excluding the parent component

C. Harsh punishment escalates the cycle — Recommending harsher punishment

D. Medication is not first-line for ODD itself — Defaulting to medication for core ODD

Q7. A. Replacing escalating commands and inconsistent follow-through with clear expectations, consistency, and positive reinforcement

Consistency and positive reinforcement

Common error: Leaving the coercive cycle in place

B. Power battles escalate the cycle — Framing discipline as a contest to win

C. Giving in reinforces defiance — Rewarding escalation with capitulation

D. Blanket ignoring is not the plan — Assuming the pattern will self-resolve

Q8. A. With consent, coordinate consistent expectations and supports across home and school given the cross-setting pattern

Coordinate home-school consistency with consent

Common error: Addressing home while ignoring school

B. Excluding school forgoes useful consistency — Refusing helpful, consented coordination

C. Disclosing without consent breaches confidentiality — Sharing information without authorization

D. Harsher punishment escalates the cycle — Recommending punitive escalation at school

Q9. A. Validate how exhausting the conflict is and reframe parent training as a skill set, not a verdict on her worth as a parent

Validate and reframe parent training

Common error: Either blaming or only sympathizing with the parent

B. Blaming the parent deepens shame — Assigning blame rather than building skills

C. Premature reassurance forgoes the work — Reassuring instead of engaging the work

D. Facts alone rarely shift the feeling — Answering emotion with numbers

Q10. A. Build rapport, find his strengths and goals, and involve him in age-appropriate problem-solving rather than only correcting him

Strengths-based, collaborative engagement

Common error: Approaching the child only as a problem to correct

B. Confrontation invites more defiance — Leading with confrontation

C. A deficit-only focus disengages him — Centering only on deficits

D. Threats erode engagement — Using threats to motivate a child

Q11. A. Clarify at the outset, in age-appropriate terms, what will be shared with his mother and what stays private, within legal limits

Set clear minor-confidentiality expectations up front

Common error: Leaving confidentiality expectations undefined

B. Absolute secrecy from the guardian is inappropriate — Guaranteeing total secrecy from the parent

C. Minors still have some privacy interests — Disregarding the minor's privacy entirely

D. Skipping the discussion undermines consent — Failing to set expectations

Q12. A. Follow mandated-reporting duties, assess safety, and make the required child-abuse report through proper channels

Mandated reporting and safety first

Common error: Treating disclosed possible abuse as ordinary confidential material

B. Confidentiality yields to mandated reporting — Treating abuse disclosure as protected

C. Delaying a report is unsafe and improper — Postponing a required report

D. Investigating yourself is not the counselor's role — Conducting your own investigation

Q13. A. Seek consultation and training, or refer to a clinician competent in parent training and child behavioral treatment

Consult, train, or refer within competence

Common error: Delivering specialized treatment without competence

B. Going it alone exceeds competence — Prioritizing continuity over competence

C. Minimizing the problem fails the family — Underestimating the need for evidence-based care

D. Abrupt transfer mishandles continuity — Dropping the family without coordination

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